

Report Concerning the New York State Department of Corrections and
Community Supervision

June 29, 2026



WILMER CUTLER PICKERING HALE AND DORR LLP ®

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EXECUTIVE SUMMARY

On December 9, 2024, Robert Brooks, an incarcerated individual in the custody of the New York State Department of Corrections and Community Supervision (DOCCS), was beaten to death at Marcy Correctional Facility (Marcy) by correction officers, while other officers and staff failed to intervene. Mr. Brooks died the next day, and his death was ruled a homicide. The medical examiner reported the cause of death was compression of the neck and multiple blunt impact injuries sustained during the incident. Body camera footage of the incident was released by the New York Attorney General's Office, showing Mr. Brooks being beaten in the infirmary by officers while he was handcuffed. The footage demonstrated that Use of Force Reports filed by DOCCS staff following the beating were false.

Approximately two months after Mr. Brooks's death, on February 17, 2025, DOCCS correction officers started an illegal strike. On February 19, 2025, Governor Kathy Hochul issued an executive order declaring a disaster emergency in the State of New York and activating the New York National Guard to assist authorities at various correctional facilities.

On March 1, 2025, during the illegal strike, Messiah Nantwi, an incarcerated individual housed at Mid-State Correctional Facility (Mid-State), was beaten to death by DOCCS correction officers. Mid-State is one mile away from Marcy. According to the medical examiner, Mr. Nantwi suffered lethal brain injuries from violent blows to his head and blunt force trauma to his body. According to the indictment, correction officers and sergeants conspired to cover up the killing: they mopped up the blood in Mr. Nantwi's unit; met at a local diner to coordinate their stories; planted a weapon in Mr. Nantwi's unit; and authored false reports to elide key incident details.

Following the killing of Mr. Brooks and continuing after the killing of Mr. Nantwi, DOCCS retained Wilmer Cutler Pickering Hale and Dorr LLP (hereinafter, "WilmerHale") to

conduct a review of DOCCS' patterns and practices regarding the use of force and an assessment of DOCCS' internal systems and procedures to identify any potential systemic issues. This report presents the findings of that review. At DOCCS' request, the review focused initially on Marcy, where Mr. Brooks was killed, and it later expanded to include a limited review of Mid-State, after Mr. Nantwi was killed. To assist in the review, WilmerHale retained subject-matter experts who specialize in corrections and related subjects, as well as a private investigator to conduct interviews of current and former DOCCS staff.

During the review, we examined the following key areas: use of force, staff training, DOCCS' Office of Special Investigations (OSI), staff discipline, DOCCS' Incarcerated Grievance Program (IGP), risk detection and mitigation, weapons, staffing, contraband, cameras, Incarcerated Liaison Committees (ILCs), programming and recreation, the classification of incarcerated individuals, mental health care, and medical care.

Key findings and themes, as discussed in greater detail in this report, are as follows:

- **Lack of Safety.** Our review found that both incarcerated individuals and DOCCS staff feel unsafe in the DOCCS system. Incarcerated individuals reported poor relations with security staff—characterized by an “us versus them” mentality—a theme we also identified in discussions with former security staff and current facility leadership. In addition, a staffing crisis has led to a cycle that exacerbates safety issues: staff shortages lead to less programming, which leads to worse conditions for incarcerated individuals, which leads to misbehavior, which leads to unsafe and stressful working conditions, which causes attrition, which in turn leads to fewer staff. The prevalence of contraband and gang activity also exacerbates all safety problems. The lack of safety at DOCCS facilities undermines the DOCCS goal of rehabilitation.
- **Training.** Our review identified that DOCCS' current training for new recruits and ongoing training for staff is outdated and insufficient for the conditions staff face today. The DOCCS Training Academy (Academy) trains correction officers using a paramilitary, classroom-based approach that is not conducive to officer engagement, knowledge retention, or preparedness for the high-stakes and potentially traumatic real-life scenarios that officers will encounter. Likewise, on-the-job training (OJT) currently offered by DOCCS does not provide officers with the ongoing education needed to stay prepared for the challenges of a correction professional.

- **Accountability.** Our review identified roadblocks to accountability for staff misconduct. These included challenges in investigating and substantiating staff misconduct, difficulties in imposing meaningful discipline to deter misconduct, and limited review functions for uses of force and staff performance. Insufficient oversight of body-worn and fixed cameras and chemical agent usage, as well as structural problems with reporting mechanisms, including peer-led spaces, further undermine efforts to incentivize compliance and mission-aligned conduct. This lack of accountability—combined with high-profile news of tragic violence at DOCCS facilities—can cause incarcerated individuals and staff alike to lose faith and hope in the goal of rehabilitation.
- **Wellness and Morale.** DOCCS lacks a coherent strategy to enable meaningful wellness and rehabilitation for both staff and the incarcerated population. For incarcerated individuals, significant deficiencies in the classification process, programming and recreation opportunities, and medical and mental health care thwart key components of their rehabilitation, including developing productive skills, preparing for less restrictive units or reentry, and maintaining positive behavior that contributes to the facility’s overall safety. On the part of officers, chronic understaffing and challenges inherent to the job pose dire challenges to their safety and well-being in their professional and personal lives.
- **Modernization.** DOCCS’ recordkeeping practices are out of date. Key systems like grievances, medical and mental health records, and performance evaluations are largely siloed, paper based and kept at the facility level. Even some DOCCS-wide systems—like special investigations into staff misconduct, staff discipline, and use of force reporting and reviews—are siloed. The absence of systemwide coordination and data tracking stymies DOCCS’ ability to identify use of force hotspots, problematic employees, or other critical trends. This makes DOCCS reactive to risks as they arise, rather than able to proactively address potential problems.

At the end of this report, we include 57 priority, short-, and long-term recommendations designed to foster rehabilitation and improve systemwide safety and efficiency. As discussed in greater detail in each section, key solutions are as follows:

- **Strengthen Oversight.** DOCCS should create the position of Chief Risk Officer (CRO) to oversee a centralized risk function. The CRO would be a senior official responsible for proactively identifying, assessing, and mitigating risks across DOCCS—including risks to both incarcerated individuals and staff. The CRO would detect and analyze data across functions and develop systemwide early warning systems with the capacity to communicate, and correct, in real time at the facility level. At the same time, DOCCS should also continue to prioritize the installation of fixed cameras, including installing cameras in high-need areas such as transport vans, and institute a systemwide protocol for facility leadership to review the use of body-worn cameras (BWCs).
- **Update Training.** DOCCS should conduct a comprehensive review of correction officer training curriculum to increase the use of scenario-based training wherever possible. We

propose that DOCCS conduct more frequent in-person training, while at the same time leveraging technology where appropriate. In particular, an enhanced training model for critical skills such as the use of force continuum, de-escalation, the duty to intervene, and supervisory responsibility can contribute to an overall safer environment and commitment to DOCCS' mission of rehabilitation.

- **Ensure Accountability.** DOCCS should establish or enhance accountability functions to include routine, centralized, and thorough reviews and audits, particularly for uses of force, grievances, complaints, and cameras. For use of force, specifically, DOCCS should implement a multidisciplinary review at the agency level for every severe use of force incident. At the same time, DOCCS should strengthen mechanisms to prevent retaliation for filing complaints. In addition, the DOCCS Commissioner should be granted explicit authority—using a transparent process with documented rationale—to discipline or terminate any employee for serious misconduct; we acknowledge that this will likely require legislative change or changes to labor agreements with staff unions.
- **Modernize Records.** DOCCS should transition all records from paper-based systems to a fully electronic format. Investing in modernized recordkeeping and early warning systems for key areas—including use of force, grievances, and health records—will improve DOCCS' oversight and response capabilities. At the same time, DOCCS should update its Use of Force Report form to standardize the collection of key information (e.g., facility location, incarcerated individual demographic information, etc.) and require detailed descriptions of compliance with DOCCS' BWC policies, de-escalation efforts, and techniques used prior to use of force, such as active listening and encouraging dialogue rather than confrontation.
- **Prioritize Mental Health.** For staff, DOCCS should establish a five-year formalized approach to employee health and wellness that emphasizes confidentiality and incorporates, among other tools, employee incentive programs, analysis of key performance indicators, referrals to treatment programs, and the employee wellness app. To bolster these efforts, DOCCS should establish a task force that oversees this formal staff wellness initiative. For the incarcerated population, DOCCS should increase coordination with the New York State Office of Mental Health (OMH), invest in mental health care and programming recordkeeping systems, prioritize patient privacy, and foster a culture of care that will improve mental health and behavioral outcomes and support equitable access to care.

It is important to recognize that meaningful change will require sustained leadership, additional financial, technological, and staff resources, and an executive and legislative commitment to achieving long-term safety, dignity, and accountability for both incarcerated individuals and staff. Our recommendations are by no means an end point; there is no quick fix here. Many of our recommendations will take years of dedicated effort, operational innovation,

and financial commitment, along with potential legislative action and updates to collective bargaining agreements. These recommendations provide the foundation for the years of work required to effectuate meaningful and sustained reform. The task ahead is substantial, but it is also an opportunity to build a correctional system defined not by its past failures, but by its commitment to safety, transparency, and human dignity.

To develop these recommendations, we conducted hundreds of confidential interviews with currently and formerly incarcerated individuals, current and former DOCCS staff, facility leadership, and executive leadership; undertook repeated, multi-day visits to Marcy and Mid-State to observe their conditions; and surveyed incarcerated individuals and staff at four facilities: Marcy, Mid-State, Sing Sing Correctional Facility (Sing Sing), and Gouverneur Correctional Facility (Gouverneur). We also reviewed records gathered from the facilities and across the system, including records related to use of force, grievances, oversight and accountability, training, discipline, staffing, medical and mental health care, and demographic information, as well as documents from OMH.

During our interviews, we received numerous thoughtful suggestions from incarcerated individuals, correction officers, and staff. Notably, many incarcerated individuals and staff expressed shared priorities: the desire for safe conditions and mutual respect. Incarcerated individuals sought to serve their sentences, pursue personal growth, and progress with their lives. Staff demonstrated both dedication to their roles—despite challenging working conditions—and deep dismay at the killings of Mr. Brooks and Mr. Nantwi. It is our hope that the report's recommendations will serve to benefit both groups.

1. KILLINGS OF ROBERT BROOKS AND MESSIAH NANTWI

1.1 Killing of Robert Brooks

In the early morning of December 10, 2024, 43-year-old Robert Brooks died at a Utica hospital following an incident at Marcy. Marcy staff had reported using force against Mr. Brooks on the evening of December 9 because, according to Marcy staff, Mr. Brooks “became irate” while being transported to the infirmary and began “swinging his arms violently.”¹ According to the consolidated Unusual Incident Report—a report filed after serious occurrences that may impact or disrupt facility operations, disrupt DOCCS’ public image, or arouse widespread public interest—one officer “used a bear [h]ug type hold and forced [Mr. Brooks] face first to the ground.”² Another officer then reportedly “applied one application of OC Pepper Spray,” after which another officer “responded and using both hands forced [Mr. Brooks’s] arms behind his back and forced mechanical wrist restrains onto both of [Mr. Brooks’s] wrists,” at which point Mr. Brooks “became compliant.”³ The report stated that Mr. Brooks was then “assisted to his feet and escorted to the infirmary.”⁴ Mr. Brooks “became unresponsive” at 9:39 p.m. (ET) on December 9, 2024.⁵ According to the report, the entire encounter—from the time that officers began transporting Mr. Brooks until the time he became nonresponsive—lasted approximately 15 minutes.⁶

On December 12, 2024, an investigator from OSI traveled to Marcy to collect the BWCs of the involved correction officers.⁷ OSI learned that, though none of the officers involved in the

¹ *People of the State of New York v. Anzalone et al.*, Indictment No. I 2025-035 (County of Oneida), Ex. E (Unusual Incident Report; Use of Force Staff Memorandum) (hereinafter “Brooks Indictment”), p. 2.

² Brooks Indictment Ex. E, p. 2.

³ Brooks Indictment Ex. E, p. 2.

⁴ Brooks Indictment Ex. E, pp. 2, 18.

⁵ Brooks Indictment Ex. E, p. 3.

⁶ Brooks Indictment Ex. E, p. 21.

⁷ *People of the State of New York v. Kingsley et al.*, Jury Trial, Tr. Volume IV at 775:6-20.

use of force incident against Mr. Brooks had activated the “record” button on their BWC, four of the officers’ cameras were passively recording during the incident and had stored the footage.⁸ DOCCS identified and proactively turned over the BWC footage to the New York Attorney General’s Office.⁹

The BWC footage contrasted sharply with the Unusual Incident Report. For example, the footage showed that on December 9, 2024, around 9 p.m. (ET), Marcy correction officers carried Mr. Brooks to the infirmary, with one officer holding his feet and legs parallel to the ground and two others holding his arms restrained above his head.¹⁰ The BWC footage then showed Mr. Brooks on an exam table in the infirmary, with his arms still restrained,¹¹ and one officer shoved a white cloth into his mouth.¹² Next, officers punched Mr. Brooks several times, repeatedly kicked him in the stomach, and one officer slipped on blue latex gloves before lifting Mr. Brooks up by the neck and pushing him to the treatment room wall, where officers fitted him with leg restraints.¹³ While officers beat Mr. Brooks, other DOCCS staff—including other correction

⁸ Joint Legislative Hearing *in re* 2025-2026 Executive Budget on Public Protection, Testimony of Daniel Martuscello III, Comm’r, Dep’t of Corr. & Cmty. Supervision (Feb. 13, 2025), pp. 385-86; Press Release, N.Y. ATTORNEY GENERAL LETITIA JAMES, *Attorney General James Releases Footage from Investigation into Death of Robert Brooks* (Dec. 27, 2024), <https://ag.ny.gov/press-release/2024/attorney-general-james-releases-footage-investigation-death-robert-brooks>; Memo from the New York State Office of the Inspector General No. 0659-090-2025 (Feb. 27, 2025), https://ig.ny.gov/system/files/documents/2025/03/2.25.25-dcjs-body-worn-camera_policy.pdf.

⁹ Joint Legislative Hearing *in re* 2025-2026 Executive Budget on Public Protection, Testimony of Daniel Martuscello III, Comm’r, Dep’t of Corr. & Cmty. Supervision (Feb. 13, 2025), pp. 385-86.

¹⁰ Body Camera Footage: CO Michael Along (Time stamp 21:22:30-21:22:44), <https://ag.ny.gov/osi/footage/robert-brooks>.

¹¹ Body Camera Footage: Sgt. Glenn Trombly (Time stamp 21:23:50-21:27:31), <https://ag.ny.gov/osi/footage/robert-brooks>.

¹² Body Camera Footage: CO Matthew Galliher (Time stamp 21:25:14-21:25:20), <https://ag.ny.gov/osi/footage/robert-brooks>.

¹³ Body Camera Footage: CO Michael Fisher (Time stamp 21:25:24-21:26:42), <https://ag.ny.gov/osi/footage/robert-brooks>; Body Camera Footage: CO Matthew Galliher (Time stamp 21:25:20-21:25:53), <https://ag.ny.gov/osi/footage/robert-brooks>; Body Camera Footage: Sgt. Glenn Trombly (Time stamp 21:25:20 to 21:32:04), <https://ag.ny.gov/osi/footage/robert-brooks>.

officers, sergeants and medical staff—stood by and watched without intervening.¹⁴ None of this was documented in the incident report.

DOCCS placed the 14 employees accused of being involved in the attack, some of whom had worked at DOCCS for more than 20 years, on administrative leave.¹⁵ On December 21, 2024, Governor Kathy Hochul released a statement noting that she had directed DOCCS Commissioner Daniel F. Martuscello III to begin the termination process for those 14 individuals and that both the New York Attorney General and OSI were conducting “ongoing reviews” of Mr. Brooks’s death.¹⁶

On January 2, 2025, the New York Attorney General’s Office recused itself from the investigation into Mr. Brooks’s death due to a conflict of interest, as the Office was already representing four of the implicated officers in other cases.¹⁷ The New York Attorney General’s Office appointed Onondaga County District Attorney William J. Fitzpatrick as a special prosecutor.¹⁸ In February 2025, the special prosecutor obtained an indictment from a special grand jury in Oneida County, charging ten Marcy officers with various crimes for their alleged

¹⁴ See Body Camera Footage: CO Michael Fisher (Time stamp 21:25:24 to 21:26:24), <https://ag.ny.gov/osi/footage/robert-brooks>; Body Camera Footage: CO Matthew Galliher (Time stamp 21:25:20-21:25:53), <https://ag.ny.gov/osi/footage/robert-brooks>; Body Camera Footage: Sgt. Glenn Trombly (Time stamp 21:25:20 to 21:29:04); Body Camera Footage: CO Michael Along (Time stamp: 21:22-21:33).

¹⁵ Ed Shanahan, *Hochul Orders Firing of 14 Prison Workers After Fatal Attack on Inmate*, N.Y. TIMES (Dec. 23, 2024), <https://www.nytimes.com/2024/12/23/nyregion/ny-prison-inmate-killed-hochul.html>; [Cite Redacted].

¹⁶ *Statement from Governor Kathy Hochul* (Dec. 21, 2024), <https://www.governor.ny.gov/news/statement-governor-kathy-hochul-45>.

¹⁷ Press Release, N.Y. ATT’Y GEN. LETITIA JAMES, *Attorney General James Updates New Yorkers on Investigation into the Death of Robert Brooks at Marcy Correctional Facility*, (Jan 2, 2025), <https://ag.ny.gov/press-release/2025/video-attorney-general-james-updates-new-yorkers-investigation-death-robert>.

¹⁸ Press Release, N.Y. ATT’Y GEN. LETITIA JAMES, *Attorney General James Updates New Yorkers on Investigation into the Death of Robert Brooks at Marcy Correctional Facility* (Jan 2, 2025), <https://ag.ny.gov/press-release/2025/video-attorney-general-james-updates-new-yorkers-investigation-death-robert>; Brooks Indictment.

involvement in the assault (the “Brooks Indictment”).¹⁹ Trial was set for October 6, 2025, in Oneida County Court.²⁰

1.2 Brooks Indictment

According to the Brooks Indictment, in the evening of December 9, 2024, Mr. Brooks was transferred by van from Mohawk Correctional Facility (Mohawk) to Marcy.²¹ Following his arrival at Marcy, during transport to the infirmary, and while restrained, he was assaulted on two occasions by three Marcy correction officers.²² Upon arrival in the infirmary, he was “restrained, beaten, choked, gagged, [and] forcibly moved and kicked,” despite “minimal resistance” and “with no legitimate law enforcement purpose.”²³ The indictment alleged that, during the beatings, correction officers “did nothing to restrain each other, did nothing to stop the beatings[,] and failed to immediately order medical assistance for Mr. Brooks,” and, as a result, Mr. Brooks suffered fatal injuries to his “head, neck, hyoid bone, thyroid cartilage, torso, liver, spleen[,] and testicles”; he also choked on his own blood.²⁴ According to the medical examiner’s report, Mr. Brooks was pronounced dead at 3:52 a.m. (ET) on December 10, 2024, due to compression of the neck and multiple blunt impact injuries.²⁵

For beating Mr. Brooks, the indictment charged six officers with murder in the second degree, six officers with manslaughter in the first degree, and two officers with gang assault in the second degree.²⁶ Several officers faced charges for failing to intervene and prevent the

¹⁹ Brooks Indictment.

²⁰ See, e.g., Case Details – Appearances, Case # IND-70121-25 / 002 (Defendant David Kingsley) (noting jury trial commenced on October 6, 2025, at 9:30 a.m.).

²¹ Brooks Indictment, p. 1.

²² Brooks Indictment, p. 1.

²³ Brooks Indictment, p. 1.

²⁴ Brooks Indictment, p. 1.

²⁵ Brooks Indictment, Ex. D (Medical Examiner’s Records).

²⁶ Brooks Indictment, pp. 1-2.

killing, though they did not directly participate in beating Mr. Brooks.²⁷ One sergeant was charged with manslaughter in the second degree because, “[b]eing aware of the risk that the beating by other Corrections Officers could result in the death of Robert Brooks, and having a duty to intervene, [he did] not order the beatings to cease and delayed ordering timely medical assistance.”²⁸ A second officer was charged with manslaughter in the second degree because, “[b]eing aware of the risk that the beating by other Corrections Officers could result in the death of Robert Brooks, and having a duty to intervene, [he did] not in any way attempt to stop the beating and did not in any way attempt to shield [Mr. Brooks] from the onslaught.”²⁹ A third officer was charged with manslaughter in the second degree for failing to intervene and, in addition, “verbally instruct[ing] the nurse not to enter the Emergency Room.”³⁰

Two officers faced additional charges of offering a false instrument for filing in the first degree for falsely stating on their Use of Force Reports that “prior to entering the Emergency Room of the Infirmary, all force ceased being used against Robert Brooks.”³¹ Another officer was charged with tampering with physical evidence for “clean[ing] the area of Robert Brooks’s blood stains in an effort to conceal the existence of said assault.”³²

1.3 Brooks Trial Proceedings

The trial in the killing of Mr. Brooks began on October 6, 2025. By the time of the trial, six out of the ten officers charged had pleaded guilty: one officer to first-degree manslaughter, agreeing to a 15-year sentence³³; two officers to first-degree manslaughter, each agreeing to 22-

²⁷ Brooks Indictment, p. 2.

²⁸ Brooks Indictment, p. 2.

²⁹ Brooks Indictment, p. 2.

³⁰ Brooks Indictment, pp. 2-3.

³¹ Brooks Indictment, p. 3.

³² Brooks Indictment, p. 3.

³³ Liam Stack & Jane Gottlieb, *Officer Pleads Guilty in Fatal Beating of Prisoner Seen on Camera*, N.Y. TIMES (May 5, 2025), <https://www.nytimes.com/2025/05/05/nyregion/christopher-walrath-guilty-robert-brooks.html>.

year sentences³⁴; and a sergeant and an officer to second-degree manslaughter (the sergeant agreed to be sentenced to three to nine years, and the officer agreed to be sentenced to two years and four months to seven years).³⁵ The officer charged in connection with cleaning the area of Mr. Brooks's blood stains pled guilty to a misdemeanor charge of attempted tampering with evidence and received a one-year conditional discharge.³⁶

Three officers proceeded to trial.³⁷ The jury returned its verdict on October 20, 2025, after three days of deliberation.³⁸ One officer was found guilty of second-degree murder and first-degree manslaughter.³⁹ He was later sentenced to 25 years to life in prison.⁴⁰ Another officer, who faced charges of second-degree murder, first-degree manslaughter, and gang assault, was acquitted, as was an officer who was charged with second-degree murder, first-degree manslaughter, second-degree gang assault, and first-degree offering a false instrument for filing.⁴¹ After trial, one officer moved to withdraw his guilty plea based on what he asserted to

³⁴ Camille Baker, *Four N.Y. Prison Officers Plead Guilty to Manslaughter in Taped Beating*, N.Y. TIMES (Sept. 22, 2025), <https://www.nytimes.com/2025/09/22/nyregion/marcy-prison-manslaughter-pleas.html>.

³⁵ Camille Baker, *Four N.Y. Prison Officers Plead Guilty to Manslaughter in Taped Beating*, N.Y. TIMES (Sept. 22, 2025), <https://www.nytimes.com/2025/09/22/nyregion/marcy-prison-manslaughter-pleas.html>.

³⁶ Camille Baker, *Four N.Y. Prison Officers Plead Guilty to Manslaughter in Taped Beating*, N.Y. TIMES (Sept. 22, 2025), <https://www.nytimes.com/2025/09/22/nyregion/marcy-prison-manslaughter-pleas.html>.

³⁷ See, e.g., Case Details – Appearances, Case # IND-70121-25 / 002 (Defendant David Kingsley) (noting jury trial commenced on October 6, 2025, at 9:30 a.m.); see also Jury Trial Transcript, *People of the State of New York v. Kingsley et al.*, Indictment Nos. 70121-25/002, 70121-25/005, 70121-25/006 (County of Oneida).

³⁸ *People of the State of New York v. Kinglsey et al.*, Jury Trial, Tr. Volume X (Oct. 20, 2025).

³⁹ *People of the State of New York v. Kinglsey et al.*, Jury Trial, Tr. Volume X (Oct. 20, 2025).

⁴⁰ Lexi Bruening, *David Kingsley Sentenced to 25 Years to Life in Prison*, 7 News WWNY (Dec. 19, 2025), <https://www.wwnynv.com/2025/12/19/david-kingsley-sentenced-25-years-life-prison/>.

⁴¹ *People of the State of New York v. Kinglsey et al.*, Jury Trial, Tr. Volume X (Oct. 20, 2025).

be confusion regarding whether DOCCS' employment directives created a legal duty to intervene during the beating of Mr. Brooks.⁴² The court denied his request.⁴³

The tenth and remaining officer, who was charged with second-degree manslaughter, went to trial in January 2026.⁴⁴ After the jury failed to reach a verdict on the manslaughter charge, that officer pled guilty to a lesser charge of second-degree reckless endangerment and received a six-month sentence.⁴⁵

1.4 Killing of Messiah Nantwi

A little more than two months after Mr. Brooks's death, on February 17, 2025, DOCCS correction officers went on an illegal strike.⁴⁶ The illegal strike started at Collins Correctional Facility and expanded to 38 of the 42 facilities across the state, ultimately involving roughly 75% of security staff.⁴⁷ The striking officers cited unsafe conditions, excessive hours, and opposition to the Humane Alternatives to Long-Term (HALT) Solitary Confinement Act as primary reasons

⁴² Carleigh Minor, *Attorney Explains Release of Ex-CO Who Pleaded Guilty in Robert Brooks' Death*, ABC 13 WHAM (Dec. 11, 2025), <https://13wham.com/news/local/attorney-explains-release-of-ex-co-who-pleaded-guilty-in-robert-brooks-death-marcy-correctional-facility-david-walters>.

⁴³ Carleigh Minor, *Attorney Explains Release of Ex-CO Who Pleaded Guilty in Robert Brooks' Death*, ABC 13 WHAM (Dec. 11, 2025), <https://13wham.com/news/local/attorney-explains-release-of-ex-co-who-pleaded-guilty-in-robert-brooks-death-marcy-correctional-facility-david-walters>.

⁴⁴ Case Details – Appearances, Case # IND-70121-25 / 008 (Michael Fisher) (noting that jury trial was scheduled for January 12, 2026 at 9:30 a.m.); Brooks Indictment.

⁴⁵ *Prison Guard Accused of Failing to Step in During Fatal Inmate Beating Pleads to Reduced Charge*, ASSOCIATED PRESS (Jan. 16, 2026), <https://www.washingtontimes.com/news/2026/jan/16/prison-guard-accused-failing-step-fatal-inmate-beating-pleads-reduced/>; *Last CO Sentenced in Connection to Robert Brooks Death*, SPECTRUM NEWS 1 (Jan. 30, 2026), <https://spectrumlocalnews.com/nys/binghamton/news/2026/01/30/michael-fisher-sentencing>.

⁴⁶ Report Pursuant to Article 14 New York State Civil Service Law Section 210(4) Concerning an Illegal Strike by Certain Public Employees of the Executive Branch of the State of New York, p. 1, NEW YORK OFFICE OF EMPLOYEE RELATIONS (May 9, 2025), https://oer.ny.gov/system/files/documents/2025/05/2025-doccs-strike-report_250509.pdf; Halena Spulveda, *Hochul to send National Guard to state prisons if strike doesn't end Wednesday*, SPECTRUM NEWS (Feb. 18, 2025), <https://spectrumlocalnews.com/nys/central-ny/public-safety/2025/02/17/collins-elmira-correctional-facilities-officers-unsanctioned-strike>.

⁴⁷ Report Pursuant to Article 14 New York State Civil Service Law Section 210(4) Concerning an Illegal Strike by Certain Public Employees of the Executive Branch of the State of New York, p. 1, NEW YORK OFFICE OF EMPLOYEE RELATIONS (May 9, 2025), https://oer.ny.gov/system/files/documents/2025/05/2025-doccs-strike-report_250509.pdf.

for striking.⁴⁸ As a result of staffing shortages during the illegal strike, DOCCS' Correctional Emergency Response Team (CERT), which was usually a specialized secondary response unit, acted as the primary responder to incidents in certain facilities, including Mid-State.⁴⁹ Governor Hochul also deployed over 6,000 National Guard members to staff the state prisons.⁵⁰ By the end of February 2025, most security staff were refusing to work, officers that were still working in the facilities were stretched thin, facilities relied on CERT to respond to incidents, and the National Guard was filling crucial roles within the facilities.

On March 1, 2025—while the illegal strike was ongoing—DOCCS officers at Mid-State allegedly beat to death another incarcerated man, 22-year-old Messiah Nantwi. Mid-State is located just across the street from Marcy. The two facilities are part of the same prison “hub,” along with Mohawk and Hale Creek Correctional Facility.⁵¹

According to the civil complaint, Mr. Nantwi was absent during “the count”—when all incarcerated individuals must be in line for an official headcount.⁵² According to public reporting, witnesses said that Mr. Nantwi had been taking a shower and that he was emotional and off of his usual psychiatric medication.⁵³ According to witnesses, Mr. Nantwi returned to his

⁴⁸ *N.Y. Prison Strike: What Led to it, State Response and What Could Come Next*, CORRECTIONS1 (Mar. 7, 2025), <https://www.corrections1.com/officer-safety/n-y-prison-strike-what-led-to-it-state-response-and-what-could-come-next>.

⁴⁹ [Cite Redacted].

⁵⁰ Report Pursuant to Article 14 New York State Civil Service Law Section 210(4) Concerning an Illegal Strike by Certain Public Employees of the Executive Branch of the State of New York, p. 2, NEW YORK OFFICE OF EMPLOYEE RELATIONS (May 9, 2025), https://oer.ny.gov/system/files/documents/2025/05/2025-doccs-strike-report_250509.pdf; Chelsia Rose Marcus, *New Deal Reached to End Wildcat Strikes by N.Y. Prison Guards*, N.Y. TIMES (Mar. 9, 2025), <https://www.nytimes.com/2025/03/09/nyregion/new-york-prison-strike-deal.html>.

⁵¹ *Facility Map*, DOCCS (updated Nov. 2024), https://doccs.ny.gov/system/files/documents/2025/01/facility-map_112024-updated.pdf.

⁵² Complaint ¶¶40-41, *Estate of Messiah Nantwi v. Bartlett*, No. 9:25-cv-775 (N.D.N.Y. June 17, 2025), ECF No. 1.

⁵³ Chris Gelardi, ‘I’m Doing This for My Friend’: Imprisoned Man Recounts Watching Guards Beat Messiah Nantwi to Death, NEW YORK FOCUS (Mar. 28, 2025), <https://nysfocus.com/2025/03/28/prisoner-nantwi-death-midstate-correctional>.

dorm complaining, which led to an argument with the National Guardsman in the dorm.⁵⁴ The National Guardsman allegedly called for backup,⁵⁵ after which a CERT team allegedly entered Mr. Nantwi's room⁵⁶ and demanded to know "Who has the problem?"⁵⁷ The National Guardsman pointed to Mr. Nantwi.⁵⁸

Several officers—many of them CERT members—then allegedly began beating Mr. Nantwi, repeatedly striking his body and head with fists, batons, and boots, while Mr. Nantwi had his hands raised and displayed no weapons.⁵⁹ The alleged beating intensified after Mr. Nantwi purportedly bit the hand of one of the officers during the beating.⁶⁰

Mr. Nantwi's cellmate reported in an interview hearing a "wet splat sound" when officers eventually slammed him onto the ground.⁶¹ Witnesses allege that Mr. Nantwi begged the officers to stop and screamed incoherently until he eventually fell silent.⁶² According to these witnesses,

⁵⁴ Chris Gelardi, 'I'm Doing This for My Friend': Imprisoned Man Recounts Watching Guards Beat Messiah Nantwi to Death, NEW YORK FOCUS (Mar. 28, 2025), <https://nysfocus.com/2025/03/28/prisoner-nantwi-death-midstate-correctional>.

⁵⁵ Complaint ¶ 44, *Estate of Messiah Nantwi v. Bartlett*, No. 9:25-cv-775 (N.D.N.Y. June 17, 2025), ECF No. 1.

⁵⁶ Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10, Oneida Cnty. Ct. (N.Y. 2025).

⁵⁷ Complaint ¶ 45, *Estate of Messiah Nantwi v. Bartlett*, No. 9:25-cv-775 (N.D.N.Y. June 17, 2025), ECF No. 1; Chris Gelardi, 'I'm Doing This for My Friend': Imprisoned Man Recounts Watching Guards Beat Messiah Nantwi to Death, NEW YORK FOCUS (Mar. 28, 2025), <https://nysfocus.com/2025/03/28/prisoner-nantwi-death-midstate-correctional>.

⁵⁸ Complaint ¶¶ 46-47, *Estate of Messiah Nantwi v. Bartlett*, No. 9:25-cv-775 (N.D.N.Y. June 17, 2025), ECF No. 1; Chris Gelardi, 'I'm Doing This for My Friend': Imprisoned Man Recounts Watching Guards Beat Messiah Nantwi to Death, NEW YORK FOCUS (Mar. 28, 2025), <https://nysfocus.com/2025/03/28/prisoner-nantwi-death-midstate-correctional>.

⁵⁹ Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10, Oneida Cnty. Ct. (N.Y. 2025).

⁶⁰ Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10, Oneida Cnty. Ct. (N.Y. 2025).

⁶¹ Chris Gelardi, 'I'm Doing This for My Friend': Imprisoned Man Recounts Watching Guards Beat Messiah Nantwi to Death, NEW YORK FOCUS (Mar. 28, 2025), <https://nysfocus.com/2025/03/28/prisoner-nantwi-death-midstate-correctional>.

⁶² Complaint ¶¶ 54-56, *Estate of Messiah Nantwi v. Bartlett*, No. 9:25-cv-775 (N.D.N.Y. June 17, 2025), ECF No. 1; Chris Gelardi, 'I'm Doing This for My Friend': Imprisoned Man Recounts Watching Guards Beat Messiah Nantwi to Death, NEW YORK FOCUS (Mar. 28, 2025), <https://nysfocus.com/2025/03/28/prisoner-nantwi-death-midstate-correctional>.

during the beating, the officers yelled “stop resisting,” in an apparent attempt to justify the use of force.⁶³ The indictment alleges that the beating lasted approximately five minutes.⁶⁴

Once Mr. Nantwi was no longer responsive, the officers allegedly transported him from his dorm in building 21 to the infirmary.⁶⁵ According to the indictment, during the transport, in the stairwell of building 21, the officers allegedly further beat Mr. Nantwi.⁶⁶ Once the officers had transported Mr. Nantwi to a holding cell in the infirmary, they allegedly beat him for a third time and left him unattended for several minutes, displaying no urgency in obtaining medical attention.⁶⁷

Ultimately, Mr. Nantwi was taken to Wynn Hospital and pronounced dead.⁶⁸ According to the prosecutor, the medical examiner reported that Mr. Nantwi had “died from traumatic brain injuries as a result of violent blows to his head.”⁶⁹ In addition to the traumatic brain injuries due to the violent blows to his head, Mr. Nantwi suffered at least 69 other serious blows to his body.⁷⁰

Several involved staff participated in an alleged scheme to undermine any investigation into Mr. Nantwi’s killing. According to a civil complaint related to his death, during the incident,

⁶³ Complaint ¶ 55, *Estate of Messiah Nantwi v. Bartlett*, No. 9:25-cv-775 (N.D.N.Y. June 17, 2025), ECF No. 1; Chris Gelardi, ‘I’m Doing This for My Friend’: Imprisoned Man Recounts Watching Guards Beat Messiah Nantwi to Death, NEW YORK FOCUS (Mar. 28, 2025), <https://nysfocus.com/2025/03/28/prisoner-nantwi-death-midstate-correctional>.

⁶⁴ Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10 (Oneida Cnty. Ct. N.Y. 2025).

⁶⁵ Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10 (Oneida Cnty. Ct. N.Y. 2025).

⁶⁶ Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10 (Oneida Cnty. Ct. (Oneida Cnty. Ct. N.Y. 2025)).

⁶⁷ Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10 (Oneida Cnty. Ct. N.Y. 2025).

⁶⁸ Complaint ¶ 71, *Estate of Messiah Nantwi v. Bartlett*, No. 9:25-cv-775 (N.D.N.Y. June 17, 2025), ECF No. 1; Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10 (Oneida Cnty. Ct. N.Y. 2025).

⁶⁹ Deena Zaru & Tesfaye Negussie, *10 NY Prison Guards Indicted, 2 for Murder, in Connection to Inmate's Beating Death*, ABC NEWS (Apr. 16, 2025), <https://abcnews.go.com/US/prison-guards-indicted-connection-inmates-death-hochul/story?id=120861314>.

⁷⁰ Deena Zaru & Tesfaye Negussie, *10 NY Prison Guards Indicted, 2 for Murder, in Connection to Inmate's Beating Death*, ABC NEWS (Apr. 16, 2025), <https://abcnews.go.com/US/prison-guards-indicted-connection-inmates-death-hochul/story?id=120861314>.

some of the officers turned off their BWCs or turned away to avoid recording the abuse.⁷¹ According to the indictment filed, after the incident, involved staff and others also allegedly collaborated to write false post-incident reports.⁷² For example, some of the involved officers allegedly agreed, at the direction of a sergeant, to plant a weapon in Mr. Nantwi's room to justify the force used.⁷³ They also allegedly agreed to omit two officers from any reporting,⁷⁴ one of whom they omitted because the officer was in the middle of a disciplinary evaluation period, meaning the officer would have faced termination for any misconduct in the killing of Mr. Nantwi.⁷⁵ All of the named defendants and other involved officers allegedly met at a local diner on March 2, 2025 to agree on a narrative to obstruct the investigation into the killing.⁷⁶ Supervisors allegedly had key roles in the cover-up: according to the indictment, one sergeant directed the officers to plant a weapon in Mr. Nantwi's room, one sergeant assisted in the plot to plant the weapon, one sergeant provided a written memorandum that described the sergeant witnessing no force being used, despite being in clear view of the beating (and turning the BWC recording device away from the altercation), and one sergeant mopped up the blood in Mr. Nantwi's cell to obscure the crime scene.⁷⁷

⁷¹ Complaint ¶ 57, *Estate of Messiah Nantwi v. Bartlett*, No. 9:25-cv-775 (N.D.N.Y. June 17, 2025), ECF No. 1.

⁷² Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10 (Oneida Cnty. Ct. N.Y. 2025).

⁷³ Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10 (Oneida Cnty. Ct. N.Y. 2025).

⁷⁴ Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10 (Oneida Cnty. Ct. N.Y. 2025).

⁷⁵ Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10 (Oneida Cnty. Ct. N.Y. 2025); [Cite Redacted].

⁷⁶ Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10 (Oneida Cnty. Ct. N.Y. 2025).

⁷⁷ Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10, Oneida Cnty. Ct. (N.Y. 2025).

1.5 Nantwi Indictment

The New York Attorney General's Office appointed Onondaga County District Attorney William J. Fitzpatrick as a special prosecutor for the death of Mr. Nantwi.⁷⁸ On April 16, 2025, the court unsealed the Oneida County indictment of ten staff members, including three sergeants and seven correction officers.⁷⁹ Based on the unsealed indictment, two officers were charged with second-degree murder, five were charged with first-degree manslaughter, two were charged with second-degree manslaughter, six were charged with gang assault, nine were charged with offering a false instrument for file, four were charged with tampering with physical evidence, and eight were charged with conspiracy.⁸⁰ Other involved staff have waived indictments and others have indictments under seal.⁸¹

⁷⁸ Attorney General Letitia James opened an investigation into Mr. Nantwi's killing but then recused herself and appointed Onondaga County District Attorney William Fitzpatrick as special prosecutor, which is the same office appointed to the Brooks trial. The New York Attorney General's Office was already defending at least five of the officers involved in the Nantwi incident in civil cases arising from their employment at DOCCS. *See* Jan Ransom, *New York Attorney General Recuses Herself from Inquiry into Prison Death*, THE NEW YORK TIMES (Mar. 6, 2025), <https://www.nytimes.com/2025/03/06/nyregion/ny-prison-death-investigation-attorney-general.html>.

⁷⁹ *See* Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10 (Oneida Cnty. Ct. N.Y. 2025).

⁸⁰ *See* Indictment, *People v. Levi et al*, Indictment No. I 2025-090-1-2-3-4-5-6-7-8-9-10 (Oneida Cnty. Ct. N.Y. 2025)

⁸¹ [Cite Redacted].

Thirteen officers publicly pleaded guilty⁸² in connection with the death of Mr. Nantwi.⁸³ One correction officer pleaded guilty to first-degree manslaughter.⁸⁴ One sergeant pleaded guilty to second-degree gang assault.⁸⁵ Eleven other security staff pleaded guilty to other charges—

⁸² For the sake of this paragraph, we are only reporting on public pleas; these numbers do not represent any additional activity that may not be reflected on the public docket.

⁸³ See Matt Saffer, *Mid-State Corrections Officer Pleads Guilty to Charges Related to Inmate's Fatal Beating*, CNY CENTRAL (June 13, 2025) (Nathan Palmer), <https://cnycentral.com/news/local/mid-state-corrections-officer-pleads-guilty-to-charges-related-to-inmates-fatal-beating-correctional-messiah-nantwi>; *Correction Officer Pleads Guilty in Mid-State Inmate Death*, SPECTRUM NEWS (July 1, 2025) (Nicholas Vitale), <https://spectrumlocalnews.com/nys/central-ny/public-safety/2025/07/01/mid-state-correction-officer-guilty-plea>; Josh McMullen, *Ferrone Accepts Plea Deal in Nantwi Case*, WUTR UTICA (Oct. 1, 2025) (David Ferrone), <https://www.yahoo.com/news/articles/ferrone-accepts-plea-deal-nantwi-184614062.html>; Josh McMullen, *Chandler Pleads Guilty in Nantwi Case*, WUTR UTICA (Oct. 16, 2025) (Francis Chandler), <https://www.yahoo.com/news/articles/chandler-pleads-guilty-nantwi-case-153318161.html>; *Mid-State CO Charged with Manslaughter Pleads to Non-violent Felony*, SPECTRUM NEWS (Aug. 14, 2025) (Daniel Burger), <https://spectrumlocalnews.com/nys/watertown/public-safety/2025/08/14/mid-state-co-charged-with-manslaughter-pleas-to-non-violent-felony>; *Former Mid State Prison Guard Pleads Guilty to Misdemeanor*, SPECTRUM NEWS (Oct. 31, 2025) (Donald Slawson), <https://spectrumlocalnews.com/nys/hudson-valley/news/2025/10/31/ex-mid-state-prison-guard-pleads-guilty-to-misdemeanor>; *Former New York Prison Guard Pleads Guilty in Connection with Inmate's Beating Death*, CNN (May 31, 2025), (Joshua Bartlett), <https://www.cnn.com/2025/05/30/us/former-prison-guard-guilty-plea-messiah-nantwi-death>; Sean I. Mills, *Two COs Admit to Blocking Body Cameras in Mid-State Beating*, DAILY SENTINEL (Oct. 30, 2025), (Adam Joseph & Frank Jacobs), https://www.romesentinel.com/news/marcy-midstate-prison-guilty-joseph-jacobs/article_d28f71aa-05e1-48a1-a1bc-daa112ae745d.html.

⁸⁴ *Former Correction Officer Makes Surprise Guilty Plea at Start of Mid-State Murder Trial*, SPECTRUM NEWS (May 4, 2026) (Caleb Blair), <https://spectrumlocalnews.com/nys/central-ny/news/2026/05/04/former-mid-state-correction-officer-makes-surprise-guilty-plea-at-start-of-trial>

⁸⁵ Josh McMullen, *Chandler Pleads Guilty in Nantwi Case*, WUTR UTICA (Oct. 16, 2025) (Francis Chandler), <https://www.yahoo.com/news/articles/chandler-pleads-guilty-nantwi-case-153318161.html>.

including filing a false instrument, tampering with physical evidence, hindering prosecution, among others—and are cooperating with the District Attorney’s Office.⁸⁶

1.6 Nantwi Trial Proceedings

One officer pleaded not guilty and faced trial in March 2026. He was convicted of first-degree manslaughter, gang assault, conspiracy, and offering a false instrument for filing in connection with the death of Messiah Nantwi.⁸⁷ He was acquitted of second-degree murder.⁸⁸ In May 2026, he was sentenced to 25 years in state prison.⁸⁹ DOCCS’ and Governor’s Responses to the Killings

⁸⁶ See Matt Saffer, *Mid-State Corrections Officer Pleads Guilty to Charges Related to Inmate’s Fatal Beating*, CNY CENTRAL (June 13, 2025) (Nathan Palmer), <https://cnycentral.com/news/local/mid-state-corrections-officer-pleads-guilty-to-charges-related-to-inmates-fatal-beating-correctional-messiah-nantwi>; *Correction Officer Pleads Guilty in Mid-State Inmate Death*, SPECTRUM NEWS (July 1, 2025) (Nicholas Vitale), <https://spectrumlocalnews.com/nys/central-ny/public-safety/2025/07/01/mid-state-correction-officer-guilty-plea>; Josh McMullen, *Ferrone Accepts Plea Deal in Nantwi Case*, WUTR UTICA (Oct. 1, 2025) (David Ferrone), <https://www.yahoo.com/news/articles/ferrone-accepts-plea-deal-nantwi-184614062.html>; *Mid-State CO Charged with Manslaughter Pleads to Non-violent Felony*, SPECTRUM NEWS (Aug. 14, 2025) (Daniel Burger), <https://spectrumlocalnews.com/nys/watertown/public-safety/2025/08/14/mid-state-co-charged-with-manslaughter-pleas-to-non-violent-felony>; *Former Mid State Prison Guard Pleads Guilty to Misdemeanor*, SPECTRUM NEWS (Oct. 31, 2025) (Donald Slawson), <https://spectrumlocalnews.com/nys/hudson-valley/news/2025/10/31/ex-mid-state-prison-guard-pleads-guilty-to-misdemeanor->; *Former New York Prison Guard Pleads Guilty in Connection with Inmate’s Beating Death*, CNN (May 31, 2025), (Joshua Bartlett), <https://www.cnn.com/2025/05/30/us/former-prison-guard-guilty-plea-messiah-nantwi-death>; Sean I. Mills, *Two COs Admit to Blocking Body Cameras in Mid-State Beating*, DAILY SENTINEL (Oct. 30, 2025), (Adam Joseph & Frank Jacobs), https://www.romesentinel.com/news/marcy-midstate-prison-guilty-joseph-jacobs/article_d28f71aa-05e1-48a1-a1bc-daa112ae745d.html; *Former Mid-State Correction Officer Makes Plea in Nantwi Death in Lieu of Trial*, SPECTRUM NEWS (May 1, 2026) (Craig Klemick), <https://spectrumlocalnews.com/nys/central-ny/news/2026/05/01/mid-state-correction-officer-makes-plea-in-nantwi-death-in-lieu-of-trial>; Greta Stuckey, *Ex-N.Y. Correctional Officer Pleads Guilty to Planting Evidence in Inmate Death*, CORRECTIONS1 (March 23, 2026) (Michael Iffert), <https://www.corrections1.com/legal/ex-n-y-correctional-officer-pleads-guilty-to-planting-evidence-in-inmate-death>; Matthew Benninger, *Last Former Mid-State Guard Tied to Inmate’s Fatal Beating Takes Plea Deal*, CNYCENTRAL (May 13, 2026) (Thomas Eck), <https://cnycentral.com/news/local/final-former-mid-state-guard-tied-to-inmates-fatal-beating-takes-plea-deal>.

⁸⁷ *Former Correction Officer Sentenced in Connection with Mid-State Prison Death*, SPECTRUM NEWS (May 27, 2026), <https://spectrumlocalnews.com/nys/central-ny/news/2026/05/27/jonah-levi-sentencing-messiah-nantwi-mid-state-death>.

⁸⁸ *Former Correction Officer Found Guilty of 5 Counts in Mid-State Prison Death*, SPECTRUM NEWS (April 1, 2026), <https://spectrumlocalnews.com/nys/central-ny/public-safety/2026/03/25/murder-trial-for-former-mid-state-officer-underway>.

⁸⁹ Casey Pritchard, *Former Mid-State CO Sentenced to 25 Years for Death of Messiah Nantwi*, UTICA OBSERVER DISPATCH (May 27, 2026), <https://www.uticaod.com/story/news/crime/2026/05/27/mid-state-co-sentenced-to-25-years-for-death-of-messiah-nantwi/90277596007/?gnt-cfr=1&gca-cat=p&gca-uir=false&gca-epti=z116844p116850c116850e008600v116844&gca-ft=7&gca-ds=sophi>.

In the immediate aftermath of Mr. Brooks's killing, both Governor Hochul and Commissioner Martuscello expressed horror and disgust at the actions of the officers involved in the killing of Mr. Brooks and offered their condolences to his family.⁹⁰ Governor Hochul and DOCCS quickly sought termination of the officers and made criminal referrals to external investigatory agencies.⁹¹

DOCCS also took steps to attend to the incarcerated populations at Marcy, including organizing leadership meetings with incarcerated individuals regarding the incident, reviewing grievances and allegations, and installing secured mailboxes through which incarcerated individuals can submit correspondence to Marcy's executive team.⁹²

In addition, on December 18, 2024, Commissioner Martuscello directed that a member of facility leadership must always work the 3 p.m. (ET) to 11 p.m. (ET) shift at every facility and must always report to the medical building after a use of force incident.⁹³ Mid-State was not in compliance with this instruction on the day Mr. Nantwi was killed, as no leadership was on-site.⁹⁴

Following the killing of Mr. Nantwi, DOCCS took additional corrective actions. DOCCS commenced disciplinary proceedings against those individuals who had not already resigned.⁹⁵ DOCCS installed fixed cameras across the Mid-State infirmary and medical building.⁹⁶ DOCCS

⁹⁰ Statement from Governor Kathy Hochul, (Dec. 27, 2024), <https://www.governor.ny.gov/news/statement-governor-kathy-hochul-46>; Joint Legislative Hearing *in re* 2025-2026 Executive Budget on Public Protection, Testimony of Daniel Martuscello III, Comm'r, DOCCS, pp. 213-14 (Feb. 13, 2025).

⁹¹ Joint Legislative Hearing *in re* 2025-2026 Executive Budget on Public Protection, Testimony of Daniel Martuscello III, Comm'r, DOCCS, pp. 213-14 (Feb. 13, 2025).

⁹² [Cite Redacted].

⁹³ [Cite Redacted].

⁹⁴ [Cite Redacted].

⁹⁵ [Cite Redacted].

⁹⁶ [Cite Redacted].

also initiated implementation of facility-wide CCTV at Mid-State, with completion expected by the end of 2028.⁹⁷

Since the murder of Mr. Brooks, DOCCS and Governor Hochul have also initiated corrective actions relating to personnel, policies, and infrastructure. On December 30, 2024, Governor Hochul visited Marcy.⁹⁸ During that visit, she directed the appointment of a new permanent Marcy superintendent; expanded mandatory stationary cameras and BWC use at all DOCCS facilities, along with auditing to ensure compliance; directed DOCCS to hire more OSI staff to respond to reports of abuse and conduct investigations; directed the expansion of DOCCS' whistleblower hotline to more effectively facilitate anonymous tips; and ordered the instant review into Mr. Brooks's death.⁹⁹

DOCCS also took agency-wide steps to enhance oversight, accountability, and staffing, beginning in late 2024. DOCCS updated its BWC policy to require activation any time security staff interacts with the incarcerated population and enhanced performance monitoring capabilities, including by training facility leadership on how to supervise staff compliance with the BWC policy.¹⁰⁰ DOCCS and the Governor accelerated the investment of more than \$413 million to install over 6,000 additional fixed surveillance cameras, including expediting fixed-camera installations at Marcy, and to implement the use of over 4,500 additional BWC

⁹⁷ [Cite Redacted].

⁹⁸ *Governor Hochul Visits Marcy Correctional Facility Demanding Answer Following Death of Robert Brooks and Announces Immediate Corrective Actions* (Dec. 30, 2024), <https://www.governor.ny.gov/news/governor-hochul-visits-marcy-correctional-facility-demanding-answers-following-death-robert>.

⁹⁹ *Governor Hochul Visits Marcy Correctional Facility Demanding Answer Following Death of Robert Brooks and Announces Immediate Corrective Actions* (Dec. 30, 2024), <https://www.governor.ny.gov/news/governor-hochul-visits-marcy-correctional-facility-demanding-answers-following-death-robert>.

¹⁰⁰ Joint Legislative Hearing *in re* 2025-2026 Executive Budget on Public Protection, Testimony of Daniel Martuscello III, Comm'r, DOCCS, p. 214 (Feb. 13, 2025); *Governor Hochul Visits Marcy Correctional Facility Demanding Answer Following Death of Robert Brooks and Announces Immediate Corrective Actions* (Dec. 30, 2024), <https://www.governor.ny.gov/news/governor-hochul-visits-marcy-correctional-facility-demanding-answers-following-death-robert>; [Cite Redacted].

devices.¹⁰¹ DOCCS also obtained funding for additional OSI personnel, including investigator positions.¹⁰² The Commissioner and Governor also sought to alleviate the staffing crisis by expanding recruitment programs and removing certain applicant age and residency restrictions.¹⁰³

1.7 CERT

In the months following Mr. Nantwi's killing, DOCCS also reformed CERT by strengthening background checks, enhancing training, and updating policies.¹⁰⁴

On July 9, 2025, Commissioner Martuscello announced immediate changes to several aspects of CERT. These changes include more stringent eligibility requirements, enhanced training, and increased accountability.¹⁰⁵ For example, where DOCCS previously conducted background checks only on supervisors (i.e., sergeants and lieutenants) who were applying for CERT, DOCCS now requires both a background check and a mental health review for any CERT applicant.¹⁰⁶ Disqualifying characteristics include, but are not limited to, time and attendance violations, disciplinary decisions resulting in sanctions, and substantiated complaints of misconduct.¹⁰⁷

In addition, DOCCS updated its training and deployment requirements for the CERT team. Before July 2025, prospective CERT members could take several years to complete the

¹⁰¹ [Cite Redacted]; Joint Legislative Hearing *in re* 2025-2026 Executive Budget on Public Protection, Testimony of Daniel Martuscello III, Comm'r, DOCCS, pp. 231-232 (Feb. 13, 2025).

¹⁰² [Cite Redacted]; Joint Legislative Hearing *in re* 2025-2026 Executive Budget on Public Protection, Testimony of Daniel Martuscello III, Comm'r, DOCCS, pp. 345-346 (Feb. 13, 2025).

¹⁰³ *Recruit, Recover, and Rebuild*, DOCCS, <https://doccs.ny.gov/recruit-recover-and-rebuild-0>; Joint Legislative Hearing *in re* 2025-2026 Executive Budget on Public Protection, Testimony of Daniel Martuscello III, Comm'r, DOCCS, pp. 216-217, 282 (Feb. 13, 2025).

¹⁰⁴ We requested the CERT Operations Manual, but only received a copy with significant redactions—particularly in the sections on deployment and tactical operations—so therefore cannot make additional specific findings as to the detailed operations of CERT.

¹⁰⁵ Memorandum from Commissioner Martuscello regarding CERT, DOCCS (July 9, 2025).

¹⁰⁶ Memorandum from Commissioner Martuscello regarding CERT, DOCCS (July 9, 2025).

¹⁰⁷ Memorandum from Commissioner Martuscello regarding CERT, DOCCS (July 9, 2025).

required 96-hour training and, in the meantime, still be deployed with a CERT team.¹⁰⁸ Today, the CERT training regime distinguishes between CERT basic training (which must be completed before any deployment) and the full regimen of CERT-specific training (which can take longer to complete).¹⁰⁹ Basic training covers professional conduct and expectations, leadership, communication, high-risk transportation, de-escalation techniques, firearms, and use of force, including the progression of force and the legal authority.¹¹⁰ The July 2025 memorandum states: “The candidate will remain in non-deployable status until CERT firearms instruction and qualification have been successfully completed. Once the required qualification is attained, the candidate will then be considered a deployable CERT member and will continue to receive the required monthly training.”¹¹¹

On October 6, 2025, DOCCS issued Directive #4952, which memorialized the July 9 changes into policy.¹¹² The directive explicitly requires all CERT members to wear and activate BWCs on deployments.¹¹³ Additionally, the new directive requires CERT members to utilize only authorized uniforms and equipment,¹¹⁴ prohibiting rogue paraphernalia.¹¹⁵ Lastly, the directive outlines a review protocol after each CERT activation, including a detailed “demobilization” report with supporting documentation for a compliance review by DOCCS’ central office.¹¹⁶

¹⁰⁸ Memorandum from Commissioner Martuscello regarding CERT, DOCCS (July 9, 2025).

¹⁰⁹ Memorandum from Commissioner Martuscello regarding CERT, DOCCS (July 9, 2025).

¹¹⁰ Memorandum from Commissioner Martuscello regarding CERT, DOCCS (July 9, 2025).

¹¹¹ Memorandum from Commissioner Martuscello regarding CERT, DOCCS (July 9, 2025).

¹¹² [Cite Redacted].

¹¹³ [Cite Redacted].

¹¹⁴ [Cite Redacted].

¹¹⁵ [Cite Redacted].

¹¹⁶ [Cite Redacted].

1.8 Conclusion

The tragic killings of Mr. Brooks and Mr. Nantwi demonstrated systemic shortcomings in oversight, accountability, and culture within DOCCS facilities. Both incidents involved misuse of force and efforts to conceal misconduct, revealing deep cultural and procedural deficiencies at Marcy and Mid-State. While subsequent prosecutions and policy reforms represent critical steps toward accountability, more must be done. Our review seeks to galvanize structural change to protect the rights and safety of incarcerated individuals, while also prioritizing improved training, resources, and working conditions for DOCCS officers and civilian staff.

2. PRIOR INCIDENTS

This section reviews significant incidents of alleged excessive force at Marcy and Mid-State that preceded the killings of Mr. Brooks and Mr. Nantwi. These incidents reveal potential patterns of escalating violence, inadequate disciplinary responses, and leadership breakdowns that fostered conditions for the tragic deaths of Mr. Brooks and Mr. Nantwi. Both deaths underscore the dire consequences that result when prisons ignore clear warning signs and rely on ineffective staff disciplinary systems.

2.1 Marcy Prior Incidents

2.1.1 August 2023—Assault in the Infirmary

In August 2023, more than a year before Mr. Brooks was killed, a verbal altercation at Marcy between an incarcerated individual and a correction officer escalated into a large use of force, resulting in injuries, after the officer allegedly poured water on the incarcerated individual's bunk. This incident allegedly involved at least one officer who later pleaded guilty for their involvement in the killing of Mr. Brooks.

The incarcerated individual alleged that the argument occurred after the officer had poured water on his bunk and that, during the ensuing argument, the officer deployed chemical spray while the incarcerated individual attempted to retreat.¹¹⁷ A second officer intervened and allegedly broke the incarcerated individual's finger after restraining him; thereafter, officers allegedly transported the incarcerated individual to the infirmary, where a group of officers allegedly threw him to the ground and assaulted him with closed-fist punches and kicks.¹¹⁸

¹¹⁷ [Cite Redacted].

¹¹⁸ [Cite Redacted].

Subsequent medical examinations found that the individual had fractures in his thumb, ribs, and lumbar spine.¹¹⁹ This was the same infirmary where Mr. Brooks was later killed.

The correction officer who allegedly poured water on the bed claimed that the incarcerated individual had threatened sexual violence, causing the officer to spray him multiple times with OC spray while he charged.¹²⁰ OSI substantiated the incarcerated individual's allegations regarding the officer's misconduct of excessive use of chemical spray, with all witnesses saying that the incarcerated individual had not threatened the officer.¹²¹ That officer resigned.¹²²

As for the second officer on the scene and the other officers who allegedly beat the individual in the infirmary, OSI found that while the alleged assault in the infirmary was consistent with the medical reports, it could not identify which officers caused the injuries.¹²³ Ultimately, none of the officers were disciplined.¹²⁴ The second officer who intervened and one of the officers who allegedly participated in the infirmary beating later pleaded guilty for their involvement in the killing of Mr. Brooks.

2.1.2 June-October 2024—Excessive Force in Cell and Vans, and Flawed Supervisory Review

Approximately one year later, other Marcy officers involved in the killing of Mr. Brooks were allegedly involved in other instances of abuse.

According to the allegations in a pending civil lawsuit filed by the incarcerated individual and communications to DOCCS from the individual's family, the alleged abuses included:

¹¹⁹ [Cite Redacted].

¹²⁰ [Cite Redacted].

¹²¹ [Cite Redacted].

¹²² [Cite Redacted].

¹²³ [Cite Redacted].

¹²⁴ [Cite Redacted].

- During a verbal altercation in June 2024, a nurse allegedly told the incarcerated individual that the nurse could kill him and that no one would know.¹²⁵ The incarcerated individual alleged that his opioid treatment was improperly administered.¹²⁶
- On September 24, 2024, the incarcerated individual was restrained to a desk and chair, held without food, water, or bathroom access for six hours, during which time he was beaten and sexually assaulted before he was transferred to another facility. The resulting injuries included abrasions and a facial laceration.¹²⁷ The incident allegedly involved one officer who was also involved in the killing of Mr. Brooks.
- On October 8, 2024, the incarcerated individual reached out of his cell door slot to get an officer's attention, and an officer kicked the door slot shut, crushing his right hand. A nurse examined him cell-side, but he was not referred for outside medical attention.¹²⁸ When he was transferred to a hospital the following day, it was confirmed that he had suffered a fractured hand.¹²⁹
- On October 9, 2024, officers took the incarcerated individual to a strip frisk room, where he was choked and assaulted.¹³⁰ A nurse examined him and noted ligature marks on his neck, as well as abrasions to his head and shoulders.¹³¹ The incident allegedly involved three officers who were also involved in the killing of Mr. Brooks.

From October 2024 through April 2025, OSI investigated the September 24 and October 9 incidents and found that, with respect to the October 9 incident, the incarcerated individual suffered closed traumatic displaced fractures to his ribs and a nondisplaced fracture to his right hand, resulting in surgery and hospitalization.¹³² Our review showed that no Use of Force Report was filed for the September 24 incident.

¹²⁵ [Cite Redacted].

¹²⁶ [Cite Redacted].

¹²⁷ Amended Complaint, ¶¶ 20-31, *Rivas v. Kessler*, 9:25-cv-00253-AJB-PJE (N.D.N.Y. Mar. 31, 2025).

¹²⁸ Amended Complaint, ¶¶ 42-46, *Rivas v. Kessler*, 9:25-cv-00253-AJB-PJE (N.D.N.Y. Mar. 31, 2025).

¹²⁹ [Cite Redacted].

¹³⁰ Amended Complaint ¶¶ 50-66, *Rivas v. Kessler*, 9:25-cv-00253-AJB-PJE (N.D.N.Y. Feb. 26, 2025).

¹³¹ Amended Complaint ¶ 69, *Rivas v. Kessler*, 9:25-cv-00253-AJB-PJE (N.D.N.Y. Feb. 26, 2025); [Cite Redacted].

¹³² [Cite Redacted].

OSI ultimately found that one officer had used excessive force during the October 9 strip-frisk incident.¹³³ That officer later pled guilty to criminal charges for his involvement in the killing of Mr. Brooks.

OSI also found that there had been a failure to supervise and a misuse of BWC during the October 9 strip frisk because a lieutenant instructed an officer to turn off the BWC recording device before the strip frisk.¹³⁴ Finally, while a Use of Force Report for the October 9 incident was filed by one of the officers the same day, it was not reviewed (and referred for further action) by the acting superintendent until December 11—two days after Mr. Brooks’s death.¹³⁵

2.1.3 November 2024—Excessive Force and Cover-up

Another incarcerated individual at Marcy reported an assault by staff that occurred in November 2024, which was later corroborated by OSI.¹³⁶ The individual said that he had been sprayed with chemical spray inside his cell, cuffed, and taken to a decontamination room.¹³⁷ Once inside and with the lights turned off, officers beat the individual.¹³⁸ A sergeant, who falsely told OSI that there was no assault, remained outside while yelling was heard from the room and can be heard telling the incarcerated individual to stop resisting.¹³⁹

According to OSI, a nurse performed only a cursory visual check (estimated around 20 seconds), without asking any questions, and falsely reported no injuries despite the individual’s facial injuries.¹⁴⁰ After the assault, an officer took photographs of the individual’s face for the required Use of Force Report but instructed the individual to look down rather than directly at

¹³³ [Cite Redacted].

¹³⁴ [Cite Redacted].

¹³⁵ [Cite Redacted].

¹³⁶ [Cite Redacted].

¹³⁷ [Cite Redacted].

¹³⁸ [Cite Redacted].

¹³⁹ [Cite Redacted].

¹⁴⁰ [Cite Redacted].

the camera.¹⁴¹ The involved officers lied in the Use of Force Reports and lied when questioned by investigators, stating that they had not been involved in the use of force and describing behavior by the incarcerated individual inconsistent with video evidence.¹⁴²

OSI ultimately determined that nine staff members engaged in misconduct, including a captain, a lieutenant, two sergeants, four correction officers, and a nurse.¹⁴³ DOCCS' Bureau of Labor Relations (BLR) pursued disciplinary action against six staff members: two resigned and four settled for suspensions and disciplinary evaluation periods.¹⁴⁴

The incident took place less than a month before Mr. Brooks was killed and included several officers, some of whom were senior staff, as well as medical personnel. This group was distinct from those implicated in the killing of Mr. Brooks, suggesting that concerns at Marcy extended beyond isolated incidents and pointed to systemic problems at the facility.

2.2 Mid-State Prior Incidents

2.2.1 September 2023—Assault in the Housing Unit, False Reports, and Supervisory Failures

At Mid-State, in September 2023, four officers assaulted five incarcerated individuals in a general population housing unit, as later substantiated by OSI. The attack also involved excessive force and racial slurs. OSI substantiated the complaints, but the disciplinary response was limited to 30-day suspensions, fines, and disciplinary evaluation periods. All the officers involved returned to duty. One of those officers is facing criminal charges for participating in Mr. Nantwi's death.¹⁴⁵

¹⁴¹ [Cite Redacted].

¹⁴² [Cite Redacted].

¹⁴³ [Cite Redacted].

¹⁴⁴ [Cite Redacted].

¹⁴⁵ [Cite Redacted].

According to statements made by involved incarcerated individuals to OSI, the incident began when a sergeant entered a dorm and was upset that it was disorderly, so the sergeant began throwing items, pouring coffee onto beds, and throwing out incarcerated individuals' personal items.¹⁴⁶ The sergeant pulled an incarcerated individual into the hallway and hit him during a pat frisk.¹⁴⁷ At one point, several individuals in the dorm became agitated as the sergeant and incarcerated individuals argued over the sergeant's activation of their BWC.¹⁴⁸ The sergeant retreated and called for a response, and approximately 15 officers arrived and sprayed incarcerated individuals with pepper spray.¹⁴⁹ One incarcerated individual attempted to spray officers with a fire extinguisher.¹⁵⁰ The incarcerated individuals retreated to a different room and barricaded a door, and the officers broke through.¹⁵¹

OSI substantiated that the officers then handcuffed five incarcerated individuals, removed them from the room, and assaulted them while handcuffed.¹⁵² According to the individuals, the assaults included smashing their faces into walls, punching them in the face, kicking and punching their bodies while handcuffed, shoving boots into their faces and mouths, and pulling their hair.¹⁵³ Though the individuals were lying on the floor handcuffed throughout the assault, one individual reported that staff repeatedly yelled "stop resisting" in an apparent attempt to justify the use of force, creating a pretext for their actions.¹⁵⁴ That individual also reported to

¹⁴⁶ [Cite Redacted].

¹⁴⁷ [Cite Redacted].

¹⁴⁸ [Cite Redacted].

¹⁴⁹ [Cite Redacted].

¹⁵⁰ [Cite Redacted].

¹⁵¹ [Cite Redacted].

¹⁵² [Cite Redacted].

¹⁵³ [Cite Redacted].

¹⁵⁴ [Cite Redacted].

OSI that an officer who was later allegedly involved in Mr. Nantwi's death held his boot against the individual's neck and called him a racial slur.¹⁵⁵

OSI reviewed limited BWC footage from the incident, finding that the officers' uses of force were not captured on camera.¹⁵⁶ OSI found that the officers lied in their Use of Force Reports, falsely claiming that they had not used force.¹⁵⁷ The officers punished several incarcerated individuals with disciplinary tickets, placement in restrictive housing, and the loss of good time credits and commissary, recreation, packages, and phone privileges.¹⁵⁸ Additionally, some officers implausibly told OSI later that the incarcerated individuals had caused their own injuries by diving into walls.¹⁵⁹

Following the investigation, OSI substantiated complaints against the officers and referred them to BLR.¹⁶⁰ BLR issued notices of discipline to four officers related to each officer's utilization of an excessive degree of physical force.¹⁶¹ Each of these four officers, including the officer later allegedly involved in Mr. Nantwi's killing, individually entered into settlement agreements in which they were suspended for 30 days without pay, fined \$6,000, and placed on a twelve-month disciplinary evaluation period that began in May 2024.¹⁶² The officer allegedly involved in the killing of Mr. Nantwi was still on that disciplinary period when Mr. Nantwi was killed. During the review, we learned that Mid-State staff, though it was not clear

¹⁵⁵ [Cite Redacted].

¹⁵⁶ [Cite Redacted].

¹⁵⁷ [Cite Redacted].

¹⁵⁸ [Cite Redacted].

¹⁵⁹ [Cite Redacted].

¹⁶⁰ [Cite Redacted].

¹⁶¹ [Cite Redacted].

¹⁶² [Cite Redacted].

which staff specifically, held a fundraiser for the four officers to cover the disciplinary fines imposed by BLR.¹⁶³

OSI also found evidence of misconduct on the part of the superintendent after the incident occurred.¹⁶⁴ Among other evidence, OSI found that the superintendent provided false testimony that he reviewed BWC footage (he did not) and that he counseled an involved sergeant and lieutenant on their failure to supervise during the incident (he did not); he also authorized signing off that the use of force was reasonable and justified, despite photographic evidence that contradicted the Use of Force Reports and indicated non-compliance with the use of force policy.¹⁶⁵ The superintendent also failed to report the potential misconduct to OSI.¹⁶⁶ DOCCS demoted the superintendent in May 2025.¹⁶⁷

2.2.2 February 27, 2025—Assault in the Housing Unit and False Reports

Another incident occurred two days before the death of Mr. Nantwi, in the midst of the ongoing illegal officer strike. Six of the officers allegedly involved in the killing of Mr. Nantwi were also involved in this incident, which involved CERT officers assaulting incarcerated individuals in a housing unit.

According to OSI, in response to a call from the directing officer of the housing unit, the CERT team engaged in “rampaging” behavior,¹⁶⁸ with officers screaming, threatening to put the incarcerated individuals in restrictive housing, and smacking the walls and lockers with their batons.¹⁶⁹ Sergeants did nothing to calm the situation.¹⁷⁰ On body camera footage, one officer

¹⁶³ [Cite Redacted].

¹⁶⁴ [Cite Redacted].

¹⁶⁵ [Cite Redacted].

¹⁶⁶ [Cite Redacted].

¹⁶⁷ [Cite Redacted].

¹⁶⁸ [Cite Redacted].

¹⁶⁹ [Cite Redacted].

¹⁷⁰ [Cite Redacted].

was seen picking up a handcuffed incarcerated man and body-slamming him for no apparent reason.¹⁷¹ None of the officers submitted Use of Force Reports.¹⁷² Instead, the supervisors filed false misbehavior reports against the incarcerated individuals.¹⁷³ OSI substantiated complaints against twelve officers for misconduct, including failure to supervise, failure to properly use BWC, failure to report excessive use of force, and filing false misbehavior reports.¹⁷⁴ Three involved officers resigned in the months following the incident, none of whom were among the officers who were allegedly also involved in the killing of Mr. Nantwi.¹⁷⁵ BLR pursued discipline against four other involved officers and settled each of their cases: one officer lost ten days of accruals and was put on a disciplinary evaluation period, two officers paid fines and were put on disciplinary evaluation periods, and one officer paid a fine only.¹⁷⁶

2.2.3 February 28, 2025—Assault in Van and Restrictive Housing

OSI found that one day before Mr. Nantwi's death, CERT officers assaulted an incarcerated individual after several days of verbal altercations between him and a correction officer. According to OSI, the incident involved three officers who were allegedly involved in the killing of Mr. Nantwi the next day, and two officers who were also involved in the incident in the restrictive housing unit the previous day.

According to OSI, days of reportedly rising tensions between the incarcerated individual and security staff—including several days of verbal altercations, threats, and narrowly avoided physical altercations—escalated into a violent altercation on February 28, 2025.¹⁷⁷ That day, an

¹⁷¹ [Cite Redacted].

¹⁷² [Cite Redacted].

¹⁷³ [Cite Redacted].

¹⁷⁴ [Cite Redacted].

¹⁷⁵ [Cite Redacted].

¹⁷⁶ [Cite Redacted].

¹⁷⁷ [Cite Redacted].

officer discovered that the incarcerated individual was a professional fighter, started threatening to “beat his ass,” and stated that the individual was not “really tough.”¹⁷⁸ According to disciplinary records, the officer inappropriately challenged the incarcerated individual to a fight.¹⁷⁹ The incarcerated individual offered to fight the officer.¹⁸⁰ That officer called CERT.¹⁸¹

OSI found that two CERT sergeants (who were later allegedly involved in Mr. Nantwi’s death) removed the individual from his housing unit, then assaulted him.¹⁸² According to OSI interviews, the individual was assaulted by staff in the mess hall, then moved to a van, where he was again assaulted, and then driven to a restrictive housing unit, where he was yet again assaulted.¹⁸³ During those assaults, witnesses described that the individual was beaten and slammed head-first into the ground¹⁸⁴; one officer sprayed chemical spray into a burn wound on the individual’s leg,¹⁸⁵ and others kicked him in the genitals, pulled out his hair, and broke his finger.¹⁸⁶ OSI also found that officers forced the incarcerated individual to play “the 50/50 game,” a game in which if he took his clothes off the “wrong way,” he was assaulted by staff and hit with a baton.¹⁸⁷ Thereafter, the officers allegedly held the incarcerated individual in a holding cell without medical attention.¹⁸⁸

OSI substantiated the allegations through witness interviews, review of medical reports and photos, and review of the limited available BWC footage, all of which corroborated the

¹⁷⁸ [Cite Redacted].

¹⁷⁹ [Cite Redacted].

¹⁸⁰ [Cite Redacted].

¹⁸¹ [Cite Redacted].

¹⁸² [Cite Redacted].

¹⁸³ [Cite Redacted].

¹⁸⁴ [Cite Redacted].

¹⁸⁵ [Cite Redacted].

¹⁸⁶ [Cite Redacted].

¹⁸⁷ [Cite Redacted].

¹⁸⁸ [Cite Redacted].

individual's accusations.¹⁸⁹ During this incident, the CERT team did not wear their BWC devices, but OSI was able to recover BWC footage from non-CERT officers.¹⁹⁰ OSI found in the footage that the CERT team closed the door and turned up the radio to cover up the sounds of the assault in the strip frisk room of the restrictive housing unit.¹⁹¹ Two officers resigned amid disciplinary arbitration proceedings, not including those allegedly involved in the death of Mr. Nantwi.¹⁹² As of December 2025, one additional officer is in the midst of disciplinary arbitration proceedings in which BLR is seeking the officer's termination.¹⁹³

2.3 Key Observations

These incidents at Marcy and Mid-State are emblematic of systemic issues at both facilities, including excessive use of force, officer misconduct, accountability, oversight, and culture.

The prior incidents demonstrate that several of the officers allegedly involved in the killings of Mr. Brooks and Mr. Nantwi had a substantiated history of misconduct at Marcy and Mid-State. Yet, they were allowed to remain on the job. And at Mid-State, the days leading up to the killing of Mr. Nantwi appear to have been marked by repeated episodes of alleged officer assault, particularly by CERT officers. These patterns should have triggered urgent intervention at Marcy and Mid-State.

These prior incidents at both facilities also involved several officers who were not involved in either Mr. Brooks's or Mr. Nantwi's deaths, which indicates that those assaults were not isolated or based on the misconduct of just "a few bad apples." As discussed further

¹⁸⁹ [Cite Redacted].

¹⁹⁰ [Cite Redacted].

¹⁹¹ [Cite Redacted].

¹⁹² [Cite Redacted].

¹⁹³ [Cite Redacted].

throughout the report, the officers who were involved in both of these prior incidents and the deaths of Mr. Nantwi or Mr. Brooks highlight the lack of accountability that allows culpable officers to stay in the system. The incidents that occurred in the days leading up to the killing of Mr. Nantwi indicate a clear escalation of violence by CERT at Mid-State during the illegal strike.

Instead, as discussed later in further detail, our review found indicia of cultural problems in these facilities, including a crisis in staffing and staff morale, frequent use of force, a lack of accountability, and insufficiently effective oversight.

3. THE REVIEW

3.1 Engagement

On December 30, 2024, following the killing of Mr. Brooks, Governor Hochul directed DOCCS to engage an outside firm to conduct a review of DOCCS' culture, patterns, and practices.¹⁹⁴ DOCCS retained WilmerHale in January 2025. DOCCS requested an evaluation of its patterns and practices regarding use of force, as well as an assessment of its internal systems and procedures to identify any potential systemic issues. At DOCCS' request, the review focused initially on Marcy, where Mr. Brooks was killed, and later expanded to include a limited review of Mid-State after Mr. Nantwi was killed. This report presents the findings of that review.

WilmerHale retained sole responsibility for the drafting, methodology, and final determinations of this report. Accordingly, unless explicitly noted otherwise, the findings, conclusions, and recommendations presented herein represent the judgment, opinions, and work product of WilmerHale.

3.2 Team

WilmerHale is an international private law firm with deep experience conducting high-stakes and sensitive internal investigations and government investigations involving the U.S. Department of Justice, Congress, the FBI and CIA, Inspectors General, state attorneys general, and numerous other agencies with investigative authority.

To assist in the review, WilmerHale retained subject-matter experts in prison-based security operations, correctional healthcare, and correctional mental health care.¹⁹⁵ These experts included three former heads of correctional systems in other states and several correctional

¹⁹⁴ *Governor Hochul Visits Marcy Correctional Facility Demanding Answers Following Death of Robert Brooks and Announces Immediate Corrective Actions* (Dec. 20, 2024), <https://www.governor.ny.gov/news/governor-hochul-visits-marcy-correctional-facility-demanding-answers-following-death-robert>.

¹⁹⁵ [Cite Redacted].

practice medical and mental health experts with decades of experience in correctional settings. WilmerHale also retained a private investigation and litigation consulting firm with extensive criminal justice experience to assist with interviews of incarcerated individuals and current and former correction officers.¹⁹⁶

3.3 Methodology

Our findings are based on several sources of information, gleaned from site visit observations, interviews, surveys, and both internal and public records. It is worth highlighting that the interviews and surveys were conducted in the wake of the tragic, and highly publicized, killings of Mr. Brooks and Mr. Nantwi and the attendant criminal proceedings against those involved, and in the wake of the illegal correction officers' strike. It is likely that these events had an impact on the interview and survey responses. In addition, out of respect for the criminal process and recognizing that all defendants are innocent until proven guilty, we have not attempted to make judgments regarding the guilt or innocence of any specific defendant in the killings of Mr. Brooks and Mr. Nantwi. However, through our review, we have attempted to draw lessons from the incidents for the purpose of making recommendations for DOCCS' policies and procedures.

3.3.1 Site Visits

The team made repeated multi-day visits to Marcy and Mid-State to observe conditions at the facilities and to interview incarcerated individuals, staff, and facility leadership. During the review, we visited Marcy five times (in February, March, April, May, August, and September 2025) and Mid-State twice (in April and September 2025). Site visits allowed our team to directly interact with incarcerated populations and facility staff and provided firsthand exposure

¹⁹⁶ [Cite Redacted]; *About Grant Fine*, FINE AND ASSOCIATES, <https://fineandassociates.com/about-grant-fine/> (last visited Feb. 2, 2026).

to each facility’s mission, organization, culture, daily operational practices, medical and mental health care, staff workloads and workflows, and other facility-specific conditions. We observed general population housing units, restrictive and specialized housing units, infirmaries, and recreation and programming areas. Additionally, we conducted brief visits to two other facilities—Sing Sing and Gouverneur—for a comparative analysis with Marcy and Mid-State. We also visited and observed classes at the officer training academy in Albany in August 2025. During all these visits, DOCCS provided us with unfettered access to the facilities.

3.3.2 Interviews

The team conducted hundreds of formal and informal interviews during these visits, including with incarcerated individuals, facility leadership, security staff, and civilian staff. We also spoke with National Guard members who were stationed at DOCCS facilities to fill staffing shortages. At the facilities, we conducted group interviews of staff by subject-matter expertise, including staff specializing in the IGP, disciplinary hearings, property and package room, and CERT. We also interviewed correction officers by post assignment, including intake, housing units (including restrictive housing and other specialized housing), watch commander, disciplinary hearings, commissary, package room, programs, recreation, and visitation. The review also drew on virtual workshops with facility staff and executive leadership at Marcy and Mid-State to explore issues related to use of force, staff discipline, special housing, staffing, the IGP, officer wellness, mental health care, commissary, food services, package rooms, and ILCs.

With respect to the incarcerated population, we conducted structured, voluntary, and confidential interviews across a stratified, random sample of incarcerated individuals at Marcy and Mid-State concerning a wide range of topics, including (1) past and recent experiences while incarcerated; (2) system strengths and areas for improvement; and (3) perceptions of facility conditions and services. At Marcy, we conducted 133 structured interviews—among them, 116

with individuals in the general population, seven in the Residential Mental Health Unit (RMHU), six in the Specialized Housing Unit, and four with individuals who did not identify their housing. At Mid-State, where the scope of our review was more limited, we interviewed 20 incarcerated individuals, including 13 in the general population, five in the Intermediate Care Program (ICP), and two in the Step-Down Program (SDP).

We met with the leadership team at Marcy and Mid-State, and also interviewed DOCCS executive leadership, representatives from the Academy, and leadership and investigators from OSI. We also spoke with relevant third parties, including the Correctional Association of New York and union representatives. We also reached out to state legislators, though only one agreed to speak with us.

In addition, as stated above, we engaged a private investigation and litigation consulting firm to assist with interviews. Grant Fine of Fine and Associates reached out to over 100 current and former correction officers, all of whom had experience at Marcy and Mid-State. Ultimately, 43 current and former correction officers spoke with Mr. Fine, some multiple times.¹⁹⁷ The majority of interviewees were former correction officers who had left DOCCS in the last five years; almost half of these interviewees had left in either 2024 or 2025. None of these officers left DOCCS for disciplinary reasons.¹⁹⁸ Throughout this report, we have included verbatim statements from some of these interviews.¹⁹⁹ These statements illustrate individual experiences of the issues described.

To encourage candor, we promised all interviewees that our interviews would remain confidential and their identities would not be revealed. Despite these assurances, some had

¹⁹⁷ The investigator spoke with some correction officers twice.

¹⁹⁸ [Cite Redacted].

¹⁹⁹ In some of these conversations, interviewees used the term “inmate,” which we have not altered. Throughout the report, we use the term “incarcerated individual.”

concerns about speaking with us. For instance, some incarcerated individuals communicated discomfort with being interviewed due to the presence of nearby facility staff. Certain facility staff members were concerned about describing issues in detail for fear of revealing their own identities. Some current and former staff members and incarcerated individuals declined to speak with us at all, with a few citing concerns of retaliation.

3.3.3 Surveys

In addition to interviews, we distributed surveys to incarcerated individuals and staff at Marcy, Mid-State, Sing Sing, and Gouverneur. The surveys were voluntary, took approximately 15 minutes to complete, and contained 76 questions in the incarcerated individual survey and 84 questions in the staff survey. For accessibility, Spanish and Mandarin Chinese versions were available upon request. Survey participants were not required to provide personally identifiable information, though some did. Surveys covered the following topics: demographics, interactions and relationships with staff members and incarcerated individuals, security and safety, experiences with differential treatment in facilities, use of force, health services, reporting mechanisms, contraband, cross-facility and longitudinal comparisons, training and documentation (staff only), and overall observations of positive and negative aspects of the facility.

We physically distributed surveys to incarcerated individuals in every general housing unit at Marcy and Mid-State, as well as in specialized housing at Marcy, Mid-State, and Gouverneur, and a subset of general population units at Sing Sing and Gouverneur. Surveys completed by incarcerated individuals were collected in person to protect respondents' confidentiality and anonymity. We also announced that individuals could mail surveys to WilmerHale as free, legal mail.

We distributed surveys to staff throughout the facilities in person where possible and placed surveys in staff mailboxes for completion and return by mail. We could not electronically distribute surveys to staff because correction officers typically do not have a work email address.

We received a strong survey response from incarcerated individuals (1,145 total surveys) but a low response from staff (119 surveys), as detailed in Figure 1.

Figure 1 Total Population and Number of Surveys Received by Facility.

	Facility Population, as of 10/1/2025	Incarcerated Individuals	Facility Staff
Marcy	782	361	66
Mid-State	1,164	440	36
Sing Sing	1,494	91	4
Gouverneur	724	253	13
TOTAL	4,164	1,145	119

3.3.4 Document Collection and Review

We submitted hundreds of document requests to DOCCS and reviewed hundreds of records from the facilities and across the system. The requests included, but were not limited to, records relating to use of force, grievances, oversight and accountability, training, discipline, staffing, medical and mental health care, and demographic information. In some cases, we also reviewed camera footage of specific instances of uses of force. We also collected and reviewed documents from OMH. We also reviewed public records relating to the killings of Mr. Brooks and Mr. Nantwi, including the publicly released BWC footage showing the murder of Mr. Brooks. Due to the ongoing criminal proceedings during the review, DOCCS did not provide us

with OSI investigation files related to the killings of Mr. Brooks and Mr. Nantwi or any other nonpublic evidence related to those killings. Where defendants in those criminal proceedings pled guilty and admitted to certain conduct, we included that information in our report. DOCCS provided all requested information that was reasonably available.

4. DEPARTMENT OF CORRECTIONS AND COMMUNITY SUPERVISION

4.1 DOCCS' Mission, Governance, and Organizational Structure

4.1.1 DOCCS' Mission Statement

According to its mission statement, DOCCS seeks “to enhance public safety by having incarcerated persons return home, under supportive supervision, less likely to revert to criminal behavior.”²⁰⁰ DOCCS intends to “improve public safety by providing a continuity of appropriate treatment services in safe and secure facilities where all incarcerated individuals’ needs are addressed and they are prepared for release, followed by supportive services for all parolees under community supervision to facilitate a successful completion of their sentence.”²⁰¹ DOCCS explains that it further seeks to “create an atmosphere where all incarcerated individuals, parolees, and staff feel secure”; “[t]each incarcerated individuals the need for discipline and respect”; and “[a]ssist staff by providing the requisite training and resources needed to perform their duties while enhancing their skills.”²⁰²

4.1.2 DOCCS' Leadership Structure

The DOCCS Commissioner is appointed by the Governor and serves as DOCCS’ chief executive officer.²⁰³ Commissioner Daniel F. Martuscello III has served in this role since June 9, 2023.²⁰⁴ The Commissioner directs the DOCCS executive team, which currently consists of 13 Deputy Commissioners and Associate Commissioners, which, with the exception of the Board of

²⁰⁰ *The Departmental Mission*, DOCCS, <https://doccs.ny.gov/departamental-mission>.

²⁰¹ *The Departmental Mission*, DOCCS, <https://doccs.ny.gov/departamental-mission>.

²⁰² *The Departmental Mission*, DOCCS, <https://doccs.ny.gov/departamental-mission>.

²⁰³ N.Y. Corr. Law §§5(1)-(2) (McKinney 2022).

²⁰⁴ *Commissioner Daniel F. Martuscello III*, DOCCS, <https://doccs.ny.gov/commissioner-daniel-f-martuscello-iii>. (Martuscello assumed the Acting Commissioner position on June 9, 2023, and was later nominated by Governor Kathy Hochul and then confirmed by the Senate on May 23, 2024).

Parole Chairman, are appointed by the Commissioner.²⁰⁵ Each Deputy Commissioner is responsible for a specific area, such as public information, mental health, medical services, parole, and program services.²⁰⁶

4.2 DOCCS Facilities

4.2.1 DOCCS Facilities and Demographics

As of June 2026, DOCCS operates 41 facilities throughout New York State, as shown in Figure 2.²⁰⁷

Figure 2 DOCCS Facility Summary²⁰⁸

	Male	Female	Mixed
Minimum	1	-	1
Medium	23	2	1
Maximum	12	1	-

DOCCS is divided into eight regions, each of which (except for the New York City region) has component hubs. Each hub, in turn, contains several prisons and other facilities, which share certain administrative resources.²⁰⁹ This helps DOCCS more efficiently distribute

²⁰⁵ *About Us*, DOCCS, <https://doccs.ny.gov/about-us>; Darryl C. Towns, DOCCS, <https://doccs.ny.gov/darryl-c-towns-0>; N.Y. Corr. Law §§7(2)-(3) (McKinney 2022).

²⁰⁶ *About Us*, DOCCS, <https://doccs.ny.gov/about-us>.

²⁰⁷ *Facilities*, DOCCS, <https://doccs.ny.gov/facilities>.

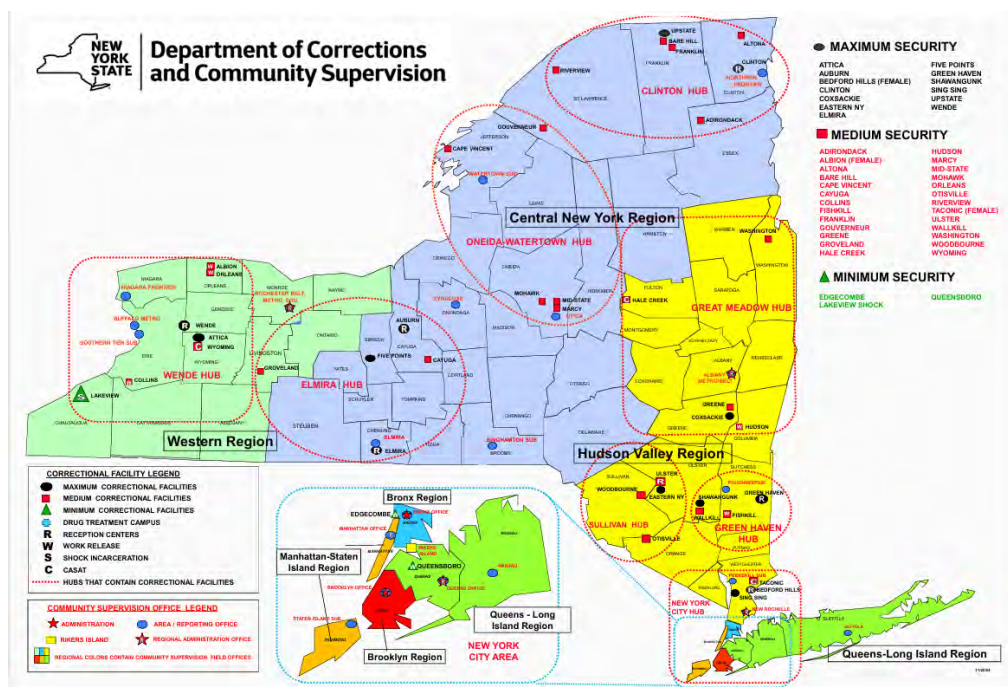
²⁰⁸ *Facilities*, DOCCS, <https://doccs.ny.gov/facilities>.

²⁰⁹ *Facility Map*, DOCCS (Updated Nov. 2024), https://doccs.ny.gov/system/files/documents/2025/01/facility-map_112024-updated.pdf.

resources by allowing, for example, shorter transport times and regional program deployment.

The hub and region system is shown in Figure 3.

Figure 3 DOCCS Facility Map²¹⁰



DOCCS’ incarcerated population has decreased from a peak of over 72,899 individuals in 1999 to 33,944 as of June 2026.²¹¹ A key driver of this decline was the reform of the Rockefeller Drug Laws, which previously mandated lengthy sentences for non-violent drug offenses; these reforms contributed to a drop in the percentage of incarcerated individuals convicted of drug felonies—since 2008, the number of convictions has dropped from 29% to 14%.²¹² The enactment of the “Less is More” Act in 2021 further reduced re-incarceration for technical parole

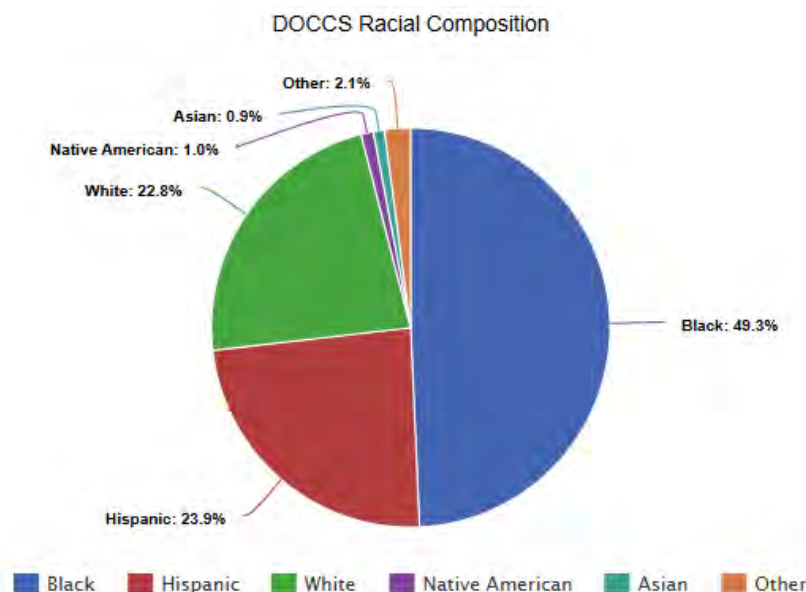
²¹⁰ Facility Map, DOCCS (Updated Nov. 2024), https://doccs.ny.gov/system/files/documents/2025/01/facility-map_112024-updated.pdf.

²¹¹ Liz Johnson, *Trends in the New York State Prison Population: 2008-2023*, p. 1, DATA COLLABORATIVE FOR JUSTICE (July 2023); DOCCS Fact Sheet June 2026, p. 6, DOCCS, <https://doccs.ny.gov/system/files/documents/2026/06/doccs-fact-sheet-june-2026.pdf>.

²¹² Liz Johnson, *Trends in the New York State Prison Population: 2008-2023*, p. 1, DATA COLLABORATIVE FOR JUSTICE (July 2023), <https://datacollaborativeforjustice.org/wp-content/uploads/2023/07/PrisonPop-1.pdf>.

violations.²¹³ Between 2008 and 2023, the population of individuals incarcerated for parole violations decreased by 90%.²¹⁴ These reforms have meant that the DOCCS population has seen a decline, particularly among the population of individuals convicted of non-violent criminal offenses.²¹⁵

The racial composition of those incarcerated at DOCCS facilities has not changed significantly in nearly two decades.²¹⁶ Black incarcerated individuals comprise the largest proportion of incarcerated individuals within DOCCS, at 49% of the incarcerated population. Racial and gender demographics are shown below:²¹⁷



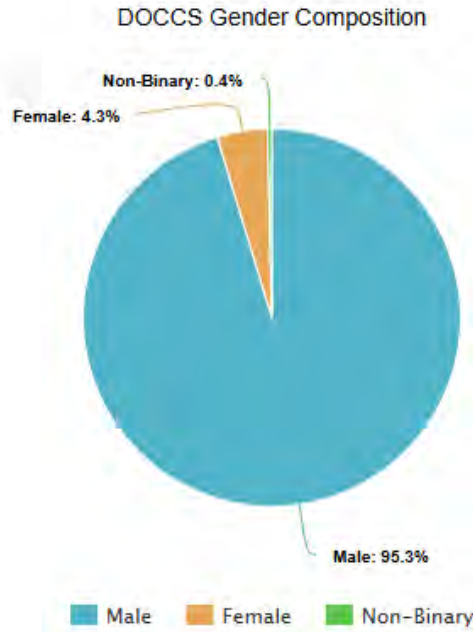
²¹³ *Less Is More Act Cuts Parole Population by 40% in New York*, PRISON LEGAL NEWS (Dec. 1, 2023), <https://www.prisonlegalnews.org/news/2023/dec/1/less-more-act-cuts-parole-population-40-new-york/>

²¹⁴ *Less Is More Act Cuts Parole Population by 40% in New York*, PRISON LEGAL NEWS (Dec. 1, 2023), <https://www.prisonlegalnews.org/news/2023/dec/1/less-more-act-cuts-parole-population-40-new-york/>; Liz Johnson, *Trends in the New York State Prison Population: 2008-2023*, p. 1, DATA COLLABORATIVE FOR JUSTICE (July 2023).

²¹⁵ *Less Is More Act Cuts Parole Population by 40% in New York*, PRISON LEGAL NEWS (Dec. 1, 2023), <https://www.prisonlegalnews.org/news/2023/dec/1/less-more-act-cuts-parole-population-40-new-york/>

²¹⁶ Liz Johnson, *Trends in the New York State Prison Population: 2008-2023*, DATA COLLABORATIVE FOR JUSTICE (JULY 2023), <https://datacollaborativeforjustice.org/wp-content/uploads/2023/07/PrisonPop-1.pdf>.

²¹⁷ Incarcerated Profile Report – February 2026 2, DOCCS (Feb. 1, 2026).

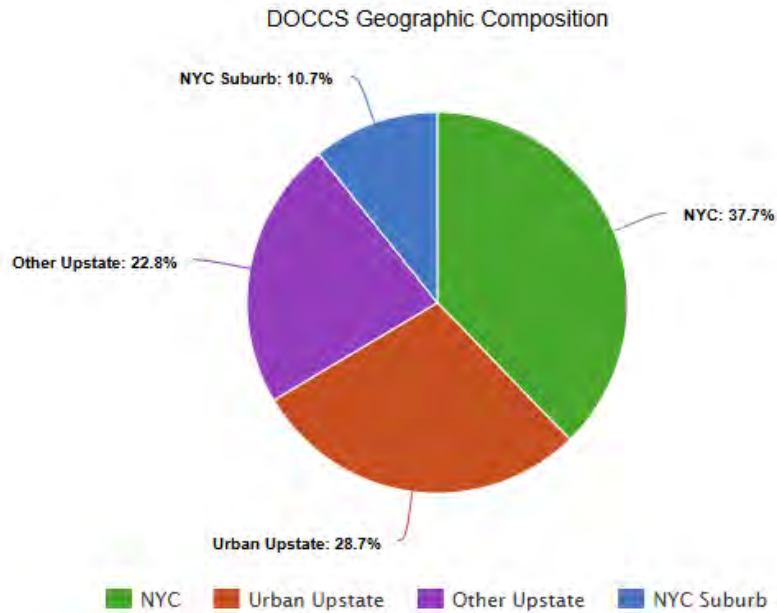


The DOCCS incarcerated population comes from mostly urban areas, with 37.7% of the population from New York City and 28.7% of the population from urban upstate locations, for a total of 66.4% of the total population.²¹⁸ Another 10.7% of the population comes from the New York City suburbs, while 22.8% come from non-urban upstate locations.²¹⁹ Given that Sing Sing is the only facility within DOCCS’ New York City Hub, many individuals serve their sentences in non-urban communities staffed by officers from those areas.²²⁰

²¹⁸ Incarcerated Profile Report, p. 2, DOCCS (Feb. 1, 2026).

²¹⁹ Incarcerated Profile Report, p. 2, DOCCS (Feb. 1, 2026).

²²⁰ *Facility Map*, DOCCS (Updated Nov. 2024), https://doccs.ny.gov/system/files/documents/2025/01/facility-map_112024-updated.pdf.



4.2.2 Special Housing Units

DOCCS operates five categories of special housing units within facilities: Special Housing Units, Residential Rehabilitation Units, Step-Down to General Population Program Units, Residential Mental Health Units, and Residential Crisis Treatment Programs.

First, DOCCS operates Special Housing Units (SHUs) intended for the confinement of incarcerated individuals who must be separated from the general population due to misbehavior and risk to others.²²¹ The SHU may house individuals who are confined due to disciplinary reasons, investigative reasons, or protective custody.²²² The HALT Act limits individuals to a maximum of 15 consecutive days in the SHU, and no more than 20 days in any 60-day period.²²³ Typically, cell confinement in the SHU totals about 20 hours per day.²²⁴ Incarcerated individuals from the following “Special Populations” are not eligible for placement in the SHU for any

²²¹ Directive #4933 (Jan. 17, 2024).

²²² Directive #4933 (Jan. 17, 2024).

²²³ NY Corr. Law §137 (HALT Act).

²²⁴ Directive #4933, p. 14 (Jan. 17, 2024).

duration: individuals who are 21 or younger, 55 or older, pregnant or postpartum, living with a disability defined under state law, or diagnosed with a serious mental illness.²²⁵

Second, DOCCS operates Residential Rehabilitation Units (RRUs), which are recently created high-security, separate housing units where therapy, treatment, and rehabilitative programming are provided for incarcerated individuals who would otherwise require more than 15 days of segregated confinement.²²⁶ According to DOCCS records, these units are designed to be therapeutic and trauma-informed, aiming to address both individual treatment needs and the underlying causes of problematic behaviors.²²⁷ Individuals who meet DOCCS' definition of "Special Populations" are permitted to be housed in the RRU.²²⁸ The Residential Rehabilitation Unit manual was revised in November 2023.²²⁹

Third, DOCCS runs SDPs to serve incarcerated individuals who have continued to receive disciplinary sanctions, refuse to participate in RRU programming, and have a minimum of six months remaining until disciplinary sanctions are completed.²³⁰ The SDP is a high-security, separate housing unit focused on therapy, treatment, and rehabilitative programming.²³¹ According to DOCCS records, the SDP is designed to be therapeutic and trauma-informed, with the goal of addressing individual treatment needs and the root causes of problematic behavior.²³²

²²⁵ Directive #4933, p. 2 (Jan. 17, 2024).

²²⁶ Residential Rehab. Unit Program Manual, DOCCS (Nov. 2023).

²²⁷ Residential Rehab. Unit Program Manual, DOCCS (Nov. 2023).

²²⁸ Directive #4933, p. 2 (Jan. 17, 2024).

²²⁹ Residential Rehab. Unit Program Manual, DOCCS (Nov. 2023).

²³⁰ Residential Rehab. Unit Program Manual, DOCCS (Nov. 2023).

²³¹ Residential Rehab. Unit Program Manual, DOCCS (Nov. 2023).

²³² Residential Rehab. Unit Program Manual, DOCCS (Nov. 2023).

Special Populations may also be housed within the SDP.²³³ Unlike the RRU, the SDP does not currently have a specific directive or operating manual.²³⁴

Fourth, DOCCS operates RMHUs to accommodate incarcerated individuals with a serious mental illness who, due to their disciplinary status, are serving time in special housing.²³⁵ The RMHU consists of patients with disciplinary issues who otherwise would be in a Special Housing Unit or other segregated housing.²³⁶ The RMHU, meant to be operated collaboratively between DOCCS and OMH, is designed to meet the safety and security needs of the correctional facility, along with the clinically assessed mental health and behavioral needs of the incarcerated patient.²³⁷ According to DOCCS records, these units and programs are designed to be therapeutic in nature and are not intended to function as traditional disciplinary units.²³⁸ The manual governing the RMHU has not been revised since 2009.²³⁹

Finally, DOCCS operates Residential Crisis Treatment Programs (RCTPs).²⁴⁰ The goal of the RCTP is to evaluate and treat incarcerated patients in need of short-term crisis mental health care.²⁴¹ The manual governing the RCTP has not been updated since 2011.²⁴²

²³³ Directive #4933 (Jan. 17, 2024); Residential Rehab. Unit Program Manual, DOCCS (Nov. 2023).

²³⁴ [Cite Redacted].

²³⁵ Mental Health Program Descriptions, DOCCS (2011).

²³⁶ Mental Health Program Descriptions, DOCCS (2011).

²³⁷ Residential Mental Health Unit, Program Operations Description, DOCCS (Dec. 7, 2009).

²³⁸ Residential Mental Health Unit, Program Operations Description, DOCCS (Dec. 7, 2009).

²³⁹ [Cite Redacted].

²⁴⁰ Directive #0080 (Jan. 19, 2024).

²⁴¹ Mental Health Program Description, DOCCS (July 5, 2011).

²⁴² Mental Health Program Description, DOCCS (July 5, 2011).

4.3 Marcy and Mid-State

As discussed in Section 3, “The Review,” the review focused on Marcy and Mid-State. Marcy and Mid-State are located across the street from each other in Oneida County, New York.²⁴³

4.3.1 Marcy Correctional Facility

Marcy is a medium-security facility for males 18 and older, where Mr. Brooks was killed in December 2024.²⁴⁴ The facility can house up to 1,218 individuals in its 20 housing units, consisting of 18 general confinement dormitories with 60 beds each, one RMHU with 100 individual cells, and one SHU with 32 individual cells.²⁴⁵ Marcy is also equipped to operate an RCTP.²⁴⁶

As of June 2026, Marcy housed 806 incarcerated individuals.²⁴⁷ Of those, 41.9% were Black, 28.7% white, 25.2% Hispanic, 1.1% Native American, 1.2% Asian, and 1.9% other races.²⁴⁸ The majority (61.2%) of the incarcerated individuals at Marcy were convicted of a violent felony, 12.5% were convicted of property offenses, 13.5% were convicted of drug offenses, 11.9% were convicted of other coercive offenses, and 0.9% were classified as youth/juvenile offenders.²⁴⁹

Marcy is equipped to provide educational services, vocational and industrial training, counseling, substance abuse treatment services, and transitional services for aggression and cognitive behavioral intervention.²⁵⁰

²⁴³ Directive #0080 (Jan. 19, 2024); Directive #0092 (May 13, 2020).

²⁴⁴ Directive #0092 (May 13, 2020).

²⁴⁵ Directive #0092 (May 13, 2020); Marcy Correctional Facility PREA Audit Report, p.6 (July 25, 2019).

²⁴⁶ Directive #0092 (Jan. 19, 2024).

²⁴⁷ Incarcerated Profile Report, DOCCS (June 2026).

²⁴⁸ Incarcerated Profile Report, DOCCS (June 2026).

²⁴⁹ Incarcerated Profile Report, DOCCS (June 2026).

²⁵⁰ Directive #0092 (May 13, 2020).

Marcy is classified as a Mental Health Service Level 2, meaning that OMH staff are assigned full-time to treat major disorders, but those disorders are not as acute as those in Level 1 facilities.²⁵¹ As of September 2025, 44% of Marcy’s population receives mental health treatment, compared to only 31% systemwide.²⁵² Similarly, 11% of Marcy’s population is OMH Level 1 (the most acute) compared to 8% systemwide.²⁵³

4.3.2 Mid-State Correctional Facility

Across the street from Marcy is Mid-State—a medium-security facility for males 18 and older in Oneida County, where Mr. Nantwi was killed in March 2025.²⁵⁴ Mid-State can house up to 1,459 individuals in 34 total housing units. Unlike Marcy, Mid-State has an RRU and an SDP.²⁵⁵ Mid-State is also equipped to operate an RCTP.²⁵⁶

As of June 2026, Mid-State housed 1,173 incarcerated individuals.²⁵⁷ The demographic breakdown of the Mid-State population at that time was 41.6% Black, 33.5% white, 21.1% Hispanic, 1.4% Asian, 1.0% Native American, and 1.4% other races.²⁵⁸ As of June 2026, 61.8% of incarcerated individuals at Mid-State were convicted of a violent felony, 15.1% were convicted of property offenses, 9.3% were convicted of drug offenses, and 13.6% were convicted of other coercive offenses.²⁵⁹

²⁵¹ Mental Health Program Description, DOCCS (July 5, 2011).

²⁵² CANY Data Dashboard, OMH Caseload and Program Census (last updated September 2025), <https://www.correctionalassociation.org/data/dashboard-omh-census-data>

²⁵³ CANY Data Dashboard, OMH Caseload and Program Census (last updated September 2025), <https://www.correctionalassociation.org/data/dashboard-omh-census-data>

²⁵⁴ Directive #0080 (Jan. 19, 2024).

²⁵⁵ Directive #0080 (Jan. 19, 2024).

²⁵⁶ Directive #0080 (Jan. 19, 2024).

²⁵⁷ Incarcerated Individual Profile Report, DOCCS (June 2026).

²⁵⁸ Incarcerated Individual Profile Report, DOCCS (June 2026).

²⁵⁹ Incarcerated Individual Profile Report, DOCCS (June 2026).

Mid-State is equipped to offer vocational training, academic education, substance abuse treatment services, residential sex offender programs, transitional services programs, community service, correctional industries, residential programs for incarcerated veterans, and other programs, including an ICP, and a Transitional Intermediate Care Program.²⁶⁰

Mid-State is a Mental Health Service Level 1 facility, the highest designation, meaning that it has full-time OMH staff on-site to provide specialized services, including residential crisis treatment, and potential commitment to the Central New York Psychiatric Center.²⁶¹ As of late January 2026, 15% of Mid-State's population had a serious mental illness designation.²⁶²

4.4 DOCCS' Security Staff

As of January 1, 2026, DOCCS employed 11,278 correction officers, sergeants, and lieutenants.²⁶³ The number of these employees has steadily decreased since 2019.²⁶⁴ Security staff population dropped 2.8% between 2019 and 2020, and has decreased by at least 6% each consecutive year.²⁶⁵ As of January 2026, DOCCS has a ratio of 3.0 incarcerated individuals for each security staff member, the highest number since 2012.²⁶⁶ According to recent estimates, this ratio ranks in the middle compared to other states.²⁶⁷

²⁶⁰ Directive #0080 (Jan. 19, 2024).

²⁶¹ Directive #0080 (Jan. 19, 2024).

²⁶² [Cite Redacted].

²⁶³ *Fact Sheet*, DOCCS (Jan. 2026), <https://doccs.ny.gov/system/files/documents/2026/01/doccs-fact-sheet-january-2026.pdf>.

²⁶⁴ *Fact Sheet*, DOCCS (Jan. 2026), <https://doccs.ny.gov/system/files/documents/2026/01/doccs-fact-sheet-january-2026.pdf>.

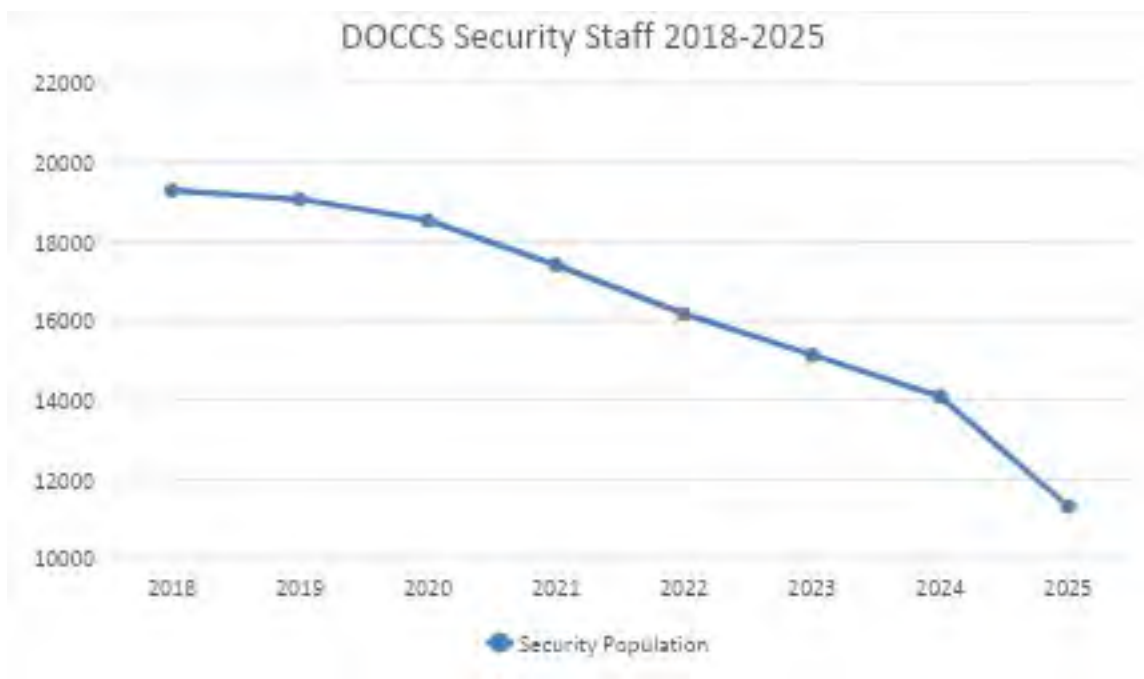
²⁶⁵ *Fact Sheet*, DOCCS (Jan. 2026), <https://doccs.ny.gov/system/files/documents/2026/01/doccs-fact-sheet-january-2026.pdf>.

²⁶⁶ *Fact Sheet*, DOCCS (Jan. 2026), <https://doccs.ny.gov/system/files/documents/2026/01/doccs-fact-sheet-january-2026.pdf>.

²⁶⁷ David Keech, *State-by-State Ranking: Highest and Lowest Prison Staff Levels in America*, ONFOCUS NEWS (Aug. 17, 2024), <https://www.onfocus.news/state-by-state-ranking-highest-and-lowest-prison-staff-levels-in-america/>.

Although they were already declining, security staff numbers decreased sharply from the end of 2024 to October 2025, likely driven by the illegal February 2025 strike. (see Figure 4).

Figure 4 DOCCS Security Staff Population Trends



4.5 The Humane Alternatives to Long-Term Solitary Confinement Law (HALT) Act

In addition to applicable federal law and constitutional requirements to “take reasonable measures to guarantee the safety of the inmates themselves,” DOCCS is bound by New York State Law.²⁶⁸ One key state law is the HALT Act, which was enacted in March 2022 and imposes limitations on segregated confinement in correctional facilities.²⁶⁹ The HALT Act restricts the criteria for justifying segregated confinement and limits how long an incarcerated individual may spend in segregated confinement.²⁷⁰ It seeks to improve confinement conditions

²⁶⁸ *Hudson v. Palmer*, 468 U.S. 517, 526-27 (1984); *Hayes v. NYC Dep’t of Corr.*, 84 F.3d 614, 620 (2d Cir. 1996); see also *Farmer v. Brennan*, 511 U.S. 825, 832 (1994); see also *United States v. Walsh*, 194 F.3d 37, 48 (2d Cir. 1999).

²⁶⁹ Humane Alternatives to Long-Term Solitary Confinement Act (HALT Act), N.Y. Corr. Law §137.

²⁷⁰ See generally HALT Act §137.

by mandating programming and out-of-cell time.²⁷¹ It also prohibits, altogether, the segregated confinement of those who are over 55, under 21, or who are disabled, pregnant, or postpartum.²⁷²

The HALT Act defines “segregated confinement” as the “confinement of an incarcerated individual in any form of cell confinement for more than seventeen hours a day other than in a facility-wide emergency or for the purpose of providing medical or mental health treatment.”²⁷³

4.6 Key Applicable Policies

4.6.1 DOCCS Directives

DOCCS uses written directives to guide its operational and administrative practices.²⁷⁴ Each directive includes a purpose statement and a policy statement, along with the definitions and provisions relevant to the directive subject matter. When applicable, each directive references supporting documents, such as New York Corrections Law, New York Codes, Rules and Regulations for DOCCS, and American Correctional Association expected practices.

4.6.2 American Correctional Association (ACA) Standards

The ACA is a national and international membership organization that develops, publishes, and promotes the implementation of operational standards and accreditation programs for correctional facilities.²⁷⁵ The ACA establishes performance-based standards emphasizing measurable outcomes, continuous self-evaluation, and ongoing updates as approved by practitioners and subject-matter experts in the ACA Performance Standard committee.²⁷⁶ The ACA’s standards and manuals address every area of correctional operations—including safety, security, services and programming, transportation of incarcerated individuals, healthcare,

²⁷¹ HALT Act §137.

²⁷² HALT Act §137(h); see N.Y. Correct. Law §2(33) (definition of “Special Populations”).

²⁷³ N.Y. Correct. Law §2(23).

²⁷⁴ *Laws Rules & Directives Listing*, DOCCS, <https://doccs.ny.gov/legal>.

²⁷⁵ [Cite Redacted].

²⁷⁶ [Cite Redacted].

records, administration, and staff training—and are intended to help agencies develop policies and procedures and to provide a framework for consistent, professional operations.²⁷⁷ The ACA standards “represent fundamental correctional practices that ensure staff and [incarcerated individual] safety and security; enhance staff morale; improve record maintenance and data management capabilities; assist in protecting the agency against litigation; and improve the function of the facility or agency at all levels.”²⁷⁸ DOCCS requires each facility to maintain compliance with ACA standards; both Marcy and Mid-State are currently accredited.²⁷⁹

4.7 Oversight

4.7.1 State Commission of Correction (SCOC)

The New York Constitution created the SCOC to “[p]romulgate[] minimum standards for the management of correctional facilities; [e]valuate[], investigate[] and oversee[] local and state correctional facilities and police lock-ups; [a]ssist[] in developing new correctional facilities; and [p]rovide[] technical assistance.”²⁸⁰ SCOC’s minimum standards for state correctional facilities touch on a wide range of topics, including health and safety, policies and procedures, maximum facility capacity, use of chemical agents, and nondiscriminatory treatment.²⁸¹ SCOC publishes annual reports to aid the Governor and the Legislature in policymaking.²⁸² SCOC has the corresponding duty to shut down “any correctional facility which is unsafe, unsanitary, or

²⁷⁷ [Cite Redacted].

²⁷⁸ [Cite Redacted].

²⁷⁹ Directive #6921 (Feb. 4, 2025).

²⁸⁰ N.Y. C.L.S. Const. Art. XVII, §5.

²⁸¹ *Regulations*, COMMISSION OF CORRECTION, <https://scoc.ny.gov/regulations>.

²⁸² *Commission Members and Organization*, COMMISSION OF CORRECTION, <https://scoc.ny.gov/commission-members-and-organization>.

inadequate.”²⁸³ SCOC also develops policies, plans, and programs for “improving the administration of ... correctional facilities and the delivery of services therein.”²⁸⁴

Two other oversight bodies operate within the SCOC: the Correction Medical Review Board and the Citizens’ Policy and Complaint Review Council.²⁸⁵ The Correction Medical Review Board investigates the cause and circumstances surrounding the death of incarcerated individuals.²⁸⁶ And the Citizens’ Policy and Complaint Review Council “[i]nvestigate[s], review[s] or take[s] action as it deems necessary with respect to grievances regarding any local correctional facility.”²⁸⁷

4.7.2 Correctional Association of New York (CANY)

CANY is “designated by law to provide independent monitoring and oversight of state prisons in New York State.”²⁸⁸ CANY “is charged with visiting and examining the state’s correctional facilities to identify and report on prison conditions, the treatment of incarcerated individuals, and the administration of policy promulgated by the executive and legislature.”²⁸⁹ Following visits to state prisons, CANY publicly releases post-visit reports with observations, findings, and a response to any findings or proposals from DOCCS.²⁹⁰ CANY also publishes

²⁸³ N.Y. Correct. Law §45.

²⁸⁴ N.Y. Corr. Law §45(2) (McKinney 1975).

²⁸⁵ *Correction Medical Review Board, Overview*, N.Y. STATE COMM’N OF CORRECT., <https://scoc.ny.gov/correction-medical-review-board>; New York Correction Law §43.

²⁸⁶ *Correction Medical Review Board, Overview*, N.Y. STATE COMM’N OF CORRECT., <https://scoc.ny.gov/correction-medical-review-board>

²⁸⁷ *Citizens’ Policy and Complaint Review Council*, N.Y. STATE COMM’N OF CORRECT., <https://scoc.ny.gov/citizens-policy-and-complaint-review-council>; New York Correction Law §42.

²⁸⁸ *About, CANY*, <https://www.correctionalassociation.org/our-mission>.

²⁸⁹ *About, CANY*, <https://www.correctionalassociation.org/our-mission>.

²⁹⁰ *Post-Visit Briefing Reports, CANY*, <https://www.correctionalassociation.org/post-visit-briefings>.

annual reports and compiles data on state prisons, which it then uses to inform recommendations to the Governor, the Legislature, and the public.²⁹¹

During its October 2022 visit to Marcy, CANY interviewed 117 incarcerated individuals in various housing units.²⁹² CANY stated that incarcerated individuals at Marcy were dissatisfied with the quality and accessibility of healthcare.²⁹³ Additionally, incarcerated individuals reported “rampant abuse by staff, including physical assaults and observations of a retaliatory environment.”²⁹⁴ CANY also reported “a significant number of instances of racialized abuse and discrimination including derogatory language and unequal treatment.”²⁹⁵

CANY also visited Mid-State in October 2022 and interviewed 122 incarcerated individuals across various housing units.²⁹⁶ Incarcerated individuals reported long wait times for medical care and delays, inattentiveness to reported grievances, and little confidence in the fairness of the staff disciplinary process.²⁹⁷ Numerous reports identified employee abuse of incarcerated individuals and retaliation after incarcerated individuals filed grievances.²⁹⁸

CANY has visited Marcy and Mid-State since 2022 but has not published any updated facility reports. However, CANY documented “serious concerns” at its visit to Mid-State in January 2025.²⁹⁹ The concerns included no progress on fixed surveillance camera installation

²⁹¹ *Monitoring and Reporting*, CANY, <https://www.correctionalassociation.org/issue-based-reports-and-fact-sheets>; *Legislative Testimony*, CANY, <https://www.correctionalassociation.org/testimony>.

²⁹² *Monitoring Visit to Marcy Correctional Facility*, (Oct. 11-12, 2022), https://static1.squarespace.com/static/62f1552c1dd65741c53bbcf8/t/64ab93bfab9c7e045ea53c01/1688966083012/2023_PVB-10-Marcy.pdf.

²⁹³ *Monitoring Visit to Marcy Correctional Facility*, (Oct. 11-12, 2022), p. 3.

²⁹⁴ *Monitoring Visit to Marcy Correctional Facility*, (Oct. 11-12, 2022), p. 4.

²⁹⁵ *Monitoring Visit to Marcy Correctional Facility*, (Oct. 11-12, 2022), p.4.

²⁹⁶ [Cite Redacted].

²⁹⁷ [Cite Redacted].

²⁹⁸ [Cite Redacted].

²⁹⁹ Statement in Response to Death of Incarcerated Individual at Mid-State Correctional Facility, CANY (Mar. 4, 2025), <https://www.correctionalassociation.org/press-releases-archive/correctional-association-of-new-york-cany-statement-in-response-to-death-of-incarcerated-individual-at-mid-state-correctional-facility>.

since the previous visit in October 2022, specific incidents of violence and abuse by correction officers, and a reported general lack of accountability, including fundamental breakdowns in trust and the functioning of essential services.³⁰⁰

4.7.3 New York State

The Governor and Legislature have key oversight responsibilities for DOCCS. The Governor nominates DOCCS' Commissioner, as well as the members of SCOC.³⁰¹

In addition, the New York State OIG has broad jurisdiction to investigate all executive branch entities, including DOCCS, for allegations of criminal wrongdoing, corruption, fraud, and abuse.³⁰² DOCCS matters account for half of OIG's caseload and are handled by a dedicated task force that is responsible for both proactive and complaint-driven investigations into misconduct, including abuse of authority, staff contraband possession, and inappropriate staff-incarcerated individual relationships.³⁰³

4.8 DOCCS Operating Budget

4.8.1 Total Spending

In fiscal year 2025, DOCCS' annual budget was approximately \$3.4 billion. This represents less than 2% of the state's overall budget, but nearly a third of what New York State

³⁰⁰ Statement in Response to Death of Incarcerated Individual at Mid-State Correctional Facility, CANY (Mar. 4, 2025).

³⁰¹ *Commissioner Daniel F. Martuscello III*, DOCCS, <https://doccs.ny.gov/commissioner-daniel-f-martuscello-iii>; N.Y. Corr. Law §41 (McKinney 1975).

³⁰² *About the Office*, N.Y. STATE OFF. OF THE INSPECTOR GEN., <https://ig.ny.gov/offices/inspectorgeneral>; *Docs Oversight*, N.Y. STATE OFF. OF THE INSPECTOR GEN., <https://ig.ny.gov/doccs-matters>; Executive Law Article 4-A, N.Y. STATE OFF. OF THE INSPECTOR GEN., <https://ig.ny.gov/executive-law-article-4>.

³⁰³ *Docs Oversight*, N.Y. STATE OFF. OF THE INSPECTOR GEN., <https://ig.ny.gov/doccs-matters>.

spends on criminal justice.³⁰⁴ In fiscal year 2025, DOCCS spent approximately \$109,000 per incarcerated individual.³⁰⁵

DOCCS also spends millions on civil payments, including for alleged staff misconduct. As a data point, DOCCS made approximately \$43 million in Court of Claims payments from 2019 through 2024.³⁰⁶ While the State estimates that a majority of this sum stems from civil rights cases, it could not provide a detailed breakdown.³⁰⁷

4.9 Unions

DOCCS' workforce is represented by an array of labor organizations. The largest in number is the New York Correctional Officers and Police Benevolent Association (NYSCOPBA), which represents correction officers and has historically been the most vocal in matters concerning safety, staffing, and labor conditions.³⁰⁸ Council 82, affiliated with the American Federation of State, County, and Municipal Employees and AFL-CIO, represents supervisory law enforcement personnel, such as lieutenants.³⁰⁹ The Civil Service Employees Association represents civilian staff across multiple bargaining units, including administrative, operational, and institutional services, as well as maintenance and support personnel within DOCCS facilities.³¹⁰ The Public Employees Federation represents professional, scientific, and

³⁰⁴ *Budget & Actuals, Open Budget: Spending Form*, N.Y. STATE DIV. OF THE BUDGET, <https://openbudget.ny.gov/spendingForm.html>.

³⁰⁵ *Budget & Actuals, Open Budget: Spending Form*, N.Y. STATE DIVISION OF THE BUDGET, <https://openbudget.ny.gov/spendingForm.html>; *UC Profile*, DOCCS (April 1, 2025), <https://doccs.ny.gov/system/files/documents/2025/04/2025.04.01-uc-profile.pdf>.

³⁰⁶ [Cite Redacted].

³⁰⁷ [Cite Redacted].

³⁰⁸ *NYSCOPA Asks for Meeting with DOCCS Officials to Address Safety, Working Conditions for Officers*, NBC 5 (Apr. 4, 2025), <https://www.mynbc5.com/article/nyscopba-doccs-meeting-corrections-officers/64390337>.

³⁰⁹ *Security Supervisors Unit (SSPU) 61 and 91*, N.Y. STATE OFFICE OF EMPLOYEE RELATIONS, <https://oer.ny.gov/security-supervisors-unit-sspu-61-and-91>.

³¹⁰ *State Government Employees*, CIVIL SERVICE EMPLOYEES ASS'N, <https://cseany.org/for-members/state-government-employees>.

technical staff, including nurses, mental health professionals, addiction specialists, and parole officers, who provide critical rehabilitative and reentry services to incarcerated individuals.³¹¹

³¹¹ *Get to Know Your Union*, N.Y. STATE PUBLIC EMPLOYEES FEDERATION, <https://www.pef.org/get-to-know-your-union>.

5. **FINDINGS**

5.1 **Introduction**

This section discusses our findings relating to key areas of focus, including use of force by DOCCS staff, safety at DOCCS facilities, and staffing challenges—principally at Marcy and Mid-State, the sites of the killings of Mr. Brooks and Mr. Nantwi. This section also integrates insights from DOCCS’ data on use of force, survey data, and interviews with current and former staff and incarcerated individuals.

5.2 **Reported Uses of Force: DOCCS, Marcy, and Mid-State**

DOCCS defines a “use of force” as “any instance in which physical action is taken to resolve an incident,”³¹² including, but not limited to, the use of body holds, batons, shields, chemical agents, mechanical restraints, tasers, or firearms.³¹³ DOCCS policy requires reporting of “all instances where physical force is used” through a Use of Force Report written by the involved officers, logged by the watch commander, and reviewed by the superintendent.³¹⁴ DOCCS policy also requires officers to prepare these reports no matter the degree of force, meaning that all uses of force—not just excessive force—must be reported. The following quantitative findings are based on our review of nonpublic use of force data from 2015-2025 produced by DOCCS.³¹⁵

Over the past decade, reported incidents of force in DOCCS facilities have more than tripled, despite a decline in the incarcerated population.³¹⁶ The number of reported uses of force

³¹² Directive #4944, §II.R (Apr. 28, 2023).

³¹³ Directive #4944, §II.R (Apr. 28, 2023).

³¹⁴ Directive #4944, §IV (Apr. 28, 2023).

³¹⁵ This data represents staff uses of force, with up to three types of force recorded per staff member in an incident; *e.g.*, if three staff members use body holds in a single incident, three body hold uses are counted here. Incidents reported at the central office have been excluded. Accordingly, the quantitative findings may not represent distinct instances of force used. [Cite Redacted].

³¹⁶ [Cite Redacted].

has increased almost every year between 2016 and 2025 (except in 2020, when the reported uses of force remained roughly the same as 2019), resulting in over a three-fold increase in reported uses of force over that period, from approximately 6,000 in 2016 to approximately 22,000 in 2025.³¹⁷

The increase in overall reported uses of force could be due to a number of factors. For example, while the population of incarcerated individuals at DOCCS facilities has been decreasing, the proportion of incarcerated individuals who have been convicted of violent offenses has increased.³¹⁸ During this period, staffing levels at DOCCS have declined,³¹⁹ with the percentage of vacant staff positions at Marcy increasing from 10.4% in 2022 to 18.9% in 2025, and Mid-State’s vacancy rate increasing from 8.5% to 35% in the same period.³²⁰ In addition, the use of BWCs has led to more accurate reporting when uses of force occur. At the same time, it could signal that use of force has become more prevalent in DOCCS facilities.

It is worth noting that only a small percentage of reported uses of force lead to a referral to OSI for investigation of excessive force, and even fewer are substantiated. For example, in 2025, OSI received a total of 1,189 complaints for “Use of Force: Excessive/Unnecessary” or “Staff on [Incarcerated Individual] Assault,” comprising approximately 5.4% of all reported uses

³¹⁷ [Cite Redacted].

³¹⁸ See, e.g., Elizabeth Johnson, *Trends in the New York State Prison Population, 2008-2023* <https://datacollaborativeforjustice.org/wp-content/uploads/2023/07/PrisonPop.pdf> (showing the proportion of the population charged with violent felonies increasing, while the overall population has sharply decreased); laws like The 2009 Rockefeller Drug Law Reforms, The Bail Reform Act of 2019, and The “Less is More” Act (2021) all decreased the number of non-violent offenders in state prison. See, e.g., *Understanding the Impact of New York Bail Reform*, VERA, <https://www.vera.org/publications/the-impact-of-new-york-bail-reform-on-statewide-jail-populations#:~:text=Bail%20reform%20led%20to%20a,rarely%20considered%20ability%20to%20pay>.

³¹⁹ *Fact Sheet*, p. 3, DOCCS (Jan. 2026), <https://doccs.ny.gov/system/files/documents/2026/01/doccs-fact-sheet-january-2026.pdf>.

³²⁰ *Data Portal, Dashboard-Staffing*, CANY, <https://www.correctionalassociation.org/data/dashboard-staffing>.

of force. Of those 1,189 cases, 47 were substantiated, including 40 substantiated specifically for “Use of Force: Excessive/Unnecessary.”³²¹

For 2025, Mid-State was the facility with the highest number of complaints to OSI for alleged staff assault or excessive or unnecessary use of force, while Marcy was the tenth highest (out of 40 DOCCS facilities).³²² The overall number of reported uses of force at Mid-State increased by approximately 66% over the prior year (which itself was a historic high), despite an average population decrease at Mid-State,³²³ while at Marcy, overall reported uses of force trended downward in 2025 compared to 2024.³²⁴ In 2025, the number of reported uses of force at Mid-State increased by approximately 66% over the prior year (which itself was a historic high), despite an average population decrease at Mid-State.³²⁵ At Marcy and Mid-State, reported use of force incidents occurred more frequently in the special housing units.³²⁶ Approximately 36.4% of reported uses of force at Marcy between 2020 and 2024 occurred within the RMHU—the designated unit for individuals with a serious mental illness—which housed approximately 10% of the Marcy incarcerated population during that period.³²⁷ Approximately 25.6% of all reported uses of force at Mid-State between 2020 and 2024 occurred within the SDP—a behavior modification program for long-term SHU individuals with the goal of safely returning them to

³²¹ [Cite Redacted].

³²² During 2025, OSI received 99 complaints from incarcerated individuals at Mid-State that were coded as “Use of Force: Excessive/Unnecessary” or “Staff on [Incarcerated Individual] Assault.” This comprised approximately 8% of the total number of complaints received by OSI during 2025. Of those 99 Mid-State complaints, OSI substantiated nine of those cases during 2025—also the highest number of substantiated complaints across all DOCCS facilities. [Cite Redacted].

³²³ [Cite Redacted].

³²⁴ [Cite Redacted].

³²⁵ [Cite Redacted].

³²⁶ Note that these statistics are based on Use of Force Reports drafted by involved officers. Accordingly, the quantitative findings may represent inaccuracies based on human error, biases, and other reporting deficiencies.

³²⁷ [Cite Redacted]; *Data Portal, Dashboard-Under Custody Reports Marcy 2020-2024*, CANY, <https://www.correctionalassociation.org/data/dashboard-under-custody>.

general population—which housed approximately 7% of the Mid-State population during that period.³²⁸

5.3 Incarcerated Individuals’ Perspectives on Safety and Use of Force

5.3.1 Survey Results on Use of Force

We distributed surveys to incarcerated individuals in both the general population and special housing units of Marcy and Mid-State, which housed 782 and 1,164 incarcerated individuals, respectively, as of October 1, 2025 (a proximate date to the distribution and collection of surveys). As explained in our Methodology, we also conducted structured interviews of incarcerated individuals, which complemented and further contextualized the topics covered in the surveys. We acknowledge that these two facilities may be unique because of the killings of Mr. Brooks and Mr. Nantwi. Therefore, our observations primarily pertain to conditions at these two facilities, not to DOCCS as a whole.

Surveys of incarcerated individuals at Marcy and Mid-State covered experiences with and perceptions of staff uses of force. On a scale from “Not At All Safe” to “Very Safe,” 62.2% of the 440 incarcerated individual respondents out of 1,164 incarcerated individuals at Mid-State and 57.2% of the 361 respondents out of 782 incarcerated individuals at Marcy reported feeling “Not At All Safe” or “Unsafe” from violence by staff.³²⁹ (see Figure 5).

³²⁸ [Cite Redacted]; *Data Portal, Dashboard-Under Custody Reports Mid-State 2020-2024*, CANY, <https://www.correctionalassociation.org/data/dashboard-under-custody>.

³²⁹ [Cite Redacted].

Figure 5 Incarcerated Individual Survey Result

“How safe do you feel from violence by staff?”						
Facility	Not At All Safe	Unsafe	Somewhat Safe	Safe	Very Safe	Total
Marcy	37.9%	19.3%	30.6%	7.6%	4.6%	100.0%
Mid-State	43.5%	18.7%	25.3%	5.4%	7.2%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility

population. Over half of the 361 incarcerated individual respondents out of 782 incarcerated individuals at Marcy and nearly half of the 440 incarcerated individual respondents out of 1,164 incarcerated individuals at Mid-State reported having “been personally involved” in uses of force at those facilities (54.8% at Marcy and 46.4% at Mid-State).³³⁰ (see Figure 6). The vast majority of those at Marcy (93.9%) and Mid-State (89.5%) who reported having been involved in the use of force reported believing that the force was unjustified.³³¹ (see Figure 7). In addition, the majority of those at Marcy and Mid-State who reported having been involved in uses of force indicated that the use of force resulted in injuries (57.5% at Marcy and 71.9% at Mid-State).³³² (see Figure 8).

Figure 6 Incarcerated Individual Survey Result

“Have you ever been personally involved in Use of Force at this facility?”			
Facility	No	Yes	Total
Marcy	45.2%	54.8%	100.0%
Mid-State	53.6%	46.4%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

³³⁰ [Cite Redacted].

³³¹ [Cite Redacted].

³³² [Cite Redacted].

Figure 7 Incarcerated Individual Survey Result

(For individuals who reported a history of Use of Force at their current facility) “In your opinion, was the Use of Force justified?”			
Facility	No	Yes	Total
Marcy	93.9%	6.1%	100.0%
Mid-State	89.5%	10.5%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

Figure 8 Incarcerated Individual Survey Result

(For individuals who reported a history of Use of Force at their current facility) “Did the Use of Force result in any injuries?”			
Facility	No	Yes	Total
Marcy	42.5%	57.5%	100.0%
Mid-State	28.1%	71.9%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population. Approximately two-thirds of the 361 incarcerated individual respondents out of 782 incarcerated individuals at Marcy and the 440 incarcerated individual respondents out of 1,164 incarcerated individuals at Mid-State said they had witnessed uses of force at the facility (63% at Marcy and 65.4% at Mid-State).³³³ (see Figure 9). Again, the vast majority of those respondents who witnessed force reported that they believed the use of force was unjustified (89.9% at Marcy and 90.4 % at Mid-State).³³⁴ (see Figure 10).

³³³ [Cite Redacted].

³³⁴ [Cite Redacted].

Figure 9 Incarcerated Individual Survey Result

“Have you ever witnessed other uses of force at this facility?”			
Facility	No	Yes	Total
Marcy	37.0%	63.0%	100.0%
Mid-State	34.6%	65.4%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

Figure 10 Incarcerated Individual Survey Result

(For individuals who reported witnessing other uses of force at their current facility) “In your opinion, was the use of force justified?”			
Facility	No	Yes	Total
Marcy	89.9%	10.1%	100.0%
Mid-State	90.4%	9.6%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

About half of the combined 801 incarcerated individual respondents of the total population of 1,946 incarcerated individuals at Marcy and Mid-State indicated that they “Often” or “Very Often” noticed differences based on race, both in how often staff use force (50.6% at Marcy and 48.9% at Mid-State) (see Figure 11) and the level of force used (52.8% at Marcy and 49.1% at Mid-State) (see Figure 12).³³⁵

³³⁵ [Cite Redacted].

Figure 11 Incarcerated Individual Survey Result

“Do you notice differences in how often staff use force on people of different racial groups?”						
Facility	Never	Once	Few	Often	Very Often	Total
Marcy	19.1%	3.4%	27.0%	25.5%	25.1%	100.0%
Mid-State	13.9%	6.2%	31.1%	24.3%	24.6%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

Figure 12 Incarcerated Individual Survey Result

“Do you notice differences in the level of force staff use on people of different racial groups?”						
Facility	Never	Once	Few	Often	Very Often	Total
Marcy	19.9%	4.1%	23.2%	27.7%	25.1%	100.0%
Mid-State	17.1%	5.4%	28.4%	26.9%	22.2%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

5.3.2 Survey Results on Other Mistreatment

Surveyed incarcerated individuals also cited other staff mistreatment, such as arbitrary discipline. For instance, 47% of the 440 incarcerated individual respondents out of 1,164 incarcerated individuals at Mid-State and 45.2% of the 361 incarcerated individual respondents out of 782 incarcerated individuals at Marcy reported that they had received false or unjust misbehavior tickets during 2025.³³⁶ (see Figure 13). More than half of the respondents at Marcy (55%) and Mid-State (58%) also said rules and expectations were inconsistent.³³⁷ (see Figure 14).

³³⁶ [Cite Redacted].

³³⁷ [Cite Redacted].

Figure 13 Incarcerated Individual Survey Result

“In 2025, have you been issued a ticket you felt was false or unjust?”					
Facility	Never	Few	Often	Very Often	Total
Marcy	54.8%	16.9%	9.6%	18.8%	100.1% ³³⁸
Mid-State	53.0%	21.6%	6.0%	19.5%	100.1%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

Figure 14 Incarcerated Individual Survey Result

“This facility generally has consistent rules and expectations.”					
Facility	Strongly Disagree	Disagree	Agree	Strongly Agree	Total
Marcy	26.1%	28.9%	39.8%	5.3%	100.0%
Mid-State	24.5%	33.5%	35.1%	6.9%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

5.3.3 Survey Results on Violence by Other Incarcerated Individuals

Many incarcerated individuals at Marcy and Mid-State—where the majority of housing units are dormitory-style—also reported feeling unsafe from violence by other incarcerated individuals.³³⁹ When asked how safe they felt from violence by other incarcerated individuals, 43.6% of the 361 incarcerated individual respondents out of 782 incarcerated individuals at Marcy and 50.5% of the 440 incarcerated individual respondents out of 1,164 incarcerated individuals at Mid-State reported feeling “Not At All Safe” or “Unsafe.”³⁴⁰ (see Figure 15).

These numbers indicate less fear of violence from other incarcerated individuals than from staff

³³⁸ Throughout this report, total percentages for survey data that are slightly above (i.e., 100.1%) or slightly below (i.e., 99.9%) 100% reflect rounding to the nearest tenth in the component percentages.

³³⁹ [Cite Redacted].

³⁴⁰ [Cite Redacted].

but still represent widespread feelings of unsafety. This result is consistent with the high level of assaults on incarcerated individuals statewide reported by DOCCS.³⁴¹

Figure 15 Incarcerated Individual Survey Result

“How safe do you feel from violence or mistreatment by other incarcerated individuals?”						
Facility	Not At All Safe	Unsafe	Somewhat Safe	Safe	Very Safe	Total
Marcy	27.3%	16.3%	37.1%	10.4%	8.9%	100.0%
Mid-State	33.6%	16.9%	32.6%	7.4%	9.5%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

5.3.4 Interviews on Use of Force

In addition to the surveys, we conducted structured, voluntary, and confidential interviews concerning a wide range of topics with a stratified, random sample of incarcerated individuals at Marcy and Mid-State. The interview results relating to use of force were consistent with the survey findings.

For example, interviewees commonly stated that force was “random” (without any provocation) or in response to a minor problem (e.g., going to the wrong mess hall or complaining about needing different medication).³⁴² One interviewee described witnessing Mid-State staff using force against an incarcerated individual who was experiencing a drug overdose.³⁴³ Another described being subject to staff use of force after accidentally going to the wrong mess hall.³⁴⁴

³⁴¹ *Fact Sheet*, p. 1, DOCCS (Jan. 2026), <https://doccs.ny.gov/system/files/documents/2026/01/doccs-fact-sheet-january-2026.pdf>.

³⁴² [Cite Redacted].

³⁴³ [Cite Redacted].

³⁴⁴ [Cite Redacted].

Several interviewees described what they viewed as concerted efforts by security staff to conceal uses of force. Interviewees reported blows to the ribs, which incarcerated individuals said were used by officers to conceal evidence of injury; bruising to the chest/rib area, for example, is more easily concealed underneath clothing, as opposed to the face.³⁴⁵ As an example, one interviewee said he witnessed an officer punch an incarcerated individual in the ribs multiple times for smoking in the dorm.³⁴⁶ Another interviewee recalled that, when another incarcerated individual did not wake up for count, an officer started punching him in the ribs, similar to the allegations that officers beat Mr. Nantwi for not appearing for count.³⁴⁷ Relatedly, some incarcerated individuals reported that officers often use force near the double doors at the entry of a building because it is a camera blind spot.³⁴⁸

Incarcerated individuals at Marcy also told us that uses of force frequently occur in van rides and in the infirmary; Mr. Brooks was killed in the infirmary.³⁴⁹ Several interviewees described witnessing others being dragged into vans to be beaten or hearing screaming coming from inside vans, and others alleged they had personally been beaten in vans.³⁵⁰ Others said staff would take incarcerated individuals to a medical treatment room in the infirmary to “deal with” them, which they said typically involved pepper spraying and beatings.³⁵¹ One interviewee told us that he heard Marcy staff beat an incarcerated individual in an infirmary room, while a lieutenant conversed with a nurse outside the room.³⁵² The incarcerated individual told us that he

³⁴⁵ [Cite Redacted].

³⁴⁶ [Cite Redacted].

³⁴⁷ See Section 1.4, “Killing of Messiah Nantwi,” [Cite Redacted].

³⁴⁸ [Cite Redacted].

³⁴⁹ [Cite Redacted].

³⁵⁰ [Cite Redacted].

³⁵¹ [Cite Redacted].

³⁵² [Cite Redacted].

heard the nurse yell at the screaming incarcerated individual to “shut up.”³⁵³ Another incarcerated individual recalled an officer moving everyone in the infirmary waiting room to a different area of the building for approximately 30 minutes, during which he heard screaming from what he perceived to be a use of force in the infirmary.³⁵⁴ Several incarcerated individuals stated they believed uses of force tended to occur in the Marcy medical exam rooms because the windows are blacked out and no other incarcerated individuals are present.³⁵⁵

Numerous incarcerated interviewees stated that the implementation of additional cameras, and especially BWCs, improved safety conditions and their dynamic with security staff.³⁵⁶ However, many other individuals expressed that some officers disregard the camera policies, “bend the corners” by manipulating their BWCs, or intentionally escalate tensions before activating their BWCs.³⁵⁷

5.3.5 Interviews on Other Mistreatment

Interviews with incarcerated individuals at Marcy and Mid-State also revealed concerns beyond staff use of force, including instances of retaliatory and/or arbitrary enforcement of rules and expectations. Some incarcerated individuals described officers planting (or threatening to plant) contraband in their housing units, resulting in false disciplinary charges.³⁵⁸ A formerly incarcerated individual also described an incident in which a correction officer threatened to “plant a weapon on [him], and then we can use it to kill [him].”³⁵⁹ As previously discussed, following Mr. Nantwi’s killing, officers allegedly conspired to plant a weapon in his room to

³⁵³ [Cite Redacted].

³⁵⁴ [Cite Redacted].

³⁵⁵ [Cite Redacted].

³⁵⁶ [Cite Redacted].

³⁵⁷ [Cite Redacted].

³⁵⁸ [Cite Redacted].

³⁵⁹ [Cite Redacted].

fabricate a justification for their use of force.³⁶⁰ Incarcerated individuals also reported receiving disciplinary tickets for “unfair” reasons, such as when they have a medical episode, are singled out for behavior when others are not punished, or receive collective punishment for one individual’s infraction.³⁶¹ For example, one interviewee with a history of strokes said that while he was suffering stroke-like symptoms, an officer accused him of being intoxicated and struck him in the genital area; a nurse intervened to tell the officer to stop, knowing the incarcerated man’s medical history.³⁶² The incarcerated man stated that he later received multiple misbehavior reports, while in recovery, after he filed a complaint about the adequacy of his medical care, which he perceived as retaliation.³⁶³ In another example, an incarcerated individual reported that he overheard officers who had responded to a seizure talk loudly about how the seizing incarcerated individual was “just high,” despite having a history of seizures.³⁶⁴

Other incarcerated individuals described being forced to remain outside during inclement weather. For example, we heard multiple accounts, including from a facility leader, that incarcerated individuals are required to wait in long lines outside the infirmary to receive medication with insufficient outerwear, which is especially challenging for elderly or medically infirm individuals.³⁶⁵ In addition, during one site visit in April 2025, we saw BWC footage of a sergeant who forced an incarcerated individual to remain outside for over three hours of nighttime recreation, despite his repeated requests to come inside because he was “freezing” in the cold weather.³⁶⁶ After reviewing the incident, facility leadership acknowledged the officer’s

³⁶⁰ See Section 1.4, “Killing of Messiah Nantwi.”

³⁶¹ [Cite Redacted].

³⁶² [Cite Redacted].

³⁶³ [Cite Redacted].

³⁶⁴ [Cite Redacted].

³⁶⁵ [Cite Redacted].

³⁶⁶ [Cite Redacted].

misconduct and told us that, regardless of temperature, officers must honor an incarcerated individual's request to end recreation and return indoors.³⁶⁷

5.4 Reported Assaults on Staff

DOCCS data illustrates that assaults by incarcerated individuals on *both* staff and other incarcerated individuals have risen since 2020, reaching their highest point in 2024.³⁶⁸ Average assaults statewide for both groups decreased in 2025, but rates remain elevated compared to earlier years.³⁶⁹ In 2023, according to DOCCS' unusual incident report data, which tracks serious occurrences at the facilities, Marcy had 95 assaults on staff and 67 assaults on other incarcerated individuals, while at Mid-State, there were 75 assaults on staff and 90 assaults on other incarcerated individuals.³⁷⁰ DOCCS reported higher rates for both facilities in its unusual incident reporting for 2024: that year saw 102 assaults on staff and 78 assaults on incarcerated individuals at Marcy, while at Mid-State, there were 87 assaults on staff and 112 assaults on incarcerated individuals.³⁷¹ Both Marcy and Mid-State have large, specialized, maximum-security units (the RMHU and SDP, respectively) that may be significant contributors to these high rates of reported assault.

Assaults were classified as "any attack by an incarcerated individual," including the throwing of an object that hits another person, regardless of whether injury occurs.³⁷² On January

³⁶⁷ [Cite Redacted].

³⁶⁸ *Fact Sheet*, p. 1, DOCCS (Jan. 2026), <https://doccs.ny.gov/system/files/documents/2026/01/doccs-fact-sheet-january-2026.pdf>.

³⁶⁹ *Fact Sheet*, p. 1, DOCCS (Jan. 2026), <https://doccs.ny.gov/system/files/documents/2026/01/doccs-fact-sheet-january-2026.pdf>. (comparing 2025 data to prior years).

³⁷⁰ *Unusual Incident Report 2024*, p. 4, DOCCS, https://doccs.ny.gov/system/files/documents/2024/08/annual-ui-report-2023_final.pdf.

³⁷¹ *Unusual Incident Report 2024*, p. 4, DOCCS, https://doccs.ny.gov/system/files/documents/2025/10/annual-ui-report-2024_final.pdf.

³⁷² *Fact Sheet*, DOCCS (Jan. 2026), <https://doccs.ny.gov/system/files/documents/2026/01/doccs-fact-sheet-january-2026.pdf>.

1, 2026, DOCCS updated this definition to “when, with intent to cause harmful or offensive contact, or physical injury to another person, they strike, shove, kick or otherwise subject such other person to physical contact.”³⁷³ In this updated definition, DOCCS emphasized that “fundamental” to an assessment of assault is the intent to injure, whether or not an injury occurs.³⁷⁴ DOCCS also clarified that the throwing of bodily fluids and/or excrement, an all-too-common occurrence known as “throwing” at DOCCS facilities, is presumed intentional and harmful and is automatically categorized as assault.³⁷⁵

Staff can sustain injuries when assaulted by incarcerated individuals, but injuries rarely occur and, when they do, they are typically minor. For example, during 2025, DOCCS reported that staff sustained injuries in 23.1% of assaults by incarcerated individuals.³⁷⁶ Of the assaults that caused injuries, approximately 91% were categorized as “minor” injuries, defined as “[i]njuries that require either no treatment, minimal treatment (scratch, bruise, aches/pain), or precautionary treatment.”³⁷⁷ A small portion of injuries (approximately 7.7%) were categorized as “moderate,” defined as “[i]njuries such as lacerations, concussions, 2nd degree burns, serious

³⁷³ [Cite Redacted].

³⁷⁴ [Cite Redacted].

³⁷⁵ [Cite Redacted].

³⁷⁶ Aggregating quarterly fact sheets: *Fact Sheet*, DOCCS (Apr. 2025), https://doccs.ny.gov/system/files/documents/2025/04/doccs-fact-sheet-april-2025_4.pdf; *Fact Sheet*, DOCCS (Jul. 2025), <https://doccs.ny.gov/system/files/documents/2025/07/doccs-fact-sheet-july-2025.pdf>; *Fact Sheet*, DOCCS (Oct. 2025), <https://doccs.ny.gov/system/files/documents/2025/10/doccs-fact-sheet-october-2025.pdf>; *Fact Sheet*, DOCCS (Jan. 2026), <https://doccs.ny.gov/system/files/documents/2026/01/doccs-fact-sheet-january-2026.pdf>.

³⁷⁷ Aggregating quarterly fact sheets: *Fact Sheet*, DOCCS (Apr. 2025), https://doccs.ny.gov/system/files/documents/2025/04/doccs-fact-sheet-april-2025_4.pdf; *Fact Sheet*, DOCCS (Jul. 2025), <https://doccs.ny.gov/system/files/documents/2025/07/doccs-fact-sheet-july-2025.pdf>; *Fact Sheet*, DOCCS (Oct. 2025), <https://doccs.ny.gov/system/files/documents/2025/10/doccs-fact-sheet-october-2025.pdf>; *Fact Sheet*, DOCCS (Jan. 2026), <https://doccs.ny.gov/system/files/documents/2026/01/doccs-fact-sheet-january-2026.pdf>.

sprains, dislocation, and muscle or ligament damage.”³⁷⁸ Approximately 1% of the injuries involved an officer sustaining a “serious” injury, which is defined as an injury “that require[s] transport to an outside hospital but [is] not considered life-threatening at the preliminary report.”³⁷⁹ And only one “severe” case was reported; a “severe” injury risks death or “cause[s] obvious disfigurement, protracted impairment of health, loss or impairment of organ function, [or] amputation.”³⁸⁰ Compared to recent years, the reported injuries for 2025 do not appear to be an outlier.³⁸¹

5.5 Staff Perspectives on Safety and Use of Force

5.5.1 Staff Survey Results

At Marcy and Mid-State, we distributed surveys to civilian staff, security officers, and leadership at Marcy and Mid-State—either through their mailbox or directly at their posts. Sixty-six out of approximately 379 Marcy staff members (17.4%) and 36 out of approximately 474 Mid-State staff members (7.6%) completed the survey.³⁸² (see Figure 16). Given the low survey response rate, the results of the surveys must be viewed with caution. And again, we

³⁷⁸ Aggregating quarterly fact sheets: *Fact Sheet*, DOCCS (Apr. 2025), https://doccs.ny.gov/system/files/documents/2025/04/doccs-fact-sheet-april-2025_4.pdf; *Fact Sheet*, DOCCS (Jul. 2025), <https://doccs.ny.gov/system/files/documents/2025/07/doccs-fact-sheet-july-2025.pdf>; *Fact Sheet*, DOCCS (Oct. 2025), <https://doccs.ny.gov/system/files/documents/2025/10/doccs-fact-sheet-october-2025.pdf>; *Fact Sheet*, DOCCS (Jan. 2026), <https://doccs.ny.gov/system/files/documents/2026/01/doccs-fact-sheet-january-2026.pdf>.

³⁷⁹ Aggregating quarterly fact sheets: *Fact Sheet*, DOCCS (Apr. 2025), https://doccs.ny.gov/system/files/documents/2025/04/doccs-fact-sheet-april-2025_4.pdf; *Fact Sheet*, DOCCS (Jul. 2025), <https://doccs.ny.gov/system/files/documents/2025/07/doccs-fact-sheet-july-2025.pdf>; *Fact Sheet*, DOCCS (Oct. 2025), <https://doccs.ny.gov/system/files/documents/2025/10/doccs-fact-sheet-october-2025.pdf>; *Fact Sheet*, DOCCS (Jan. 2026), <https://doccs.ny.gov/system/files/documents/2026/01/doccs-fact-sheet-january-2026.pdf>.

³⁸⁰ Aggregating quarterly fact sheets: *Fact Sheet*, DOCCS (Apr. 2025), https://doccs.ny.gov/system/files/documents/2025/04/doccs-fact-sheet-april-2025_4.pdf; *Fact Sheet*, DOCCS (Jul. 2025), <https://doccs.ny.gov/system/files/documents/2025/07/doccs-fact-sheet-july-2025.pdf>; *Fact Sheet*, DOCCS (Oct. 2025), <https://doccs.ny.gov/system/files/documents/2025/10/doccs-fact-sheet-october-2025.pdf>; *Fact Sheet*, DOCCS (Jan. 2026), <https://doccs.ny.gov/system/files/documents/2026/01/doccs-fact-sheet-january-2026.pdf>.

³⁸¹ From 2023 to 2024, the rates of minor, moderate, serious, and severe injuries to staff due to assaults by incarcerated individuals decreased by less than 1%. *Compare Unusual Incident Report 2023*, p. 12, DOCCS, https://doccs.ny.gov/system/files/documents/2024/08/annual-ui-report-2023_final.pdf with *Unusual Incident Report 2024*, p. 12, DOCCS, https://doccs.ny.gov/system/files/documents/2025/10/annual-ui-report-2024_final.pdf.

³⁸² [Cite Redacted].

acknowledge that these two facilities may be unique because of the killings of Mr. Brooks and Mr. Nantwi. The findings reflect the perspectives of respondents when surveyed and should not be assumed to generalize the overall staff population. Although the survey response rate did not meet expectations for a representative sample of the respective facilities, we are presenting the findings as the lived experiences of the respondents and to ensure that their perspectives are represented.

Figure 16 Response Rates for Surveys of Facility Staff by Facility

Facility	Allocated Positions	Filled Positions	Surveys Received	Staff Response Rate ³⁸³
Marcy	518.2	378.6	66.0	17.4%
Mid-State	747.7	474.4	36.0	7.6%

Note: Percentages reflect the proportion of survey respondents, not the total facility staff.

The majority of staff respondents at Mid-State and many at Marcy said they felt unsafe at their facilities. At Mid-State, 60% of the 36 staff respondents reported feeling “Not Safe At All” or “Unsafe” while working at the facility; at Marcy, 45.9% of the 66 staff respondents reported feeling “Not Safe At All” or “Unsafe” while working at the facility.³⁸⁴ (see Figure 17).

³⁸³ We calculated the staff response rate by dividing the number of surveys received from a facility by the number of filled staff positions. Allocated positions are reported only for reference, but they show a large number of unfilled positions at both facilities.

³⁸⁴ [Cite Redacted].

Figure 17 Staff Survey Result

“Overall, how safe do you feel at work at this facility?”						
Facility	Not Safe At All	Unsafe	Somewhat Safe	Safe	Very Safe	Total
Marcy	26.2%	19.7%	29.5%	13.1%	11.5%	100.0%
Mid-State	31.4%	28.6%	20.0%	14.3%	5.7%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility staff.

Most staff survey respondents also reported feeling unsafe when asked specifically about violence by incarcerated individuals. At Mid-State, 65.7% of the 36 staff respondents reported feeling “Not Safe At All” or “Unsafe” from violence by incarcerated individuals; at Marcy, 55.7% of the 66 staff respondents reported feeling “Not Safe At All” or “Unsafe” from violence by incarcerated individuals.³⁸⁵ (see Figure 18).

Figure 18 Staff Survey Result

“How safe do you feel form violence by incarcerated individuals?”						
Facility	Not Safe At All	Unsafe	Somewhat Safe	Safe	Very safe	Total
Marcy	31.1%	24.6%	21.3%	13.1%	9.8%	100.0%
Mid-State	28.6%	37.1%	14.3%	11.4%	8.6%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility staff.

³⁸⁵ [Cite Redacted].

These survey responses are consistent with the high number of assaults on staff reported by DOCCS statewide.³⁸⁶ While DOCCS data shows that most staff injuries are minor,³⁸⁷ correctional facilities present inherent safety risks. Even minor injuries, coupled with ongoing exposure to serious risks and traumatic incidents discussed below, undoubtedly contribute to staff feelings of not being safe.

No staff survey respondents reported feeling that safety conditions were better in 2025 as compared to previous years.³⁸⁸ In fact, 87% of the 66 staff survey respondents at Marcy and 80.6% of the 36 staff survey respondents at Mid-State reported that safety issues were “Worse.”³⁸⁹ (see Figure 19).

Figure 19 Staff Survey Result

“Generally, are safety issues better, worse, or the same in 2025 vs. previous years?”			
Facility	Worse	Same	Total
Marcy	87.0%	13.0%	100.0%
Mid-State	80.6%	19.4%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility staff.

5.5.2 Staff Interviews

Alongside interviews at facility visits, we reached out to over 100 current and former correction officers between August and October 2025. Most were former officers from Marcy and/or Mid-State who retired or resigned between 2020 and 2025. More than 40 officers agreed to speak, some more than once. Altogether, these officers represented perspectives from 15 different correctional facilities in the DOCCS system, though the number of officers

³⁸⁶ See Section 5.4, “Reported Assaults on Staff.”

³⁸⁷ See Section 5.4, “Reported Assaults on Staff.”

³⁸⁸ [Cite Redacted].

³⁸⁹ [Cite Redacted].

participating was not sufficient to draw conclusions about all 15 facilities. None of the former officers interviewed had a disciplinary history, and only one of the current officers had ever been disciplined, due to an administrative infraction. Interviewees were promised confidentiality to encourage candor, but many declined to participate, citing fear of retaliation or discomfort with the process.

5.5.3 Interviews on Safety

Virtually all officers interviewed reported having been exposed to traumatic events on the job, including, but not limited to³⁹⁰:

- Assaults by incarcerated individuals, including stabbings, spittings, and other physical assaults.
- Being “thrown on,” which is when incarcerated individuals throw fluids onto staff, including feces, urine, and other bodily and non-bodily substances.
- Threats from incarcerated individuals, including the stalking of loved ones and verbal threats to the effect of, “I’m going to kill your family when I get out.”
- Witnessing incarcerated individuals engage in extreme acts of self-harm, such as hanging themselves, cutting open wounds, slitting wrists, ligatures, consuming their own feces, and inserting or ingesting foreign objects.

A correction officer who retired from Mid-State in 2020 stated, “I’ve been assaulted numerous times. I only pressed charges once. I had to go to the State Police because the facility wouldn’t do it.”³⁹¹ Another correction officer who retired from Mid-State in 2023 stated, “I was attacked as a CO. I had shit thrown in my face (at the RMHU at Marcy). I’ve been spit at in the face.”³⁹² Another correction officer who retired from Mid-State in 2024 described a time the officer was “doing rounds in the SHU and found a guy hanging himself. A rope was around his neck, attached to the gate. He wasn’t really hanging himself as much as he was sort of

³⁹⁰ [Cite Redacted].

³⁹¹ [Cite Redacted].

³⁹² [Cite Redacted].

asphyxiating himself. Some of his weight was on the ground. We were cutting the rope when he went off. He kicked me, bit my buddy. With the help of the sergeant, we got him down and there was a violent struggle until we got him restrained and cuffed.”³⁹³ The officer described that the incident resulted in severe injuries, including a concussion, vertebrae injuries, and a torn labrum.³⁹⁴

Current staff also described such incidents during our site visits, and we personally witnessed several of them.³⁹⁵ In one case, we observed a correction officer moments after being “thrown on.”³⁹⁶ In another, an incarcerated individual showed us a recently self-inflicted laceration on his wrist and we then observed him ingesting his own feces.³⁹⁷

Immediately following traumatic incidents, employees may need to continue their duties.³⁹⁸ A current correction officer opined that, even after witnessing or being involved in traumatic events, “No one comes to check in with [the correction officers]. They don’t follow up with anyone.”³⁹⁹ An incarcerated person with whom we spoke empathized with the officers facing these challenges, stating that just as his own past trauma led him to hurt others, the correction officers’ trauma can cause them to disregard incarcerated individuals’ needs, including access to programming and recreation.⁴⁰⁰

³⁹³ [Cite Redacted].

³⁹⁴ [Cite Redacted].

³⁹⁵ [Cite Redacted].

³⁹⁶ [Cite Redacted].

³⁹⁷ [Cite Redacted].

³⁹⁸ [Cite Redacted].

³⁹⁹ [Cite Redacted].

⁴⁰⁰ [Cite Redacted].

5.5.4 Interviews on Use of Force

No former officer interviewed admitted to ever having personally used excessive force on an incarcerated individual,⁴⁰¹ and when asked if they had ever witnessed another correction officer use excessive force, all but two former officers said “no.”⁴⁰² However, in discussing specific incidents, several indicated that they had, in fact, either personally engaged in or witnessed conduct that would constitute excessive force.⁴⁰³ The most frequently reported occurrence involved officers physically striking incarcerated individuals when they were “on the wall” for a pat frisk.⁴⁰⁴ One former officer stated, “When the inmate was on the wall, they’d punch him in the ribs or smack him in the head. Or they’d hold his head hard against the brick wall during the frisk. That hurts.”⁴⁰⁵ DOCCS investigators stated that complaints of blows to an incarcerated individual’s ribs “happen all the time.”⁴⁰⁶ This is consistent with the incarcerated interviewees at Marcy who frequently reported experiencing or witnessing incidents of excessive force involving blows to the ribs.⁴⁰⁷ Several former officers admitted that some of their colleagues “crossed the line,” but, when asked directly, they declined to share details or names.⁴⁰⁸

Nearly all former officers interviewed acknowledged that Marcy and Mid-State had “Goon Squads,” an informal group of correction officers who “took things too far, used excessive force.”⁴⁰⁹ All but one of the officers interviewed told us about such groups, also

⁴⁰¹ [Cite Redacted].

⁴⁰² [Cite Redacted].

⁴⁰³ [Cite Redacted].,

⁴⁰⁴ [Cite Redacted].

⁴⁰⁵ [Cite Redacted].

⁴⁰⁶ [Cite Redacted].

⁴⁰⁷ See Section 5.3, “Incarcerated Individuals’ Perspectives on Safety and Use of Force.”

⁴⁰⁸ [Cite Redacted].

⁴⁰⁹ [Cite Redacted].

referring to them as the “Beat Down Squad,” “Clint Eastwood Guys,” “Cowboys,” “Red Apple Crew,” and “Gold Boys.”⁴¹⁰ Some recently retired officers told us that they knew certain officers involved in the killings of Mr. Brooks and Mr. Nantwi were a part of these squads.⁴¹¹

According to the interviewees, knowledge of the “Goon Squads” at Marcy and Mid-State was common. Though impossible to verify, a handful of the interviewees claimed that “everybody,” including supervisors and facility leadership, knew about these squads.⁴¹² Some stated that these informal squads existed beyond Marcy and Mid-State (“There’s been a Goon squad at every prison I’ve been in”).⁴¹³ While some of the interviewed former officers wondered why “bad apples,” who they believed were known to prison leadership, were not stopped (“They know who the bad apples are. Why don’t they get rid of these guys? I don’t know.”), others suggested that they played the role of “enforcers” (“The Goon Squad is like a hockey team. You need your enforcers.”).⁴¹⁴

In March 2025, the Commissioner terminated approximately 2,000 officers in response to the illegal officers’ strike.⁴¹⁵ The Commissioner told us that these officers included many who did not share in the Department’s mission and policies, suggesting that problematic officers are now gone.⁴¹⁶ Echoing this, one facility executive suggested that there are no “bad apples” left at the facility, but in the same meeting also stated that “we’re realizing we have more bad apples than we thought.”⁴¹⁷ In the same meeting, all present facility executives said that there remained

⁴¹⁰ [Cite Redacted].

⁴¹¹ [Cite Redacted].

⁴¹² [Cite Redacted].

⁴¹³ [Cite Redacted].

⁴¹⁴ [Cite Redacted].

⁴¹⁵ Sarah Roebuck, *N.Y. Fires Over 2,000 Corrections Officers as Prison Strike Ends After 22 Days*, CORRECTIONS1 (Mar. 10, 2025), <https://www.corrections1.com/jail-management/n-y-corrections-officers-reach-agreement-with-state-to-end-prison-strike>.

⁴¹⁶ [Cite Redacted].

⁴¹⁷ [Cite Redacted].

problematic officers who had not been fired; one said that they sometimes report these officers, but nothing happens.⁴¹⁸ At another facility, an executive opined that ten percent of the remaining correction officers were “problem children” who “may need more convincing” to change their ways.⁴¹⁹ Similarly, numerous incarcerated individuals at Marcy and Mid-State expressed that, while some of the “bad apples” were gone, they believed that many others still remained.⁴²⁰

5.5.5 Interviews on Discipline

Current and former staff indicated that the facility disciplinary system provides officers with broad discretion to administer punishment. For example, one former officer from Marcy recalled, “You could give an inmate a ticket for damn near anything. [...] You can write a ticket for literally anything. Tickets are kind of a joke.”⁴²¹ Relatedly, we heard accounts of officers planting contraband to cover up misconduct or trigger a false disciplinary ticket, just as Mid-State officers allegedly did in the wake of killing Mr. Nantwi.⁴²²

Some former Marcy and Mid-State staff also acknowledged that some officers manipulate the system to punish incarcerated individuals. One former officer described “Fight Nights” at Marcy, during which officers would encourage incarcerated individuals in one unit to fight each other for their own entertainment; when the practice was discovered, two incarcerated individuals were disciplined and sent to the SHU.⁴²³

Other reported examples of manipulation of the disciplinary system included:

A correction officer who resigned from Mid-State in 2023 stated: “An inmate was messing with [an officer]. [The officer] told me, ‘I’m gonna get him today. I want him off my gallery. Watch and learn.’ So we go to the guy’s cell. The guy tells [the officer], ‘Fuck you. I ain’t doing nothin’, and there’s nothin’ you can do

⁴¹⁸ [Cite Redacted].

⁴¹⁹ [Cite Redacted].

⁴²⁰ [Cite Redacted].

⁴²¹ [Cite Redacted].

⁴²² See Section 5.4, “Reported Assaults on Staff.”

⁴²³ [Cite Redacted].

about it.’ The officer pulled his pin while looking at the inmate. You could hear guys [from the response team] coming. Just before they get there, the officer grabs the inmate and falls down with him so it looks like they’re struggling. The team comes in and cuffs the inmate and takes him away... That was an eye opener... It wasn’t right, but desperate measures for desperate times.”⁴²⁴

A correction officer who retired from Marcy in 2020 stated: “I’d say 50% of the [correction officers] could talk to [incarcerated individuals] and get across [...] legally, and 50% did other things that were not above board.” The officer described that if a correction officer doesn’t like an incarcerated individual who wants to go to recreation, “they ask him for his ID. Then they tell him he first needs to straighten up his cell. By the time the inmate returns, rec is over. Is it illegal? Yeah. You’re depriving them of a right. But will it kill them? No.”⁴²⁵

A correction officer who retired from Marcy in 2020 stated: “40% [of the COs] use and abuse the system to manipulate the inmates, to get them moved out of their unit, or to get them to the box ... If they didn’t want an inmate around, they’d make something up.”⁴²⁶

In addition, we saw firsthand during site visits how officers administer collective punishment for the actions of one or a few individuals in the unit by, for example, confiscating an entire dormitory’s amenities, like the microwave, stove, refrigerator, hot pot, toaster, or television.⁴²⁷ In one unit, we observed that standard cooking equipment was missing and that the freezer was warm, suggesting it was either broken or had just been turned on.⁴²⁸ When asked about these conditions, a security supervisor explained that sergeants can confiscate common area equipment—like televisions or cooking equipment—for up to two weeks without approval if there is a general disturbance or if they identify contraband but cannot tie it to a specific individual.⁴²⁹ In fact, the supervisor stated that the refrigerator and television had just been

⁴²⁴ [Cite Redacted].

⁴²⁵ [Cite Redacted].

⁴²⁶ [Cite Redacted].

⁴²⁷ [Cite Redacted].

⁴²⁸ [Cite Redacted].

⁴²⁹ [Cite Redacted].

returned to that unit.⁴³⁰ We later confirmed this collective discipline on the Watch Commander's "Deprivation Log," which tracks which dorms had been deprived, the duration, and a minimal explanation (e.g., "disturbance").⁴³¹

Facility leadership, staff, and incarcerated individuals at nearly all facilities we visited told us about the use of deprivations as collective punishment; however, there appeared to be little supervision or direction relating to when such punishment is appropriate, how long it can last, and who is authorized to institute it.⁴³² Indeed, one facility executive member told us that they viewed at least one recent deprivation as "unnecessary" and "causing too much disruption" and, therefore, directed that the punishment be lifted.⁴³³ Without further guidance or parameters, collective punishment is not conducive to the fair treatment of incarcerated individuals, ultimately affecting the safety of the responsible individuals, fueling resentment toward staff, and exacerbating a sense of arbitrariness and lack of faith in the fair administration of justice at the facility.

5.5.6 Interviews on Stressful Working Conditions and Mental Health

Both staff and leadership consistently described the stress of working at DOCCS facilities, and the majority acknowledged that their daily work has taken a toll on their mental health.⁴³⁴ One retired officer told us, "I would get a feeling in the pit of my stomach when I was on vacation. About two days before I was due at work, I'd get that feeling. I knew I'd be going back to hell."⁴³⁵ Union leadership reported that staff feel that their safety and well-being

⁴³⁰ [Cite Redacted].

⁴³¹ [Cite Redacted].

⁴³² [Cite Redacted].

⁴³³ [Cite Redacted].

⁴³⁴ [Cite Redacted].

⁴³⁵ [Cite Redacted].

concerns are ignored or met with retaliation.⁴³⁶ Many former correction officers voiced disappointment in how DOCCS has failed to provide mental health support,⁴³⁷ an issue that DOCCS and facility leadership acknowledged and said DOCCS is working to improve.⁴³⁸ As one DOCCS executive stated, “[It’s] absurd. No one is the same person after watching that behavior or having to conduct that type of life-saving measure. There’s no debrief. It’s terrible what we do to staff.”⁴³⁹ A correction officer who retired from Mid-State in 2020 told us, “I never understood the amount of stress I was under until after I retired. While I was working, I just dealt with it day to day. It took several months after retiring to finally realize how stressed out I was.”⁴⁴⁰ The former officer described still, to this day, waking up with nightmares of violent incidents in prison.⁴⁴¹ A correction officer who retired from Marcy in 2020 recalled receiving no offer for mental health services after witnessing an incarcerated individual slice another incarcerated individual from ear to mouth: “I saw that type of thing so often. No one asked if we needed to talk to someone.”⁴⁴²

Multiple former correction officers mentioned that correction officers turn to alcohol to cope. A correction officer who retired from Mid-State in 2023 stated, “There were a lot of guys who got help for alcohol. There were a lot of big drinkers. A lot of guys would buy a six pack for the car on the way home. That’s how they coped.”⁴⁴³ Similarly, a correction officer who retired from Mid-State in 2024 stated, “A lot of the guys – not me – drank. And they take it out on their

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⁴⁴¹ [Cite Redacted].

⁴⁴² [Cite Redacted].

⁴⁴³ [Cite Redacted].

kids and family. That’s why the divorce rate is so high. It’s stressful. I tried not to take the job home, but it’s hard.”⁴⁴⁴

A DOCCS correction officer and a parole officer died by suicide in 2023.⁴⁴⁵ In 2024, a correction officer and a Bureau Chief also died by suicide.⁴⁴⁶ On November 7, 2025, a staff prison chaplain fatally shot himself at Marcy after entering Marcy’s administration building.⁴⁴⁷

At the same time, DOCCS and union leadership reported staff’s extremely low utilization of available wellness resources, which they attributed to a stigma around mental health.⁴⁴⁸ About 33% of the 66 staff survey respondents from Marcy and 60% of the 36 staff survey respondents from Mid-State indicated they would describe the policies and practices to support staff well-being at those facilities as “Poor.” (see Figure 20).

Figure 20 Staff Survey Result

“How would you describe the policies and practices to support staff well-being at this facility?”						
Facility	Poor	Fair	Good	Very Good	Excellent	Total
Marcy	33.3%	28.6%	20.6%	12.7%	4.8%	100.0%
Mid-State	60.0%	14.3%	20.0%	5.7%	0.0%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility staff.

⁴⁴⁴ [Cite Redacted].

⁴⁴⁵ *Law Enforcement Suicides and Attempted Suicides in New York State Department of Corrections and Community Supervision*, p. 2, DOCCS (2023), <https://doccs.ny.gov/system/files/documents/2024/01/2023-law-enforcement-suicides-and-attempted-suicides-in-new-york-state-department-of-corrections-and-community-supervision.pdf>.

⁴⁴⁶ *Law Enforcement Suicides and Attempted Suicides in New York State Department of Corrections and Community Supervision*, p. 2, DOCCS (2024), <https://doccs.ny.gov/system/files/documents/2025/03/2024-law-enforcement-suicides-and-attempted-suicides-in-nys-doccs.pdf>.

⁴⁴⁷ Greta Stuckey, *NY Prison Chaplain from Syracuse Fatally Shoots Himself at Marcy Correctional Facility*, SYRACUSE.COM (Nov. 8, 2025), <https://www.syracuse.com/news/2025/11/ny-prison-chaplain-from-syracuse-fatally-shoots-himself-at-marcy-correctional-facility.html>.

⁴⁴⁸ [Cite Redacted].

During one site visit at Mid-State, we observed a staff wellness room that featured amenities like air conditioning, a lounge chair, mindfulness resources, and snacks.⁴⁴⁹ Although facility leadership told us that the wellness room was underutilized, we observed that the room was covered in a layer of dust and we learned that the room could only be unlocked by maintenance personnel.⁴⁵⁰ DOCCS also recently rolled out an app that offers mental health and wellness resources, including emergency hotlines, but staff, facility leadership, and DOCCS leadership indicated that officers do not use it because of its perception as “touchy feely” in a “hyper-masculin[e]” environment and because officers fear the app is not confidential.⁴⁵¹

As discussed later in this report, Marcy and Mid-State security staff’s concerns about their well-being—and their ability to cope with trauma—are exacerbated by understaffing and the consequent “unprecedented” levels of mandatory overtime; this overtime can amount to more than 24 consecutive hours and severely impact their life outside of work.⁴⁵² Many former correction officers interviewed described “getting stuck” working multiple shifts as the worst part of the job, and many resigned or retired early for that reason.⁴⁵³ One correction officer who retired from Mid-State in 2023 stated, “I got mandated (stuck) all the time... about four or five times a week. I’d do 16-hour shifts. I never had to work a 24-hour shift. I would have quit. But I hated getting stuck. I got stuck at Mid-State from the beginning of my time there to the end. I usually worked 80-hour weeks.”⁴⁵⁴ One current officer, now working at another facility, reported that, “[a]t Marcy, the staff shortages were so severe that correction officers had to stack 24-hour

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⁴⁵² [Cite Redacted].

⁴⁵³ [Cite Redacted].

⁴⁵⁴ [Cite Redacted].

shifts. They were being forced to come in to work on their regular days off.”⁴⁵⁵ Another officer who retired from Mid-State in 2023 stated, “I did ten shifts in a row once (80 hours).”⁴⁵⁶

Several former officers said that this mandatory overtime negatively affected their work-life balance and personal relationships, describing how they struggled to recover and cope after long shifts that left them in a “zombie” state.⁴⁵⁷ One former officer shared that, before retiring from Marcy, shifts could last up to 16 hours and made it impossible to get enough sleep.⁴⁵⁸ A current correction officer said that the job does not allow for time to have a personal life and described waiting for news about facility closures, to improve staffing, in order to know “when I’ll be home.”⁴⁵⁹ Both current and former officers pointed to marriages falling apart and missing time with their children and loved ones.⁴⁶⁰ One correction officer who retired from Mid-State in 2024 stated that mandatory overtime “was a big part of my marriage falling apart ... I was hardly around at home. And when I was there, I was exhausted. I was a zombie. We [the married couple] sort of fell out of touch.”⁴⁶¹ Some staff and incarcerated individuals alike expressed concern about officers’ ability to drive home safely after working more than 24 hours straight.⁴⁶² One former officer told us of a fellow officer who, after a long shift, rushed home because it was the night before his son’s first day of school; the officer died in a car accident that night.⁴⁶³ Another correction officer who retired from Mid-State in 2023 stated, “The mandatory [overtime] was terrible. You’d miss your kid’s ball game, Christmas Eve. It didn’t matter.

⁴⁵⁵ [Cite Redacted].

⁴⁵⁶ [Cite Redacted].

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⁴⁶¹ [Cite Redacted].

⁴⁶² [Cite Redacted].

⁴⁶³ [Cite Redacted].

They'd tell you, 'You ain't goin' home.' You can't make no plans."⁴⁶⁴ One staff interview respondent noted, "We need more time with our loved ones. People are beyond their breaking point. We are broken. We are going to see staff suicides if nothing is done."⁴⁶⁵ One former correction officer stated, "No matter how much people are paid, if they can't see their families, they're not gonna be interested in that job."⁴⁶⁶

At Marcy and Mid-State, leadership, staff, and incarcerated individuals described a vicious cycle: staff shortages lead to less programming, which leads to worse conditions for incarcerated individuals, which leads to misbehavior, which leads to unsafe and stressful working conditions, which leads to attrition, which in turn leads to fewer staff. This cycle ultimately results in reduced rehabilitative services for the incarcerated population and greater tensions within the prisons that can lead to the excessive use of force and, sometimes, fatal tragedies.⁴⁶⁷

5.6 Gangs Exacerbate Contraband and Safety Issues

Gangs exacerbate safety issues for both staff and incarcerated individuals at Marcy and Mid-State.⁴⁶⁸ Facility leadership and former officers told us that gangs prey on those with drug debts, those who are members of rival gangs, and other vulnerable individuals.⁴⁶⁹ One member of DOCCS leadership stated that, while much of the violence in prisons is gang-related, facility staff do not have the tools to protect individuals.⁴⁷⁰ DOCCS investigators reported that, in some instances, gang members will file false reports of misconduct to target officers who identify them

⁴⁶⁴ [Cite Redacted].

⁴⁶⁵ [Cite Redacted].

⁴⁶⁶ [Cite Redacted].

⁴⁶⁷ [Cite Redacted].

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⁴⁶⁹ [Cite Redacted].

⁴⁷⁰ [Cite Redacted].

and their contraband.⁴⁷¹ While DOCCS attempts to gather intelligence on gang activity proactively,⁴⁷² we heard about the ongoing prevalence of gangs and the resulting effect on perceived safety at DOCCS facilities.⁴⁷³ Indeed, during one of our site visits, we witnessed the immediate aftermath of a fight between incarcerated individuals who staff believed to be gang members; we witnessed the challenges posed to investigation, including the victim's unwillingness to provide names.⁴⁷⁴

5.7 Mutual Mistrust and the Existence of an “Us versus Them” Mentality

Survey results indicated that both incarcerated individuals and staff characterized their interactions with each other as predominantly negative.

At Mid-State, 67.7% of the 440 surveyed incarcerated individuals rated the relationship between the incarcerated population and security staff as “Bad” or “Very Bad,” while 62.7% of the 361 surveyed incarcerated individuals at Marcy reported the same perception.⁴⁷⁵ (see Figure 21). With respect to non-security staff (e.g., medical personnel, counselors, programming instructors), 35.5% of incarcerated individual respondents at Marcy and 33.7% of incarcerated individual respondents at Mid-State reported that their relationship was “Bad” or “Very Bad.”⁴⁷⁶ (see Figure 22).

⁴⁷¹ [Cite Redacted].

⁴⁷² [Cite Redacted].

⁴⁷³ [Cite Redacted].

⁴⁷⁴ [Cite Redacted].

⁴⁷⁵ [Cite Redacted].

⁴⁷⁶ [Cite Redacted].

Figure 21 Incarcerated Individual Survey Result

“How would you describe the current relationships between the incarcerated population and security staff here?”						
Facility	Very Bad	Bad	Neutral	Good	Very Good	Total
Marcy	30.9%	31.8%	31.2%	4.0%	2.1%	100.0%
Mid-State	40.2%	27.5%	24.6%	4.4%	3.4%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

Figure 22 Incarcerated Individual Survey Result

“How would you describe the current relationships between the incarcerated population and non-security staff here?”						
Facility	Very Bad	Bad	Neutral	Good	Very Good	Total
Marcy	18.2%	17.3%	43.2%	13.9%	7.4%	100.0%
Mid-State	16.2%	17.5%	45.3%	11.5%	9.4%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

On the other hand, 60% of the 36 surveyed staff at Mid-State (which included both security and non-security staff) rated their relationships with the incarcerated population as “Bad” or “Very Bad,” and 60.6% of the 66 surveyed staff at Marcy reported the same perception.⁴⁷⁷ (see Figure 23).

Figure 23 Staff Survey Result

“How would you describe the current relationships between the incarcerated population and security staff here?”						
Facility	Very Bad	Bad	Neutral	Good	Very Good	Total
Marcy	29.5%	31.3%	26.2%	8.2%	4.9%	100.0%
Mid-State	31.4%	28.6%	28.6%	11.4%	0.0%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility staff.

Current and former staff (at both the facility level and DOCCS level), as well as currently and formerly incarcerated individuals, described an “us versus them” or “blue versus green” mentality at Marcy and Mid-State (referencing the color of outfits worn by security staff versus incarcerated individuals).⁴⁷⁸ One former officer told us, “A lot of correction officers [...] felt that these guys [the incarcerated individuals] deserved more punishment.”⁴⁷⁹ We heard from one former officer that the term “inmate lover” has been used to disparagingly describe an officer who is too kind to incarcerated individuals: “If a correction officer said good morning to an incarcerated individual in max [a maximum security unit], they’d call the correction officer an ‘inmate lover.’”⁴⁸⁰ We observed a staff area of the Mid-State SDP with offensive drawings and profanity marked on plexiglass, including a drawing stating that an “[officer] [heart emoji, loves] Cons.”⁴⁸¹ DOCCS executives, former staff, and current and former incarcerated individuals also stated that officers who took acts that would help an incarcerated individual—including steps to help with an investigation into officer misconduct—risk intimidation, retaliation, ostracization, and abuse by their peers.⁴⁸²

We also reviewed BWC footage of one incident, in particular, that raised concerns about the “us versus them” mentality, especially in the context of power dynamics between officers and the incarcerated population. The footage, dated from June 6, 2025 (i.e., several months into the review), captures a dorm officer speaking on the facility phone with an unidentified individual.⁴⁸³ During the call, the dorm officer talks about incarcerated individuals who do not

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⁴⁷⁸ [Cite Redacted].

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⁴⁸² [Cite Redacted].

⁴⁸³ [Cite Redacted].

listen and states that the officer “wishes” the porters (i.e., those who are given more freedom in the facility based on good behavior) “would really push up on” and “make miserable” those who do not listen.⁴⁸⁴ The dorm officer also can be heard saying, “Now I don’t know if it’s because of these cameras or what, or if it’s because of everything that happened, but they [incarcerated individuals] just feel that much more entitled.”⁴⁸⁵

Nearly 67% of the 36 surveyed staff at Mid-State and 84% of the 66 surveyed staff at Marcy reported believing that incarcerated individuals receive fair and consistent treatment in the facilities.⁴⁸⁶ (see Figure 24). Almost all former correction officers interviewed claimed they treated incarcerated individuals with respect (“You give respect, you get respect”)⁴⁸⁷—a sentiment that we observed in real time among some current officers.⁴⁸⁸ Facility leaders acknowledged that security staff condition their dynamic with the incarcerated population on receiving their respect.⁴⁸⁹ Yet, former and current correction officers interviewed also reported that many officers did not treat incarcerated individuals respectfully, citing officers who “felt that these guys deserved more punishment,” treated incarcerated individuals as “less than humans,” acted as “bullies,” “had a lot of testosterone,” “abused their authority,” and “liked to fight.”⁴⁹⁰

⁴⁸⁴ [Cite Redacted].

⁴⁸⁵ [Cite Redacted].

⁴⁸⁶ [Cite Redacted].

⁴⁸⁷ [Cite Redacted].

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⁴⁹⁰ [Cite Redacted].

Figure 24 Staff Survey Result

“Incarcerated individuals receive fair and consistent treatment in this facility.”					
Facility	Strongly Disagree	Disagree	Agree	Strongly Agree	Total
Marcy	5.4%	10.7%	55.4%	28.6%	100.0%
Mid-State	12.1%	21.2%	48.5%	18.2%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility staff.

For staff, 71.4% of the 36 respondents at Mid-State and 55% of the 66 respondents at Marcy reported that they “Disagree” or “Strongly Disagree” that there is generally mutual respect between staff and the incarcerated population.⁴⁹¹ (see Figure 25).

Figure 25 Staff Survey Result

“I feel there is generally mutual respect between staff and incarcerated individuals in this facility.”					
Facility	Strongly Disagree	Disagree	Agree	Strongly Agree	Total
Marcy	26.7%	28.3%	38.3%	6.7%	100.0%
Mid-State	31.4%	40.0%	28.6%	0.0%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility staff.

For incarcerated individuals, 60.8% of the 440 respondents at Mid-State and 55% of the 361 respondents at Marcy reported that they “Disagree” or “Strongly Disagree” that they are generally treated with respect.⁴⁹² (see Figure 26). An overwhelming majority of incarcerated individual respondents at Marcy (73.5%) and at Mid-State (77.4%) reported that they “Disagree” or “Strongly Disagree” that security staff are generally fair to the incarcerated populations.⁴⁹³

⁴⁹¹ [Cite Redacted].

⁴⁹² [Cite Redacted].

⁴⁹³ [Cite Redacted].

(see Figure 27). Some incarcerated individuals stated that security staff generally view them “like they’re not human.”⁴⁹⁴ However, 54.4% of Marcy incarcerated individual respondents and 55.8% of Mid-State incarcerated individual respondents reported that they “Agree” or “Strongly Agree” that non-security staff are generally fair to the incarcerated populations.⁴⁹⁵ (see Figure 28).

Figure 26 Incarcerated Individual Survey Result

“I feel like I am generally treated with respect.”					
Facility	Strongly Disagree	Disagree	Agree	Strongly Agree	Total
Marcy	19.3%	35.7%	41.3%	3.7%	100.0%
Mid-State	26.5%	34.3%	34.8%	4.4%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

Figure 27 Incarcerated Individual Survey Result

“Security staff are generally fair to people incarcerated here.”					
Facility	Strongly Disagree	Disagree	Agree	Strongly Agree	Total
Marcy	33.8%	39.7%	24.6%	1.9%	100.0%
Mid-State	40.7%	36.7%	19.9%	2.6%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

⁴⁹⁴ [Cite Redacted].

⁴⁹⁵ [Cite Redacted].

Figure 28 Incarcerated Individual Survey Result

“Non-Security staff are generally fair to people incarcerated here.”					
Facility	Strongly Disagree	Disagree	Agree	Strongly Agree	Total
Marcy	16.1%	29.5%	48.8%	5.6%	100.0%
Mid-State	17.1%	27.0%	48.8%	7.0%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

5.8 Cultural Issues Underlying Lack of Accountability

Former and current staff described how the close-knit community around Marcy and Mid-State fosters a “Thin Blue Line” and “No Snitching” culture that can compromise staff’s willingness to intervene, report, and discipline each other for misconduct, including in excessive force cases.⁴⁹⁶ Difficult working conditions, in part, foster this tight-knit community: staff interviewees described camaraderie and “brotherhood” among staff, noting that “[a]ll staff seems to look out for each other” because “we all just want to make it home after our shifts without being exposed to drugs or assaulted.”⁴⁹⁷ The locations of the facilities also contribute to this dynamic. Marcy and Mid-State are in a single small community, and, historically, it was common for multiple members or generations of a family to work in the same facility.⁴⁹⁸ Many facility staff members are family, neighbors, friends, or otherwise loved ones.⁴⁹⁹ Former correction officers interviewed stated that officers typically stay in the communities in which they grew up and thus have pre-existing relationships with each other.⁵⁰⁰ We heard about long-

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⁴⁹⁷ [Cite Redacted].

⁴⁹⁸ [Cite Redacted].

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⁵⁰⁰ [Cite Redacted].

lasting friendships forged among facility staff and about their kids attending school together.⁵⁰¹ We learned of staff fundraisers when coworkers faced personal and professional strife.⁵⁰² In particular, we learned about a 2024 fundraiser at Mid-State for four officers who had been fined in connection with an excessive use of force; one of those officers would later be indicted in the killing of Mr. Nantwi.⁵⁰³

These dynamics can undermine fairness and accountability. Only one former correction officer interviewed said that, had they been in the room during the fatal beatings of Mr. Brooks and Mr. Nantwi, they would have intervened.⁵⁰⁴ All others interviewed referenced cultural norms against intervening to stop fellow officers.⁵⁰⁵ “You don’t tell another CO what to do. There’s a code.”⁵⁰⁶ “Never turn in another officer. They protect you.”⁵⁰⁷ Others made similar comments:

- A current staff member at Mid-State stated, “People cover up a lot and they are all related or friends,” which makes speaking up risky.⁵⁰⁸
- Another current staff member at Mid-State stated that, if reporting use of force, a correction officer should do so “externally, for fear of retaliation directly and indirectly.”⁵⁰⁹
- A current staff member at Marcy stated, “I will not last long if I snitch.”⁵¹⁰
- A former Marcy and Mid-State correction officer who retired in 2023 stated, “There’s a blue line and the green line. You never cross it. You never cross it. Never take an inmate’s word over an officer’s word. If an officer says this inmate hit him, and it turned out he didn’t—if he was lying to you—we’d deal with it after work [by fighting].”⁵¹¹

⁵⁰¹ [Cite Redacted].

⁵⁰² [Cite Redacted].

⁵⁰³ [Cite Redacted].

⁵⁰⁴ [Cite Redacted].

⁵⁰⁵ [Cite Redacted].

⁵⁰⁶ [Cite Redacted].

⁵⁰⁷ [Cite Redacted].

⁵⁰⁸ [Cite Redacted].

⁵⁰⁹ [Cite Redacted].

⁵¹⁰ [Cite Redacted].

⁵¹¹ [Cite Redacted].

- A correction officer who retired from Marcy in 2022 stated, “As an officer, I can’t say ‘stop.’ [The involved officers] would slash my tires ... and they wouldn’t have stopped anyway. They would have beat me up... snitches get stiches.”⁵¹²

It is important to note that the “Thin Blue Line” culture is not unique to DOCCS.

However, when “left unchecked and unchallenged an established code of silence results in an increase in violence and in the dangerousness of our prisons.”⁵¹³

5.9 Perceptions of Racial Bias at Marcy and Mid-State

A significant majority of surveyed incarcerated individuals reported experiencing or witnessing differential treatment based on race or ethnicity. At Marcy, 59.1% of the 361 respondents reported that they have been treated differently based on their race or ethnicity, and 75% reported witnessing others being treated differently based on their race or ethnicity.⁵¹⁴ (see Figures 29 and 30). At Mid-State, 59.6% of the 440 respondents reported that they have been treated differently based on their race or ethnicity, and 78.5% reported witnessing others being treated differently based on their race or ethnicity.⁵¹⁵ (see Figures 29 and 30).

⁵¹² [Cite Redacted].

⁵¹³ Kathleen Dennehy and Kelly Nantel, *Improving Prison Safety: Breaking the Code of Silence*, p. 177, WASHINGTON UNIVERSITY JOURNAL OF LAW AND POLICY (Jan. 2006), https://openscholarship.wustl.edu/cgi/viewcontent.cgi?article=1352&context=law_journal_law_policy&utm_source.

⁵¹⁴ [Cite Redacted].

⁵¹⁵ [Cite Redacted].

Figure 29 Incarcerated Individual Survey Result

“In this facility, do you believe you have ever been treated differently based on race or ethnicity?”			
Facility	Yes	No	Total
Marcy	40.9%	59.1%	100.0%
Mid-State	40.4%	59.6%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

Figure 30 Incarcerated Individual Survey Result

“In this facility, have you observed others being treated differently based on race or ethnicity?”			
Facility	Yes	No	Total
Marcy	25.0%	75.0%	100.0%
Mid-State	21.5%	78.5%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

In 2024, CANY published reports on Marcy and Mid-State that touched on these same issues. In its report on Marcy, CANY highlighted “pervasive allegations of racial discrimination.”⁵¹⁶ Around 67% of the 55 incarcerated individuals CANY surveyed said they had seen or experienced racialized abuse by staff at Marcy.⁵¹⁷ By contrast, in its report on Mid-State, CANY described that 22% of the 36 incarcerated individuals surveyed had seen or experienced racialized abuse by staff at the facility.⁵¹⁸

⁵¹⁶ Press Release, CANY, *CANY Releases Report on Marcy Correctional Facility*, Monitoring Visit to Marcy Correctional Facility (Oct. 2022), <https://www.correctionalassociation.org/press-releases-archive/2023-marcy-pvb-release>.

⁵¹⁷ Monitoring Visit to Marcy Correctional Facility, Post-Visit Briefing and Recommendations (Oct. 2022), https://static1.squarespace.com/static/62f1552c1dd65741c53bbcf8/t/64ab93bfab9c7e045ea53c01/1688966083012/2023_PVB-10-Marcy.pdf.

⁵¹⁸ Monitoring Visit to Mid-State Correctional Facility, Post-Visit Briefing and Recommendations (Oct. 2022), https://static1.squarespace.com/static/62f1552c1dd65741c53bbcf8/t/649b0eb221eb640a14d63056/1687883445206/2023_PVB-11-MidState.pdf.

Interviews with incarcerated individuals also revealed concerns about persistent racial bias and discrimination. For example, one interviewee from Marcy stated: “I’m Black so I already got a strike right there.”⁵¹⁹ Another individual from Marcy reported: “It’s mostly a green and blue issue, but we see a lot of racists too. The officers will talk with most white guys, but the attitude changes quickly if it’s a Black guy—tell them to get the fuck away.”⁵²⁰ A Mid-State respondent simply stated, “race plays a part in everything here.”⁵²¹

By contrast, the majority view among current and former officers was that DOCCS does not have race or bias issues. One former Mid-State officer summed up the issue: “[Race was] not an issue. It was blue and green, blue and green.”⁵²²

However, in interviews, some former officers admitted to hearing other officers using racial slurs, especially when referring to Black incarcerated individuals and, at least occasionally, when interacting with Black incarcerated individuals.⁵²³ For example, one former Marcy officer stated that there were some correction officers that used the “N” word directly when addressing incarcerated individuals, but the most prevalent use occurred when the correction officers were talking among themselves.⁵²⁴ From the officer’s perspective, “most officers [only use the “N” word] if they have back-up.”⁵²⁵ Another former Mid-State officer explained: “I heard the ‘N’ word used by COs, within my earshot, never directly. I knew which ones were racist. They would refer to the Black inmates as pieces of shit. They didn’t like Blacks. They had hate.”⁵²⁶

⁵¹⁹ [Cite Redacted].

⁵²⁰ [Cite Redacted].

⁵²¹ [Cite Redacted].

⁵²² [Cite Redacted].

⁵²³ [Cite Redacted].

⁵²⁴ [Cite Redacted].

⁵²⁵ [Cite Redacted].

⁵²⁶ [Cite Redacted].

5.10 Summary

During a period of declining incarcerated populations and staffing levels, our review identified pervasive safety concerns on the part of both incarcerated individuals and all levels of staff. Staff are afraid because they are too frequently subject to traumatic events on the job, work too many hours, and report not feeling supported in their important public service roles. Incarcerated individuals fear for their safety in light of increasing rates of reported and witnessed force, perceived arbitrary enforcement of rules and a lack of accountability for staff misconduct, and staff exhibitions of racial bias. Although recent reforms in response to the killings of Mr. Brooks and Mr. Nantwi have led to tangible improvements, a deeply entrenched “blue versus green” divide persists that undermines the rehabilitation of incarcerated individuals, accountability measures for officers, and ultimately the safety and well-being of everyone. As detailed in this report, systematic efforts must be made to overcome this schism and alleviate the daily tensions that can lead to tragedy.

The following sections of this report build upon these findings, providing a detailed topic-by-topic analysis with accompanying recommendations. The sections proceed as follows: staff training, Office of Special Investigations, staff discipline, Incarcerated Grievance Program, risk detection and mitigation, weapons, staffing, contraband, cameras, Incarcerated Liaison Committee, programming and recreation, classification, mental health care, and medical care. The report ends with a summary of our recommendations.

6. STAFF TRAINING

Our review revealed that DOCCS should work to update and improve its staff training, particularly with respect to use of force, de-escalation, and intervention. Several current and former officers interviewed could not accurately describe DOCCS' use of force policy, which is laid out in writing at DOCCS Directive #4944. Both current and former correction officers (including a DOCCS-designated training officer) revealed a lack of familiarity with DOCCS' basic use of force rules and techniques. Several examples are provided below:

- When Use of Force Is Justified: Under Directive #4944, use of force is authorized in six situations.⁵²⁷ None of the correction officers interviewed—including former CERT members and a weapons training officer—could list all six.⁵²⁸
- Duty to Intervene: DOCCS Directive #4944 imposes a duty to intervene for any staff member who observes another staff member using excessive force if there is a realistic opportunity to prevent harm, and to always report such behavior to a supervisor.⁵²⁹ Only one former correction officer interviewed could accurately paraphrase the most important elements of the rule.⁵³⁰ A Mid-State correction officer who resigned in 2024 said “I know that’s a thing—the duty to intervene if you see misconduct. I never had to do it.”⁵³¹ Similarly, a Mid-State correction officer who retired in 2025 recalled “I know you’re supposed to do something, but I don’t know what. You’re supposed to say ... I can’t remember what.”⁵³²

In addition, many officers interviewed claimed never to have used excessive force, while at the same time admitting to conduct disallowed by current DOCCS policy. Perhaps no policy or training could have prevented the killings of Mr. Brooks and Mr. Nantwi, but the need for more and better training is manifest.

Though DOCCS' policies largely align with national standards, its training model can be improved. Today, DOCCS relies on an almost entirely paramilitary approach to new-recruit

⁵²⁷ [Cite Redacted].

⁵²⁸ [Cite Redacted].

⁵²⁹ [Cite Redacted].

⁵³⁰ See [Cite Redacted].

⁵³¹ [Cite Redacted].

⁵³² [Cite Redacted].

training. DOCCS would benefit by increasing new staff training’s focus on a formal use of force continuum, de-escalation, and scenario-based learning. Further, there is limited refresher training for veteran officers. DOCCS should, therefore, invest in a modernized, interactive, and ongoing training framework—including by hiring a dedicated training curriculum consultant.

6.1 Timeline of Officer Training

Staff training occurs in three main stages: the Academy; OJT for new correction officers; and annual in-service training.⁵³³ The Academy provides eight weeks of pre-service training, consisting of classroom and hands-on learning.⁵³⁴ After the Academy, graduates transfer to a correctional facility, where they undergo three-week OJT and complete the remainder of their one-year probationary period.⁵³⁵ OJT entails shadowing a more experienced officer across various facility posts.⁵³⁶ After the probationary period is completed, the trainee is considered to be a correction officer and is thereafter responsible for completing annual in-service training.⁵³⁷

As we will discuss herein, DOCCS should review and update staff training across the board: from the Academy, to in-service training, to OJT. To assist in this effort, DOCCS should hire formal curriculum specialists to perform a comprehensive review of the agency’s training requirements and offerings.

6.2 DOCCS Should Update Academy Training Culture

During our visit to the Academy, we observed a class on use of force policies and a live hands-on defensive tactics training.⁵³⁸ The classroom training was primarily lecture-based,

⁵³³ [Cite Redacted].

⁵³⁴ *Correction Officer Trainee, About the Job*, DOCCS, <https://doccs.ny.gov/employment/correction-officer-trainee>.

⁵³⁵ *Correction Officer Trainee, About the Job*, DOCCS, <https://doccs.ny.gov/employment/correction-officer-trainee>; [Cite Redacted].

⁵³⁶ [Cite Redacted].

⁵³⁷ See Directive #4944 (Apr. 28, 2023).

⁵³⁸ [Cite Redacted].

although the instructor did answer some trainee questions.⁵³⁹ During the lecture, the class showed mixed engagement levels; most of them actively took notes, but a small number appeared to doze off.⁵⁴⁰ Tactics training began immediately after the lecture and covered physical applications of defensive and handcuff techniques.⁵⁴¹ The hands-on instructors were professional, spent quality time with recruits to teach the correct techniques, and spent extra time with those who needed it.⁵⁴²

DOCCS adopted the current pre-service training curriculum from the District of Columbia Department of Corrections approximately three years ago.⁵⁴³ The structure and content of the curriculum are largely consistent with those of others nationally.⁵⁴⁴ DOCCS last re-accredited the Academy in June 2024 and auditors report 100% compliance with ACA standards.⁵⁴⁵

The Academy curriculum is not flexible. In previous years, there were hours in the training schedule for optional Commissioner initiatives to train recruits on emerging trends.⁵⁴⁶ However, the curriculum's mandated material has expanded, eliminating the time for such creative additions to the curriculum.⁵⁴⁷

As with the majority of U.S. correctional academies, the Academy uses a paramilitary approach in the environment and curriculum.⁵⁴⁸ Paramilitary training is intended to help recruits

⁵³⁹ [Cite Redacted].

⁵⁴⁰ [Cite Redacted].

⁵⁴¹ [Cite Redacted]; Practical use of force sessions require a 1:10 student teacher ratio. Instructors are correction officers who have received a specific training certification.

⁵⁴² [Cite Redacted].

⁵⁴³ [Cite Redacted].

⁵⁴⁴ [Cite Redacted].

⁵⁴⁵ [Cite Redacted].

⁵⁴⁶ [Cite Redacted].

⁵⁴⁷ [Cite Redacted].

⁵⁴⁸ [Cite Redacted].

react consistently and appropriately under stress.⁵⁴⁹ However, a paramilitary environment can promote aggression and an “us versus them” mentality, which can undermine the effective teaching of non-violent de-escalation strategies.⁵⁵⁰ These include, for instance, using communication and tactical patience, slowing things down, building rapport, and creating time and space for peaceful resolution.⁵⁵¹

A paramilitary training approach may also result in higher dropout rates because recruits are sometimes unable to manage the intensity; this may undercut DOCCS’ efforts to recruit and retain correction officers in the face of staffing shortages.⁵⁵² DOCCS’ paramilitary approach could be enhanced with more interactive and scenario-based training, especially for training on use of force and de-escalation. Scenario-based training is an instructional method that immerses officers in realistic, high-fidelity scenarios designed to replicate operational conditions. When implemented correctly, scenario-based training improves judgment under pressure, enhances skill transfer to real-world encounters, and supports critical thinking and problem-solving in authentic contexts, making it more effective than traditional training models.⁵⁵³ During the review, we learned that DOCCS is exploring implementing statewide virtual-reality defensive

⁵⁴⁹ Eliav Lieblich & Adam Shinar, *The Case Against Police Militarization*, 23 MICH. J. RACE & L. 105 (2018), <https://repository.law.umich.edu/mjrl/vol23/iss1/4>.

⁵⁵⁰ Joseph B. Doherty, *Us v. Them: The Militarization of American Law Enforcement and the Psychological Effect on Police Officers & Civilians*, 25 S. CAL. INTERDISC. L. J. 415 (2016), <https://gould.usc.edu/why/students/orgs/ilj/assets/docs/25-2-Doherty.pdf>.

⁵⁵¹ See, e.g., National Policing Institute, De-Escalation FAQs, <https://www.policinginstitute.org/de-escalation/#:~:text=De%20escalation%20involves%20%22taking%20action,reduction%20in%20the%20force%20necessary.%22>. See also Rusty Ringler, *The 8 most effective De-escalation Techniques in Corrections*, CORRECTIONS1 (July 31, 2024), <https://www.corrections1.com/corrections-training/articles/the-8-most-effective-de-escalation-techniques-in-corrections-jNORMERiIsox1LuE/>.

⁵⁵² Samantha Max, *Rethinking The Police Academy: Why are many departments still using military-style training when research suggests it’s not the best option?*, 90.3 WPLN NEWS (Feb. 10, 2022), <https://wpln.org/post/rethinking-the-police-academy-why-are-many-departments-still-using-military-style-training-when-research-suggests-its-not-the-best-option/>.

⁵⁵³ Christopher Cushion, *Changing Police Personal Safety Training Using Scenario-Based-Training: A Critical Analysis of the ‘Dilemmas of Practice’ Impacting Change*, 6 Frontiers in Educ. (Jan. 7, 2022).

tactics training, which could be an effective way to provide more immersive learning without the injury risks of in-person training.⁵⁵⁴ However, the review also identified a likely need for expanded scenario-based training to help officers learn about best practices in, for example, safe and compliant body holds and carries and identifying and addressing mental health crises.⁵⁵⁵

In addition, the Academy's curriculum does not specifically use a formal use of force continuum, which provides a framework to guide the appropriate use of force during evolving situations. Nor does Directive #4944 provide DOCCS' written policy on a use of force continuum.⁵⁵⁶ The primary goal of a continuum is to ensure that force remains proportional, justified, and consistent with legal standards. For example, the use of force continuum used by the U.S. Bureau of Prisons discusses the difference between a reaction to passive resistance compared to scenarios where more elevated uses of force would be permissible.⁵⁵⁷ An example continuum from the Massachusetts correctional system is pictured below in Figure 31:

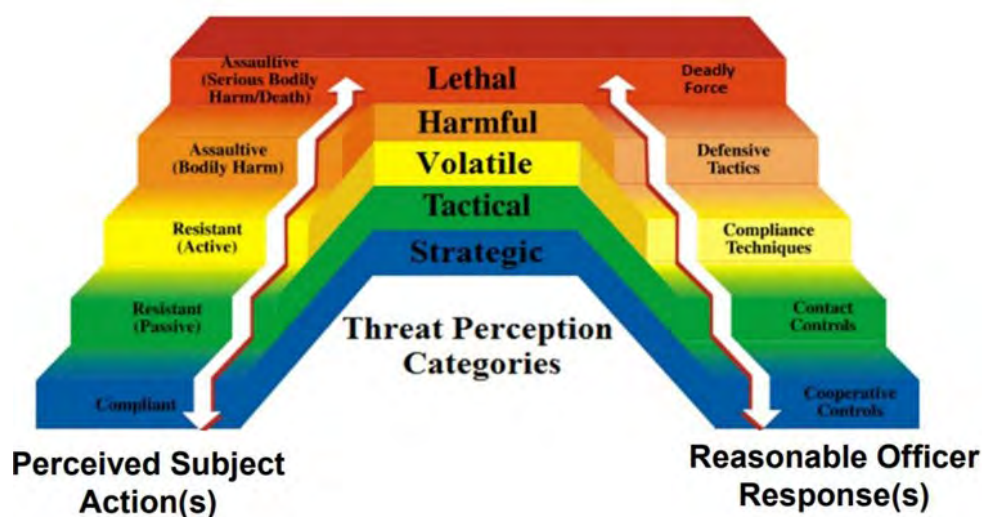
⁵⁵⁴ [Cite Redacted].

⁵⁵⁵ [Cite Redacted].

⁵⁵⁶ Directive #4944 (Apr. 28, 2023).

⁵⁵⁷ DOJ, Federal Bureau of Prisons, *Use of Force, Application of Restraints, and Firearms* (Program Statement P5566.07, July 17, 2024), Attachment C, <https://www.bop.gov/policy/progstat/5566.07.pdf>; see also Standard Operating Procedure to 103 CMR 505 Use of Force, pp.4-6, Mass. Dep't of Corr. (Oct. 28, 2025), <https://www.mass.gov/doc/505-sop-mptc-use-of-force-model-totality-triangle-and-defensive-tactics/download>.

Figure 31 Use of Force Continuum⁵⁵⁸



DOCCS should also rely more on modern adult learning approaches, especially in the classroom. “Adult learning,” which has been endorsed by the National Institute of Corrections, is an educational concept designed to engage motivation, promote collaboration, emphasize self-directed thinking by drawing on the learner’s prior experience, and focus on immediately relevant and real-world challenges.⁵⁵⁹ Adult learning activities can include table-top exercises, discussion and dialogue, case studies and scenarios, and collaborative learning that involves group exercises to leverage collective experience and work through complex, real-life

⁵⁵⁸ Standard Operating Procedure to 103 CMR 505 Use of Force, p. 4, Mass. Dep’t of Corr. (Oct. 28, 2025), <https://www.mass.gov/doc/505-sop-mptc-use-of-force-model-totality-triangle-and-defensive-tactics/download>.

⁵⁵⁹ Correctional Learning and Performance: A Vision for the 21st Century, White paper, NIC Accession No. 026506, DOJ, NATIONAL INSTITUTE OF CORRECTIONS (Dec. 2012), <https://s3.amazonaws.com/static.nicic.gov/koha/026506.pdf>; Jannette Collins, *Education Techniques for Lifelong Learning: Principles of Adult Learning*, 24 RADIOGRAPHICS 1483 (Sept. 2004), <https://doi.org/10.1148/rg.245045020>; Reed, Suzanne et al., *Applying Adult Learning Practices in Medical Education*, 44 CURRENT PROBLEMS IN PEDIATRIC AND ADOLESCENT HEALTH CARE 170, <https://doi.org/10.1016/j.cppeds.2014.01.008>; Palis, Ana G., & Peter A. Quiros, *Adult Learning Principles and Presentation Pearls*, 21 MIDDLE EAST AFRICAN JOURNAL OF OPHTHALMOLOGY 114, <https://doi.org/10.4103/0974-9233.129748>.

challenges.⁵⁶⁰ For example, the Connecticut Department of Corrections partners with an organization that hosts panel and discussion sessions for new recruits, where they hear from formerly incarcerated individuals who have successfully rehabilitated.⁵⁶¹ These sessions allow recruits to see what success looks like and hear firsthand how staff affect an incarcerated individual's journey.⁵⁶²

To assist in this effort, DOCCS should hire formal curriculum specialists to perform a comprehensive review of the Academy and in-service curriculum.

6.3 DOCCS Should Improve On-the-Job Training

Some former officers reported that OJT quality can vary widely and depends on the assigned training officer.⁵⁶³ A Marcy correction officer who resigned in 2024 stated, "It depends on who you get matched up with ... a couple of COs took it seriously. One guy made me run the housing unit on first day. He taught me everything, all day. He showed me how to frisk, how to do count. He made me keep the log book. That was one of the few quality training experiences I had while working for DOCCS."⁵⁶⁴ A Mid-State correction officer who retired in 2024 stated, "It is only as good as the guy you're shadowing is the problem."⁵⁶⁵ Another Mid-State correction officer who retired in 2024 recalled, "I had my OJT with a guy who had three weeks more [time]

⁵⁶⁰ Farrell, Victoria L., *The Effectiveness of Training for Correction Officers in the Performance of their Job*, UNIV. ALBANY, SUNY (Dec. 2015); DOJ, National Institute of Corrections, *Correctional Learning and Performance: A Vision for the 21st Century* (White paper, NIC Accession No. 026506, Dec. 2012), <https://s3.amazonaws.com/static.nicic.gov/koha/026506.pdf>.

⁵⁶¹ [Cite Redacted].

⁵⁶² [Cite Redacted].

⁵⁶³ [Cite Redacted].

⁵⁶⁴ [Cite Redacted].

⁵⁶⁵ [Cite Redacted].

than me.”⁵⁶⁶ In addition, new officers may be trained only on a limited number of posts, leaving them unequipped for every aspect of their role as correction officers.⁵⁶⁷

In light of this, DOCCS should implement a standardized curriculum for all trainees as part of OJT, ensuring that they receive training on all potential posts. In addition, DOCCS should establish standards for training officers so that trainees learn from the most experienced officers in the facility. Finally, to ensure comprehension and improve retention during this critical phase of learning, DOCCS should also test officers at the conclusion of OJT.

6.4 DOCCS Should Improve In-Service Training

Directive #2406 requires facility-based training on the third Wednesday of every month. Officers receive 40 hours of in-service training each year.⁵⁶⁸

However, the current training has gaps. In-service training can include some scenario-based training, although the effectiveness of this kind of training depends on the trainer.⁵⁶⁹ Correction officers also reported in interviews that the in-service training offered does not necessarily require officers to take it seriously—as a former Mid-State officer said: “We like going to training because we can do what we want ... but we’re not refreshed on anything.”⁵⁷⁰ For example, a Mid-State correction officer who retired in 2023 said, “They show the same videos every year. Guys don’t pay attention to it. Some of it is useless, like baton training. No one carries a baton in medium. It’s a waste.”⁵⁷¹ Another Mid-State correction officer who retired

⁵⁶⁶ [Cite Redacted].

⁵⁶⁷ [Cite Redacted].

⁵⁶⁸ [Cite Redacted].

⁵⁶⁹ [Cite Redacted].

⁵⁷⁰ [Cite Redacted].

⁵⁷¹ [Cite Redacted].

in 2023 stated, “We didn’t get good training ... They’ve been showing the same videos since the time I started. They never updated it.”⁵⁷²

6.4.1 Prioritize Key Issues and Interactive Elements

Except for those who undergo CERT training, most DOCCS correction officers do not receive hands-on defensive tactics training after graduating from the Academy, as the required annual trainings are lecture-based.⁵⁷³ This can create inconsistencies among staff. For example, around 2017, DOCCS shifted the guiding principles of its hands-on tactics from an aikido-type system to more of a jiu jitsu system; but the lack of mandatory hands-on training after graduation means that only those officers who attended the Academy after 2017 have had complete, hands-on training with these new techniques.⁵⁷⁴ The gap in training is especially stark where previously-taught techniques—like “softening blows” to an incarcerated individual’s groin or ribs—are no longer allowed.⁵⁷⁵ Even if tactics or lessons do not change, without ongoing standardized scenario-based training, over time, staff may forget the details of what they learned at the Academy.

DOCCS should also revamp its in-service training to emphasize key issues confronting officers. In-service training on use of force is minimal compared to its importance. Only seven of the 40 required hours of annual training are dedicated to weapons, chemical agents, and use of force training.⁵⁷⁶ Of those seven hours, only two hours are dedicated to use of force, while weapons and chemical agents combined typically have five or six hours of dedicated training.⁵⁷⁷

⁵⁷² [Cite Redacted].

⁵⁷³ [Cite Redacted].

⁵⁷⁴ [Cite Redacted].

⁵⁷⁵ [Cite Redacted].

⁵⁷⁶ [Cite Redacted].

⁵⁷⁷ [Cite Redacted].

The use of force training manual that is covered during in-service training is 178 pages, which is too long for effective teaching, and contains background information not tailored to correctional settings. For example, the manual contains 15 pages on case law and criminal procedure, covering topics such as the four stages of responding to an ongoing felony (prevent, terminate, arrest, prevent from escape), the affirmative defense of justification to any committed offense (assault, murder, etc.), and how the justification defense applies to police and peace officers, as well as private citizens.⁵⁷⁸ This includes, for example, an example illustration regarding whether it is justified to trespass onto private property to prevent someone from drowning.⁵⁷⁹ These detailed explanations have limited relevance in corrections and are not an effective use of limited training time; they do not belong in the first 15 pages of the manual. DOCCS should streamline its use of force training manual so that it includes only information directly relevant to corrections.

The manual and in-service training also do not emphasize key issues. For example, the training manual does not refer to a DOCCS formal use of force continuum.⁵⁸⁰ Nor does the in-service lesson on use of force devote a discrete section to the duty to intervene.⁵⁸¹ Instead, the duty to intervene is only mentioned twice: first in a list of 26 definitions at the beginning of the lesson and again, briefly, during the discussion of the use of force directive.⁵⁸² The in-service lesson plans do not require instructors to emphasize the duty to intervene, nor do they provide examples of when intervention could be necessary and how to effectively intervene against

⁵⁷⁸ [Cite Redacted].

⁵⁷⁹ [Cite Redacted].

⁵⁸⁰ [Cite Redacted].

⁵⁸¹ [Cite Redacted].

⁵⁸² [Cite Redacted].

colleagues.⁵⁸³ Therefore, DOCCS should update in-service training materials to emphasize the duty to intervene and a DOCCS-wide formal use of force continuum, among other key topics.

In-service training should also be more interactive. Like scenario-based training at the Academy, interactive use of force in-service training would go beyond lectures and static presentations to actively engage participants in decision-making, problem solving, and skill application. Today, DOCCS’ in-service training requires officers to watch a video at the beginning of the use of force section.⁵⁸⁴ But this video is inadequate. The 79-minute video, from the New York State Division of Criminal Justice Services, devotes only 43 seconds to the duty to intervene.⁵⁸⁵ Notably, this training was so limited that certain defendants indicted in the murder of Mr. Brooks raised this limited training on the duty to intervene as an affirmative defense.⁵⁸⁶ DOCCS should require mandatory, in-person, scenario-based in-service training for all correction officers, every two years, and include the most up-to-date techniques with a focus on use of force, the use of force continuum, de-escalation, and the duty to intervene.

6.4.2 Improve the In-Service Training Environment

Because instructors are fellow correction officers, they sometimes do not strictly enforce classroom rules.⁵⁸⁷ For example, a Mid-State correction officer who retired in 2021 said, “The trainings are mainly ... they just put on videos. The instructors are friends of ours. You just sit and BS with the buddy next to you. You’re all sitting in a room. There’s no testing.”⁵⁸⁸ And a Mid-State correction officer who retired in 2024 stated, “We like going to training, because we

⁵⁸³ [Cite Redacted].

⁵⁸⁴ [Cite Redacted].

⁵⁸⁵ [Cite Redacted].

⁵⁸⁶ *Day 3 of Marcy Inmate Murder Trial Focuses on ‘Use of Force,’ Health of Robert Brooks*, CNYCENTRAL (Oct. 8, 2025), <https://cnycentral.com/news/local/day-3-testimony-focuses-on-use-of-force-policies-in-marcy-inmate-murder-trial-robert-brooks-correctional-facility-messiah-nantwi-mid-state-correctional-facility-bodycam>.

⁵⁸⁷ [Cite Redacted].

⁵⁸⁸ [Cite Redacted].

can do what we want. They give you long lunches. But you don't learn anything. I go in, sit at my desk and read the paper. The TV comes on, they show a video, then we take a break. There are a lot of breaks. Some guys walk out while the video is on. Other people are sleeping, reading the newspaper. It's well known you get a long lunch – 11:00 to 1:30. We loved it, but we're not refreshed on anything.”⁵⁸⁹ Likewise, we heard from both former and current officers that the rules against cell phones during in-service trainings have not been enforced, which also makes it difficult to ensure focus, learning, and retention during these trainings.⁵⁹⁰

Nor are in-service testing requirements followed strictly. Though the in-service training handbook contains a “Pre-Test” and a “Post-Test,” instructors often do not administer them.⁵⁹¹ Going forward, DOCCS should require mandatory testing at the end of each in-service training session—for both lecture and scenario-based lessons—to measure comprehension. Testing results should be maintained for each officer.

Compounding these issues, instructors themselves are not always trained effectively. Officers must attend a two-week course to earn an instructor certification, but that curriculum focuses on lesson planning as opposed to how officers can be engaging and effective instructors.⁵⁹² One current officer who was responsible for in-service weapons training shared, “Frankly, [the instructor course] is almost useless. They teach you how to write lesson plans – something we don't need to do. It does help a little with public speaking. But it's not geared to teaching about weapons or use of force.”⁵⁹³

⁵⁸⁹ [Cite Redacted].

⁵⁹⁰ *See, e.g.*, [Cite Redacted].

⁵⁹¹ [Cite Redacted].

⁵⁹² [Cite Redacted].

⁵⁹³ [Cite Redacted].

If the instructor cannot effectively deliver material and manage the classroom, officers tune out. As a Marcy correction officer who retired in 2024 said, “Now, the instructors just read the directives. It’s boring. If the instructors can’t keep it interesting, no one’s gonna pay attention.”⁵⁹⁴ With this feedback in mind, DOCCS should create an environment where in-service training is taken seriously by, at a minimum, enforcing a no-cell-phone policy, requiring written tests.

DOCCS can also efficiently deliver classroom content by leveraging a Learning Management System (LMS) in content areas where it is most appropriate. LMS is a computer-based management system that offers staff more flexibility in meeting their annual training requirements by allowing for remote attendance; it also reduces transportation and facilitation costs, and promotes continuity in content delivery.⁵⁹⁵ Many correctional systems and government-sector entities have augmented their annual training requirements with a combined use of instructor facilitation and LMS technologies.⁵⁹⁶ However, some critics note that participants may not fully engage in remote training, and that overusing LMS can decrease content retention.⁵⁹⁷ To balance these considerations, DOCCS could allow security staff to participate in some agency-approved LMS training modules on-site in a designated training room under observation to ensure attention. Bottom line, DOCCS should revamp its training to include a hybrid approach to in-service training, with regular in-person training on key issues such as use

⁵⁹⁴ [Cite Redacted].

⁵⁹⁵ Oudat, Qutaibah & Mohammad Othman, *Embracing Digital Learning: Benefits and Challenges of Using Canvas in Education*, 14 J. Nursing Educ. Prac. 39 (2024).

⁵⁹⁶ *e-Learning Resources*, Ind. Dep’t Corr., <https://www.in.gov/idoc/about/commissioners-office/staff-development-and-training/e-learning-resources/?a=116259>; *How Technology is Advancing Correctional Officer Training*, CORRECTIONS1 (Nov. 21, 2018), <https://www.corrections1.com/corrections-training/articles/how-technology-is-advancing-correctional-officer-training-VzeOfRj0Re15tzUW/>; Training Programs, Events, and Networks, NIC, <https://nicic.gov/training>.

⁵⁹⁷ Qutaibah Oudat & Mohammad Othman, *Embracing Digital Learning: Benefits and Challenges of Using Canvas in Education*, 14 J. Nursing Educ. Prac. 39 (2024).

of force, defensive tactics, the duty to intervene, and de-escalation, supported by the delivery of virtual classroom learning (where appropriate for the topic) and a LMS for security officers.

6.5 Recommendation Summary

- 6.5.1 Require every facility to conduct mandatory, in-person, scenario-based in-service training for all correction officers every two years. Training should provide officers with the most up-to-date techniques with a focus on appropriate use of force, the use of force continuum, de-escalation, and the duty to intervene. The training should leverage technology, such as virtual reality, to incorporate scenario-based training. It should incorporate formal testing at the end of each session to ensure comprehension of both lecture and scenario training and DOCCS should maintain records of participation and assessment outcomes. DOCCS should also hire dedicated curriculum consultants to assist in creating and enhancing the in-service training program.
- 6.5.2 Conduct comprehensive review of correction officer training curriculum at the Academy and in-field training to increase use of scenario-based training wherever possible; to emphasize the most up-to-date techniques on appropriate use of force, the use of force continuum, de-escalation, and the duty to intervene; and to incorporate formal testing of covered concepts. At the same time, implement lesson plans to complement the paramilitary approach with state-of-the art adult learning methods, such as table-top exercises, discussion and dialogue, case studies and scenarios, and collaborative learning involving group exercises to leverage collective experience and work through complex, real-life challenges.
- 6.5.3 Include in standardized curriculum for all trainees as part of on-the-job training protocols that ensure trainees receive training on all posts conducted by experienced officers, with testing at the end of the field training.
- 6.5.4 Revamp training to include a hybrid approach with regular in-person training on key issues such as use of force, defensive tactics, the duty to intervene, and de-escalation, supported by delivery of virtual classroom learning (where appropriate for the topic) and a Learning Management System for security officers. This recommendation will require access to the necessary technology for computer-based trainings.

7. **OFFICE OF SPECIAL INVESTIGATIONS (OSI)**

OSI serves as DOCCS' investigative body.⁵⁹⁸ According to DOCCS, OSI supports the Department's mission by promoting accountability, justice, and integrity, and safety.⁵⁹⁹ OSI investigates crime, staff misconduct, abuse, and corruption across DOCCS.⁶⁰⁰ It is led by a Deputy Commissioner/Chief of Investigations, who reports directly to the Commissioner.⁶⁰¹ OSI is divided into five major divisions, three of which are responsible for investigations and managing complaints: (1) Public Integrity (responsible for investigating criminal and internal policy violations, such as allegations of physical and sexual abuse of incarcerated individuals by employees and staff misconduct),⁶⁰² (2) Criminal Investigations (responsible for investigating serious criminal activities, such as violence, trafficking, extortion, and criminal organizations), and (3) Intake, Training, and Analysis (ITAD) (responsible for processing and managing OSI complaints).⁶⁰³ Criminal Investigations focuses more on crimes between incarcerated individuals, whereas Public Integrity largely covers interactions involving staff, administrators, and officers, as well as any investigations not covered by another OSI division. OSI also has a Canine Division responsible for contraband detection, and a Fugitive Investigations Division responsible for returning parole absconders.⁶⁰⁴

OSI has faced significant challenges to its effectiveness: (1) it is overwhelmed by the volume of complaints it receives; (2) it is considered ineffective by many incarcerated

⁵⁹⁸ Directive #0700 (May 16, 2025).

⁵⁹⁹ Directive #0700 (May 16, 2025).

⁶⁰⁰ Directive #0700 (May 16, 2025).

⁶⁰¹ Directive #0700 (May 16, 2025).

⁶⁰² In March 2025, OSI merged the Internal Affairs Division and Sex Crimes Division to create a new Public Integrity Division. Office of Special Investigations Monthly Report – September 2025.

⁶⁰³ Directive #0700 (May 16, 2025).

⁶⁰⁴ Directive #0700 (May 16, 2025).

individuals; (3) it does not have enough tools to take proactive measures; and (4) it is short on staff, with roughly 50 current open positions as of August 2025.⁶⁰⁵

DOCCS has already taken steps to address staffing issues in OSI. In January 2025, Governor Hochul proposed an additional \$7.2 million to expand OSI.⁶⁰⁶ As of January 2026, OSI has 142 new applicants in its pipeline, with a goal of filling 167 positions.⁶⁰⁷ OSI continues to lose staff due to retirements, transfers, and promotions.⁶⁰⁸ To prevent and address misconduct, it is essential that OSI is adequately resourced and works cooperatively with other DOCCS-wide risk prevention, tracking, and mitigation systems.

7.1 How OSI Works

When OSI receives a complaint or becomes aware of an incident, ITAD assigns the investigation a case number.⁶⁰⁹ A review committee at OSI then processes the complaint and sorts it into one of five categories: (1) assigned for OSI investigation; (2) referred to a separate Central Office Division Head (i.e., referred to a specific division within DOCCS' central office); (3) referred back to the facility for investigation (with or without an obligation to respond with investigation results); (4) referred back the facility for other appropriate action; or (5) referred to a local, state, or federal agency.⁶¹⁰

⁶⁰⁵ [Cite Redacted].

⁶⁰⁶ James Moss, *Hochul Seeks \$400 Million to Expedite Installation of Surveillance Cameras in All NY Prisons*, SYRACUSE.COM (Jan. 27, 2025), <https://www.syracuse.com/politics/cny/2025/01/hochul-seeks-400-million-to-expedite-installation-of-surveillance-cameras-in-all-ny-prisons.html>; Robert Harding, *Following Inmate Death, N.Y. Governor Unveils \$425M Prison Safety Overhaul*, CORRECTIONS1.COM (Jan. 22, 2025), <https://www.corrections1.com/finance-and-budgets/following-inmate-death-n-y-governor-unveils-425m-prison-safety-overhaul>.

⁶⁰⁷ [Cite Redacted].

⁶⁰⁸ [Cite Redacted].

⁶⁰⁹ Directive #0700 (May 16, 2025).

⁶¹⁰ Directive #0700 (May 16, 2025).

The review committee faces a daunting task. OSI received roughly 24,000 complaints in 2024.⁶¹¹ However, many of the complaints received are not appropriate for OSI; at least one third of the 2024 complaints raised issues that should have been submitted and handled via the grievance process, such as commissary complaints or tablet problems.⁶¹² Mid-State and Marcy ranked second and fourth, respectively, in overall complaint volume between January 2024 and April 2025, with 1,583 and 1,072 complaints, respectively.⁶¹³

Each incident referred for investigation is assigned to an investigator. Each OSI investigator is typically responsible for approximately 75 active investigations at a time.⁶¹⁴ The investigations can be time-consuming. The investigators must interview (or attempt to interview) the complainant, the accused staff, and potential witnesses, and they must collect evidence, conduct searches and surveillance, subpoena records, and review available footage.⁶¹⁵

Aside from demanding caseloads, OSI investigators face other challenges to conducting their investigations. For example, OSI investigators report that they cannot take their laptops into facilities, making their work less efficient.⁶¹⁶ In addition, OSI must work quickly to refer incidents to the BLR because, under the relevant collective bargaining agreements, security officers cannot be disciplined for any non-criminal actions if a BLR notice of discipline is issued more than nine months after the incident.⁶¹⁷ Therefore, OSI simultaneously faces immense

⁶¹¹ [Cite Redacted].

⁶¹² [Cite Redacted].

⁶¹³ [Cite Redacted].

⁶¹⁴ [Cite Redacted].

⁶¹⁵ [Cite Redacted].

⁶¹⁶ [Cite Redacted]. There is no directive preventing investigators from taking laptops into facilities, but they nonetheless report being denied.

⁶¹⁷ Agreement Between the State of New York and the New York State Correctional Officers and Police Benevolent Association, Inc., Security Services Unit, 2023-2026 (Mar. 28, 2024), <https://oer.ny.gov/system/files/documents/2024/09/2023-2026-security-services-unit-contract-agreement.pdf>.

caseloads, resource and technology constraints, and serious time pressures; this inhibits investigation and undermines accountability.

OSI makes findings based on a preponderance of the evidence standard. If OSI concludes that the allegations are substantiated based on the preponderance of the evidence, the case is referred either to BLR for discipline or to law enforcement authorities for criminal prosecution.⁶¹⁸ If OSI determines that the preponderance of the evidence does not support the allegations, it closes the investigation as unfounded.⁶¹⁹ And if OSI lacks sufficient evidence to decide, OSI closes the investigation as unsubstantiated.⁶²⁰ OSI does not discipline staff; when OSI substantiates allegations, those results are provided to BLR, which makes the decision whether to pursue formal discipline.⁶²¹

7.2 DOCCS Should Improve the OSI Complaint Process

Timely reporting of staff misconduct helps protect both staff and incarcerated individuals, and OSI is a key DOCCS mechanism for doing so. Therefore, OSI must ensure that (1) incarcerated individuals are well informed of, and have easy access to, OSI reporting avenues, and (2) staff and incarcerated individuals feel safe making complaints without fearing retaliation. While OSI has made efforts on both fronts, it can continue to improve.

7.2.1 Ensure Incarcerated Individuals Can Access OSI Reporting Avenues

It is imperative that OSI reporting avenues remain well-publicized, accessible, and secure. Today, incarcerated individuals have three methods of filing an OSI complaint: they can submit a complaint in writing via complaint boxes throughout the facility; they can call the OSI

⁶¹⁸ Directive #0700 (May 16, 2025).

⁶¹⁹ Directive #0700 (May 16, 2025).

⁶²⁰ Directive #0700 (May 16, 2025).

⁶²¹ Directive #0700 (May 16, 2025).

tip-line during business hours from a dedicated payphone inside each facility housing unit; or they can mail complaints directly to an OSI mailing address.⁶²²

During the review, we observed several issues with OSI reporting avenues. For example, during one visit to Marcy, we observed that at least one dormitory phone was not working, and another was covered by a sock, potentially indicating that it was under gang control.⁶²³ In addition, at both Marcy and Mid-State, we observed that the official DOCCS flyers advertising OSI reporting avenues were not posted in every dormitory, and in some dorms where they were posted, the flyers were outdated, obstructed, or placed too high to be legible.⁶²⁴ DOCCS must work with facilities to ensure that all incarcerated individuals have access to the OSI reporting mechanism of their choice.

7.2.2 Prevent Retaliation Against Incarcerated Individuals

Our review also revealed that, even where incarcerated individuals could access an OSI reporting avenue, other factors—such as a perceived lack of confidentiality and fears of retribution—prevented them from doing so. For example, some incarcerated individuals reported that they do not submit OSI complaints in writing because they have witnessed or heard of staff reading or tampering with complaints.⁶²⁵ Others reported that they do not file OSI complaints, via any mechanism, due to fear of retaliation.⁶²⁶ They believe that staff can infiltrate the process—either at the complaint stage or during the OSI investigation stage, especially during interviews—and then target the complaining incarcerated individual and/or their family (if the family complained on their behalf).⁶²⁷ We also learned that when OSI visits facilities to address

⁶²² Directive #0700 (May 16, 2025).

⁶²³ [Cite Redacted].

⁶²⁴ [Cite Redacted].

⁶²⁵ [Cite Redacted].

⁶²⁶ [Cite Redacted].

⁶²⁷ [Cite Redacted].

complaints, staff members may be able to identify individuals who have submitted those complaints—further fueling the risk, or at least the perceived risk, of retaliation.⁶²⁸ Incarcerated individuals reported that officers subject them to physical and psychological abuse following OSI complaints, such as issuing false disciplinary tickets or depriving them of food or personal hygiene items.⁶²⁹ DOCCS could eliminate many of these issues by enabling incarcerated individuals to file complaints using their individual tablets. Of course, this presents both technological challenges—i.e., developing a system to facilitate this additional submission mechanism—and logistical challenges—i.e., confronting and addressing a likely increase in complaints. However, we recommend that DOCCS make it a priority to expand OSI reporting outlets to include a virtual, confidential, tablet-based mechanism.

At the same time, OSI should strengthen processes to prevent retaliation. For example, DOCCS should create a formal protocol for protecting incarcerated individuals against retaliation. This might include, for example, systems for tracking incarcerated individuals after their interviews with OSI investigators, tracking and investigating instances in which incarcerated individuals refuse to sit for an OSI interview, providing dedicated post-reporting retaliation complaint forms, creating a separate intake process for retaliation complaints received, and developing a formal mechanism for tracking retaliation investigations.

7.3 DOCCS Should Expand OSI’s Toolkit

As of September 2025, OSI reported 7,566 total cases closed year to date, a 32% decrease compared to the same period the previous year.⁶³⁰ OSI attributed the reduction in case closures to the illegal 2025 strike and the assignment of OSI staff to support facility operations.⁶³¹ In

⁶²⁸ [Cite Redacted].

⁶²⁹ [Cite Redacted].

⁶³⁰ [Cite Redacted].

⁶³¹ [Cite Redacted].

September 2025, OSI referred 97 individuals for further action: 58 to BLR, 35 for criminal prosecution, and four to the Personnel Office.⁶³² The number of referrals increased 90% from August 2025 to September 2025. In light of this, DOCCS should continue hiring additional OSI staff to lighten investigative caseloads.

Although OSI monitors the initiation and closure of the cases it investigates, there is room for improvement in tracking OSI trends. For example, OSI does not maintain centralized records on the ultimate results of cases that are sent back to the facilities for investigation.⁶³³ More generally, OSI representatives told us that they have recently piloted efforts aimed at more sophisticated tracking.⁶³⁴ For example, they have instituted technological tools such as geo-mapping (which uses location-based data to visualize trends) to identify problematic “hotspots,” facilitate interventions by supervisors, and guide camera placement.⁶³⁵ OSI is also considering additional tools, such as tools to identify abuse in medical areas and developing systems for tracking “red-flag” employees.⁶³⁶

We were impressed by OSI’s leadership and its proactive approach to making improvements. OSI has acknowledged the current lack of strong analytical tools for identifying patterns in excessive use of force, contraband, and other types of staff misconduct, and is actively working to address these shortcomings. Continuing these efforts is important, as better data collection will allow DOCCS to design proactive policies to enhance safety and accountability. At the same time, gathering and communicating these data will contribute to greater public trust in the investigative process. As discussed further in Section 10, “Risk

⁶³² [Cite Redacted].

⁶³³ [Cite Redacted].

⁶³⁴ [Cite Redacted].

⁶³⁵ [Cite Redacted].

⁶³⁶ [Cite Redacted].

Detection and Mitigation,” we recommend that DOCCS continue developing analytical tracking tools internally, including to track investigations and resolutions for alleged staff misconduct. This includes utilizing the full suite of tools offered by existing service providers. For example, Axon, DOCCS’ BWC supplier, also provides an integrated evidence management system that centralizes video, audio, and incident reports.⁶³⁷ And SECURUS, which also has an existing contract with DOCCS, provides big data analysis and automatic transcription of audio calls.⁶³⁸

7.4 DOCCS Should Address Mistrust of OSI by Incarcerated Individuals

In interviews and surveys, incarcerated individuals at Marcy and Mid-State reported having a negative opinion of OSI. According to surveys, more than 75% of the combined 801 incarcerated individual respondents at Marcy and Mid-State who filed complaints with OSI rated the process as “Poor” or “Very Poor/Ineffective.” (see Figure 32).⁶³⁹

Figure 32 Incarcerated Individual Survey Result

“If you have ever filed an OSI complaint online or called the hotline while housed in this facility, how would you rate the process?”						
Facility	Very Poor / Ineffective	Poor	Satisfactory	Very Satisfactory / Effective	Never Filed / Don’t Know	Total
Marcy	34.5%	8.1%	4.9%	2.0%	50.5%	100.0%
Mid-State	29.4%	9.7%	3.9%	4.2%	52.9%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

⁶³⁷ Investigation, SECURUS TECHNOLOGIES, <https://securustechnologies.tech/investigative/investigation/> (last visited March 3, 2026).

⁶³⁸ Investigation, SECURUS TECHNOLOGIES, <https://securustechnologies.tech/investigative/investigation/> (last visited March 3, 2026).

⁶³⁹ [Cite Redacted].

One contributor to this negative perception appears to be the sentiment that reporting misconduct to OSI does not result in meaningful accountability or remediation. In interviews, incarcerated individuals told us, for example, that calls to OSI go unanswered or are sent back to the facility and result in no further action.⁶⁴⁰ When complaints are referred back to the facility for further investigation, the local investigations are often problematic; for example, some individuals reported that after OSI referred their complaints to the facility, they were interviewed by a sergeant in the presence of the accused officer and instructed to repeat their allegations.⁶⁴¹ Some declined due to safety concerns, resulting in case closures.⁶⁴² Others repeated their allegations, which the officers denied, and complainants were allegedly pressured to recant.⁶⁴³ Furthermore, we heard from incarcerated individuals that, from their perspective, complaints rarely, if ever, result in staff discipline, which is discussed in the next section (Section 8).⁶⁴⁴

7.5 Recommendation Summary

- 7.5.1 Continue hiring additional OSI staff to lighten investigative caseloads. At the same time, leverage technology and data to identify risk and investigative patterns, facility hot spots, and trends, as well as potential red flags at the facility or staff level.
- 7.5.2 Review existing technology partnerships to assess opportunities to expand by leveraging each vendor's full suite of applicable technology capabilities.
- 7.5.3 Strengthen mechanisms to prevent retaliation for filing of complaints, including by creating formal follow-up protocol that includes checking in on subjects after their interviews with OSI, providing dedicated retaliation complaint forms, and creating a separate intake process for any retaliation complaints that OSI receives. The protocol should also include a formal mechanism for tracking outcomes of retaliation investigations.

⁶⁴⁰ [Cite Redacted].

⁶⁴¹ [Cite Redacted].

⁶⁴² [Cite Redacted].

⁶⁴³ [Cite Redacted].

⁶⁴⁴ [Cite Redacted].

8. STAFF DISCIPLINE

Timely discipline and corrective action for staff misconduct are critical to deter wrongdoing and establish a culture of safety and respect. The killings of Mr. Brooks and Mr. Nantwi highlight serious flaws in DOCCS processes for seeking and ensuring staff discipline. While DOCCS suspended many of the officers involved in the deaths of Mr. Brooks and Mr. Nantwi and initiated termination proceedings, actual firings are rare and difficult due to structural restraints and misuse of the arbitration system. Most of the allegedly involved officers resigned.⁶⁴⁵ Governor Hochul ordered DOCCS to fire all involved officers, but despite that order, at least one officer remains employed due to union protections.⁶⁴⁶ Moreover, multiple officers involved in the deaths of Mr. Brooks and Mr. Nantwi had previously faced complaints related to excessive use of force and other misconduct, some of which were substantiated. While the issues discussed in this section likely apply to DOCCS' ability to seek and impose staff discipline writ-large, our review focused on disciplining staff for excessive use of force.

8.1 Applicable Collective Bargaining Agreements and DOCCS Policies

DOCCS staff discipline is constrained by the collective bargaining agreements between DOCCS and the employee unions: NYSCOPBA, which represents correction officers and sergeants,⁶⁴⁷ and Council 82, which represents lieutenants (together, the "Union Agreements").⁶⁴⁸ By their terms, the Union Agreements govern discipline instead of the civil

⁶⁴⁵ [Cite Redacted].

⁶⁴⁶ [Cite Redacted].

⁶⁴⁷ *Security Services Unit (SSU) – 01 and 21*, NEW YORK OFFICE OF EMPLOYEE RELATIONS, <https://oer.ny.gov/security-services-unit-ssu-01-and-21>.

⁶⁴⁸ *Security Supervisors Unit (SSPU) – 61 and 91*, NEW YORK OFFICE OF EMPLOYEE RELATIONS, <https://oer.ny.gov/security-supervisors-unit-sspu-61-and-91>.

service laws that govern discipline for other state employees.⁶⁴⁹ The two Union Agreements provide similar procedures regarding discipline.⁶⁵⁰

DOCCS Directive #2111 outlines the procedures for reporting, investigating, and disciplining staff misconduct. All employees must report misconduct to their supervisor or to OSI, and no investigation may begin without OSI consultation.⁶⁵¹

8.2 The Process of Seeking Discipline

BLR, under the direction of the Executive Deputy Commissioner, is responsible for seeking employee discipline, including actions taken pursuant to employee union agreements.⁶⁵² As stated earlier, OSI investigates staff misconduct, and BLR then reviews OSI's findings and referrals to determine whether to pursue disciplinary action.⁶⁵³ As discussed later, disciplinary proceedings are often resolved through settlement agreements before the disciplinary process runs its course. BLR has a lean staff, relative to OSI, that handles approximately 500 to 1,000 cases referred for discipline annually.⁶⁵⁴ Given this workload, BLR—like OSI—needs increased staffing to more effectively manage review workload.⁶⁵⁵ BLR finds it difficult to fill positions with qualified applicants, given the heavy workload.⁶⁵⁶ However, in February 2026, BLR

⁶⁴⁹ Union Agreements §8.1.

⁶⁵⁰ Agreement Between the State of New York and the New York State Correctional Officers and Police Benevolent Association, Inc., Security Services Unit, 2023-2026 (Mar. 28, 2024), <https://oer.ny.gov/system/files/documents/2024/09/2023-2026-security-services-unit-contract-agreement.pdf>; Agreement Between the State of New York and New York State Law Enforcement Officers Union, Council 82, AFSCME, AFL-CIO, Security Supervisors Unit, 2016-2023, <https://oer.ny.gov/system/files/documents/2024/11/2016-2023-sspu-contract-agreement.pdf>. For the updated 2023-2026 contract, the State and Council 82 entered into a Memorandum of Understanding to renew the contract with edits. See Memorandum of Understanding Between the State of New York and the District Council 82, April 1, 2023 to March 31, 2026, https://www.council82.org/vs-uploads/PDF/1715355503_SSU-2023-2026-MOU.pdf.

⁶⁵¹ Directive #2111 §II (Nov. 17, 2025).

⁶⁵² Directive #2111§§I & V (Nov. 17, 2025); the organization and scope of the BLR is described in detail in Directive #2114 (June 5, 2025).

⁶⁵³ Directive #2111 (Nov. 17, 2025).

⁶⁵⁴ [Cite Redacted].

⁶⁵⁵ [Cite Redacted].

⁶⁵⁶ [Cite Redacted].

leadership indicated that the bureau was making progress with filling vacant positions.⁶⁵⁷

DOCCS should continue to prioritize hiring for new positions in BLR to more effectively manage review workload.

The full life cycle of a disciplinary proceeding—from referral to the imposition of discipline—is briefly summarized below.

8.2.1 Referral

As discussed above, OSI refers misconduct to BLR after substantiating the misconduct based on a preponderance of the evidence.⁶⁵⁸ BLR may address misconduct either informally, such as through facility-level counseling, or formally, through documented intervention such as retraining or suspension.⁶⁵⁹ There is no written policy specifying which types of misconduct may be addressed informally versus formally.⁶⁶⁰ If informal discipline is pursued, it can be done at the facility level—often without documentation, immediate escalation, or DOCCS oversight. The goal of informal facility-level discipline is to impose timely corrective action, so that staff have a chance to improve before formal discipline becomes necessary. At Mid-State, for example, the disciplinary process usually starts with informal counseling, then escalates to formal counseling, which is documented and entails coaching by facility leadership.⁶⁶¹ At Marcy, two captains review footage of officer interactions with incarcerated individuals daily.⁶⁶² They provide informal coaching to officers when they identify room for improvement.⁶⁶³ For example, we saw informal counseling of an officer regarding the officer’s use of profane language after being

⁶⁵⁷ [Cite Redacted].

⁶⁵⁸ Directive #2111, §II (Nov. 17, 2025); Directive #0700, §V.E, F (May 16, 2025).

⁶⁵⁹ [Cite Redacted].

⁶⁶⁰ [Cite Redacted].

⁶⁶¹ [Cite Redacted].

⁶⁶² [Cite Redacted].

⁶⁶³ [Cite Redacted].

thrown on.⁶⁶⁴ There is no centralized log for tracking these kinds of interventions because, according to DOCCS leadership, they are meant to be informal.⁶⁶⁵ We recommend implementing a centralized process to document and log instances of informal interventions for facility staff. This data, in turn, should be used to identify staff with frequent informal interventions, analyze underlying trends, and inform targeted interventions or additional training, thereby supporting a culture of continuous improvement.

8.2.2 BLR Consideration

BLR may exercise discretion and decide not to seek formal discipline in certain cases, including cases where OSI has substantiated the misconduct by a preponderance of the evidence. BLR maintains a database that tracks DOCCS staff disciplinary outcomes dating back to 2000. We received an export from that database in July 2025. Therefore, our analysis runs through 2024, the most recent full year of information. We identified 94 incidents investigated in 2024 in which OSI substantiated allegations of staff abuse against incarcerated individuals (referred to as “Inmate Abuse” internally) and referred those cases to BLR.⁶⁶⁶ In approximately one third of those cases, BLR determined that the referral did not require formal disciplinary action.⁶⁶⁷

Typically, BLR issues a declination memorandum when it declines to take disciplinary action on a substantiated case.⁶⁶⁸ We reviewed a sample of these declination memoranda from the last two years.⁶⁶⁹ The memoranda ranged from formal memoranda on DOCCS letterhead with detailed paragraphs to handwritten notes briefly describing the investigative steps and

⁶⁶⁴ [Cite Redacted].

⁶⁶⁵ [Cite Redacted].

⁶⁶⁶ [Cite Redacted].

⁶⁶⁷ [Cite Redacted].

⁶⁶⁸ [Cite Redacted].

⁶⁶⁹ [Cite Redacted].

decision-making process.⁶⁷⁰ In some cases, BLR looked beyond the particular charges to determine whether to pursue discipline. For example, one of the cases involved an altercation between an incarcerated individual and an officer, where both allegedly punched each other in the head and/or face.⁶⁷¹ The officer admitted to his involvement, although there was conflicting witness information about who initiated the conflict.⁶⁷² OSI determined that the officer had used a high-impact strike in violation of Directive #4944 and referred the case to BLR for discipline.⁶⁷³ The BLR declination memo states that OSI’s conclusion “lacks pragmatism,” noting that “staff have a reasonable [sic] ability to defend themselves when being involved in an altercation where an [incarcerated individual] is delivering or attempting to deliver closed fist strikes to their own head.”⁶⁷⁴ The memo also notes that the officer had no disciplinary record in his six years of service, while the incarcerated individual had a gang affiliation and 15 prior disciplinary tickets.⁶⁷⁵

If BLR decides to pursue discipline, BLR issues a Notice of Discipline to the officer.⁶⁷⁶ Of the 94 incidents OSI substantiated in 2024, 63 resulted in Notices of Discipline.⁶⁷⁷ After receiving a Notice of Discipline, the accused officer can enter into a settlement agreement by negotiating and agreeing to the discipline, or contest the disciplinary action and proceed to arbitration.⁶⁷⁸

⁶⁷⁰ [Cite Redacted].

⁶⁷¹ [Cite Redacted].

⁶⁷² [Cite Redacted].

⁶⁷³ [Cite Redacted].

⁶⁷⁴ [Cite Redacted].

⁶⁷⁵ [Cite Redacted].

⁶⁷⁶ Directive #2111, §IV (Nov. 17, 2025).

⁶⁷⁷ [Cite Redacted].

⁶⁷⁸ Union Agreements §8.2(e); [Cite Redacted].

DOCCS internal discipline records found that, between 2023 and 2024, approximately 39% of cases categorized as “Inmate Abuse” were resolved through negotiated settlements short of dismissal, despite BLR starting with a Notice of Discipline calling for dismissal in almost all of those cases.⁶⁷⁹

8.2.3 Arbitration

Under the Union Agreements, employees are entitled to pursue neutral arbitration before DOCCS can dock privileges, issue a written reprimand, fine, demote, or terminate employment.⁶⁸⁰ This arbitration process, governed by the Union Agreements, determines whether the accused officer is guilty or innocent and if the proposed penalty is appropriate, “taking into account mitigating and extenuating circumstances.”⁶⁸¹ The Union Agreements require that DOCCS prove the charges by a preponderance of the evidence and, if applicable, that the accused officer prove any affirmative defenses.⁶⁸² The Union Agreements state that the arbitration panel “shall not have the authority to impose any other burden of proof upon the employer.”⁶⁸³ The arbitration decision is final and binding on the employee and on DOCCS.⁶⁸⁴

The Union Agreements provide that the arbitration decision-maker must be neutral—either a single neutral arbitrator or a panel of three arbitrators. According to DOCCS leadership, in practice, the process of identifying a neutral arbitrator involves DOCCS and the union each eliminating and ranking arbitrators from a shortlist.⁶⁸⁵ One DOCCS executive explained that this selection process creates incentives for arbitrators to establish a reputation for not deciding

⁶⁷⁹ [Cite Redacted].

⁶⁸⁰ Union Agreements §8.2(a-e).

⁶⁸¹ Union Agreements §8.2(h).

⁶⁸² Union Agreements §8.9(c).

⁶⁸³ Union Agreements §8.9(c).

⁶⁸⁴ Union Agreements §8.2(h).

⁶⁸⁵ [Cite Redacted].

overwhelmingly for one side or the other; as a result, “basically, an arbitrator who decides about 50/50.”⁶⁸⁶ Based on public reporting in late 2023, following an investigation into the process, just two arbitrators handled over half of the cases reviewed.⁶⁸⁷

In allegations of misconduct relating to sexual misconduct, distribution of contraband, or excessive force, the Union Agreements require a tripartite panel of arbitrators.⁶⁸⁸ In these cases, DOCCS selects one arbitrator, the union selects one arbitrator, and the third arbitrator is assigned from a list of neutral arbitrators on a rotating basis.⁶⁸⁹ A guilty finding requires the agreement of two of the three panel members.⁶⁹⁰ However, in practice, we learned that a tripartite panel has never been constituted in any discipline cases, including cases involving serious misconduct.⁶⁹¹

Moreover, in these types of cases that require tripartite panels, such as excessive use of force cases, the Union Agreements require termination when the arbitrator finds that the officer used excessive force in bad faith.⁶⁹² However, in practice, arbitrators are applying lesser penalties than termination even when finding an officer guilty of abuse.⁶⁹³ In fact, in the excessive force cases that went to arbitration from 2023-2024, when an arbitrator found an officer guilty, the arbitrator did not uphold termination in a single case.⁶⁹⁴ Therefore, neither the

⁶⁸⁶ [Cite Redacted].

⁶⁸⁷ Alysia Santon & Joseph Neff, “A Crazy System”: *How Arbitration Returns Abusive Guards to New York Prisons* THE MARSHALL PROJECT (Dec. 14, 2023), <https://www.investigativepost.org/2023/12/14/a-crazy-system-how-arbitration-returns-abusive-guards-to-new-york-prisons/>.

⁶⁸⁸ Union Agreements §8.9(a-b).

⁶⁸⁹ Union Agreements §8.9(c).

⁶⁹⁰ Union Agreements §8.9(c).

⁶⁹¹ [Cite Redacted].

⁶⁹² Union Agreements §8.9(e) (“[W]here the panel finds the actions of the employee were not taken in a good-faith effort to maintain or restore discipline but were done maliciously and sadistically to cause harm, the penalty shall be termination.”).

⁶⁹³ [Cite Redacted].

⁶⁹⁴ [Cite Redacted].

tripartite panel requirement nor the termination requirements from the Union Agreements are followed in practice.

While the disciplinary proceedings are pending, DOCCS can suspend an employee without pay in two circumstances: (1) DOCCS finds “probable cause” that the “employee’s continued presence on the job represents a potential danger to persons or property or would severely interfere with [DOCCS’] operations” and provides the employee in writing with the specific reasons for the suspension, or (2) the employee is charged with the commission of a crime.⁶⁹⁵ In the first scenario, DOCCS must promptly serve a notice of discipline, providing the reasons for the suspension; this decision is reviewable by an arbitrator.⁶⁹⁶ In the second scenario, if the employee is found not guilty of the criminal charges, DOCCS may either serve a notice of discipline and proceed as in scenario one, or the employee must be reinstated with full back pay.⁶⁹⁷

8.2.4 Outcomes

Disciplinary outcomes may range from informal counseling to termination.⁶⁹⁸ Intermediate steps include mandatory training or retraining, fines, forfeiture of accruals, disciplinary evaluation periods (DEPs), and demotions.⁶⁹⁹ DEPs provide for a period of evaluation, such that DOCCS can seek further punishment or termination if another incident occurs during the DEP.⁷⁰⁰ There are two types of DEPs: (1) general DEPs, which apply to any misconduct, and (2) same-or-similar DEPs, which limit further punishment to disciplinary action

⁶⁹⁵ Union Agreements §8.4(a)(1)-(2).

⁶⁹⁶ Union Agreements §8.4(a)(1).

⁶⁹⁷ Union Agreements §8.4(a)(2).

⁶⁹⁸ [Cite Redacted].

⁶⁹⁹ [Cite Redacted].

⁷⁰⁰ [Cite Redacted].

with the “same or similar” circumstances.⁷⁰¹ Retraining is also a frequent settlement outcome, which is targeted toward specific areas related to the misconduct and can include training on use of force and chemical weapons.⁷⁰² BLR representatives told us that retraining is effective, pointing out that the majority of those who receive retraining after a disciplinary issue do not reoffend.⁷⁰³

During our review, BLR leadership told us that, more than ever before, and especially in egregious cases, accused employees choose to resign or retire before proceeding to discipline at all, as was seen with many of the officers involved in the deaths of Mr. Brooks and Mr. Nantwi.⁷⁰⁴

8.3 Shortcomings

The Union Agreements’ provisions for staff discipline hamper DOCCS’ ability to hold culpable officers accountable, and even when misconduct results in discipline, the discipline does not meaningfully deter future misconduct.

8.3.1 The Process Makes It Difficult to Impose Discipline

The biggest shortcoming in this process is the lack of effective discipline, which is due at least in part to the Union Agreements limiting or preventing BLR from enacting discipline against individuals who deserve it.⁷⁰⁵

Based on DOCCS internal discipline records, between 2023 and 2024, BLR opened 140 cases that were categorized as “Inmate Abuse.”⁷⁰⁶ BLR declined to pursue discipline in at least 36 of those cases, at least two were opened in error, at least two were withdrawn, at least 54 were

⁷⁰¹ [Cite Redacted].

⁷⁰² [Cite Redacted].

⁷⁰³ [Cite Redacted].

⁷⁰⁴ [Cite Redacted].

⁷⁰⁵ [Cite Redacted].

⁷⁰⁶ [Cite Redacted].

settled for lesser discipline (despite the original notice of discipline seeking termination in 50 of those), at least twelve resulted in the officer resigning before any disciplinary decisions were made, at least one resulted in a termination during the probationary period (and therefore did not require arbitration), at least eight went to arbitration, and the remaining are pending closure or otherwise unclear about the outcome.⁷⁰⁷ Of the eight cases we confirmed that went to arbitration, the arbitrators did not uphold any of the terminations, even though DOCCS sought termination in all eight cases; four of the officers were given lesser punishments and four were found not guilty.⁷⁰⁸ In other words, the majority of abuse cases that BLR opens after a substantiated investigation are either settled or declined, and very few result in termination, even though notices of discipline frequently seek termination.⁷⁰⁹ This lack of meaningful results points to a systemic problem in which DOCCS is incentivized to settle for lesser charges because pursuing termination is often overturned in arbitration. At the same time, offending officers may be incentivized to fight justified terminations because they might be reinstated with backpay, even if the arbitrator imposes a lesser penalty.

8.3.2 The Process Faces Evidentiary Challenges

The arbitration and discipline process, generally, also faces challenges related to evidence. Disciplinary decisions often hinge on limited documentary evidence and credibility assessments of a small number of witnesses. The evidence may consist of the word of an officer against the word of one or several incarcerated individuals—which raise credibility challenges that are difficult to overcome.⁷¹⁰ For example, in two arbitration decisions we reviewed, the arbitrator found the incarcerated witnesses not credible (one for making a false statement and the

⁷⁰⁷ [Cite Redacted].

⁷⁰⁸ [Cite Redacted].

⁷⁰⁹ [Cite Redacted].

⁷¹⁰ [Cite Redacted].

other for making inconsistent statements), and therefore declined to consider any of their testimony; ultimately, the arbitration found in favor of the accused officers in both cases.⁷¹¹

Another challenge is that BLR must rely on the evidence from the OSI investigation, so a missed interview or an equivocating report can sink the case in arbitration.⁷¹²

Compounding these evidentiary challenges, we reviewed cases in which arbitrators cited extenuating circumstances in limiting punishment despite findings of wrongdoing. In one case, after finding an officer wrongly used high-impact force multiple times, the arbitrator did not impose termination because the officer had a clean record and a prior commendation for taking life-saving measures to help another incarcerated individual.⁷¹³ Similarly, for an officer who was found to have lied in connection with the murder of Mr. Brooks, the arbitrator considered that the officer was contrite and “otherwise exemplary,” and therefore was “to be afforded this one grave mistake.”⁷¹⁴

These kinds of evidentiary and other challenges may act as a barrier to officer accountability and might explain why DOCCS agrees to so many disciplinary settlements (often for settlements less than the originally sought discipline).

Promisingly, DOCCS investigators noted that increased use of stationary cameras and BWC devices assist them in investigating and proving alleged misconduct,⁷¹⁵ so increasing camera usage will likely contribute to better accountability over time.

⁷¹¹ [Cite Redacted].

⁷¹² [Cite Redacted].

⁷¹³ [Cite Redacted].

⁷¹⁴ [Cite Redacted].

⁷¹⁵ [Cite Redacted].

8.3.3 Discipline Is Rarely Meaningful

Even when discipline is successfully imposed, the punishment often lacks efficacy and does not deter misconduct.

For example, many punishments involve a monetary fine—even in cases of relatively serious misconduct—and these financial penalties might even be split among employees. We reviewed one example of an officer who was disciplined after being found guilty of excessive force: the officer swiped the legs out from under an incarcerated individual and pushed the individual face-first into the floor. The officer incurred a \$1,000 fine.⁷¹⁶ In another example, an officer used excessive physical force and lied about the justification for the force in a Use of Force Report; that officer also received a \$1,000 fine.⁷¹⁷

In some cases, officers may receive the money to cover disciplinary fines through officer-promoted fundraisers. For example, we were made aware of fundraisers to cover disciplinary fines imposed from excessive-force incidents (*see* Section 2.2, “Mid-State Prior Incidents”), in violation of Directive #2780, which prohibits any individual from “conduct[ing] fundraising and/or solicitation activities on Department premises for the benefit of an individual.”⁷¹⁸ In addition, we heard in interviews that at least one of the unions has a “rainy day fund” used to pay officers who are out on disciplinary suspension.⁷¹⁹ These actions diminish the effects of discipline and undermine efforts toward accountability.

We also identified the prevalence of same-or-similar DEPs (i.e., disciplinary evaluation periods, during which DOCCS can impose additional discipline if the officer engages in conduct

⁷¹⁶ [Cite Redacted].

⁷¹⁷ [Cite Redacted].

⁷¹⁸ [Cite Redacted]. *See* Directive #2780, §1 (Nov. 19, 2024) (“No individual or unit of the Department shall conduct fundraising and/or solicitation activities on Department premises for the benefit of an individual”).

⁷¹⁹ [Cite Redacted].

that is the “same or similar” to the initial misconduct) as a barrier to meaningful discipline. Several DOCCS stakeholders singled out same-or-similar DEPs as a challenge to implementing discipline when officers demonstrate repeated instances of misconduct.⁷²⁰ We identified several instances in which same-or-similar DEPs were imposed in cases of serious misconduct. In one instance, an officer who struck an incarcerated individual in the head and face 14 times with closed-fist strikes received a two-week suspension and a twelve-month same-or-similar DEP.⁷²¹ And in another case, an officer who repeatedly kicked, stomped, and stood on an incarcerated individual who was lying on the floor, and then lied on the Use of Force Report and to OSI investigators, received a five-day suspension, a \$6,000 fine, and a twelve-month same-or-similar DEP.⁷²² And one of the officers allegedly involved in the killing of Mr. Nantwi was on two separate same-or-similar DEPs at the time of the incident.⁷²³ He received the first same-or-similar DEP due to his involvement in a physical assault of a group of incarcerated individuals at Mid-State.⁷²⁴ Part of the officer’s settlement agreement included a twelve-month same-or-similar DEP in mid-2024—less than a year before he allegedly participated in the killing of Mr. Nantwi.⁷²⁵ Months later, this officer also had an altercation with a romantic partner at Mid-State.⁷²⁶ He received a second same-or-similar DEP for that incident.⁷²⁷ Had the original same-or-similar DEP been a general DEP, the incident with the romantic partner could have been sufficient to terminate his DOCCS employment, and he may not have been a DOCCS employee

⁷²⁰ [Cite Redacted].

⁷²¹ [Cite Redacted].

⁷²² [Cite Redacted].

⁷²³ [Cite Redacted].

⁷²⁴ [Cite Redacted].

⁷²⁵ [Cite Redacted].

⁷²⁶ [Cite Redacted].

⁷²⁷ [Cite Redacted].

at the time of Mr. Nantwi’s killing.⁷²⁸ Indeed, the correction officer was still under both same-or-similar DEPs at the time of Mr. Nantwi’s death, making these prior incidents missed warning signs.⁷²⁹

DOCCS’ current methods of discipline are insufficient to meaningfully and effectively deter misconduct by officers. To make discipline more impactful, we recommend that DOCCS take steps to ensure that officers returning after disciplinary actions or on DEPs are not placed into high-risk assignments (e.g., special housing, emergency response, strip searches, planned uses of force, transport, or housing units). We acknowledge that this may require legislative changes or changes to the Union Agreements. Relatedly, although DOCCS made reforms to the training, deployment, and monitoring of the CERT team in the wake of Mr. Nantwi’s death (see Section 1.8, “CERT”), DOCCS should also continue to develop and implement policies to ensure that security staff returning after disciplinary actions or on DEPs are not placed on CERT deployments for a period of time proportionate to the severity of the misconduct.

8.3.4 Attempts to Reform DOCCS Staff Discipline

Change is necessary to address deficiencies in imposing meaningful staff discipline; otherwise, under the Union Agreements, DOCCS’ hands are tied. The NYSCOPBA and Council 82 union contracts are up for renegotiation in 2026, and DOCCS should make it a priority to improve accountability through the staff discipline process.

Efforts at reforming this staff discipline system are underway in New York. In January 2025, New York State Senator Julia Salazar proposed N.Y. Senate Bill S1671, which would, among other things, establish a new disciplinary procedure for allegations of “serious misconduct” outside the scope of the Union Agreements. If passed, the new law would grant the

⁷²⁸ [Cite Redacted].

⁷²⁹ See Section 2.2, “Mid-State Prior Incidents” for a detailed description of this incident.

DOCCS Commissioner final decision-making authority in cases of “serious misconduct,” which would include any excessive use of force (not just “malicious excessive force”), sexual abuse, false reporting of excessive use of force, intentional failure to report excessive use of force, introduction of contraband (e.g., cell phones, drugs, etc.) into facilities, and inappropriate sexual relationships with incarcerated individuals or those under supervision.⁷³⁰ For these cases, a designated hearing officer would conduct a hearing and make a recommendation to the Commissioner, who would then make a final decision on discipline; there would be no immediate right to arbitration.⁷³¹ Employees could appeal by filing suit in court.⁷³² This new procedure would align the disciplinary process for correction officers with the process that already exists for non-security staff.⁷³³ Though they did not endorse any specific legislation, BLR leadership agreed that the changes must be made to the current process to adequately address and deter misconduct.⁷³⁴

In addition, the law would permit security staff involved in disciplinary proceedings for serious misconduct to be suspended without pay during the pendency of proceedings.⁷³⁵ As noted, today, DOCCS cannot suspend security staff without the employee being charged with a crime or without probable cause that the employee’s continued presence on the job would severely interfere with DOCCS’ operations (a determination that is reviewable by an arbitrator).⁷³⁶ Last, the bill would prohibit security staff removed for serious misconduct from

⁷³⁰ N.Y. Senate Bill S1671, <https://www.nysenate.gov/legislation/bills/2025/S1671>; N.Y. Senate Bill S1671, Section 1 (§12.1) <https://www.nysenate.gov/legislation/bills/2025/S1671>.

⁷³¹ N.Y. Senate Bill S1671, §1 (§12.3-4), <https://www.nysenate.gov/legislation/bills/2025/S1671>.

⁷³² N.Y. Senate Bill S1671, §1 (§12.6), <https://www.nysenate.gov/legislation/bills/2025/S1671>.

⁷³³ N.Y. Senate Bill S1671, §2, <https://www.nysenate.gov/legislation/bills/2025/S1671>.

⁷³⁴ [Cite Redacted].

⁷³⁵ N.Y. Senate Bill S1671, §1 (§12.5) <https://www.nysenate.gov/legislation/bills/2025/S1671>.

⁷³⁶ Union Agreements §8.4(a)(1)-(2).

maintaining civil service eligibility for purposes of future employment and benefits.⁷³⁷ In May 2026, the bill passed in the New York Senate’s Crime Victims, Crime and Correction Committee; as of June 2026, the bill is under consideration in the Civil Service and Pensions Committee.⁷³⁸

While we do not endorse any specific legislation, we recommend that DOCCS strengthen its discipline system by granting the Commissioner explicit authority to discipline or terminate employees for serious misconduct. This authority should provide for a transparent process with documented rationale, ensuring fair and consistent outcomes across the organization. Such reforms could be achieved through legislative action—like the pending bill—or through negotiated amendments to the Union Agreements with security staff.

Timely and meaningful discipline is essential to address individual misconduct and deter future violations, and enhanced tracking and oversight of informal discipline can also contribute to more effective accountability. The shortcomings in DOCCS’ disciplinary process—which include a lack of meaningful consequences—underscore the need for a complementary system of oversight.

8.4 Recommendation Summary

- 8.4.1 Prioritize hiring for new positions in BLR to more effectively manage review workload.
- 8.4.2 Implement a centralized process to document and log instances of informal interventions for facility staff. Use this data to identify staff with multiple instances of informal interventions, analyze underlying trends, and inform targeted interventions or additional training, thereby supporting a culture of continuous improvement.
- 8.4.3 Continue to develop and implement policies to ensure that security staff returning after disciplinary actions or on disciplinary evaluation periods are not placed on Corrections Emergency Response Team (CERT)

⁷³⁷ N.Y. Senate Bill S1671, §3 <https://www.nysenate.gov/legislation/bills/2025/S1671>.

⁷³⁸ N.Y. Senate Bill S1671, §1 (§12.5) <https://www.nysenate.gov/legislation/bills/2025/S1671>.

deployments for a period of time proportionate to the severity of the misconduct. DOCCS should also ensure that security staff returning after disciplinary actions or on DEPs are not placed into other high-risk assignments (e.g., special housing, emergency response, strip searches, planned uses of force, transport, or housing units). We acknowledge that this recommendation would require a legislative change or changes to the collective bargaining agreements with security staff.

- 8.4.4 Grant the Commissioner explicit authority to discipline or terminate any employee for serious misconduct. This authority should encompass a transparent process with documented rationale, ensuring fair and consistent outcomes across the organization. We acknowledge that this recommendation would require a legislative change or changes to the collective bargaining agreements with security staff.

9. INCARCERATED GRIEVANCE PROGRAM (IGP)

DOCCS' grievance process—known as the IGP—faces persistent challenges concerning timeliness, oversight, and consistency with overall accountability standards. We examined the IGP, with a focus on Marcy and Mid-State, because of its importance to a well-functioning prison system. The IGP is designed to serve as a formal process for incarcerated individuals to submit concerns and seek timely resolution regarding facility conditions. Issues addressed through this program include the denial of necessities such as food, toiletries, and medical care, as well as matters related to discrimination and staff conduct.

A functioning grievance system serves several purposes. In addition to providing individual remedies, the IGP is a pressure relief valve that takes the temperature of a particular facility or housing unit. An effective grievance system can thus prevent addressable individual issues from escalating into tensions that broadly impact the secure and orderly running of the facility. A grievance system that does not function properly hampers DOCCS' ability to identify and effectively redress problematic behaviors or trends.

In addition, the IGP, along with OSI, is a key DOCCS reporting mechanism. Incarcerated individuals must know what the grievance system can do and how to access it. Grievance submission mechanisms must be secure and allow for timely filing, investigation, response, and appeal—all steps that must be tracked in real time. Ultimately, incarcerated individuals must trust that the system works; otherwise, there is little incentive to report in the first place, which ultimately deprives DOCCS of the information it needs to identify issues and track problematic trends.

Finally, effective oversight of the IGP requires periodic audits, along with integrated accountability measures, to help DOCCS identify breakdowns early and recommend ongoing steps to ensure the grievance program is functioning efficiently and effectively.

9.1 The Grievance Process

Directive #4040 provides that the IGP should be an “orderly, fair, simple, and expeditious” means of resolving problems.⁷³⁹ The IGP involves three steps. Individuals first submit a grievance form within 21 calendar days of an alleged occurrence to the Incarcerated Grievance Resolution Committee (IGRC), which consists of two peer-elected incarcerated individuals, two staff members, and one non-voting chairperson.⁷⁴⁰ The IGRC either facilitates an informal resolution or, if that fails, conducts a hearing within 16 days at which the grievant appears, after which the IGRC may accept, deny, or dismiss the grievance.⁷⁴¹ For incarcerated individuals in special housing units, the IGRC may make a decision without an in-person appearance by the grievant after an investigation.⁷⁴² Second, the grievant may appeal the IGRC’s decision to the facility superintendent.⁷⁴³ Finally, the grievant may appeal the superintendent’s decision to the Central Office Review Committee (CORC), which comprises DOCCS’ central office staff who review grievance appeals; DOCCS policies require CORC to make a decision and inform the grievant within 30 calendar days from the time it receives an appeal.⁷⁴⁴ In addition, Directive #4040 requires “Code 49” grievances—which cover allegations of staff harassment, discrimination, and misconduct during frisk searches—to be expedited for a decision by the superintendent in the first instance.⁷⁴⁵

⁷³⁹ Directive #4040, §701.1 (Jan. 20, 2016); *see also* N.Y. Comp. Codes R. & Regs. tit. 7, §701.1(a) (2025).

⁷⁴⁰ Directive #4040, §§701.4, 701.5(a) (Jan. 20, 2016).

⁷⁴¹ Directive #4040, §701.5(b) (Jan. 20, 2016).

⁷⁴² Directive #4040, §701.5(b) (Jan. 20, 2016).

⁷⁴³ Directive #4040, §701.5(c) (Jan. 20, 2016).

⁷⁴⁴ Directive #4040, §701.5(d)(3) (Jan. 20, 2016); *see also* N.Y. Comp. Codes R. & Regs. tit. 7, §701.5(d)(3)(ii).

⁷⁴⁵ Directive #4040, §§701.8, 701.9, 701.10 (Jan. 20, 2016).

9.2 Limitations in the Grievance Process

The review identified several limitations to the effectiveness of the grievance process. While this three-tiered process is consistent with prevailing correctional practices, we found that superintendent-level reviews at both Marcy and Mid-State are frequently limited to clerical or procedural checks without a qualitative evaluation.⁷⁴⁶ This narrow approach deprives the grievant of a meaningful superintendent review and inhibits the facilities' ability to identify systemic concerns, ensure consistency in outcomes, and demonstrate fairness in handling complaints. Instead, a more comprehensive review of grievance appeals should include the incorporation of BWC reviews; cross-referencing against related misconduct and trends; investigating the most grieved issues, involved staff, and locations; and ensuring that all issues raised in the initial grievance are sufficiently addressed. Today, the process contemplates this review at the superintendent level, but this may also be a task well suited for the proposed DOCCS Chief Risk function.

The grievance system also suffers from systemic delays. Although IGP Supervisors reported minimal issues with delivering timely responses at the facility level,⁷⁴⁷ many incarcerated individuals stated that facilities either failed to respond to their grievances or provided untimely responses—an issue that some facility leadership acknowledged.⁷⁴⁸ Reviews of grievances and interviews with grievance staff confirmed persistent tardiness in facility-level response rates.⁷⁴⁹ For example, from January 2023 through May 2025, 25.8% of IGRC hearings at Marcy were conducted after the 16-day limit required under Directive #4040, with 7.9%

⁷⁴⁶ [Cite Redacted].

⁷⁴⁷ [Cite Redacted].

⁷⁴⁸ [Cite Redacted].

⁷⁴⁹ [Cite Redacted].

exceeding 60 days.⁷⁵⁰ More broadly, in a 2022 analysis of the grievance system across 41 DOCCS facilities, CANY found that 46.9% of grievances heard by the IGRC exceeded the 16-day time limit.⁷⁵¹ Our review also revealed significant delays at the CORC level, including appeals that stalled for more than two years.⁷⁵² For example, in one instance, an appeal from the superintendent’s decision at Mid-State was filed at CORC on March 7, 2023; the CORC decision was made on February 8, 2024, almost one year later, and the facility was notified of CORC’s decision on August 26, 2025, more than two years after the appeal was initially filed.⁷⁵³ Such delays diminish confidence in the system and fall short of both DOCCS policies and ACA standards.⁷⁵⁴

9.3 Marcy Code 49 Grievance Review

As part of the review, we analyzed 98 Code 49 grievances—which cover allegations of staff harassment, discrimination, and misconduct during frisk searches—filed at Marcy between 2020 and 2024.⁷⁵⁵ Ten of the 98 grievances fully or partially concerned allegations of sexual abuse and were reclassified as Prison Rape Elimination Act (PREA) complaints, per DOCCS policy, and therefore not part of this review.⁷⁵⁶ The review sought to identify patterns, such as inconsistent documentation of injuries, indicia of independence and objectivity, timeliness, and failure to review available evidence. For each grievance, we analyzed the alleged conduct, involved officers, alleged and documented injuries, the facility’s investigative steps (including

⁷⁵⁰ [Cite Redacted].

⁷⁵¹ CANY, Administrative Data Findings from New York State’s Incarcerated Grievance Program §4.1 (June 17, 2024), <https://www.correctionalassociation.org/grievance-data>.

⁷⁵² [Cite Redacted].

⁷⁵³ [Cite Redacted].

⁷⁵⁴ American Correctional Ass’n., *Performance-based Standards & Expected Practices for Adult Correctional Institutions*, Standard 5-ACI-3D-19 (5th ed. 2001).

⁷⁵⁵ [Cite Redacted].

⁷⁵⁶ Directive #4040, §701.3(i) (Jan. 20, 2016).

review of evidentiary materials), dispositions, and any appeals (including additional investigative steps and dispositions).

The review identified several inconsistencies, in practice, with DOCCS policy. For example, although Directive #4040 requires Code 49 grievances to be expedited for a decision by the superintendent in the first instance, we identified six that were first addressed by the IGRC.⁷⁵⁷ Out of the 88 grievances reviewed, all but one were denied in the first instance; among the reasons for denial were, for example, technical errors (like the grievant requesting impermissible relief), mootness (like the grievant being transferred), and the officer denying misconduct (discussed in more detail below).⁷⁵⁸ Forty-two of those denied grievances were subsequently appealed to CORC, resulting in 36 decisions, none of which sustained the grievance.⁷⁵⁹ The database did not include decisions for the other six that were sent to CORC.⁷⁶⁰

The grievance responses were also largely untimely. In 70 of the 88 cases, the length of time between the grievance filing and the initial facility-level decision exceeded 16 days.⁷⁶¹ And in 52 of those cases, the length of time between the grievance filing and the initial facility-level decision exceeded 30 days.⁷⁶² Cases that were appealed to CORC also experienced delay. The length of time between CORC's receipt of an appeal and CORC's decision ranged from about eleven days (mooting a grievance where the incarcerated individual had been transferred) to two years (halfway through which the grievant was released to community supervision).⁷⁶³

⁷⁵⁷ [Cite Redacted].

⁷⁵⁸ [Cite Redacted].

⁷⁵⁹ [Cite Redacted].

⁷⁶⁰ [Cite Redacted].

⁷⁶¹ [Cite Redacted].

⁷⁶² [Cite Redacted].

⁷⁶³ [Cite Redacted].

We also identified some recurring issues in Code 49 grievance investigations. First, a significant number of the investigations into these 88 grievances relied on non-descript statements from the involved officers merely denying the grievant’s allegations.⁷⁶⁴ In one of these cases, the officer accused of misconduct even conducted the grievance investigation.⁷⁶⁵ In several other cases, investigation files failed to document any outreach to witnesses identified in the grievance or named by the grievant; even if these investigative steps occurred, they were not documented.⁷⁶⁶ These investigatory practices are especially notable because, on appeal, grievance decisions were commonly upheld based solely on an analysis of the initial investigation.⁷⁶⁷

Second, we identified a pattern of denying grievance claims, without trying to address the reported fear, discomfort, or need. For example, in several of the Code 49 grievances reviewed, the incarcerated individual reported a fear for their health or safety because of allegations about officer abuse or harassment.⁷⁶⁸ Rather than engage with that reported fear, the grievance dispositions would often cite to the accused officer’s statements merely denying wrongdoing or to medical records showing little to no injury.⁷⁶⁹ This, of course, would do nothing to remedy the fear reported by the incarcerated individual—much less any fear of resulting retaliation. In several other cases alleging assault, investigation files lacked sufficient medical documentation to indicate whether the allegation had been fully considered.⁷⁷⁰ For example, the investigation file for a grievance alleging injuries from a use of force that resulted in outside hospital treatment

⁷⁶⁴ [Cite Redacted].

⁷⁶⁵ [Cite Redacted].

⁷⁶⁶ [Cite Redacted].

⁷⁶⁷ [Cite Redacted].

⁷⁶⁸ [Cite Redacted].

⁷⁶⁹ [Cite Redacted].

⁷⁷⁰ [Cite Redacted].

did not include any documentation from the hospital and only relied on statements by DOCCS nurses; that grievance was denied.⁷⁷¹ In addition, while numerous investigations into use of force allegations indicated that the investigation had appropriately reviewed Use of Force Reports and Unusual Incident Reports, as well as available video footage, other investigatory files failed to document the availability of BWC footage requested by the grievant or indicated that the facility had failed to preserve the footage in time.⁷⁷²

9.4 Incarcerated Individuals’ Perceptions of the IGP

Surveys completed by incarcerated individuals at Marcy and Mid-State show that the majority of individuals who reported filing a grievance at those facilities described the process as “Very Poor/Ineffective.”⁷⁷³ (see Figure 33). A majority of respondents from Marcy and Mid-State also responded that reporting mechanisms—including grievances, OSI complaints, and the ILC—were “Very Poor/Ineffective” for making meaningful changes or improvements.⁷⁷⁴ (see Figure 34).

Figure 33 Incarcerated Individual Survey Result

“If you have filed a grievance while housed in this facility, how would you rate the overall grievance process?”						
Facility	Very Poor / Ineffective	Poor	Satisfactory	Very Satisfactory / Effective	Never Filed / Don’t Know	Total
Marcy	48.5%	7.8%	2.6%	0.3%	40.7%	100.0%
Mid-State	45.1%	8.7%	2.2%	1.1%	42.9%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

⁷⁷¹ [Cite Redacted].

⁷⁷² [Cite Redacted].

⁷⁷³ [Cite Redacted].

⁷⁷⁴ [Cite Redacted].

Figure 34 Incarcerated Individual Survey Result

“Overall, how effective are the [IGP, OSI, Incarcerated Liaison Committee] reporting systems for making meaningful changes or improvements?”						
Facility	Very Poor / Ineffective	Poor	Satisfactory	Very Satisfactory / Effective	Never Filed / Don’t Know	Total
Marcy	54.6%	14.1%	2.9%	2.2%	26.2%	100.0%
Mid-State	54.9%	13.1%	3.3%	2.2%	26.5%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

Though a few interviewed incarcerated individuals discussed positive outcomes, many expressed that the facilities’ failure to respond to grievances and delayed responses have undermined their trust or confidence in the system.⁷⁷⁵ Moreover, former and currently incarcerated individuals—and even a DOCCS executive—reported that fears of staff intimidation or retribution undermine trust in the IGP, which they viewed as being administered with an “us versus them” mentality that discourages staff from substantiating grievances against coworkers.⁷⁷⁶

Incarcerated individuals also reported that grievances go missing.⁷⁷⁷ During one visit to Marcy’s IGP office, we observed that the filing cabinets with grievance folders were not locked.⁷⁷⁸ After facility leadership found that “too many officers” had keys to the grievance offices, access was limited to the area sergeants.⁷⁷⁹ While area sergeants may need access to the IGP office in limited circumstances, such as emergencies or rounds, allowing security staff to have unrestricted access without properly securing grievance records can discourage incarcerated

⁷⁷⁵ [Cite Redacted].

⁷⁷⁶ [Cite Redacted].

⁷⁷⁷ [Cite Redacted].

⁷⁷⁸ [Cite Redacted].

⁷⁷⁹ [Cite Redacted].

individuals from submitting grievances because they may reasonably fear retaliation or harassment. Restricting access to staff members designated for the IGP reduces the risk of tampering with grievances and reinforces the system as legitimate and safe to use, even when grievances involve staff. Indeed, some incarcerated individuals at Marcy acknowledged feeling better about the grievances submitted to executive mailboxes (where the incarcerated population can submit correspondence directly to facility leadership), including because those grievances were more consistently answered.⁷⁸⁰ Nevertheless, as described below, incarcerated individuals might be reluctant to submit paper grievances, altogether, based on security concerns.

9.5 DOCCS Should Modernize the Antiquated Paper-Based System

The grievance program would be vastly improved if it were electronic. DOCCS' grievance system is entirely paper based.⁷⁸¹ DOCCS currently does not permit incarcerated individuals to submit grievances electronically via their tablets, and instead requires that grievances be submitted on paper. This system should be replaced with an electronic system that allows incarcerated individuals to submit grievances on their tablets and/or kiosks and permits DOCCS to process, track, and analyze grievance trends more effectively. A comprehensive grievance database would enable DOCCS to track trends, pinpoint facilities or issues needing intervention, deploy resources to known or persistent risks and vulnerabilities, and proactively address violence-related or serious grievances. Because transitioning to an electronic grievance system will require significant investment and will be a powerful source of trend data, this will likely be best implemented as part of the broader Chief Risk function discussed in this report.

⁷⁸⁰ [Cite Redacted].

⁷⁸¹ [Cite Redacted].

Incarcerated individuals at Marcy and Mid-State explained that obtaining grievance forms can be difficult.⁷⁸² During our site visits, we observed that many general population housing units at Marcy and Mid-State lacked grievance forms. On May 1, 2025, we checked for grievance forms in three of Marcy’s general population housing units; all three lacked grievance forms.⁷⁸³ On August 20-21, 2025, we checked for grievance forms in all 14 of Marcy’s general population housing units and found that half lacked grievance forms.⁷⁸⁴ On September 4, 2025, we checked for grievance forms in Mid-State’s 24 general population housing units and found that at least 14 lacked grievance forms.⁷⁸⁵ These findings are consistent with a 2023 CANY report that reviewed the entire DOCCS grievance system. That report found that “54% of incarcerated individuals said that they cannot obtain grievance forms when they need them.”⁷⁸⁶

Directive #4040 requires that grievances be submitted on grievance forms, except that plain paper may be used if the form “is not readily available.”⁷⁸⁷ Marcy and Mid-State personnel confirmed that they do not reject grievances submitted on regular paper.⁷⁸⁸ This is not ideal, and not having access to the form can result in grievances being overlooked or lost.

Even where individuals can obtain forms, the collection of completed forms poses challenges. At Marcy, we observed grievance boxes in the mess hall and outside the IGP Supervisor’s office,⁷⁸⁹ which the IGP Supervisor reported were checked, respectively, two times

⁷⁸² [Cite Redacted].

⁷⁸³ [Cite Redacted].

⁷⁸⁴ [Cite Redacted].

⁷⁸⁵ [Cite Redacted].

⁷⁸⁶ CANY, “Smoke Screen”: *Experiences with the Incarcerated Grievance Program in New York State Prisons*, p. 16 (Oct. 2023), https://static1.squarespace.com/static/62f1552c1dd65741c53bbcf8/t/651ec66e5505c5122ed0154a/1696515700783/CANY_GrievanceReport_2023Oct.pdf.

⁷⁸⁷ Directive #4040, §701.5(a) (Jan. 20, 2016).

⁷⁸⁸ [Cite Redacted].

⁷⁸⁹ [Cite Redacted].

per week (mess hall) and daily (outside the office).⁷⁹⁰ By contrast, Mid-State appeared to suffer major challenges to grievance collection. For example, two incarcerated individuals at Mid-State stated that the designated grievance boxes cannot be reliably used because people—whom they believe to be officers—have historically tampered with them by dumping cigarettes and coffee grounds into them; a Mid-State staff member acknowledged that trash is often placed in the grievance boxes.⁷⁹¹ Another group of incarcerated individuals informed us that one of the boxes inside the mess hall—the designated submission location for half of the general population—had been broken and unusable for eight months, which we confirmed.⁷⁹² When we notified members of the leadership team at Mid-State during our September 3-4, 2025 site visit, they neither denied it nor offered to address the situation.⁷⁹³ On September 22, 2025, we raised the issue again.⁷⁹⁴ Mid-State leadership acknowledged that the box was broken and that repairs could be finished within the week, showing that the facility had allowed the issue to persist despite the availability of a quick fix.⁷⁹⁵

Beyond the mess hall, we observed common areas at Mid-State that had no designated grievance box, as well as grievance boxes that were unusable due to liquid being in the box.⁷⁹⁶ In light of all of these challenges, we observed that intra-facility mail was the most common way the IGP Supervisor at Mid-State received grievances, and grievance clerks stated that

⁷⁹⁰ [Cite Redacted].

⁷⁹¹ [Cite Redacted].

⁷⁹² [Cite Redacted].

⁷⁹³ [Cite Redacted].

⁷⁹⁴ [Cite Redacted].

⁷⁹⁵ [Cite Redacted].

⁷⁹⁶ [Cite Redacted].

incarcerated individuals in the general population may bring their grievances directly to the grievance office, by sliding the form under the door of the grievance office.⁷⁹⁷

These challenges can discourage incarcerated individuals from utilizing the grievance boxes.⁷⁹⁸ This observation is consistent with the 2023 CANY grievance report, which noted that 45% of incarcerated individuals reported problems accessing the drop box systemwide.⁷⁹⁹ Until DOCCS can adopt an electronic grievance system, it is important to at least make interim improvements to the paper system. For example, DOCCS should, at a minimum, install secure grievance boxes in living areas and common areas, such as the mess hall, and limit access to these boxes to the IGP Supervisor and designated IGP staff.⁸⁰⁰

9.6 DOCCS Should Regularly Revise and Clarify IGP Policies And Handbooks

DOCCS can also update its policies and guidance to incarcerated individuals about how to use the grievance process. In surveys, 72.4% of the 361 incarcerated individual respondents at Marcy and 76.2% of the 440 incarcerated individual respondents at Mid-State reported that their respective facilities do a “Poor” or “Very Poor” job of informing or educating about the grievance or complaint processes.⁸⁰¹ (see Figure 35). For a grievance system to function effectively, incarcerated individuals must receive clear information about their right to submit grievances and have accessible means to do so.

⁷⁹⁷ [Cite Redacted].

⁷⁹⁸ [Cite Redacted].

⁷⁹⁹ CANY, “Smoke Screen”: *Experiences with the Incarcerated Grievance Program in New York State Prisons*, p. 16 (Oct. 2023), https://static1.squarespace.com/static/62f1552c1dd65741c53bbcf8/t/651ec66e5505c5122ed0154a/1696515700783/CANY_GrievanceReport_2023Oct.pdf.

⁸⁰⁰ See e.g., Commonwealth of PA DOC, Policy Statement: Inmate Grievance System, #DC-ADM 804, p. 1-5 (May 1, 2015).

⁸⁰¹ [Cite Redacted].

Figure 35 Incarcerated Individual Survey Result

“How well does this facility inform or educate you about processes for reporting grievances or complaints?”						
Facility	Very Poor	Poor	Somewhat Clear	Clear	Very Clear	Total
Marcy	46.9%	25.5%	19.5%	4.4%	3.8%	100.0%
Mid-State	56.7%	19.5%	17.4%	2.9%	3.4%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

While DOCCS provides some guidance through Directive #4040 and the Orientation Handbook, which incarcerated individuals receive upon admission to a facility, these resources lack clarity and consistency.⁸⁰² Directive #4040 outlines the grievance process, defines terminology, and emphasizes informal resolution.⁸⁰³ The Orientation Handbook similarly explains the IGP and describes what a grievance is, who participates in resolutions, and how to request a visit to the IGP office.⁸⁰⁴ However, the Orientation Handbook omits critical details, including how to obtain and submit grievance forms.⁸⁰⁵ Orientation sessions for incoming incarcerated individuals at Marcy and Mid-State additionally outline the IGP, though Mid-State could not confirm if the limited orientation for those transferred to the SDP includes IGP information.⁸⁰⁶

The materials are also inconsistent. For example, while the Orientation Handbook states that a grievance must be submitted within 14 days of the alleged occurrence, Directive #4040 provides up to 21 days.⁸⁰⁷ Directive #4040 also needs revision on what constitutes a grievance: it notes that a letter addressed to facility leadership is not a grievance, yet it also states that

⁸⁰² [Cite Redacted].

⁸⁰³ Directive #4040 (Jan. 20, 2016).

⁸⁰⁴ [Cite Redacted].

⁸⁰⁵ [Cite Redacted].

⁸⁰⁶ [Cite Redacted].

⁸⁰⁷ Compare [Cite Redacted] with Directive #4040 (Jan. 20, 2016).

grievances do not need to be submitted on a grievance form.⁸⁰⁸ The Orientation Handbook should align with an updated version of Directive #4040, as these current discrepancies create confusion and invite unresolvable grievances, which ultimately undermine confidence in the grievance system and in DOCCS' accountability as a whole.

Finally, best practice would be to implement annual updates to both the Orientation Handbook (last updated in 2024) and Directive #4040 (last updated in 2016) to incorporate supplemental memoranda, ensure staff follow current practices, and provide the incarcerated population with current information.⁸⁰⁹ For example, a new DOCCS-wide procedure requiring written notification of receipt to be provided to grievants when their grievance is filed has been in effect since January 2, 2023, but has not been incorporated into either of the materials.⁸¹⁰ Improving clarity and consistency in these materials would enhance reporting, efficiency, and trust in the IGP.

9.7 DOCCS Should Modernize Grievance System Technology

In the longer term, investing in electronic grievance submission technologies, such as via individual tablets or at facility kiosks, would improve submission, tracking, and processing—which would benefit both the incarcerated population and the IGP Supervisor.

Indeed, staff at Marcy acknowledged that making the system electronic would make the grievance process “so much easier,”⁸¹¹ and incarcerated individuals voiced support for adding grievance-filing capabilities to their individual tablets.⁸¹² DOCCS leadership stated that an

⁸⁰⁸ Directive #4040 §§701.2, 701.5(a) (Jan. 20, 2016).

⁸⁰⁹ [Cite Redacted].

⁸¹⁰ CANY, “Smoke Screen:” *Experiences with the Incarcerated Grievance Program in New York State Prisons*, Appendix F (October 2023) (supplemental memorandum changed grievance practice since Directive #4040 was last updated in 2016); [Cite Redacted]; Directive #4040 (Jan. 20, 2016).

⁸¹¹ [Cite Redacted].

⁸¹² [Cite Redacted].

electronic grievance system would also help reduce concerns of retaliation by facility staff, who, under the current paper-based system, can observe which incarcerated individuals are submitting paper grievances, which is particularly concerning in light of the reports about grievance box tampering.⁸¹³ Although DOCCS leadership stated that they plan to build a system to enable incarcerated individuals to submit grievances on their tablets, they acknowledged that work to begin developing a system for the electronic submission of grievances has not yet begun.⁸¹⁴ We recommend that this effort be accelerated.

An electronic system would also help DOCCS address grievance volume and improve efficiency by restricting duplicate or excessive grievance filings in appropriate cases. As an example of such cases, we were told that, in 2022, four incarcerated individuals at Mid-State filed more than 70 grievances each.⁸¹⁵ Implementing a limit on grievance volume and length based on a standardized, electronic form could help reviewers work more efficiently.

Finally, electronic reporting would promote efficiencies in integrating IGP and OSI reporting mechanisms. Incarcerated individuals who wish to report staff misconduct, sexual abuse, or criminal actions can skip the usual facility-based grievance process and send their complaints directly to OSI.⁸¹⁶ However, as noted earlier, at least one third of OSI's complaints in 2024 were issues that otherwise should have been submitted and addressed via the grievance process, such as commissary complaints or tablet problems.⁸¹⁷ Indeed, many of the issues communicated directly to OSI are referred back to the facility for investigation because they do not rise to the level of an OSI investigation (i.e., failure to receive prescribed medication,

⁸¹³ [Cite Redacted].

⁸¹⁴ [Cite Redacted].

⁸¹⁵ [Cite Redacted].

⁸¹⁶ [Cite Redacted].

⁸¹⁷ [Cite Redacted].

packages, cold cell, etc.).⁸¹⁸ According to a DOCCS executive, OSI received approximately 24,000 complaints in 2024, of which an estimated 8,000 to 9,000 cases could have been addressed by a healthy grievance system at the facility level.⁸¹⁹ This leads to OSI receiving and handling complaints that should go through the grievance program, taking away time and resources that OSI could use for its serious investigations.⁸²⁰ A well-designed system would either stop these kinds of complaints from reaching OSI entirely or promptly redirect them to the IGP. DOCCS can achieve this kind of coordination by improving the IGP system and by technologically integrating the OSI and grievance systems.

Modernization of the IGP system will enable comprehensive early identification and tracking of patterns of misconduct, equipping DOCCS with the information to take timely corrective actions to ensure safety (discussed in further detail at Section 10, “Risk Detection and Mitigation”). DOCCS may also leverage an enhanced recordkeeping system to reduce litigation expenses by efficiently verifying incarcerated individuals’ compliance with exhaustion requirements under the Prison Litigation Reform Act (PLRA).

9.8 DOCCS Should Improve IGP Accessibility, Integrity, and Oversight

DOCCS should also address two other areas to improve IGP accessibility, integrity, and oversight: revisiting the IGP’s reliance on incarcerated grievance clerks and expanding IGP oversight.

9.8.1 Incarcerated Individuals Should Not Serve As Grievance Representatives

Per Directive #4040 and our observations, the IGRC is comprised of two voting incarcerated individuals: a non-voting chairperson who may be an incarcerated individual and a

⁸¹⁸ [Cite Redacted].

⁸¹⁹ [Cite Redacted].

⁸²⁰ [Cite Redacted]; *see also* Section 7, “Office of Special Investigations (OSI).”

non-voting incarcerated clerk.⁸²¹ The assignment of incarcerated individuals as grievance representatives raises significant concerns. Although DOCCS may be inclined to entrust these individuals with review of peers' grievances for constructive reasons, this approach is problematic. It places them in positions of authority over fellow incarcerated individuals, lacks sufficient privacy safeguards, and does not appear to include adequate training. These factors influence the integrity and overall effectiveness of the grievance program.

We also observed that incarcerated individuals predominantly lead IGRC hearings.⁸²² In one instance, an incarcerated individual called the next grievant into the room, read the grievance, explained the process, and answered any questions.⁸²³ On only a few occasions did we observe a staff member offer a clarification or solution, which frequently overlooked more practical remedies or failed to address the grievant's concern.⁸²⁴ For example, we observed an IGRC hearing for a grievant who had not been returned his heart monitor—which he described as life-saving medical equipment due to multiple serious heart conditions and post-operative conditions—after he was transferred from another facility.⁸²⁵ The IGP Supervisor did not offer any meaningful solutions, such as confirming his medical records or authorization forms or contacting the last facility; instead, the IGP Supervisor repeatedly referred the grievant to medical forms, which the grievant had already completed.⁸²⁶

Relying on incarcerated grievance clerks for these roles creates the appearance and/or reality of placing one incarcerated individual in a position of authority over another. It is not best

⁸²¹ Directive #4040, §701.4 (Jan. 20, 2016); [Cite Redacted].

⁸²² [Cite Redacted].

⁸²³ [Cite Redacted].

⁸²⁴ [Cite Redacted].

⁸²⁵ [Cite Redacted].

⁸²⁶ [Cite Redacted].

practice to place incarcerated individuals in an authority position over their peers.⁸²⁷ Doing so risks retaliation and retribution between incarcerated individuals. Furthermore, incarcerated individuals may feel pressure to side with staff.⁸²⁸ In typical correctional practice, a trained grievance officer or investigator independently investigates the grievance and/or interviews the grievant without the involvement of other incarcerated individuals and then responds to the reviewing body.⁸²⁹ We recommend adding civilian and security staff at the facility level dedicated to the grievance program to help maintain its integrity and expedite review.

9.8.2 Adequately Train Incarcerated Grievance Personnel

So long as incarcerated individuals serve as representatives, DOCCS must adequately train them. Though IGP Supervisors reported that they provide initial training for the incarcerated IGRC members,⁸³⁰ there is no annual training for the incarcerated grievance clerks (or, indeed, for the civil IGP Supervisors and grievance investigators).⁸³¹ And without a formal training process in place, training is typically passed from incarcerated individual to incarcerated individual.⁸³² The high turnover rate for IGRC appointments exacerbates issues with a lack of training. We learned of cases of IGRC appointees who were involved for only a short period of time before being released from the facility.⁸³³ In addition, while Directive #4040 requires incarcerated IGRC members to be elected by their peers and meet certain educational requirements, most of the incarcerated representatives at Marcy are appointed based on

⁸²⁷ See Commonwealth of PA DOC Policy Statement: Inmate Grievance System, #DC-ADM 804, p. 1-6 – 1-1-8 (May 1, 2015); Massachusetts DOC, 103 CMR 491.00, Inmate Grievances, at 491.15 (May 5, 2017); [Cite Redacted].

⁸²⁸ [Cite Redacted].

⁸²⁹ Policy Statement: Inmate Grievance System, #DC-ADM 804, p. 1-5, Commonwealth of PA DOC (May 1, 2015); Massachusetts DOC, 103 CMR 491.21, Abuse of the Grievance Process.

⁸³⁰ [Cite Redacted].

⁸³¹ Directive #4040 (Jan. 20, 2016); [Cite Redacted].

⁸³² [Cite Redacted].

⁸³³ [Cite Redacted].

recommendations from existing IGRC members.⁸³⁴ This informal selection method further undermines the integrity of the IGP and risks circumvention of the directive's safeguards.

9.9 Recommendation Summary

- 9.9.1 Evaluate and develop a plan to improve the grievance process and resolution of grievances, including recommendations for technology improvements, additional staffing, and analysis of trends.
- 9.9.2 Enhance processes and controls around the grievance system, including by ensuring the availability and accessibility of separate, secured grievance boxes with access limited to the IGP Supervisor and designee in living areas and common areas.
- 9.9.3 Revise the Orientation Handbook and Directive #4040 to be consistent with each other regarding the time limit to file a grievance and how to file a grievance, along with ongoing annual revisions to incorporate supplemental memoranda.
- 9.9.4 Leverage the plan created from recommendation 9.9.1 (a plan to improve the grievance process) and trends identified through body camera data, and strengthen the grievance system by adding civilian and security staff at the facility level dedicated to the grievance program.
- 9.9.5 Transition to an electronic grievance process, including through tablets and/or kiosks, to enable more efficient tracking, response, identification of "hotspots," and auditing.

⁸³⁴ Directive #4040, §701.4 (Jan. 20, 2016); [Cite Redacted].

10. RISK DETECTION AND MITIGATION

DOCCS' siloed, paper-based systems prevent it from effectively identifying trends and detecting risk. Data collected across various parts of DOCCS, including OSI, grievances, disciplinary records, Use of Force Reports, and BWC footage, can be leveraged more effectively to assess and mitigate risks. Through the creation of a centralized risk function under the leadership of a new CRO, DOCCS can more effectively collect and monitor data across the agency and leverage that data to create heat maps that classify risks at the regional, facility, and individual levels and provide early warnings of interpersonal tensions, staff misconduct, and violent incidents.

10.1 Role of Chief Risk Officer: Enhance Coordination and Oversight

To improve its efforts to identify, track, and effectively mitigate risk, DOCCS should establish the position of CRO. The CRO would be a senior official responsible for proactively identifying, assessing, and addressing risks to both incarcerated individuals and staff. The CRO should evaluate risk using quantitative and qualitative methods—including data across the various DOCCS systems discussed in this section—to develop policies and frameworks to reduce and manage risks, ensure adherence to regulatory requirements and internal standards, communicate risk exposure and mitigation plans to the Commissioner and senior leadership, and prepare an annual report. The CRO should also proactively detect and analyze data across functions to reduce silos among DOCCS departments and manage appropriate resource allocation for corrective actions. Some of these responsibilities presently reside with the OSI and Research departments, so they should be reallocated to the CRO to centralize risk management and free up other departments to focus on immediate needs.⁸³⁵

⁸³⁵ [Cite Redacted].

Under the proposed CRO, DOCCS should expand the scope of data collection and analysis regarding key trends, including by moving away from paper-based systems, by enhancing coordination with and real-time monitoring of facilities, and effectively leveraging existing data streams from OSI complaints, grievance submissions, use of force data, BWC footage, and disciplinary records. At the same time, the CRO should work to develop even more robust datasets to create comprehensive real-time heat maps that track incidents and patterns across regions, individual facilities, and even specific locations and individuals within facilities. DOCCS should regularly review and update these tools to support decision-making and policy adjustments. Ultimately, centralized data analytics should, in turn, develop systemwide early warning systems with the capacity to communicate with correctional facilities in real time.

The following discusses the initial systemwide areas that require enhanced tracking, data analysis, and oversight under a CRO. This is not an exhaustive list, but rather a guide to illustrate some of the key areas that might fall under the purview of a CRO.

10.2 DOCCS' Current Paper-Based System Limits Coordination

Today, communication and coordination between the DOCCS central office and facility leadership are not as timely and effective as needed. In its August 2024 report, the New York State Office of Inspector General found that DOCCS' "antiquated paper-based recordkeeping systems" hinder comprehensive monitoring across all facilities, "which employ divergent and often deficient recordkeeping processes."⁸³⁶ The report indicated that even where the DOCCS

⁸³⁶ New York State Office of Inspector General, *Review of the First Two Years of HALT at the New York State Department of Corrections and Community Supervision*, p. 2 (Aug. 2024), https://www.prisonlegalnews.org/media/publications/NYS_DOCCS_Halt_Report_08-05-24.pdf.

central office could access these divergent systems, manual cross-referencing and verification were necessary.⁸³⁷

We observed these shortcomings across a variety of disciplines both at the DOCCS central office and the facility level. For example, we confirmed that incarcerated individuals' medical and mental health records, incarcerated individuals' grievance forms, staff logbooks, and staff performance evaluations are all archived at the facility level and remain paper based. This makes it impossible for DOCCS' central office to track, much less address, trends in these key correctional benchmarks.

The current process for disseminating new DOCCS directives or facility policies is also antiquated and ineffective. New guidance or rules (from DOCCS or the facility) are read at the start of each shift during staff lineup for up to one week, and then displayed on the lineup room bulletin board.⁸³⁸ This approach to communicating new information inevitably misses some recipients, can lack critical context, and might be subject to misinterpretation.⁸³⁹

DOCCS can also do more to track the implementation and effectiveness of its directives and staff compliance, including by leveraging enhanced technology. With several hundred directives and over 40 prison facilities, as well as community-based operations, it is undeniably difficult to ensure consistent implementation of directives and uniform policy applications across sites.⁸⁴⁰ This vast system of unconnected, paper-based, or otherwise manual records and communications impedes the central office's ability to communicate effectively in real time with facilities and ensure that facilities are complying with new directives.

⁸³⁷ New York State Office of Inspector General, *Review of the First Two Years of HALT at the New York State Department of Corrections and Community Supervision*, p. 10 (Aug. 2024), https://www.prisonlegalnews.org/media/publications/NYS_DOCCS_Halt_Report_08-05-24.pdf.

⁸³⁸ [Cite Redacted].

⁸³⁹ [Cite Redacted].

⁸⁴⁰ Directive #9999 (June 9, 2025).

10.3 Limited Tracking and Oversight Capabilities

DOCCS can also do more to track facility and systemwide trends. Although DOCCS has made recent efforts to improve tracking and analysis capabilities, DOCCS' outdated recordkeeping and communication systems compromise the efficacy of these oversight tools.

For example, DOCCS' central office relies on manual processes for understanding facility programming, including legally required programming in special housing.⁸⁴¹ At one facility, a deputy superintendent sends two daily emails to the central office to confirm the day's offered and accepted programming in a special housing unit.⁸⁴² This is completely inefficient—both for the facility staff, who have to remember to send the email, and DOCCS' central office staff, who receive and archive the email. DOCCS should leverage technology to collect, compile, and analyze this kind of important data in a more powerful and effective manner.

DOCCS has taken some steps to enhance oversight through tracking technology. For example, OSI told us that it has recently piloted efforts aimed at more sophisticated tracking.⁸⁴³ As part of these efforts, OSI has instituted technological tools such as incident location geo-mapping (which uses location-based data to visualize trends) to identify problematic “hotspots,” facilitate interventions by supervisors, and guide camera placements.⁸⁴⁴ OSI is also developing systems for tracking “red-flag” employees.⁸⁴⁵

At the same time, OSI acknowledged the current lack of strong analytical tools for identifying patterns in use of force, contraband incidents, or various types of staff misconduct, and is actively working to address these shortcomings. However, effective risk mitigation and

⁸⁴¹ [Cite Redacted].

⁸⁴² [Cite Redacted].

⁸⁴³ [Cite Redacted].

⁸⁴⁴ [Cite Redacted].

⁸⁴⁵ [Cite Redacted].

identification of trends require more coordination across DOCCS, including by leveraging data collected across the agency.

10.4 DOCCS Should Enhance its Reporting, Tracking, and Analysis of Uses of Force

10.4.1 Update the Use of Force Report Form

DOCCS should also implement enhancements to its use of force reporting processes and analytics. Although DOCCS has an electronic recordkeeping mechanism and a systematic reporting policy in place for uses of force, our review identified critical weaknesses that hinder the potential for proactive and prompt action. DOCCS must ensure that its policies regarding use of force translate into responsible and consistent practice, through oversight measures that proactively identify and appropriately address prohibited conduct, warning signs, and problem areas.

Directive #4944 requires involved officers to report all uses of force.⁸⁴⁶ Officers must “accurately” and “independently” complete the two-page Use of Force Report, including “specific details regarding the incident.”⁸⁴⁷ The report has fields for certain details, such as the time and location of the incident, all involved personnel and incarcerated individuals, and a summary of the medical examination.⁸⁴⁸ Officers then provide, in their own words, an account of the events leading up to the use of force and the actual force used, which typically includes a description of all actions by both the staff and the incarcerated individuals.⁸⁴⁹ Currently, officers typically fill out Use of Force Reports on paper, though the form is also available on computers.⁸⁵⁰

⁸⁴⁶ Directive #4944, §IV (Apr. 28, 2023).

⁸⁴⁷ Directive #4944, §IV (Apr. 28, 2023).

⁸⁴⁸ [Cite Redacted].

⁸⁴⁹ [Cite Redacted].

⁸⁵⁰ [Cite Redacted].

In our review of hundreds of Use of Force Reports from Marcy and Mid-State, we identified several areas for improvement in the design and process for DOCCS’ use of force reporting, which would enhance the integrity of post-incident review and tracking, strengthen oversight, and provide opportunities for corrective action and/or commendation.

One area for improvement is DOCCS’ tracking of demographic data of incarcerated individuals subject to force. Although the facility Use of Force Reports log demographic data, the data is incomplete and inconsistent with DOCCS-wide reporting on demographics and use of force.⁸⁵¹ For instance, the facility-based reports include only Black, Hispanic, and white within a single field.⁸⁵² This conflates race (e.g., Black) and ethnicity (Hispanic) and ignores other racial/ethnic identities (e.g., Asian, Native American, etc.). This limited collection is also inconsistent with DOCCS’ other collection of demographic data: DOCCS-wide reporting on incarcerated population demographics, for example, includes white, Black, Hispanic, Native American, Asian, and Other as racial/ethnic categories.⁸⁵³ These issues hindered our ability to assess whether use of force is applied disproportionately against a certain race or ethnicity. Adding checkboxes or a drop-down menu for racial and ethnic data—harmonized with DOCCS’ other demographic data collections—would help address this issue.

As another example, the current Use of Force Reports make it difficult to reliably track uses of force by location. The report form has three fields for location: “Facility,” “General Location,” and “Specific Location.”⁸⁵⁴ Our review of Use of Force Reports revealed that the officers usually complete the “Facility” section by inputting the facility name (e.g., Marcy) to

⁸⁵¹ [Cite Redacted].

⁸⁵² [Cite Redacted].

⁸⁵³ [Cite Redacted].

⁸⁵⁴ [Cite Redacted].

likely refer to general population areas, but at other times input the type of housing unit (e.g., RMHU).⁸⁵⁵ The “General Location” identifier generally describes the location within the housing unit (such as toilet area, day area, walkways).⁸⁵⁶ The “Specific Location” identifier is commonly left blank, though some reports indicated a specific cube number.⁸⁵⁷ Confusing things further, officers might also include location information in the freeform description of the incident.⁸⁵⁸ These inconsistencies hindered our ability to assess whether there are any location “hotspots” for use of force. Again, adding checkboxes or a drop-down menu for location information would help address this issue.

The Use of Force Report form does not require other key details, such as a description of de-escalation efforts attempted prior to force, any staff interventions, and compliance with BWC capture policies (discussed in additional detail herein). In practice, this means that only some officers include these details in their reports and that they do so inconsistently. Officers should be encouraged to document all efforts at de-escalation, including, for example, active listening, encouraging dialogue, slowing down the encounter, building rapport, creating space, avoiding threatening body language, and using Crisis Intervention Training (CIT), along with religious, mental health, or other wellness resources; again, this might be achieved via checkboxes or other standardized methods of data collection. Not requiring officers to log data relating to de-escalation, intervention, and BWC compliance limits DOCCS’ ability to capture data relating to hallmarks of use of force and stymies the organization’s ability to identify and address any problematic trends. Adding required fields for these kinds of incident details would not only

⁸⁵⁵ [Cite Redacted].

⁸⁵⁶ [Cite Redacted].

⁸⁵⁷ [Cite Redacted].

⁸⁵⁸ [Cite Redacted].

improve DOCCS' data collection abilities, but also reinforce that de-escalation and intervention are key tools for officers involved in uses of force.

To summarize, we recommend that DOCCS revise the Use of Force Report form to include required fields with checkboxes that consolidate and streamline options for key details like the location(s) and demographic data (harmonized with DOCCS' own system of measuring such data), location information, and compliance with BWC policy, along with required fields for de-escalation and intervention efforts (if applicable). The form should add optional comment fields and preserve the text boxes to allow officers to provide details as accurately and comprehensively as possible. Facilitating officers' ability to submit robust information about a use of force—and holding them to account to do so—will create a more extensive range of data to understand patterns, including hotspot locations and trends of officers engaging in best practices.

Importantly, this kind of use of force data must be analyzed, not just filed. Once DOCCS is able to more effectively collect this data, its centralized risk function (under the Chief Risk Officer) should analyze the data to identify and mitigate risks. Indeed, patterns among staff and in specific units may provide early warning signs and allow supervisors to provide coaching, retraining, or reassignment before problems escalate. In addition, “hotspot mapping” by housing area, shift, day, and incident type could guide staffing, camera placement, and supervisory presence. Leaders could also identify trends that may inform decisions on overtime, equipment, and staffing coverage.

We also recommend that use of force trend data, based on this enhanced reporting structure, be periodically published. Most correctional systems publish regular public reports on

use of force trends.⁸⁵⁹ DOCCS does not yet compile and share its facility-level numbers in a centralized, easy-to-use way. An up-to-date public dashboard with monthly, quarterly, and annual reports would turn DOCCS’ incident data into an actual management, transparency, and oversight tool.⁸⁶⁰ In addition, publishing these results in one place would strengthen public trust by illustrating DOCCS’ real-time challenges and trajectory of improvement.

10.4.2 Strengthen the Use of Force Review Protocol

Once Use of Force Reports are logged, DOCCS policy requires that facility supervisors and superintendents review the reports to ensure compliance with DOCCS policies.⁸⁶¹ However, our review identified delays and lapses in this oversight function at Marcy and Mid-State, illustrating that there is room for improvement in facility leadership’s ability—and, by extension, DOCCS’ ability—to monitor officer compliance with DOCCS policies, impose timely corrective action, and ensure the safety of incarcerated individuals and staff.

According to Directive #4944, uninvolved supervisors are the first to review an officer’s Use of Force Report.⁸⁶² Thereafter, the facility superintendent is required, within ten days of the incident, to review the report and determine whether the force used was appropriate.⁸⁶³ The superintendent’s findings—including any corrective actions, disciplinary measures, and/or referrals for further investigation—are added to the Use of Force Report in a section entitled “Review and Evaluation by Superintendent.” If the superintendent determines that the use of

⁸⁵⁹ See, e.g., Office of the Inspector General, *Monitoring the Use-of-force Review Process of the California Department of Corrections and Rehabilitation*, pp. 9-32 (Aug. 2024), <https://www.oig.ca.gov/wp-content/uploads/2024/08/OIG-2023-Use-of-Force-Monitoring-Report.pdf>.

⁸⁶⁰ See, e.g., Wisconsin Dep’t of Corrections, Restrictive Housing Dashboards, Restrictive Housing: Disciplinary Separation and Administrative Confinement Dashboards, <https://doc.wi.gov/Pages/DataResearch/RestrictiveHousingDashboard.aspx> (last accessed Dec. 3, 2025).

⁸⁶¹ Directive #4944, §IV.A.4 (Apr. 28, 2023).

⁸⁶² Directive #4944, §IV.B.1 (Apr. 28, 2023).

⁸⁶³ Directive #4944, §IV.A.4 (Apr. 28, 2023).

force was appropriate, the facility logs the Use of Force Report and there is no further action.⁸⁶⁴ If the superintendent identifies red flags in the review (e.g., has reasonable suspicion about non-compliance with policy), the superintendent must notify the Deputy Commissioner for Correctional Facilities and refer the incident to OSI for further investigation.⁸⁶⁵

In practice, we found that this timeline was rarely met at Marcy and Mid-State. Indeed, one facility superintendent admitted to falling behind on this required review by several months.⁸⁶⁶ Of the Use of Force Reports we reviewed, only 26.4% of reports at Marcy and 4.7% of reports at Mid-State were reviewed by the superintendent within the required ten-day timeline.⁸⁶⁷ At Mid-State, 43.7% of the reports were reviewed by the superintendent between 50 and 100 days after the incident occurred, and 18.6% were reviewed after more than 100 days.⁸⁶⁸ At Marcy, although a much smaller percentage of the reports (8.2%) were reviewed by the superintendent past the 50-day mark, 65.5% of the reports were still reviewed after the ten-day deadline.⁸⁶⁹ Recall, too, that the acting superintendent of Marcy took over two months to review a use of force incident that involved several of the officers involved in the killing of Mr. Brooks; the acting superintendent's review occurred just two days after Mr. Brooks's death.⁸⁷⁰ Some of these untimely reviewed (or never reviewed) incidents involved, for example, the potentially deliberate obstruction of camera recording, excessive chemical agent deployment, officers inexplicably turning off the room lights during post-chemical-agent decontamination, improper transport procedures, deficient de-escalation efforts, documented injuries to the incarcerated

⁸⁶⁴ Directive #4944, §IV (Apr. 28, 2023).

⁸⁶⁵ Directive #4944, §IV.B.2 (Apr. 28, 2023).

⁸⁶⁶ [Cite Redacted].

⁸⁶⁷ [Cite Redacted].

⁸⁶⁸ [Cite Redacted].

⁸⁶⁹ [Cite Redacted].

⁸⁷⁰ See Section 2.1, "Marcy Prior Incidents."

individual that did not match with the force described in the Use of Force Report, and force and injuries to the incarcerated individual that were captured on BWC but not reported in the Use of Force Report or other post-incident documentation.⁸⁷¹

We also identified issues with inconsistent review and reporting of BWC footage. In one instance, the Use of Force Report indicated that available BWC footage of the incident was not reviewed.⁸⁷² For another incident, the Use of Force Report erroneously indicated that there was no available camera footage of the use of force, even though it was captured on BWC.⁸⁷³ Lastly, for another incident, the officer reported grabbing an incarcerated individual when BWC footage clearly showed the officer pushing him.⁸⁷⁴

These lengthy delays and incomplete reviews can cause the degradation of evidence and lost witnesses, which prevent effective investigations. Moreover, late and insufficient reviews can leave officers untrained or uncounseled when they are out of compliance with policy or, even worse, allow abusive officers to continue to interact with incarcerated individuals. We recommend tracking compliance with the ten-day requirement for superintendent review of Use of Force Reports, as well as identifying and documenting facilities that are out of compliance and providing corrective action timeframes. If the ten-day superintendent review timeframe proves unworkable, DOCCS should consider amending the policy to provide a clear and realistic deadline that ensures timely oversight, while maintaining rigorous standards for review and accountability. We also recommend that, in addition to the current chain-of-command review, DOCCS implement a process for outside-facility review of uses of force. DOCCS should

⁸⁷¹ [Cite Redacted].

⁸⁷² [Cite Redacted].

⁸⁷³ [Cite Redacted].

⁸⁷⁴ [Cite Redacted].

implement a multidisciplinary review at the DOCCS agency level for every severe use of force incident. Minor uses of force should be reviewed by an appointed individual, such as a regional administrator or a designated agency-level staff member. This review should include an evaluation of the facility superintendent's own review, the involved officers' adherence to DOCCS' policies, use of BWC, de-escalation and intervention practices, response protocols, and all other pertinent information. In appropriate cases, OSI and law enforcement agencies may request holds on institutional review for cases referred for criminal misconduct. Ultimately, the multidisciplinary review would evaluate more incidents than just those flagged by the superintendent's review.⁸⁷⁵

10.4.3 DOCCS Should Implement a Structured BWC Audit System

BWC footage is another useful tool for oversight and transparency, and DOCCS can do more to reliably collect, study, and learn from data recorded on officers' BWC devices during uses of force.

10.4.3.1 Current DOCCS BWC Policies

Several current DOCCS policies provide for review of BWC. Supervisors are required by Directive #4942 to complete a weekly review of at least one hour of fixed video footage from non-business days and hours.⁸⁷⁶ In addition, following the killing of Mr. Brooks, the Commissioner directed the superintendent, first deputy superintendent, and deputy superintendent for security at each facility to randomly select and review three BWC devices per day and immediately report policy violations to OSI and BLR.⁸⁷⁷ While these reviews are

⁸⁷⁵ See Massachusetts Dep't of Corrections, *Standard Operating Procedure to 103 CMR 505, Use of force*, <https://www.mass.gov/doc/505-sop-use-of-force/download> (MA CMR 505 Admin Review) (May 2025).

⁸⁷⁶ Directive #4942, §V.H.1 (Feb. 21, 2024).

⁸⁷⁷ [Cite Redacted].

designed to ensure policy compliance among security staff, our review found them to often be inconsistent, poorly documented, and not linked to staff performance or corrective action.⁸⁷⁸

DOCCS Directive #4943, which was updated in February 2025 following Mr. Brooks’s murder, requires staff to use BWC during certain events, including interactions with incarcerated individuals or visitors, emergency responses, escorts, transports, weapon use, searches, disciplinary hearings without cameras, and any situation where staff feel unsafe.⁸⁷⁹ The directive also sets retention rules for the footage, which require the footage to be uploaded to secure digital storage and preserved according to a detailed categorization schedule—ranging from one to 20 years.⁸⁸⁰

10.4.3.2 Strengthen Enforcement of BWC Requirements

Despite these policies, our review did not identify any uniform, systemwide protocol for facility or for DOCCS leadership to track adherence to these BWC policies or to effectively review and analyze BWC footage. The use of BWC, and the consistency with which staff activate them across shifts, was difficult to assess with confidence because DOCCS does not track or report failures to activate in a way that allows meaningful shift-by-shift review.⁸⁸¹ Additionally, we observed varying levels of reviewer proficiency with the BWC system, with some struggling to locate incidents or conduct random checks.⁸⁸² Our review identified at least one use of force incident in which the camera view was obstructed during significant parts of the incident due to the placement of the BWC and the incarcerated individual, and no other BWC

⁸⁷⁸ [Cite Redacted].

⁸⁷⁹ Directive #4943 (Feb. 11, 2025).

⁸⁸⁰ Directive #4943, Attachment A (Feb. 11, 2025).

⁸⁸¹ [Cite Redacted].

⁸⁸² [Cite Redacted].

footage was available, despite the involvement of at least one other officer.⁸⁸³ In another instance, a supervisor apparently deliberately pointed the BWC at a wall during the use of force.⁸⁸⁴

The lack of consistent enforcement of the various BWC policies means that officers are not held to account for failing to deploy their BWC and facilities are not held to account for failing to conduct the required BWC review. Without reliable BWC enforcement mechanisms, DOCCS will never be able to effectively collect and analyze the valuable information captured on officers' BWC, and supervisors, superintendents, and other oversight mechanisms will not be able to reliably assess whether an officer's force was proper or reasonable. As such, we recommend that DOCCS institute a uniform, systemwide protocol for supervisors to review the use of BWC to ensure facility-specific compliance with DOCCS' BWC policy, including mandatory written records for any discipline, counseling, coaching, or training resulting from failure to comply with the BWC policy. This protocol should include the steps taken and the disciplinary consequences for failure to comply with the BWC policy, including potential mandatory written records of non-compliance, counseling, coaching, or training. The protocol should also clearly outline responsibilities for managers, timelines for investigation, and requirements for documentation to promote transparency and accountability at every stage. In addition, even where BWC devices are appropriately activated, systematic reviews will be beneficial to identify patterns about how cameras are handled, assess the need for refresher training based on performance, and address problematic trends or misconduct. Strengthening the

⁸⁸³ [Cite Redacted].

⁸⁸⁴ [Cite Redacted].

mechanisms for ensuring that BWC footage is collected and analyzed (as discussed next) will keep staff and incarcerated individuals safer.

10.4.3.3 Enhance Required Analysis of BWC Footage

Like with data gleaned from the Use of Force Reports, information on officers' BWC footage must be reviewed and analyzed with an eye toward identifying, assessing, and mitigating risk. Together with fixed cameras, BWC footage provides a wealth of information about officers' and incarcerated individuals' behavior, including misconduct and compliance with policies—information that can be leveraged to improve facility conditions for both staff and incarcerated individuals and to keep everyone at DOCCS safe.

We recommend that DOCCS establish a structured routine program for auditing camera footage at least annually. The need for a structured and documented camera audit program has been identified nationally, including by the U.S. Department of Justice Office of the Inspector General, which reports on correctional camera practices within the Federal Bureau of Prisons.⁸⁸⁵ This audit should assess compliance with DOCCS directives, including the use of force directive, and enhance systemwide tracking of officer conduct, hotspots, and other weaknesses. The program should specify the scope of audits, define the criteria for selecting footage, identify performance or integrity issues, assess areas of elevated risk, require documentation of findings and any corrective actions taken (using standardized checklists and/or narratives), and compile findings in a centralized log that can be proactively leveraged to address potential problems before they escalate.⁸⁸⁶

⁸⁸⁵ See e.g., DOJ OIG, Management Advisory Memo. No. 22-001, *Notification of Needed Upgrades to the Federal Bureau of Prisons' Security Camera System* (Oct. 2021), <https://oig.justice.gov/sites/default/files/reports/22-001.pdf>; DOJ OIG, *Top Management and Performance Challenges Facing the Department of Justice—2024* (Oct. 10, 2024), <https://oig.justice.gov/sites/default/files/2024-11/TMPC-2024.pdf>.

⁸⁸⁶ [Cite Redacted].

10.5 DOCCS Should Track and Expand Audit Tools and Data Analysis for the IGP

Incarcerated individuals' grievance submissions—along with the existing required audits of those grievances—provide another existing source of data that the CRO can leverage to centrally identify and address risk areas.

Although Directive #4040, which governs the IGP, does not specifically mention an audit process for grievances, we learned that the IGP Director and one civilian staff member conduct an annual audit of the IGP process and practices at each facility through an on-site review of grievance files.⁸⁸⁷ The 2024 audits for Mid-State and Marcy required numerous corrective actions.⁸⁸⁸ They included ensuring that IGRC members annually sign the IGP's code of ethics per Directive #4040; properly closing or dismissing grievances; and maintaining appropriate documentation regarding informal resolutions, among other requirements. When asked about these audits, the IGP supervisors at both Marcy and Mid-State recalled receiving a copy of the audit memos but did not recall them requiring any corrective action.⁸⁸⁹

The audit process, however, is limited. The last audit at Mid-State took less than three hours and did not clearly indicate whether it included interviews of incarcerated individuals.⁸⁹⁰ While an audit can appropriately focus on administrative tasks, it should also review the quality of investigations, training, data trends, corrective actions, information conveyed, and overall compliance with the grievance policy.⁸⁹¹ Upon identifying deficiencies, the audit should also

⁸⁸⁷ [Cite Redacted].

⁸⁸⁸ [Cite Redacted].

⁸⁸⁹ [Cite Redacted].

⁸⁹⁰ [Cite Redacted].

⁸⁹¹ Md. Code Regs. 12.02.28.20 – Audits; Commonwealth of PA DOC Policy Statement, #DC-ADM 804 (May 1, 2015); Massachusetts DOC, 103 CMR 491.22 Evaluation of the Grievance Process.

recommend corrective training.⁸⁹² Currently, there is no identified method for IGP Supervisors or others to track and analyze multiple grievances for trends, hotspots, or a problematic correction officer or staff member.⁸⁹³ This is a missed opportunity to be proactive.

We recommend that DOCCS develop and implement a grievance system plan that also leverages body camera data to strengthen the grievance process and resolution of grievances, including through the addition of civilian and security staff at the facility level dedicated to the grievance program and trend analysis capabilities.

We also strongly recommend transitioning the current paper-based grievance system into an electronic one, including through tablets and/or kiosks, to enable more efficient tracking, response, identification of “hotspots,” and auditing. Together, DOCCS and the prison facilities can collect robust data for the oversight of resolution and review timelines, trend analysis, and hotspots. In turn, risk analysis rooted in grievance data can complement patterns identified from OSI data, allowing for proactive and cross-functional resolutions.

10.6 DOCCS Should Track Facility-Level Discipline and Corrective Action

Finally, as part of its centralized risk function, DOCCS should collect and analyze facility-level data about corrective action and discipline. This includes, for example, data about any logged interventions—like training or counseling. It also includes, critically, existing formal discipline records from BLR.

Together with real-time monitoring of incident data, DOCCS can identify patterns of problematic behavior involving the same officers before they escalate into serious incidents, enabling proactive targeted interventions like retraining or reassignment rather than only reactive

⁸⁹² Commonwealth of PA DOC Policy Statement, #DC-ADM 804 (May 1, 2015); Massachusetts DOC, 103 CMR 491.22; Connecticut Dep’t of Correction Administrative Directive 9.6, p. 5 (Apr. 30, 2021); South Carolina Dep’t of Correction, Policy # GA-01.12 §17, Inmate Grievance System (Sept. 1, 2023).

⁸⁹³ [Cite Redacted].

interventions through formal discipline. Linking discipline with robust oversight ensures that corrective action is not merely reactive but part of a broader strategy to prevent harm, promote accountability, and strengthen a culture of safety across facilities.

In sum, isolated systems at any facility can fail to paint a complete picture of critical patterns. Frequent, excessive, or injurious uses of force may be an indicator of an overzealous, overworked, or abusive officer. They may also be a culmination of long-standing tensions or aggravated dynamics between officers and incarcerated individuals. Therefore, DOCCS should monitor data to proactively identify hotspots, conduct targeted investigations, and consider appropriate corrective action. By developing real-time heat maps, DOCCS and its facilities can have a comprehensive view of trends across use of force incidents and reports, OSI data, staff discipline and corrective actions, grievances, and camera footage. Systematic evaluations of trends will empower DOCCS to implement proactive solutions for intervention that may stop repeat issues and forego tragic outcomes.

In the near term, DOCCS should create a task force to develop a work plan to modernize technology related to maintaining electronic records and leveraging data analytics in the longer term.

10.7 Recommendation Summary

- 10.7.1 Create the position of CRO to oversee a centralized risk function. The CRO would be a senior official responsible for proactively identifying, assessing, and mitigating risks across DOCCS. These include risks to both incarcerated individuals and staff. The CRO should evaluate risk using quantitative and qualitative methods (e.g., demographic dashboard, disciplinary and use of force trends, PREA, federal funding, etc.), develop policies and frameworks to reduce or manage risks, ensure adherence to regulatory requirements and internal standards, communicate risk exposure and mitigation plans to the Commissioner and senior leadership, and prepare an annual report. The CRO should proactively detect and analyze data across functions to reduce silos among DOCCS departments and manage appropriate resource allocation and implementation for corrective actions. Where these responsibilities

presently reside with the OSI and Research departments, they should be reallocated to the CRO.

- 10.7.2 Under the proposed CRO, expand the scope of data collection and analysis regarding trends (e.g., use of force, grievances, OSI, BLR/discipline data) to develop systemwide early warning systems with the capacity to communicate with and correct in real time at the facility level. Implement comprehensive real-time heat maps that track incidents and patterns across regions, individual facilities, and specific locations within facilities, based on Use of Force Reports, OSI data, and BLR data, among other sources. These maps should include data on involved officers, demographic information such as race, and other relevant indicators. Regularly review and update these heat maps to support decision-making and policy adjustments.
- 10.7.3 Transition all records from the current paper-based system to a fully electronic format, including grievances, Use of Force Reports, and medical and mental health documentation.
- 10.7.4 Procure software that can use data already being collected to allow for proactive staff intervention.
- 10.7.5 Revise the Use of Force Report form to require detailed descriptions of compliance with the BWC directive, de-escalation efforts, and other techniques used prior to the use of force (such as active listening, encouraging dialogue rather than confrontation, slowing down the encounter, avoiding immediate force, building rapport, creating space, avoiding threatening body language, and using Crisis Intervention Training, religious, mental health, or other wellness resources). Revisions should also include required fields with checkboxes that consolidate and streamline options for key details like the location(s) and demographic data. The form should add optional comment fields and preserve the text boxes to allow officers to provide details as accurately and comprehensively as possible.
- 10.7.6 Track compliance with the ten-day requirement for superintendent review of Use of Force Reports. Identify facilities that are out of compliance, document, and provide corrective action timeframes. If the ten-day review timeframe proves unworkable, consider amending the policy to provide a clear and realistic deadline that ensures timely oversight while maintaining rigorous standards for review and accountability.
- 10.7.7 Implement a multidisciplinary review at the DOCCS agency level for every severe use of force incident. Minor incidents involving the use of force should be reviewed by an appointed individual, such as a regional administrator or designated agency level staff member. The review

should include an evaluation of the facility superintendent's own review, as well as the involved officers' adherence to policies, use of BWC, de-escalation and intervention practices, response protocols, and all other pertinent information. In appropriate cases, OSI and law enforcement agencies may request holds on institutional review for cases referred for criminal misconduct.

- 10.7.8 Implement a centralized process to document and log instances of informal interventions for facility staff. Use this data to identify staff with multiple instances of informal interventions, analyze underlying trends, and inform targeted interventions or additional training, thereby supporting a culture of continuous improvement.
- 10.7.9 Leverage the plan created from recommendation 9.9.1 (a plan to improve the grievance process) and trends identified through body camera data, and strengthen the grievance system by adding civilian and security staff at the facility level dedicated to the grievance program.
- 10.7.10 Transition to an electronic grievance process, including through tablets and/or kiosks, to enable more efficient tracking, response, identification of "hotspots," and auditing.
- 10.7.11 Establish a structured DOCCS-wide or external audit program for camera footage at least annually as a mechanism to assess compliance with DOCCS directives, including the use of force directive, and to enhance systemwide tracking of officer conduct, hotspots, and other weaknesses. The audit program should define the criteria for selecting footage and require documentation of findings and any corrective actions taken, enabling the organization to proactively identify and address patterns of inappropriate behavior before they escalate.
- 10.7.12 Institute a uniform, systemwide protocol for facility leadership to review the use of BWC to ensure facility-specific compliance with DOCCS' BWC policy. This protocol should include the steps taken and the disciplinary consequences for failure to comply with the BWC policy, including potential mandatory written records of non-compliance, counseling, coaching, or training. The protocol should clearly outline responsibilities for managers, timelines for investigation, and requirements for documentation to promote transparency and accountability at every stage.
- 10.7.13 Create a task force to develop a work plan to modernize technology related to maintaining electronic records and leveraging data analytics in the longer term.

11. CHEMICAL AGENTS

During our review, we also identified potential enhancements to DOCCS' policies and practices relating to the deployment of chemical agents. Directive #4944, which governs the use of force generally and the deployment of firearm and non-firearm weapons specifically, lacks robust tracking and accountability measures for chemical agents and does not provide sufficient guidance on when their use is appropriate. We recommend implementing updated inventory controls and clearer protocols for chemical agents to align DOCCS policy with recognized best practices.

The use of chemical agents at DOCCS has increased significantly since 2015, when all staff became authorized to carry chemical agents.⁸⁹⁴ According to DOCCS data, for example, statewide incidents of chemical agent use rose from 124 instances in 2015 to 4,758 in 2024.⁸⁹⁵ Similar rates of increase were seen at Marcy (seven in 2015 to 185 in 2024, with a peak of 223 in 2022) and Mid-State (four in 2015 to 156 in 2024, with a peak of 182 in 2023).⁸⁹⁶ Just like uses of force, generally, this increased chemical agent use should be viewed in the context of both the decreased incarcerated population and the simultaneous increased proportion of incarcerated individuals convicted of violent crimes.⁸⁹⁷ That said, the increase is still quite stark.

⁸⁹⁴ [Cite Redacted].

⁸⁹⁵ [Cite Redacted].

⁸⁹⁶ [Cite Redacted].

⁸⁹⁷ See, e.g., Elizabeth Johnson, *Trends in the New York State Prison Population, 2008-2023*, DATA COLLABORATIVE FOR JUSTICE, <https://datacollaborativeforjustice.org/wp-content/uploads/2023/07/PrisonPop.pdf> (last visited Feb. 28, 2026) (showing the proportion of the population charged with violent felonies increasing, while the overall population has sharply decreased). Laws like the 2009 Rockefeller Drug Law Reforms, the Bail Reform Act of 2019, and the “Less is More” Act of 2021 all decreased the number of non-violent offenders in state prison. See, e.g., *Understanding the Impact of New York Bail Reform*, VERA (May 2024), <https://www.vera.org/publications/the-impact-of-new-york-bail-reform-on-statewide-jail-populations#:~:text=Bail%20reform%20led%20to%20a,rarely%20considered%20ability%20to%20pay>.

Our review of use of force information, unusual incident data, and interviews indicated that chemical agents are being used at Marcy and Mid-State for low-level non-compliance situations where verbal de-escalation was underutilized.⁸⁹⁸ This suggests that security staff at those facilities, at least, use chemical agents as a quick compliance tool, not as a last-resort safety measure.⁸⁹⁹ For example, our review identified two instances in which officers sprayed multiple bursts of chemical agents in succession—as many as six in a row—without assessing compliance between discharges.⁹⁰⁰ In one of these instances, officers immediately resorted to chemical agents without any verbal de-escalation.⁹⁰¹ In the other, a planned use of force, officers merely repeated commands without changing tone, even as the situation escalated, which were inadequate attempts at de-escalation.⁹⁰²

Some other state correction systems limit chemical agent use through restrictions, like requiring authorization from facility leadership before use or limiting who can carry chemical agents on their body.⁹⁰³ Given the high usage of chemical agents, we recommend that DOCCS reassess whether it is necessary for all staff to carry chemical agents, or whether chemical agents could be limited to supervisors, responders, and designated areas. We acknowledge that chemical agents can be a form of less-than-lethal protection for staff, but it is also important to consider limitations that would guard against overuse while still preserving chemical agents as a critical safety tool.

⁸⁹⁸ [Cite Redacted].

⁸⁹⁹ [Cite Redacted].

⁹⁰⁰ [Cite Redacted].

⁹⁰¹ [Cite Redacted].

⁹⁰² [Cite Redacted].

⁹⁰³ See, e.g., Massachusetts Dep't of Corrections, Policy 103 DOC 509.03 (July 3, 2025), <https://www.mass.gov/doc/doc-509-oc-chemical-agents-specialty-impact-munitiondistraction-devices-batons-and-electronic-control-devices/download>; Connecticut Dep't of Corrections, Administrative Directive 6.5 (Feb. 13, 2013), <https://portal.ct.gov/-/media/doc/pdf/ad/ad0605pdf.pdf?la=en>.

In addition, the American Correctional Association recommends that facilities implement sufficient inventory and accountability protocols for the use of chemical agents,⁹⁰⁴ and DOCCS has already begun implementing technology updates to improve controls over the use of chemical agents. DOCCS should continue these efforts. For example, during our review, we heard that DOCCS was in the initial stages of working with a third-party provider to develop technology that would activate an officer's BWC device as soon as the chemical agent or baton is pulled out of its holster.⁹⁰⁵

DOCCS should also update its use of force directive to enhance accountability for the use of chemical agents. Currently, Directive #4944 does not provide for a way to monitor unauthorized disbursements of chemical agents.⁹⁰⁶ Nor does the directive address failure to report the deployment of chemical agents.⁹⁰⁷ We recommend that DOCCS require that canisters of chemical agents be weighed and serialized at least once a month, as well as after each use. Such controls would not only confirm that staff have sufficient levels available for appropriate use, but also ensure oversight and accountability regarding the frequency and amount of chemical agents used.⁹⁰⁸

11.1 Recommendation Summary

- 11.1.1 Require that canisters of chemical agents issued as duty equipment and carried during the shifts are weighed and serialized at least once a month to confirm how much is being used, as well as after each use, to confirm sufficient levels for subsequent use, ensure chemical agent usage complies with policy, and enhance accountability. At the same time,

⁹⁰⁴ American Correctional Ass'n, *Performance-Based Standards for Adult Correctional Institutions*, Standards 5-ACI-3A-28,29,30 (5th ed., Mar. 2021 updates).

⁹⁰⁵ [Cite Redacted].

⁹⁰⁶ See Directive #4944 (Apr. 28, 2023).

⁹⁰⁷ See Directive #4944 (Apr. 28, 2023).

⁹⁰⁸ See American Correctional Ass'n, *Performance-Based Standards for Adult Correctional Institutions*, Standard 5-ACI-3A-30.

reassess the need for all staff to carry chemical agents by focusing on supervisors, responders, and designated areas.

12. STAFFING

Like other correctional systems, DOCCS is facing a staffing crisis.⁹⁰⁹ As of April 2025, following the illegal correction officers' strike, DOCCS vacancy rates rose to 27.4%, up from 13.3% at the start of 2024.⁹¹⁰ Correction officer positions had the highest vacancy rate among DOCCS positions, reaching 31.8% compared to 11% the prior year.⁹¹¹ As a result, officers face mandatory overtime, facilities grapple with insufficient staff coverage, and DOCCS must close certain posts, which limits programming opportunities such as mental health, educational, vocational, and recreational activities. The increased pressure on staff and limitations on programming increase tensions at the facilities and heighten safety risks for staff members and incarcerated individuals.

Marcy has a Budgeted Fill Level (BFL) of 363 security staff but, as of April 2025, there were 80 vacant positions.⁹¹² At Mid-State, the BFL was 547 security staff but, as of April 2025, only 326 positions were filled.⁹¹³ Marcy leadership explained that its vacancies are not due to underfunding—all vacant positions are fully funded—but rather due to resignations and retirement from current staff, as well as difficulty recruiting new talent to fill those positions.⁹¹⁴

To deal with staffing shortages, officers at Marcy and Mid-State are now required to work twelve-hour shifts rather than the typical eight-hour shifts.⁹¹⁵ On top of the longer shifts,

⁹⁰⁹ [Cite Redacted].

⁹¹⁰ CORRECTIONAL ASS'N OF NEW YORK, Data Dashboard, Staffing (last updated Apr. 4, 2025), <https://www.correctionalassociation.org/data/dashboard-staffing>.

⁹¹¹ CORRECTIONAL ASS'N OF NEW YORK, Data Dashboard, Staffing (last updated Apr. 4, 2025), <https://www.correctionalassociation.org/data/dashboard-staffing>.

⁹¹² [Cite Redacted]. CORRECTIONAL ASS'N OF NEW YORK, Data Dashboard, Staffing (last updated April 4, 2025), <https://www.correctionalassociation.org/data/dashboard-staffing>.

⁹¹³ [Cite Redacted]. CORRECTIONAL ASS'N OF NEW YORK, Data Dashboard, Staffing (last updated April 4, 2025), <https://www.correctionalassociation.org/data/dashboard-staffing>.

⁹¹⁴ [Cite Redacted].

⁹¹⁵ [Cite Redacted].

and as discussed in Section 5, “Findings,” officers are frequently “stuck,” or told they must work mandatory overtime, including on regular days off. Nor are staff assured of consecutive regular days off (RDO). For instance, based on Marcy’s RDO schedule in September 2025, someone might have Tuesday, September 1 off, followed by Monday, September 7 off—resulting in a six-day gap between days off.⁹¹⁶ Marcy staff also reported that staff sick call-ins have significantly increased on Mondays and Fridays.⁹¹⁷ The amount of overtime officers must work is costly for the State: for the fiscal year 2024 through 2025, New York appropriated approximately \$311 million for overtime pay for more than 6.7 million hours of overtime, at the average correction officer overtime rate of \$62.58 per hour and the average correction sergeant overtime rate of \$76.30 per hour.⁹¹⁸

As also described in Section 5, “Findings,” these conditions are driving staff to leave the job, worsening the staffing crisis. One correction officer described the vicious cycle: “If COs keep retiring, if we’re not retaining them, and the rate of recruiting stays about same, we’re headed for more mandatory OT, and then more people quitting or even striking.”⁹¹⁹ Marcy leadership reported that almost every staff member eligible to retire (i.e., those with 25 years of service) did so.⁹²⁰ Marcy leadership also reported losing more experienced officers with ten to 15 years of experience, which is a new and concerning trend.⁹²¹

⁹¹⁶ [Cite Redacted]. Superintendents are instructed to honor regular days off when possible. Memorandum of Agreement Between State of New York and NYSCOPBA (Mar. 8, 2025), <https://doccs.ny.gov/system/files/documents/2025/03/moa-doccs-nyscopba-3.8.2025.pdf>.

⁹¹⁷ [Cite Redacted].

⁹¹⁸ Report of Security Staffing, Annual Legislative Report, DOCCS (2025), p. 6.

⁹¹⁹ [Cite Redacted].

⁹²⁰ [Cite Redacted].

⁹²¹ [Cite Redacted].

Interviewed incarcerated individuals highlighted the ongoing staffing crisis and expressed empathy for staff who are forced to work twelve hours per day.⁹²² They suggested that these long shifts affect correction officers psychologically. One incarcerated individual said he had drawn up a blueprint for a correction officer dormitory that would be outside the fence near the facility so that correction officers could sleep at the facility.⁹²³

Current and former staff indicated that DOCCS is not doing enough to proactively respond to the staffing crisis, which they claim DOCCS should have anticipated.⁹²⁴ A current correction officer from Mohawk stated, “DOCCS is reactive. They only do anything when something gets fucked up.”⁹²⁵ A correction officer who retired from Mid-State in 2023 stated, “The State doesn’t do anything until something happens. They’re reactive. They did nothing proactively. They never came to see us. They never spoke to us, the guys in the field, to see what they should do. They never asked us any questions to see what our experiences were, how to retain us. It took them from 1971, Attica, ‘til now -- more than 50 years – for them to do a serious evaluation.”⁹²⁶

12.1 DOCCS Should Revise Facility Plot Plans

DOCCS uses “plot plans” for staffing, which are plans that specify how security staff will be allocated for maintaining safe and routine supervision of incarcerated individuals. The current plot plans were designed for eight-hour shifts rather than the current twelve-hour shifts.⁹²⁷ Due

⁹²² [Cite Redacted].

⁹²³ [Cite Redacted].

⁹²⁴ [Cite Redacted].

⁹²⁵ [Cite Redacted].

⁹²⁶ [Cite Redacted].

⁹²⁷ Both Marcy and Mid-State operations showed manual manipulation of plans to accommodate a computer program that was set up for eight-hour shifts. *See also* Report of Security Staffing, Annual Legislative Report, p. 4, DOCCS (2025), <https://doccs.ny.gov/system/files/documents/2026/02/2025-doccs-report-of-sisu-legislative-report-final-002.pdf> (“The current plot plans do not accurately reflect the twelve-hour schedules of which most facilities are operating, and almost all facility plot plans have remained the same since the beginning of the strike.”).

to ongoing staff shortages, Marcy and Mid-State fulfill their primary responsibilities of housing and feeding incarcerated individuals only by consolidating posts and suspending multiple programs and activities each day.⁹²⁸ Both Marcy and Mid-State reported frequent closures of lower-priority posts such as yards, gyms, packaging, and programming to prioritize housing and meal services.⁹²⁹

Closures of posts may result in unsafe conditions. For example, during peak movement periods (especially movement between units or to mess hall and/or recreation areas), walkway coverage becomes sparse when shifts operate at minimum strength, contributing to an increased risk of blind spots.⁹³⁰ One staff survey respondent indicated that the mess hall, yard, ICP, and walkways were unsafe areas due to severe understaffing.⁹³¹ Another staff survey respondent noted that there were safety risks “[a]nywhere there is a lack of security,” which, according to the respondent, was “every place.”⁹³²

Additionally, while facilities document daily program closures,⁹³³ there appeared to be no standardized weekly or monthly summaries with programming trend analysis (we requested this information for both Marcy and Mid-State but did not receive it). Without that information, the same posts are at risk of repeated closures.⁹³⁴ For instance, the same programming posts may be closed for consecutive days without any rotation or direct evaluation of the impact on

⁹²⁸ [Cite Redacted].

⁹²⁹ [Cite Redacted].

⁹³⁰ [Cite Redacted].

⁹³¹ [Cite Redacted].

⁹³² [Cite Redacted].

⁹³³ [Cite Redacted].

⁹³⁴ [Cite Redacted].

incarcerated individuals' rehabilitation needs.⁹³⁵ Delays in updating the staffing plan drive overtime, force recurring program closures, and can affect facility security.

We recommend that DOCCS revise facility plot plans to maximize facility efficiencies, while meeting legal obligations and programming requirements—accounting for twelve-hour shifts, closures, and other staffing realities. We also recommend that DOCCS track staff deployment, including closed job assignments by location. DOCCS should aim to rotate closed posts appropriately to improve opportunities for programming and recreation. In addition, DOCCS should continue collaborating with consultants, like Creative Corrections, to identify and execute efficiencies to optimize safety, efficiency, and compliance while supporting staff well-being in the long term.

12.2 DOCCS Should Enhance Mental Health and Wellness Opportunities for Staff

Research shows that correction professionals are overwhelmingly unwell and dying at significantly elevated rates relative to the general population.⁹³⁶ As discussed in Section 5, “Findings,” DOCCS officers face severe mental health challenges due to staffing shortages and other issues inherent to the job. According to surveys, 20% of the 36 staff survey respondents at Mid-State and 20% of the 66 staff survey respondents at Marcy rated their own mental health and wellness as “Poor,” and about 40% of survey respondents at both facilities rated their sleep quality as “Poor.” (see Figures 36 and 37).

⁹³⁵ [Cite Redacted].

⁹³⁶ Caterina G. Spinaris, et al., *Posttraumatic Stress Disorder in United States Corrections Professionals: Prevalence and Impact on Health and Functioning*, pp. 18-21, Desert Waters Correctional Outreach (2012); Jaime Brower, *Correctional Office Wellness and Safety Literature Review*, pp. 10-11, DOJ OJP Diagnostic Center, <https://www.ojp.gov/library/publications/correctional-officer-wellness-and-safety-literature-review>; Christi Mathis, *SIU Researchers Find Prison Guards Suffer from PTSD and Other Issues But Get Little Help*, SIU Carbondale (June 26, 2024), <https://news.siu.edu/2024/06/062624-siu-researchers-find-prison-guards-suffer-ptsd-and-other-issues-but-get-little-help.php>.

Figure 36 Staff Survey Result

“How would rate your own mental health and wellness, including your mood and ability to think clearly?”						
Facility	Poor	Fair	Good	Very Good	Excellent	Total
Marcy	20.6%	23.8%	30.2%	19.0%	6.3%	100.0%
Mid-State	20.0%	17.1%	25.7%	20.0%	17.1%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility staff.

Figure 37 Staff Survey Result

“How would you rate your recent sleep quality?”						
Facility	Poor	Fair	Good	Very Good	Excellent	Total
Marcy	41.3%	23.8%	20.6%	9.5%	4.8%	100.0%
Mid-State	42.9%	28.6%	14.3%	11.4%	2.9%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility staff.

In interviews, we generally heard that facility morale is low. Facility leadership told us that the staffing shortages, combined with the highly publicized incidents of officer violence, have resulted in severe morale issues, including issues with staff retention.⁹³⁷ A current correction officer stated: “The COs are demoralized. We took a lot of shit ... All the COs are miserable ... Morale is low. No one cares. Guys retire as soon as they are eligible.”⁹³⁸ A former correction officer from Mid-State stated: “It’s a thankless job.”⁹³⁹ One survey respondent noted that they “have a guaranteed job as nobody wants to work in this hell hole.”⁹⁴⁰ Correction officers also cite safety concerns on the job, explaining that assaults on correction officers are

⁹³⁷ [Cite Redacted].

⁹³⁸ [Cite Redacted].

⁹³⁹ [Cite Redacted].

⁹⁴⁰ [Cite Redacted].

common and that they believe that the HALT Act worsens facility safety by reducing consequences for incarcerated individuals who may assault staff or other incarcerated individuals.⁹⁴¹ Improving staff mental health and wellness is critical to staff well-being, recruitment, and retention.

12.2.1 Recent Steps to Improve Staff Health and Wellness

In recent years, DOCCS has taken steps to address staff wellness. For instance, DOCCS has assigned a dedicated mental health clinician to each regional hub to provide support services to staff.⁹⁴² With eight hubs, there are eight Licensed Master Social Workers (LMSW) positions allocated to provide mental health support services to staff, along with one Licensed Clinical Social Worker (LCSW) to supervise the other eight clinicians.⁹⁴³ As of November 21, 2025, DOCCS had filled the supervising LCSW position and five of the eight LMSW positions for the hubs and was actively recruiting for the other positions.⁹⁴⁴ DOCCS also recently trained hundreds of staff in Desert Waters, a program that teaches staff to identify and treat trauma.⁹⁴⁵

DOCCS also offers an Employee Assistance Program (EAP). The EAP is a staff wellness initiative structured via facility-level, hub-level, and statewide committees.⁹⁴⁶ For example, we were told that at Marcy, facility-level EAP coordinators are non-mental health professionals serving as wellness ambassadors to bridge frontline staff to mental health professionals.⁹⁴⁷ Further, there are over 250 DOCCS professionals trained in Critical Incident Stress Management

⁹⁴¹ [Cite Redacted].

⁹⁴² [Cite Redacted].

⁹⁴³ [Cite Redacted].

⁹⁴⁴ [Cite Redacted].

⁹⁴⁵ [Cite Redacted].

⁹⁴⁶ Directive #2116 (Feb. 5, 2025).

⁹⁴⁷ [Cite Redacted].

(CISM) who can be deployed after critical incidents; for example, after a suicide attempt.⁹⁴⁸

CISM teams are comprised of trained peers and at least one mental health professional.⁹⁴⁹

On May 28, 2025, DOCCS also launched a newly created NYS DOCCS Wellness mobile app for employees and their families.⁹⁵⁰ The app provides contact information for both local and national resources in mental health, wellness, and healthcare, as well as articles and videos related to stress management, nutrition, financial support, physical wellness, and more.⁹⁵¹

Although these recent efforts are commendable, DOCCS lacks a standardized, holistic framework for addressing employee health and wellness. Staff recruitment, retention, and wellness initiatives are not governed by a clear, cohesive, well-communicated strategy for improving staff wellness. For instance, at Marcy, facility staff were unable to describe a structured, department-wide wellness program or framework, though they noted morale-boosting activities such as a yearly wellness fair, a yearly blood drive, holiday parties, donut days, and Friday jean days.⁹⁵² Facility site visits also indicated inconsistent and unreliable access to wellness resources. During one site visit, Marcy staff reported that while the facility previously had an employee wellness room, it has since been re-purposed for other functions.⁹⁵³ On another site visit, as described in Section 5, “Findings,” we visited the wellness room at Mid-State and found that it appeared to be underutilized—it was covered in a layer of dust.⁹⁵⁴ We did not see any indications at Marcy of any facility- or headquarters-level tracking of wellness key performance indicators, nor of formalized evaluations of wellness initiatives.

⁹⁴⁸ [Cite Redacted].

⁹⁴⁹ [Cite Redacted].

⁹⁵⁰ [Cite Redacted].

⁹⁵¹ [Cite Redacted].

⁹⁵² [Cite Redacted].

⁹⁵³ [Cite Redacted].

⁹⁵⁴ [Cite Redacted].

Further, some former staff interviewed explained that staff do not trust the confidentiality of the EAP program and believe it to be “leaky.”⁹⁵⁵ Two former Mid-State correction officers separately recalled issues with EAP confidentiality and that it “got around the jail” that “so and so has a drinking problem.”⁹⁵⁶ DOCCS leadership confirmed that, because EAP representatives are peer correction officers, the risk that the EAP will share confidential information with others disincentivizes the use of EAP—especially given the stigma regarding mental health issues in the law enforcement profession.⁹⁵⁷

In addition to continuing the morale-boosting activities cited by facility staff, such as holiday parties and other perks to recognize staff, we recommend that DOCCS establish a five-year formal health and wellness strategy that addresses a comprehensive approach to employee health and well-being and emphasizes confidentiality. DOCCS should track alignment with its wellness strategy by using key performance indicators, such as reductions in staff sick time and workers’ compensation claims, referrals to treatment programs, wellness app usage and downloads, and by analyzing anonymized healthcare claims submitted by employees. DOCCS should also establish a department-wide oversight committee or task force to review wellness data annually and report that data annually to steer the focus and direction of regional- and facility-level staff wellness committees and initiatives. Finally, DOCCS should address staff morale and retention by developing a quarterly staff recognition program at each facility, including individual public commendations, social media blasts, or personalized notes from leadership.

⁹⁵⁵ [Cite Redacted].

⁹⁵⁶ [Cite Redacted].

⁹⁵⁷ [Cite Redacted].

12.3 DOCCS Should Continue to Recruit

DOCCS faces a significant challenge recruiting new officers due to the difficult working conditions at DOCCS, including a lack of work-life balance due to staffing shortages and negative perceptions of the job caused by the killings of Mr. Brooks and Mr. Nantwi. To address these challenges, DOCCS has launched a systemwide campaign called “Recover, Recruit, Rebuild.”⁹⁵⁸ Announced on March 11, 2025 (just a few days after Mr. Nantwi’s killing), the initiative has five pillars: safe staffing at facilities, transition to a new normal for the incarcerated population, recruitment of new security staff, evaluation of opportunities to reduce the incarcerated population, and development of a long-term vision for a next generation of corrections system.⁹⁵⁹ Key components of the initiative include a multi-channel outreach strategy in partnership with OGS Media Services, legislation passed to lower the age of eligible correction officer candidates to 18, and recruitment and retention bonuses, among other things.⁹⁶⁰ This initiative shows promise. By June 2025, these efforts resulted in a 63% increase in trainees and an 81% increase in trainee graduates compared to 2024.⁹⁶¹ This program also added

⁹⁵⁸ Office of New York State Governor Kathy Hochul, *Recover, Recruit, Rebuild: Governor Hochul Updates New Yorkers on Future of State’s Correctional System Following End of Illegal Work Stoppage*, (Mar. 11, 2025), <https://www.governor.ny.gov/news/recover-recruit-rebuild-governor-hochul-updates-new-yorkers-future-states-correctional-system>.

⁹⁵⁹ Office of New York State Governor Kathy Hochul, *Recover, Recruit, Rebuild: Governor Hochul Updates New Yorkers on Future of State’s Correctional System Following End of Illegal Work Stoppage*, (Mar. 11, 2025), <https://www.governor.ny.gov/news/recover-recruit-rebuild-governor-hochul-updates-new-yorkers-future-states-correctional-system>; *Martuscello Affirmation* ¶ 33, *Smalls v. Martuscello*, Index No. 903926-25 (Sup. Ct., Albany County May 23, 2025).

⁹⁶⁰ Press Release, DOCCS, *Recover, Recruit, Rebuild: New York State Department of Corrections and Community Supervision Advances Recruitment and Recovery Efforts*, p. 2 (July 26, 2025), https://doccs.ny.gov/system/files/documents/2025/07/recover-recruit-rebuild_nys-doccs-advances-recruitment-and-recovery-efforts.pdf.

⁹⁶¹ Press Release, DOCCS, *Recover, Recruit, Rebuild: New York State Department of Corrections and Community Supervision Advances Recruitment and Recovery Efforts*, p.1 (July 26, 2025), https://doccs.ny.gov/system/files/documents/2025/07/recover-recruit-rebuild_nys-doccs-advances-recruitment-and-recovery-efforts.pdf.

to the momentum of regional hiring initiatives, which recently produced the largest Academy class, with 105 trainees.⁹⁶² DOCCS should continue its investment in this program.

DOCCS should also consider investing in messaging and strategy that re-frame DOCCS as a career in public service and rehabilitation—either through a public relations consultant or otherwise. During our visits, we met officers and staff who took great pride in their work and expressed a strong desire for better tools, greater respect, and recognition for what they do. DOCCS should reinforce this messaging through training, marketing, and recruitment. Further, DOCCS should build upon New York State’s existing Excelsior Service Fellowship Program and seek to create a three-year paid developmental training program to attract more talent to DOCCS. In its current form, the Excelsior Service Fellowship Program places recent graduates of law, graduate, and professional schools in the Executive Chamber, or a government agency or other authority, for a two-year fellowship.⁹⁶³ The new DOCCS fellowship would emphasize commitment to public service and would allow recipients to rotate through positions at DOCCS while developing leadership skills, receiving training in correctional operations, and building a community of peers. The capstone of the fellowship would include developing and drafting a plan to implement a systemwide policy initiative to improve an element of DOCCS’ operations. We recognize that such a program may require civil service law changes.

12.4 DOCCS Should Digitize the Performance Evaluation Process and Ensure Supervisor Input Is Meaningful

DOCCS’ current performance evaluation system makes it difficult to track and develop qualified staff for promotion. To start, performance evaluations are paper based, which makes them time-consuming to fill out and inhibits the development of digital performance tracking

⁹⁶² [Cite Redacted].

⁹⁶³ N.Y. Fellowship Programs, *Excelsior Service Fellowship Program*, <https://www.ny.gov/new-york-state-fellowship-programs/excelsior-service-fellowship-program> (last visited Dec. 16, 2025).

metrics.⁹⁶⁴ The evaluations also consist primarily of checkbox ratings—“Excellent,” “Good,” “Needs Improvement,” or “Unsatisfactory” for security staff, and “Satisfactory” or “Unsatisfactory” for civilian staff—with minimal space for comments from the sergeant evaluator and supervisor reviewer.⁹⁶⁵ The evaluation process also fails to include goal-setting or formal assessments of growth and achievements.⁹⁶⁶

The shared union representation of correction officers and sergeants by NYSCOPBA may further inhibit meaningful evaluation by discouraging negative feedback.⁹⁶⁷ Each DOCCS facility has a NYSCOPBA chief sector steward—typically an elected correction officer—who represents both correction officers and sergeants.⁹⁶⁸ Under this structure, sergeants may report to a subordinate correction officer for union matters.⁹⁶⁹ This dynamic could influence the sergeant’s evaluation of the subordinate officer, resulting in a more positive and less honest assessment of that officer’s conduct.⁹⁷⁰

These structural issues may discourage supervisors from conducting meaningful evaluations. We heard that evaluations of security staff generally lack constructive feedback and are treated as a low priority by supervisors—a missed opportunity in identifying corrective action where needed and in supporting staff development.⁹⁷¹ DOCCS should ensure regular (at least every three years) training of supervisors on key issues, including completing evaluations,

⁹⁶⁴ [Cite Redacted].

⁹⁶⁵ [Cite Redacted].

⁹⁶⁶ [Cite Redacted].

⁹⁶⁷ [Cite Redacted].

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⁹⁶⁹ [Cite Redacted].

⁹⁷⁰ [Cite Redacted].

⁹⁷¹ [Cite Redacted].

reviewing Use of Force Reports, and other supervisory responsibilities, and should incorporate formal testing of topics covered.

The current evaluation system also makes it difficult for DOCCS to identify staff who exhibit unsatisfactory performance or, conversely, excellent performance. Though DOCCS tracks “Unsatisfactory” performance ratings, our review identified that there have only been four “Unsatisfactory” evaluations at Marcy over the last four years.⁹⁷² And DOCCS does not track “Excellent,” “Good,” or “Needs Improvement” ratings at all.⁹⁷³ Though we acknowledge that New York Civil Service Law §52, rather than performance evaluations, may primarily dictate transfers or promotions to sergeant and lieutenant roles,⁹⁷⁴ we recommend that DOCCS digitize the performance evaluation process so that facilities can use data analytics to track staff performance.

12.5 Recommendation Summary

- 12.5.1 Continue investment in the “Recover, Recruit, Rebuild” initiative. Invest in messaging and strategy that reframe DOCCS as a career of public service and rehabilitation; reinforce this messaging through training, marketing, and recruitment.
- 12.5.2 Revise facility staffing (plot) plans to maximize facility efficiencies, while meeting legal obligations and programming requirements. Evaluate plot plans to account for twelve-hour shifts, closures, and other staffing limitations. Continue collaborating with consultants on identifying and executing efficiencies to optimize safety, efficiency, and compliance while supporting staff well-being in the long term.
- 12.5.3 Address staff morale and retention by developing a quarterly staff recognition program for staff at each facility, including individual public

⁹⁷² [Cite Redacted].

⁹⁷³ [Cite Redacted].

⁹⁷⁴ New York Civil Service Law §52 requires internal promotions from the same line of command, “as far as practicable,” based on a multiple-choice examination. N.Y. Civ. Serv. Law §52 (McKinney 2024). The New York Civil Service Law expands eligibility in limited circumstances. Section 52(3) allows for persons on “preferred lists” and on leave of absence to be eligible, while §52(4) allows for interdepartmental eligibility once the internal list is exhausted.

commendations, social media blasts, or personalized notes from leadership.

- 12.5.4 Track staff deployment and prioritize staffing for programming and recreation areas, including tracking closed job assignments (posts) by location and rotating closed posts appropriately to improve opportunities for programming and recreation.
- 12.5.5 Build upon New York State's existing Excelsior Service Fellowship Program by creating three-year paid developmental training program to attract more talent to DOCCS. We acknowledge that this may require civil service approvals or changes.
- 12.5.6 Establish a five-year formal health and wellness strategy that addresses a comprehensive approach to employee health and well-being and emphasizes confidentiality.
- 12.5.7 Track alignment with wellness strategy by using key performance indicators, such as reductions in staff sick time and workers' compensation claims, referrals to treatment programs, and wellness app usage and downloads, and by analyzing anonymized healthcare claims submitted by employees.
- 12.5.8 Establish department-wide oversight committee or task force to review wellness data annually and report that data annually to steer the focus and direction of regional- and facility-level staff wellness committees and initiatives.
- 12.5.9 Fully digitize the performance evaluation process so that facilities can use data analytics to track staff performance.
- 12.5.10 Ensure regular (at least every three years) training of supervisors on key issues, including review of Use of Force Reports, evaluations, and other supervisory responsibilities. Incorporate formal testing of concepts covered.

13. **CONTRABAND**

Drugs and other contraband inside facilities pose a major challenge to DOCCS. Drugs and other contraband exacerbate safety issues for both incarcerated individuals and staff by increasing the risk of assaults. Staff also risk exposure to harmful chemicals when attempting to interdict dangerous substances, and drug incidents and overdoses increase staffing challenges, as staff are required to provide emergency care and hospital transport in urgent cases. When asked what changes or improvements would be important at DOCCS facilities, multiple staff highlighted the need to remove contraband like drugs and weapons.⁹⁷⁵

13.1 DOCCS Faces a Drug Crisis

DOCCS faces a crisis posed by dangerous drugs, including fentanyl and synthetic drugs such as “K2.”⁹⁷⁶ These drugs “have resulted in overdoses, deaths, anxiety, and violent behaviors ... as well as other health implications” at DOCCS facilities.⁹⁷⁷ DOCCS data on unusual incidents—defined as “serious occurrences that may impact or disrupt facility operations, have the potential for disrupting the Department’s public image, or might arouse widespread public interest”—reflects 5,147 unusual incidents involving contraband in 2024.⁹⁷⁸ Of these, 778 incidents were drug related, a 35% increase from the previous year.⁹⁷⁹ In 2024, synthetic drugs accounted for approximately 39% of unusual incidents related to drug contraband (a 72% increase from 2023), and marijuana accounted for approximately 43% of drug

⁹⁷⁵ [Cite Redacted].

⁹⁷⁶ [Cite Redacted].

⁹⁷⁷ [Cite Redacted].

⁹⁷⁸ *Unusual Incident Report, January – December 2024, with Ten-Year Trends from 2015-2024*, pp. 1, 2, 6, DOCCS, https://doccs.ny.gov/system/files/documents/2025/10/annual-ui-report-2024_final.pdf.

⁹⁷⁹ *Unusual Incident Report, January – December 2024, with Ten-Year Trends from 2015-2024*, p. 6, DOCCS, https://doccs.ny.gov/system/files/documents/2025/10/annual-ui-report-2024_final.pdf.

contraband incidents (a 52% increase from 2023).⁹⁸⁰ Notably, due to contraband reporting procedures implemented in 2021, drug contraband is no longer reported as an unusual incident until it is confirmed by external laboratory analysis, meaning that the extent of the drug crisis in facilities almost certainly extends beyond these numbers.⁹⁸¹

According to surveys, 29.6% of the 361 incarcerated individual respondents out of 782 incarcerated individuals at Marcy and 46.6% of the 440 incarcerated individual respondents out of 1,164 incarcerated individuals at Mid-State said that the level of drugs or contraband was “Very High/Out of Control.”⁹⁸² (see Figure 38). Another 11.9% of respondents at Marcy and 11.5% of respondents at Mid-State said that drugs or contraband were “Mostly Controlled.”⁹⁸³ (see Figure 38).

Figure 38 Incarcerated Individual Survey Result

“How would you describe the level of drugs or contraband in this facility?”						
Facility	Very High / Out of Control	Mostly Uncontrolled	Neutral	Mostly Controlled	Very Low / Well Controlled	Total
Marcy	29.6%	11.9%	27.0%	8.7%	22.8%	100%
Mid-State	46.6%	11.5%	24.9%	3.8%	13.2%	100%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

In interviews, current and former staff confirmed the urgency and scope of the drug problem in DOCCS facilities. One correction officer who retired in 2021 told us that “[t]he last

⁹⁸⁰ *Unusual Incident Report, January – December 2024, with Ten-Year Trends from 2015-2024*, p. 6, DOCCS, https://doccs.ny.gov/system/files/documents/2025/10/annual-ui-report-2024_final.pdf.

⁹⁸¹ *Unusual Incident Report, January – December 2024, with Ten-Year Trends from 2015-2024*, pp. 2, 6, DOCCS, https://doccs.ny.gov/system/files/documents/2025/10/annual-ui-report-2024_final.pdf.

⁹⁸² [Cite Redacted].

⁹⁸³ [Cite Redacted].

few years, drugs have been more of a problem. The overdoses are out of control.”⁹⁸⁴ Other former correction officers stated that drugs are “everywhere”⁹⁸⁵ and “the biggest problem in the prisons.”⁹⁸⁶ Former staff opined that drugs get into facilities through outside packages,⁹⁸⁷ through visitors,⁹⁸⁸ through staff,⁹⁸⁹ and through mail that has been “sprayed with drugs.”⁹⁹⁰ Current staff—especially medical staff—described frequent intoxication events, often clustered after visitations and often more than one at a time.⁹⁹¹

Various incarcerated individuals described the issues with drugs. At Marcy, incarcerated individuals told us that “[d]rugs are the main concern” and there is “no accountability for people who use K2.”⁹⁹² Others at Marcy stated that “[d]rugs are huge” and “cause the [problems], even if you aren’t on them”; that drugs are “beyond out of control”; and that “K2 is everywhere.”⁹⁹³ One incarcerated individual stated that the drug and contraband problems were “[a]bsolutely abysmal ... You can come to jail with an associate[’]s [degree] in marijuana and leave with a PhD in LSD.”⁹⁹⁴ We heard similar statements from several incarcerated individuals at Mid-State, who described the drug issue as “out of control,” “all over the place,” “pretty bad,” and a “big” problem.⁹⁹⁵

⁹⁸⁴ [Cite Redacted].

⁹⁸⁵ [Cite Redacted].

⁹⁸⁶ [Cite Redacted].

⁹⁸⁷ [Cite Redacted].

⁹⁸⁸ [Cite Redacted].

⁹⁸⁹ [Cite Redacted].

⁹⁹⁰ [Cite Redacted].

⁹⁹¹ [Cite Redacted].

⁹⁹² [Cite Redacted].

⁹⁹³ [Cite Redacted].

⁹⁹⁴ [Cite Redacted].

⁹⁹⁵ [Cite Redacted].

Several incarcerated individuals said that the drug problem extends beyond certain facilities. One incarcerated individual said that “drugs are around like every other place,” and another said he had not been in a facility that does not have a drug problem.⁹⁹⁶ Another stated it was a “whole state problem,” not a facility problem.⁹⁹⁷

During our site visits, we observed evidence of the drug problem firsthand. On one occasion, an individual at Gouverneur experienced an overdose, required ten minutes of CPR, and was then transported to the hospital; he survived.⁹⁹⁸ During that visit, the staff noted that there had been eleven write-ups for intoxication the night before we arrived and that drugs were the biggest problem facing the facility.⁹⁹⁹ Similarly, the superintendent at Mid-State explained that if our team had not been visiting that day, the facility “would have been on lock down because of intoxication.”¹⁰⁰⁰

13.2 Drugs Drive Violence in Facilities

Facility leadership explained how drugs can lead to violence: incarcerated individuals who use drugs may incur debts, which leads to violence among incarcerated individuals; incarcerated individuals who use drugs may get violent with staff or need to be restrained to prevent harm to themselves; and incarcerated individuals who use drugs may need to go to the hospital if they overdose or get hurt as a result of related violence, which also pulls staff away from their posts (and, therefore, affects facility safety).¹⁰⁰¹ Another current member of DOCCS’ staff described a violent incident in which a gang ran out of drugs when their “drug guy” was

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⁹⁹⁷ [Cite Redacted].

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⁹⁹⁹ [Cite Redacted].

¹⁰⁰⁰ [Cite Redacted].

¹⁰⁰¹ [Cite Redacted].

caught returning from visitation with drugs.¹⁰⁰² They wanted another incarcerated individual's supply and, when he did not comply, they attacked him in the shower and rammed a broomstick up his rectum.¹⁰⁰³

Former DOCCS staff also described the link between drugs and violence. One former correction officer explained, "When [incarcerated individuals are] on fentanyl, it's no joke. One day we had five ambulances on the midnight shift alone."¹⁰⁰⁴ Another said that when incarcerated individuals are "on drugs like fentanyl or K2 ... they're either lying on the floor like a puddle of liquid, or they're extremely violent. Officers are the ones who are gonna get hurt. The State knows this. They know we have a drug problem. They don't care."¹⁰⁰⁵ One former Mid-State correction officer said that "drugs became rampant" at the facility around 2018 or 2019.¹⁰⁰⁶ He told us that "[d]rug use by inmates causes violence, then that trickles down to everything else—workers comp, staff shortages, mandatory [overtime]," and he pointed to increased violence between incarcerated individuals and attacks on officers.¹⁰⁰⁷

Current and former staff expressed frustration at DOCCS for failing to take steps to curb drug use and associated violence inside facilities. One former correction officer stated, "There are steps that can be taken (to control drugs), but they don't do it for some reason. Part of me thinks DOCCS wants there to be drugs in the prisons. Otherwise, they would have taken the steps."¹⁰⁰⁸ Another said that drugs were the "biggest problem" in the facility, but that the former officer believed DOCCS "gave up on trying to address the root problems," even though

¹⁰⁰² [Cite Redacted].

¹⁰⁰³ [Cite Redacted].

¹⁰⁰⁴ [Cite Redacted].

¹⁰⁰⁵ [Cite Redacted].

¹⁰⁰⁶ [Cite Redacted].

¹⁰⁰⁷ [Cite Redacted].

¹⁰⁰⁸ [Cite Redacted].

“[e]verything stems from drugs[, which] cause hospital trips, uses of force, overtime, everything.”¹⁰⁰⁹ One member of the National Guard expressed shock about the quantity of drugs getting into the facilities, noting that “it’s almost by design, it’s almost like New York State wants the drugs in these prisons.”¹⁰¹⁰

13.3 DOCCS’ Ongoing Efforts to Address the Drug Crisis

DOCCS has undertaken several efforts to interdict drugs and to address the risks presented by drug use at its facilities. For example, on September 23, 2025, Commissioner Martuscello issued a memorandum to all staff entitled “Package Room – Unknown Substance Safety.”¹⁰¹¹ The memorandum called for the polling of all Crisis Intervention Unit Team Leaders to identify trends and intelligence relating to drugs, the formation of a working group comprised of experienced package room correction officers to solicit additional drug interdiction strategies and best practices, the engagement with facility ILCs to solicit ideas regarding drug interdiction measures, and the piloting of additional technology to aid in identifying illicit drugs and foreign substances.¹⁰¹²

In addition, through the Incarcerated Individual Drug Test Program, DOCCS tests urine for consumed drugs, and through the Contraband Testing Program, DOCCS tests suspicious substances found in facilities for the presence of illegal drugs.¹⁰¹³ DOCCS also takes various interdiction actions, including routinely screening staff and outside visitors, imposing limitations on homemade packages sent to incarcerated individuals, and using canine units for drug

¹⁰⁰⁹ [Cite Redacted].

¹⁰¹⁰ [Cite Redacted].

¹⁰¹¹ [Cite Redacted].

¹⁰¹² [Cite Redacted].

¹⁰¹³ New York Offices of the Inspector General, *Investigation of the New York State Department of Corrections and Community Supervision Contraband Drug Testing Program*, p. 1 (Nov. 2023), <https://ig.ny.gov/system/files/documents/2023/11/doccs-drug-testing-program-report.pdf>.

identification.¹⁰¹⁴ Additionally, DOCCS offers a range of programs for incarcerated individuals to address addiction, including the Intensive and Comprehensive Alcohol and Substance Abuse Treatment Program, Medication Addiction Treatment Program, and Relapse Treatment Program.¹⁰¹⁵

Despite these efforts, drugs continue to enter DOCCS facilities.¹⁰¹⁶ Facility leadership at Gouverneur told us that while canine units are good at detecting marijuana, they struggle to sniff out papers that are sprayed with drugs; for example, in a book, it is difficult for dogs to detect a few pages sprayed with drugs within hundreds of unsprayed pages.¹⁰¹⁷ The trained dogs also struggle to identify drugs in dorms when many individuals are present, as opposed to when they can focus on a single individual.¹⁰¹⁸ Indeed, OSI has not yet found a good way to “weed out” paper laced with K2 or other drugs that come in through the mail.¹⁰¹⁹ A former correction officer at Mid-State said that even when outside packages to incarcerated individuals are required to come from certain vendors, drugs still get introduced into facilities through them, and drugs nonetheless get in through the mailroom and visitors.¹⁰²⁰

Facility leadership also told us that while body scanners are a good tool and have been useful in detecting contraband drugs, small children are exempt from the scanner, and they

¹⁰¹⁴ New York Offices of the Inspector General, *Investigation of the New York State Department of Corrections and Community Supervision Contraband Drug Testing Program*, p. 1 (Nov. 2023), <https://ig.ny.gov/system/files/documents/2023/11/doccs-drug-testing-program-report.pdf>.

¹⁰¹⁵ New York Offices of the Inspector General, *Investigation of the New York State Department of Corrections and Community Supervision Contraband Drug Testing Program*, p. 1 (Nov. 2023), <https://ig.ny.gov/system/files/documents/2023/11/doccs-drug-testing-program-report.pdf>.

¹⁰¹⁶ New York Offices of the Inspector General, *Investigation of the New York State Department of Corrections and Community Supervision Contraband Drug Testing Program*, p. 1 (Nov. 2023), <https://ig.ny.gov/system/files/documents/2023/11/doccs-drug-testing-program-report.pdf> (“Nonetheless, drugs continue to infiltrate state correctional facilities on a broad scale, posing a threat to the health and safety of all incarcerated persons, and civilian and security staff.”).

¹⁰¹⁷ [Cite Redacted].

¹⁰¹⁸ [Cite Redacted].

¹⁰¹⁹ [Cite Redacted].

¹⁰²⁰ [Cite Redacted].

suspect that visiting children may be used to smuggle drugs into facilities.¹⁰²¹ Multiple former correction officers also told us that drugs can be brought into facilities by correction officers or other staff.¹⁰²² Additionally, medical staff told us that synthetic drugs do not show up on drug tests, which exacerbates these issues.¹⁰²³

13.4 Weapons in Facilities Pose Safety Concerns

Contraband weapons are also an acute challenge in DOCCS facilities. DOCCS' unusual incident data reflects that in 2024, contraband-related unusual incidents that involved cutting or stabbing weapons comprised 68% of all contraband incidents.¹⁰²⁴ Between 2015 and 2024, contraband involving cutting or stabbing weapons increased by 85%; this included weapons like can lids, cutting instruments, hardback razorblades, ice picks, manufactured knives, razor blades, scalpels, shanks, toothbrushes, and utility razorblades.¹⁰²⁵ Between 2023 and 2024, the number of unusual incidents involving other weapon contraband—including ammunition, explosives, manufactured guns, weighted cloth, and other weapons—increased by 60%.¹⁰²⁶ At Marcy in 2024, there were 117 contraband unusual incidents involving cutting and stabbing weapons and 30 incidents involving other weapons, and at Mid-State in 2024, there were 157 contraband-related unusual incidents involving cutting and stabbing weapons and 22 incidents involving other weapons.¹⁰²⁷

¹⁰²¹ [Cite Redacted].

¹⁰²² [Cite Redacted].

¹⁰²³ [Cite Redacted].

¹⁰²⁴ *Unusual Incident Report, January – December 2024, with Ten-Year Trends from 2015-2024*, p. 5, DOCCS, https://doccs.ny.gov/system/files/documents/2025/10/annual-ui-report-2024_final.pdf.

¹⁰²⁵ *Unusual Incident Report, January – December 2024, with Ten-Year Trends from 2015-2024*, pp. 5-6, DOCCS, https://doccs.ny.gov/system/files/documents/2025/10/annual-ui-report-2024_final.pdf.

¹⁰²⁶ *Unusual Incident Report, January – December 2024, with Ten-Year Trends from 2015-2024*, pp. 2, 6, DOCCS, https://doccs.ny.gov/system/files/documents/2025/10/annual-ui-report-2024_final.pdf.

¹⁰²⁷ *Unusual Incident Report, January – December 2024, with Ten-Year Trends from 2015-2024*, p. 7, DOCCS, https://doccs.ny.gov/system/files/documents/2025/10/annual-ui-report-2024_final.pdf.

DOCCS leadership and incarcerated individuals shared observations about contraband and homemade weapons. For example, incarcerated individuals at Marcy described the presence of ceramic weapons, with one noting that these weapons can be smuggled in during visitations.¹⁰²⁸ During a site visit at Marcy, correction officers showed us contraband recovered during the course of our time there, which included a ceramic blade that had been recovered from an incarcerated individual’s mouth.¹⁰²⁹ DOCCS leadership also said that DOCCS “[does not] have a normal anymore” in terms of contraband and noted a recent reappearance of weapons that had not been observed in a while.¹⁰³⁰ For example, they stated that there were around 300 incidents during 2024 that involved weapons fashioned by a “lock in a sock,” whereas in previous years, such incidents seldom occurred.¹⁰³¹ One civilian staffer at Marcy recounted an incident in which one incarcerated individual attacked another incarcerated individual with a “lock in a sock”; the assaulted individual ended up on life support.¹⁰³²

Like drugs, the presence of weapons poses safety risks to DOCCS staff and incarcerated individuals. One incarcerated individual at Marcy said fighting was a main safety concern and described an attempted shanking the same morning he responded to our interview.¹⁰³³ A former correction officer described an attack at Mid-State by an incarcerated individual with a shank when the correction officer was searching the individual’s cube for weapons.¹⁰³⁴ A contraband weapon can also pose safety risks to the incarcerated individual who possesses it. For example,

¹⁰²⁸ [Cite Redacted].

¹⁰²⁹ [Cite Redacted].

¹⁰³⁰ [Cite Redacted].

¹⁰³¹ [Cite Redacted].

¹⁰³² [Cite Redacted].

¹⁰³³ [Cite Redacted].

¹⁰³⁴ [Cite Redacted].

we observed an individual who was in the RMHU at Marcy who, we were told, would repeatedly harm himself with a small blade that he would swallow, pass, and reuse.¹⁰³⁵

13.5 Recommendations To Address Contraband

DOCCS has been working to address the proliferation of contraband for years.¹⁰³⁶ Our recommendations aim to augment these existing efforts to address incoming contraband through outside visitors, staff, packages, and mail.

To address contraband entering facilities through outside visitors and staff, DOCCS, in the past year, has implemented body scanners at facilities to assist in preventing the introduction of contraband into facilities.¹⁰³⁷ DOCCS also uses canine units for drug identification.¹⁰³⁸ In addition to these existing efforts, we recommend that DOCCS conduct a security screening—through either existing or new technology vendors—of all individuals (including staff, visitors, and contractors) before entry to any facility to help ensure that contraband and other unauthorized items are not being illegally introduced.

With respect to contraband introduced into facilities via packages and mail, in the past year, DOCCS has implemented some non-intrusive mail scanners at facilities.¹⁰³⁹ We observed package room staff at Marcy and Gouverneur utilizing the property-scanning technologies to detect contraband.¹⁰⁴⁰ To the extent not already underway, DOCCS should use scanning

¹⁰³⁵ [Cite Redacted].

¹⁰³⁶ [Cite Redacted].

¹⁰³⁷ See, e.g., Directive 4943 (Sep. 5, 2025).

¹⁰³⁸ New York Offices of the Inspector General, *Investigation of the New York State Department of Corrections and Community Supervision Contraband Drug Testing Program*, p. 1 (Nov. 2023), <https://ig.ny.gov/system/files/documents/2023/11/doccs-drug-testing-program-report.pdf>.

¹⁰³⁹ See, e.g., Alex Gault, *New York Rolls Out New Legal Mail Scanning at Prisons*, ADIRONDACK DAILY ENTERPRISE (Apr. 12, 2025), <https://www.adirondackdailyenterprise.com/news/local-news/2025/04/new-york-rolls-out-new-legal-mail-scanning-at-prisons>.

¹⁰⁴⁰ [Cite Redacted].

technology to screen all mail for contraband, including legal mail, with due regard to protecting privileged legal communications.

DOCCS should also create a two-year pilot program at selected facilities that relies on approved state vendors for delivery of goods to incarcerated individuals, with appropriate exceptions to ensure access to items to which individuals are legally entitled, including religious items. Post-pilot program, DOCCS should evaluate whether the secure vendor program meaningfully reduced contraband at facilities and, if so, consider expanding it.

13.6 Recommendation Summary

- 13.6.1 Screen all individuals (including staff, visitors, and contractors) before entry to any facility to help ensure that contraband and other unauthorized items are not being illegally introduced. In addition, to the extent that it is not underway, DOCCS should use scanning technology to screen all mail for contraband, including legal mail, with due regard to protecting privileged legal communications.
- 13.6.2 Create a two-year pilot program at selected facilities that relies on approved state vendors for delivery of goods to incarcerated individuals, with appropriate exceptions to ensure access to items to which individuals are legally entitled, including religious items. After the pilot program, evaluate whether the secure vendor program meaningfully reduced contraband at those facilities.

14. CAMERAS

Fixed cameras and BWC play a critical role in promoting safety, ensuring accountability, and providing objective evidence. As a correction officer who retired from Marcy in 2024 said: “Why do these facilities not have cameras everywhere? I’m a huge advocate for cameras. All they do is prevent shitbags from being shitbags. If you’re afraid of cameras, it’s because you’re doing something wrong. I don’t think [the] Brooks [incident] would have happened if there were cameras in those buildings.”¹⁰⁴¹

DOCCS launched an initiative in 2015 to outfit every prison facility with fixed cameras.¹⁰⁴² But, at the time Mr. Brooks was murdered, Marcy did not have fixed cameras installed throughout the facility and did not have full camera coverage of the infirmary.¹⁰⁴³ Marcy was not an outlier—as of June 2026, only twelve facilities have fixed cameras installed sitewide, with 35 additional facilities in process.¹⁰⁴⁴ After the murder of Mr. Brooks, Governor Hochul accelerated \$400 million to help expedite camera installations in facilities.¹⁰⁴⁵ DOCCS considers fixed camera installations in infirmaries to be an “Emergency Project.”¹⁰⁴⁶ As a result, fixed cameras have now been installed in Marcy’s infirmary, although sitewide installation is ongoing, with an expected completion date in November 2028.¹⁰⁴⁷ Fixed camera installations in the infirmary and medical building have been completed at Mid-State and Gouverneur.¹⁰⁴⁸ Fixed camera installations continue at Ulster and Riverview, with expected completion in February and

¹⁰⁴¹ [Cite Redacted].

¹⁰⁴² Rebecca McCray & Chris Gelardi, *A Decade and \$600 Million Later, New York Prisons Still Lack Cameras*, NEW YORK FOCUS: CRIMINAL JUSTICE (Feb. 3, 2025) <https://nysfocus.com/2025/02/03/robert-brooks-doccs-prisons-cameras>.

¹⁰⁴³ [Cite Redacted].

¹⁰⁴⁴ [Cite Redacted].

¹⁰⁴⁵ [Cite Redacted].

¹⁰⁴⁶ [Cite Redacted].

¹⁰⁴⁷ [Cite Redacted].

¹⁰⁴⁸ [Cite Redacted].

March 2026, respectively.¹⁰⁴⁹ DOCCS estimates that Orleans, Franklin, Cape Vincent, and Lakeview will have completed fixed camera installations in infirmaries by spring 2026.¹⁰⁵⁰ Albion, Eastern, Wende, and Wyoming do not have estimated completion dates.¹⁰⁵¹

DOCCS estimates that certain facilities will have sitewide installations between spring 2026 (Wende), spring 2027 (Taconic), spring 2028 (Bedford Hills, Eastern), and fall 2028 (Cayuga, Albion, Marcy). The remaining facilities (Fishkill, Sing Sing, Five Points, Mid-State, Orleans, Greene, Auburn, Groveland, Clinton, Elmira, Washington, Gouverneur, Mohawk, Ulster, Wyoming, Altona, Cape Vincent, Collins, Wallkill, Woodbourne, Hale Creek, Lakeview, Otisville, Riverview) are in various stages of bidding, design, or construction, and do not have estimated completion dates.¹⁰⁵²

14.1 Understanding DOCCS' Timeline to Install Fixed Cameras

While DOCCS launched an initiative in 2015 to outfit every facility with fixed cameras,¹⁰⁵³ installation has been slow. Most facilities were designed prior to camera technology and required extensive rewiring across large, aging buildings.¹⁰⁵⁴ Additionally, facility leadership reported that although funding and equipment were in place, there were delays in installing the cameras due to low staffing levels.¹⁰⁵⁵ Officers must supervise and transport installation teams around the facility, which is difficult given the ongoing staffing issues, as

¹⁰⁴⁹ [Cite Redacted].

¹⁰⁵⁰ [Cite Redacted].

¹⁰⁵¹ [Cite Redacted].

¹⁰⁵² [Cite Redacted].

¹⁰⁵³ Rebecca McCray & Chris Gelardi, *A Decade and \$600 Million Later, New York Prisons Still Lack Cameras*, NEW YORK FOCUS: CRIMINAL JUSTICE (Feb. 3, 2025), <https://nysfocus.com/2025/02/03/robert-brooks-doccs-prisons-cameras>.

¹⁰⁵⁴ Rebecca McCray & Chris Gelardi, *A Decade and \$600 Million Later, New York Prisons Still Lack Cameras*, NEW YORK FOCUS: CRIMINAL JUSTICE (Feb. 3, 2025), <https://nysfocus.com/2025/02/03/robert-brooks-doccs-prisons-cameras>.

¹⁰⁵⁵ [Cite Redacted].

discussed in Section 12, “Staffing.”¹⁰⁵⁶ The installation timeline is also constrained by the state contracting system’s eccentricities.¹⁰⁵⁷ Nevertheless, following the Governor’s acceleration of fixed camera funding after the murder of Mr. Brooks, DOCCS expects all facilities to have complete installation within the next five years.¹⁰⁵⁸

Marcy and Mid-State have both finished installing cameras in their respective infirmaries.¹⁰⁵⁹ Both facilities reported the next proposed work would focus on exterior areas rather than on housing units.¹⁰⁶⁰ Facility-based staff were unclear as to why those locations were chosen, reporting that they were just given an installation schedule and that they have not been involved in decision-making regarding camera installation.¹⁰⁶¹ DOCCS leadership disputed this characterization, stating that facilities have been included in discussions and noting that the confusion was likely due to miscommunication.¹⁰⁶² DOCCS estimates that Marcy will complete its fixed camera installation facility-wide by 2028.¹⁰⁶³ Mid-State does not have a formal completion date but, in line with expectations for other facilities, DOCCS estimates that it will finish installation within the next five years.¹⁰⁶⁴

14.2 DOCCS Should Equip All Transport Vans with Fixed Cameras

DOCCS does not currently have a plan for installing fixed cameras in transport vans and does not consider it a priority.¹⁰⁶⁵

¹⁰⁵⁶ [Cite Redacted].

¹⁰⁵⁷ [Cite Redacted].

¹⁰⁵⁸ [Cite Redacted].

¹⁰⁵⁹ [Cite Redacted].

¹⁰⁶⁰ [Cite Redacted].

¹⁰⁶¹ [Cite Redacted].

¹⁰⁶² [Cite Redacted].

¹⁰⁶³ [Cite Redacted].

¹⁰⁶⁴ [Cite Redacted].

¹⁰⁶⁵ [Cite Redacted].

Facilities use vans to move incarcerated individuals around the facility; for example, from the dormitory to the infirmary. Throughout the review, incarcerated individuals consistently expressed that they feared transport in facility vans; they reported that the absence of cameras in transport vans enabled abuse and recounted prior instances where they experienced, witnessed, or heard of physical assaults in transport vans.¹⁰⁶⁶ In the case of Mr. Nantwi, after assaulting Mr. Nantwi in his cell, officers allegedly continued to beat him in the transport van on the way to the infirmary.¹⁰⁶⁷ Other incarcerated individuals at Marcy reported frequent uses of force during van rides—including reports of being beat up themselves, witnessing others being dragged into vans to be beaten, or hearing screaming coming from inside vans.¹⁰⁶⁸ According to a former Marcy correction officer, these “bumpy rides” were not uncommon.¹⁰⁶⁹ One former officer told us: “Had there been dash cameras in the transport vans used to transport incarcerated individuals, there would have been a camera in the van that transported Brooks from Mohawk to Marcy. I doubt he would have been beaten in the van.”¹⁰⁷⁰

We recommend that DOCCS install fixed cameras in all transport vans. While officers are required to wear BWCs, which should be active during transports, the identified issues with ensuring BWC compliance necessitate fixed camera surveillance in this high-risk area.

14.3 Recommendation Summary

- 14.3.1 Continue prioritizing installation of fixed cameras, including installing cameras in high-need areas, such as transport vans.

¹⁰⁶⁶ See Section 5, “Findings.”

¹⁰⁶⁷ See Section 1.4, “Killing of Messiah Nantwi”

¹⁰⁶⁸ See Section 2.1, “Marcy Prior Incidents;” Section 5.3, “Incarcerated Individuals’ Perspectives on Safety and Use of Force;” [Cite Redacted].

¹⁰⁶⁹ [Cite Redacted].

¹⁰⁷⁰ [Cite Redacted].

15. **INCARCERATED LIAISON COMMITTEE (ILC)**

According to Directive #4002, each general confinement correctional facility is required to establish an ILC unless specifically exempted.¹⁰⁷¹ The purpose of the ILC is to:

- Facilitate effective communication between incarcerated individuals and facility administration;
- Provide a structured forum for suggestions and feedback from incarcerated individuals regarding facility operations; and
- Promote the general welfare of the incarcerated population through regular dialogue with institutional officials.¹⁰⁷²

The ILC can be a valuable tool for strengthening collaboration between facility leadership and incarcerated individuals. This group provides a structured forum for communication that promotes both actual and perceived transparency, responsiveness, and trust—ultimately supporting DOCCS’ mission. Additionally, the ILC can help achieve rehabilitative objectives by encouraging constructive dialogue, resolving conflicts, and reaching compromises. Our review found that DOCCS can do more to ensure that the ILCs are given the necessary tools to achieve these goals and support the incarcerated population. We saw inconsistencies in the treatment of ILCs across facilities and opportunities for process improvements that can make a significant difference in giving the incarcerated population a voice.

¹⁰⁷¹ DOCCS Directive #4002 provides for the creation and operation of an ILC at each DOCCS facility. The objectives of these ILCs are twofold: (1) to “provide ... [e]ffective communications between incarcerated individuals and administration for accurate dissemination and exchange of information” and (2) to “provide ... [a] means to facilitate consideration and analysis of suggestions from incarcerated individuals relative to facility operations.” Directive #4002, §I (Aug. 5, 2024).

¹⁰⁷² Directive #4002, §§I, II.A.1 (Aug. 5, 2024).

15.1 DOCCS Should Improve the ILCs' Effectiveness

Our survey data suggests that the ILCs are not reaching their full potential at Marcy and Mid-State. The purpose of the ILC is to provide a line of communication between incarcerated individuals and the facility administration, including a mechanism for suggestions from incarcerated individuals to be considered. But our review found that ILCs are not consistently serving that purpose.

Most incarcerated individuals at Marcy and Mid-State reported the way the ILC addresses concerns at the facility as either “Poor” or “Very Poor” (57.2% of the 361 incarcerated individual respondents at Marcy and 54.7% of the 440 incarcerated individual respondents at Mid-State).¹⁰⁷³ Less than 10% of those surveyed at Marcy and Mid-State reported that the ILC addresses concerns at the facility in a way that is either “Satisfactory” or “Very Satisfactory.”¹⁰⁷⁴

Many ILC members shared frustration about the facility and the incarcerated population undermining or ignoring their hard work and advocacy victories. For example, we heard that one ILC convinced the deputy superintendent of security to allow incarcerated individuals to bring water bottles onto the yard during the summer.¹⁰⁷⁵ However, when individuals tried to bring the bottles to recreation time, correction officers confiscated the bottles and threw them away.¹⁰⁷⁶ This occurred after the ILC informed the incarcerated population about the change to the water bottle policy, undermining both the ILC’s integrity and the deputy superintendent’s authority.¹⁰⁷⁷ Some ILC members from Marcy and Gouverneur told us that they had not achieved a single “win” for several years, while Mid-State only had one.¹⁰⁷⁸ These perceptions—whether accurate

¹⁰⁷³ [Cite Redacted].

¹⁰⁷⁴ [Cite Redacted].

¹⁰⁷⁵ [Cite Redacted].

¹⁰⁷⁶ [Cite Redacted].

¹⁰⁷⁷ [Cite Redacted].

¹⁰⁷⁸ [Cite Redacted].

or not—spread among the incarcerated population, undermine the population’s trust in the ILC, and ultimately worsen the relationship between the incarcerated population and facility staff.

Because the ILC is a partner in the rehabilitation mission, DOCCS and facility leadership should highlight ILC accomplishments and ensure that commitments to the ILC are kept and clearly communicated to both staff and the incarcerated population. This collaboration can be as simple as asking the ILCs what they think about a key DOCCS challenge. For example, a memorandum from the Commissioner dated September 23, 2025, committed to “engag[ing] with various Incarcerated Liaison Committees to solicit ideas on additional drug interdiction measures that will safeguard our institutions.”¹⁰⁷⁹ Facility leadership should follow this instruction and engage with ILCs on the current drug problems.

In addition, as described herein, we recommend that DOCCS do more to follow the guidance laid out in its existing ILC directive, while finding ways to strengthen meaningful ILC participation and engagement.

15.2 DOCCS Should Follow the ILC Directive

Although Directive #4002 provides a reliable framework for achieving the promise of the ILCs, our review found that many of the procedures may not be faithfully followed in practice.

15.2.1 Adhere to Meeting and Minutes Procedures

Directive #4002 provides that meetings should be held monthly between the ILC and the superintendent or facility executive team.¹⁰⁸⁰ These meetings were not being held monthly at either Marcy or Mid-State when we visited.¹⁰⁸¹ And even when these meetings do occur, key facility leadership often miss the meetings.¹⁰⁸² In addition, we saw firsthand that ILC meetings

¹⁰⁷⁹ [Cite Redacted].

¹⁰⁸⁰ Directive #4002, §II.A.4 (Sept. 23, 2025).

¹⁰⁸¹ [Cite Redacted].

¹⁰⁸² [Cite Redacted].

may not be scheduled and publicized with sufficient advanced notice to attend, so members show up to the meeting without relevant paperwork or are otherwise unprepared.¹⁰⁸³ Unsurprisingly, Sing Sing and Gouverneur—where meetings appear to be held regularly with engagement from facility leadership—had ILCs that seemed to be more effective at achieving staff and incarcerated individual buy-in.¹⁰⁸⁴ Facilities should adhere to the monthly meeting cadence to ensure that ILCs realize their full potential.

There also appear to be shortcomings in the required review, signing, and archiving of ILC meeting minutes; this, in turn, makes it nearly impossible to memorialize the issues and communicate them to the incarcerated population.¹⁰⁸⁵ The directive provides that the minutes are to be signed by both the superintendent and the ILC president and then forwarded to the assigned Assistant Commissioner for Facility Operations.¹⁰⁸⁶ At Marcy, we learned that the facility superintendent neither takes minutes nor requests the ILC’s minutes, limiting the actual and perceived effectiveness of the meeting.¹⁰⁸⁷ ILCs at Marcy, Mid-State, and Sing Sing reported challenges in sharing meeting information with their constituents.¹⁰⁸⁸ There are no formal guidelines regarding how the ILCs should distribute information to the broader population. We recommend that facility leaders develop a consistent method of taking, authorizing, and distributing meeting minutes to staff and incarcerated individuals. Moreover, we recommend that DOCCS modify the directive to include a method of formal distribution of meeting minutes

¹⁰⁸³ [Cite Redacted].

¹⁰⁸⁴ Note that our interviews at Gouverneur and Sing Sing indicated that these procedures were generally followed at their facilities.

¹⁰⁸⁵ Directive #4002, §II.A.4 (Sept. 23, 2025).

¹⁰⁸⁶ Directive #4002, §II.A.4 (Sept. 23, 2025).

¹⁰⁸⁷ [Cite Redacted].

¹⁰⁸⁸ [Cite Redacted].

across the facility, for example, by requiring the monthly minutes to be posted in each housing unit and in the administrative building.

The facilities should follow Directive #4002, which requires ILC meetings, and ensure that executive staff with decision-making authority attend these meetings. In addition, DOCCS should consider designating staff representatives from different facility departments (e.g., medical, mental health, etc.) to attend the meetings to foster cross-disciplinary staff support for the ILC.

15.2.2 Provide Adequate Space and Supplies to Carry Out ILC Functions

Directive #4002 also requires that the facility provide the ILC with adequate space and supplies to carry out its function, including a room, typewriter, desks, supplies, and stationary.¹⁰⁸⁹ However, we learned that at Marcy and Sing Sing, there is no designated office space and little to no opportunity for the ILC to meet as a group to prepare and discuss agendas.¹⁰⁹⁰ We heard that the Marcy ILC lacks access to basic tools such as printers or computers, which further limits its ability to communicate with the broader population.¹⁰⁹¹ While these logistics may seem minor, they are necessary for members to function effectively. Therefore, we recommend that facilities ensure that ILCs have access to meeting spaces and supplies to communicate with each other and the incarcerated population, in accordance with Directive #4002.

15.2.3 Provide Access to ILC Financial Information

In accordance with Directive #4002, ILCs may raise funds for the Incarcerated Benefit Fund (IBF) by holding hobby shop sales, organizing photo fundraisers in visiting rooms, and

¹⁰⁸⁹ Directive #4002, §II.E (Sept. 23, 2025).

¹⁰⁹⁰ [Cite Redacted].

¹⁰⁹¹ [Cite Redacted].

engaging in similar activities.¹⁰⁹² With the approval of facility leadership, the IBF can be used to purchase items for the benefit of the entire incarcerated population, like premium TV channels, video entertainment, items for the family visiting center, etc.¹⁰⁹³ All ILCs interviewed stated they did not benefit from their fundraising efforts.¹⁰⁹⁴ Even more concerning, three of the four ILCs alleged that the facility had misappropriated ILC funds, including by purchasing items that were installed in the administrative building or, even worse, taken home for personal staff use.¹⁰⁹⁵ If ILC funds have been withheld or redirected inappropriately, they should be restored to the ILC; the practice should be prohibited and investigated where suspected.

Going forward, there should be full transparency for the ILC on how the IBF funds are used. Indeed, although DOCCS' policies say that the ILC or its treasurer may receive a copy of the IBF ledger upon request,¹⁰⁹⁶ we heard that either the ILCs did not know of this provision or were denied this information.¹⁰⁹⁷ Two ILCs stated that their members had never been able to see copies of their account ledgers, despite multiple requests.¹⁰⁹⁸ Furthermore, ILCs at Marcy and Gouverneur stated that staff had instructed incarcerated individuals to sign forms confirming the supposed purchase of certain goods, but the incarcerated individuals never saw those purchases.¹⁰⁹⁹

Considering these findings, we recommend that facilities prioritize compliance with DOCCS' policies relating to ILC funds by supplying copies of IBF account ledgers to the ILCs

¹⁰⁹² Directive #4002, §II.F (Sept. 23, 2025); *see also* Directive #2771 (Feb. 12, 2024).

¹⁰⁹³ Directive #2771, §II, II.B.2 (Feb. 12, 2024).

¹⁰⁹⁴ [Cite Redacted].

¹⁰⁹⁵ [Cite Redacted].

¹⁰⁹⁶ Directive #2771, §III.E (Feb. 12, 2024).

¹⁰⁹⁷ [Cite Redacted].

¹⁰⁹⁸ [Cite Redacted].

¹⁰⁹⁹ [Cite Redacted].

when requested and providing documentation of supplies purchased through the IBF or otherwise for the ILCs' benefit.

15.2.4 Encourage and Facilitate ILC Membership

The ILCs run, in large part, on the dedicated work of their members. As such, facility leadership should do everything possible to encourage full and active membership in its ILC.

These efforts should include fostering the democratic process underlying ILC member selection. According to the directive, the members of the ILC are to be elected by the general population and, at the very least, each housing unit is to be represented.¹¹⁰⁰ However, two of the ILCs we visited did not have representation for all housing units and reported difficulty filling complete membership.¹¹⁰¹ ILC representatives attributed this low membership to several factors, including fear of retaliation, constantly changing housing, and a perception that ILCs are not effective.¹¹⁰² This presents a vicious cycle: low membership leads to low effectiveness, which discourages participation and contributes to the perception that the ILC is ineffective as the voice of those it should be representing.¹¹⁰³ Facility leadership should encourage participation on the ILC by widely publicizing ILC elections, highlighting successful ILC members and achievements, and otherwise appropriately incentivizing ILC membership and participation.

15.3 Recommendation Summary

- 15.3.1 Ensure that ILC processes at facilities are consistent with the guidance in the existing ILC directive.

¹¹⁰⁰ Directive #4002, §II.B.2-6 (Sept. 23, 2025).

¹¹⁰¹ [Cite Redacted].

¹¹⁰² [Cite Redacted].

¹¹⁰³ [Cite Redacted].

16. PROGRAMMING AND RECREATION

Consistent, meaningful programming and recreation opportunities are important in maintaining a positive, safe correctional culture, and offering adequate programming is an essential component of correctional facilities' rehabilitative roles. Executive staff at Gouverneur, for example, reported that keeping incarcerated individuals productively and meaningfully occupied through programming and out-of-cell time is essential for maintaining a positive mood among incarcerated individuals and keeps life better for staff at the facility.¹¹⁰⁴ Prioritizing consistent and meaningful programming demonstrates that DOCCS values its rehabilitative role and contributes to maintaining a humane and respectful facility culture.

However, the lack of staff at DOCCS has created significant challenges in providing effective programming.¹¹⁰⁵ Staff at Sing Sing reported pride in having a culture that is “rehabilitative in nature” for over 20 years but said that the culture had begun to deteriorate, at least in part as a result of low staffing levels that prevented the facility from offering programming.¹¹⁰⁶ A lack of programming not only hinders incarcerated individuals' well-being and rehabilitation, but also inhibits incarcerated individuals from earning credits toward earlier release. Incarcerated individuals at Sing Sing expressed frustrations about diminished programming,¹¹⁰⁷ including because incarcerated individuals can earn incentives for sentence reductions after meeting program goals subject to consideration of their disciplinary history.¹¹⁰⁸

¹¹⁰⁴ [Cite Redacted].

¹¹⁰⁵ [Cite Redacted].

¹¹⁰⁶ [Cite Redacted].

¹¹⁰⁷ [Cite Redacted].

¹¹⁰⁸ Directive #4792 (Nov. 17, 2024).

16.1 Programming

DOCCS matches individuals with available facility programming based on a risk and needs assessment tool—the Correctional Offender Management Profiling for Alternative Sanctions (COMPAS).¹¹⁰⁹ COMPAS is a predictive computer-based tool to measure an incarcerated individual’s risk and needs.¹¹¹⁰ When an individual is admitted to DOCCS, they receive a COMPAS risk assessment.¹¹¹¹ Upon transfer to a facility, an assigned Offender Rehabilitation Coordinator, in coordination with the incarcerated individual, uses the COMPAS risk assessment to develop a case plan that matches the individual’s risk and needs with the facility’s available programming.¹¹¹² Case plans are reviewed on a regular basis—every three months for those within four years of their earliest release date, or twice a year for those further from release.¹¹¹³ We conducted random reviews of individual case plans and found that both Marcy and Mid-State are reviewing case plans consistent with DOCCS policy.¹¹¹⁴ Individuals also receive a reentry COMPAS assessment prior to release from custody to transition during post-release supervision.¹¹¹⁵

Agency directives and the HALT Act require out-of-cell programming and congregate activity opportunities that range from four to seven hours per day, depending on unit type.¹¹¹⁶ At Marcy and Mid-State, each facility’s deputy superintendent of programs is responsible for overseeing the facility’s rehabilitative programming efforts.¹¹¹⁷ Expected programming at these

¹¹⁰⁹ Directive #8500 (Oct. 11, 2023).

¹¹¹⁰ Global Institute of Forensic Research, *COMPAS On Demand Training* (MHS Public Safety), <https://gifrinc.com/product/compas>.

¹¹¹¹ Directive #8500 (Oct. 11, 2023).

¹¹¹² Directive #8500 (Oct. 11, 2023).

¹¹¹³ Directive #8500 (Oct. 11, 2023).

¹¹¹⁴ [Cite Redacted].

¹¹¹⁵ Directive #8500 (Oct. 11, 2023).

¹¹¹⁶ Directive #4933 (Jan. 17, 2024); Directive #8500 (Oct. 11, 2023); HALT Act (N.Y. 2021), §5.

¹¹¹⁷ Directive #4803 (Nov. 19, 2021).

facilities includes academic, vocational, industry, transitional services, work assignment, aggression replacement training, sex offender treatment, substance abuse treatment, cognitive behavioral therapy, Trauma, Addiction, Mental Health and Recovery (TAMAR), and orientation classes.¹¹¹⁸ A mix of program counselors, teachers and instructors, volunteers, and health service professionals facilitates the various programs.¹¹¹⁹

We found that the classroom space and educational resources for out-of-cell programming provided participation opportunities for a significant portion of the general populations at both Marcy and Mid-State, and their quality tended to exceed that of other facilities in jurisdictions across the country.¹¹²⁰ The designated programming classrooms at Marcy and Mid-State provide a conducive learning environment. The classrooms had sufficient space and included smartboards and recently upgraded computers.¹¹²¹

At both Marcy and Mid-State, programming in SHUs primarily consists of in-cell programming: program staff or counselors distribute and collect self-study materials tailored to individuals based on their COMPAS risk assessments and case plans.¹¹²² Both facilities also offer limited out-of-cell congregate opportunities which, in both facilities' SHUs and at Mid-State's RRU, occur in secure chairs near each individual's assigned cell.¹¹²³ Out-of-cell programming at Marcy's RMHU and Mid-State's SDP occurs in classrooms equipped with secure programming chairs or booths and separate secure spaces for individual therapy sessions.¹¹²⁴ We found that the design, setup, and visual aids in these rooms meet expected

¹¹¹⁸ *Programs*, DOCCS, <https://doccs.ny.gov/programs>; [Cite Redacted].

¹¹¹⁹ [Cite Redacted].

¹¹²⁰ [Cite Redacted].

¹¹²¹ [Cite Redacted].

¹¹²² [Cite Redacted].

¹¹²³ [Cite Redacted].

¹¹²⁴ [Cite Redacted].

programming practices and support an appropriate therapeutic and learning environment.¹¹²⁵

Televisions support video-based learning, and out-of-cell time may also include recreational viewing of movies or television.¹¹²⁶

During site visits, we did not observe any out-of-cell programming in Mid-State's RRU or in the SHU at either Marcy or Mid-State.¹¹²⁷ Out-of-cell programming was generally absent in Marcy's RMHU, too, with the exception of one orientation session.¹¹²⁸ In explaining the lack of programming, staff cited staffing shortages and a lack of participant interest.¹¹²⁹

The lack of participant interest is consistent with our review of SHU records, which indicated high rates of program refusals, and with interviews with SHU residents that confirmed a pattern of refusals.¹¹³⁰ However, we observed several incarcerated individuals who spent more time than allowed in the SHU at Marcy. During site visits in March and May 2025, we identified incarcerated individuals who were being held in the SHU, with some enduring more than 50 or 60 days.¹¹³¹ We also received information about an incarcerated individual who had spent 140 consecutive days in SHUs and RRUs at Marcy and Greene (following a transfer) between March and July 2025.¹¹³²

The absence of therapeutic programming and congregate activity in the RMHU at Marcy and in the RRU, SHU, and SDP units at Mid-State has adversely impacted the ability of staff at both facilities to complete a meaningful assessment and evaluation of incarcerated individuals'

¹¹²⁵ [Cite Redacted].

¹¹²⁶ [Cite Redacted].

¹¹²⁷ [Cite Redacted].

¹¹²⁸ [Cite Redacted].

¹¹²⁹ [Cite Redacted].

¹¹³⁰ [Cite Redacted].

¹¹³¹ [Cite Redacted].

¹¹³² [Cite Redacted].

readiness to safely return to a less restrictive housing setting or the general population—potentially lengthening incarcerated individuals’ stay in restrictive housing. Facility leadership also stated that, in rare instances, incarcerated individuals from the SDP unit are released directly into the community, which raises concerns about the adequacy of their rehabilitation upon reentry.¹¹³³ Incarcerated individuals in these units should have the ability to earn and experience distinct levels of programming and activity; this incentivizes positive behavior and progress toward an individual’s rehabilitation goals.¹¹³⁴

The absence of therapeutic programming and activities, combined with staff interactions and attitudes at the time of our site visits, left us with the impression that the units were operating for the sole purpose of serving time for disciplinary sanctions as opposed to being safe and structured program units, and that leadership was not creative in responding to the staffing challenge to provide some consistent degree of services in the RMHU at Marcy and the RRU and SDP units at Mid-State.¹¹³⁵ Indeed, a DOCCS executive attributed the Marcy RMHU’s prolonged non-compliance with programming requirements to the facility staff’s failure to think creatively about solutions.¹¹³⁶

We understand that circumstances may have changed after our site visits. On July 1, 2025, because of litigation arising out of Commissioner Martuscello’s February 2025 suspension of programming required by the HALT Act, Judge Daniel Lynch ordered DOCCS not to suspend any HALT provisions without first finding that there was a facility-wide emergency.¹¹³⁷ On July

¹¹³³ [Cite Redacted].

¹¹³⁴ Ryan M. Labrecque, et al., *Reforming Solitary Confinement: The Development, Implementation, and Processes of a Restrictive Housing Step Down Reentry Program in Oregon*, 9 HEALTH & JUSTICE, p. 3 (Aug. 2021), <https://link.springer.com/article/10.1186/s40352-021-00151-9>.

¹¹³⁵ [Cite Redacted].

¹¹³⁶ [Cite Redacted].

¹¹³⁷ Decision and Order, *Smalls v. Martuscello*, Index No. 903926-25 (Sup. Ct., Albany County July 1, 2025).

14, 2025, Commissioner Martuscello stated in court filings that an emergency continued to exist across all facilities and that 14 facilities had completely restored required programming, 17 had partially restored programming, and three facilities, including Mid-State, had not restored any HALT programming elements.¹¹³⁸ As of June 2026, DOCCS is continuing to respond to the litigation.

Despite these challenges with out-of-cell programming, both Marcy and Mid-State issue a tablet to each individual in special housing on a daily basis, which provides access to phone calls, radio, select games, and limited self-help resources.¹¹³⁹ Both facilities could enhance special housing programming by expanding tablet-based offerings, including with suites like Edovo that include thousands of programs and can award certificates of completion but are currently only on the tablets for the general population.¹¹⁴⁰

16.2 Recreation

Recreation provides another opportunity for incarcerated individuals to spend time outside of their cells. Current best practices call for at least four hours a day of out-of-cell time for incarcerated individuals in restrictive housing to be spent on activities that should emphasize enrichment, including structured programming, treatment opportunities, exposure to nature with physical activity, and meaningful human contact.¹¹⁴¹ Particularly for those in restrictive housing,

¹¹³⁸ Martuscello Affirmation, *Smalls v. Martuscello*, Index No. 903926-25 (Sup. Ct., Albany County July 14, 2025).

¹¹³⁹ [Cite Redacted].

¹¹⁴⁰ Edovo Support, Edovo's Content Catalog, <https://support.edovo.com/portal/en/kb/articles/edovo-content-lists>; [Cite Redacted].

¹¹⁴¹ Washington State Department of Corrections et al., *Solitary Confinement Transformation Project; Requirements for Sustainable Reduction* (Sept. 2023), p. 20; Falcon Inc., *Connecticut Department of Correction (CTDOC) Restrictive Housing System Study: Comprehensive Study, Program Validation, and Best Practice Recommendations* (Nov. 27, 2024); David H. Cloud et al., *The Resource Team: A Case Study of a Solitary Confinement Reform in Oregon*, 18 *PLoS ONE*, p. 11 (2023), <https://doi.org/10.1371/journal.pone.0288187>.

among whom severely mentally ill incarcerated individuals are overrepresented, engagement-focused out-of-cell opportunities can mitigate harms related to solitary confinement.¹¹⁴²

The indoor and outdoor recreation areas for general population individuals at Marcy and Mid-State include a large outdoor recreation yard with strength-training equipment and space for organized sports or cardiovascular activities, as well as sizeable indoor gymnasiums.¹¹⁴³ The recreational spaces at both facilities' SHUs, Marcy's RMHU, and Mid-State's RRU and SDP units are caged-in, outdoor, individual exercise areas that are adjacent to each cell, remain unlocked for extended periods to comply with policy, and lack strength training or other equipment, which are commonly available in such settings in other correctional systems.¹¹⁴⁴ Both facilities should re-evaluate whether those spaces offer out-of-cell recreation consistent with the facilities' otherwise forward-thinking design of program spaces.

While SHU residents at both facilities and residents of the Mid-State RRU were regularly offered outdoor recreation opportunities as required by policy, staff reported that participation remained low.¹¹⁴⁵ And although Marcy's RMHU and Mid-State's RRU also offer congregate recreational areas for those at higher program levels,¹¹⁴⁶ staff similarly reported that they had not been used for over a year,¹¹⁴⁷ which was consistent with our review of incarcerated individuals' and unit records, which indicated high rates of program refusals.¹¹⁴⁸

¹¹⁴² B. Desrochers, *Associations Between Mental Health and Restrictive Housing*, pp. 4-5, Massachusetts Dep't of Corrections, Office of Strategic Planning and Research (2020).

¹¹⁴³ [Cite Redacted].

¹¹⁴⁴ [Cite Redacted].

¹¹⁴⁵ [Cite Redacted].

¹¹⁴⁶ [Cite Redacted].

¹¹⁴⁷ [Cite Redacted].

¹¹⁴⁸ [Cite Redacted].

16.3 Staffing Shortages Negatively Impact Programming And Recreation Availability

Current correction-officer staffing shortages are having a detrimental effect on program effectiveness at both facilities because security staff are needed to support the programming. Many program providers are unable to carry out their core duties due to insufficient numbers of security officers assigned to programming posts.¹¹⁴⁹ A current correction officer shared with us that facilities were too short-staffed to run programming, noting, “You can’t cut staff and expect us to run prisons like we are fully staffed ... We are all pro-programming. Programming keeps the [incarcerated individuals] busy, but it’s hard to be done safely. You need staff.”¹¹⁵⁰

Programming was paused altogether at Marcy and Mid-State in February 2025 due to the illegal strike by correction officers.¹¹⁵¹ As one incarcerated individual noted, things were worse after the illegal strike because there were “no programs[,] nothing to do.”¹¹⁵² Following the illegal strike, some program staff were reassigned to support other services such as visitation, the law library, and package rooms.¹¹⁵³ During site visits in May 2025 at Marcy and in September 2025 at Marcy and Mid-State, we found empty classrooms and high levels of idleness on the part of incarcerated individuals and that neither Marcy nor Mid-State was meeting the Commissioner’s programming expectations.¹¹⁵⁴ During site visits to both facilities in September 2025, program providers were grossly underutilized, working in offices or classrooms with no

¹¹⁴⁹ [Cite Redacted].

¹¹⁵⁰ [Cite Redacted].

¹¹⁵¹ Memorandum of Agreement Between the State of New York and NYSCOPBA (Mar. 8, 2025), <https://doccs.ny.gov/system/files/documents/2025/03/moa-doccs-nyscopba-3.8.2025.pdf>; see also Chris Gelardi, *‘This Place is a Circus’: Eight Months Since a Guard Strike, State Prisons Remain in Crisis*, NYS FOCUS (Oct. 30, 2025), <https://nysfocus.com/2025/10/30/new-york-prison-strike-lockdowns>; see also Rebecca McCray, *New York’s Prison Guard Strike Ended Months Ago. For Some, Life-Threatening Effects Persist*, THE MARSHALL PROJECT (July 26, 2025), <https://www.themarshallproject.org/2025/07/26/new-york-prison-guard-strike-effects>.

¹¹⁵² [Cite Redacted].

¹¹⁵³ [Cite Redacted].

¹¹⁵⁴ [Cite Redacted].

participants,¹¹⁵⁵ and some programs had not restarted even as of August 2025.¹¹⁵⁶ Marcy and Mid-State staff reported that beginning the week of September 8, 2025, they began a slow and limited re-opening of educational and vocational programs,¹¹⁵⁷ but this has not been confirmed.

During site visits in May 2025 to Marcy and in September 2025 to Marcy and Mid-State, the general population use of the outdoor recreation yard at both facilities was minimal.¹¹⁵⁸ Staff at both Marcy and Mid-State attributed this limited access to ongoing staff shortages, which directly impacted the ability to open indoor and outdoor recreation areas; the outdoor recreation yards had not been regularly in use since the illegal strike.¹¹⁵⁹ As of the September 4, 2025 site visit at Mid-State, facility staff reported that recreation area schedules were determined on a day-to-day basis depending on available staffing.¹¹⁶⁰

Despite the staffing challenges, we found DOCCS' policies related to programming for specialized and high-risk populations to be innovative, evidence-guided, and appropriately secure. Multiple incarcerated individuals stated that programming can be one of the most positive aspects of their facilities.¹¹⁶¹ We recommend that Marcy and Mid-State review staffing to ensure sufficient in-person programs and recreation area use.

16.4 Recommendation Summary

- 16.4.1 Expand tablet-based offerings in special housing units, with an emphasis on adding programming opportunities.
- 16.4.2 Track staff deployment and prioritize staffing for programming and recreation areas, including tracking closed job assignments (posts) by

¹¹⁵⁵ [Cite Redacted].

¹¹⁵⁶ [Cite Redacted].

¹¹⁵⁷ [Cite Redacted].

¹¹⁵⁸ [Cite Redacted].

¹¹⁵⁹ [Cite Redacted].

¹¹⁶⁰ [Cite Redacted].

¹¹⁶¹ [Cite Redacted].

location and rotating closed posts appropriately to improve opportunities for programming and recreation.

17. CLASSIFICATION

“Classification” is the process by which incarcerated individuals are generally assigned to various custody levels such as maximum, medium, or minimum custody facilities within a correctional system based on offense severity, criminal history, remaining time on sentence, risk level, medical and mental health or other individualized needs, behavioral characteristics, and other factors.¹¹⁶² Classification systems are vital for the safe operation of a correctional facility, efficient use of staff, and for maximizing programming and reentry benefits for incarcerated individuals. We reviewed classification because it is critical to ensuring that incarcerated individuals are housed appropriately based on their security, rehabilitation, medical, and mental health needs. Without proper classification, incarcerated individuals can face safety threats and other roadblocks to rehabilitation.

17.1 DOCCS Should Revalidate the Classification System

Upon admission to one of DOCCS’ designated intake centers, an incarcerated individual receives a series of tests and assessments, including a medical and mental health screening, a PREA risk screening,¹¹⁶³ and a Gender Identity Interview to understand an individual’s gender identity, expression, and preferences.¹¹⁶⁴ The system provides extended classification guidelines for special-needs populations and transgender or intersex incarcerated individuals.¹¹⁶⁵ Classification scores result in an assignment of maximum, medium, or minimum custody, along with a mental health classification (one of five levels as determined by OMH,

¹¹⁶² See Directive #4020 (Oct. 23, 2025); Directive #4021 (Jan. 23, 2019).

¹¹⁶³ Directive #4021 (Jan. 23, 2019).

¹¹⁶⁴ Directive #4021 (Jan. 23, 2019).

¹¹⁶⁵ Directive #4021 (Jan. 23, 2019).

Bureau of Forensic Services) and a medical classification (one of three levels as determined by the Division of Health Services (DHS)).¹¹⁶⁶

Today, a much lower percentage of DOCCS' population is classified as minimum security than nationwide.¹¹⁶⁷ As of March 2026, DOCCS' primary custody levels consist of maximum security (45%), medium security (54%), and minimum security or below (2%).¹¹⁶⁸ DOCCS has only three minimum security facilities to house individuals classified as minimum security.¹¹⁶⁹ Nationwide, a significantly higher percentage of both incarcerated men (36.5%) and women (53.4%) are in minimum or community custody.¹¹⁷⁰ The proportion of incarcerated individuals across DOCCS at each security classification has remained consistent over the past ten years, except for during the COVID-19 pandemic between 2020 and 2022, when the number of incarcerated individuals in maximum-security facilities temporarily exceeded the number of incarcerated individuals in medium-security facilities.¹¹⁷¹ (see Figure 39).

¹¹⁶⁶ [Cite Redacted]. Directive #9230 (Aug. 28, 2024); Directive #4017 (June 6, 2022).

¹¹⁶⁷ See, e.g., Brian Mann, *The Drug Laws That Changed How We Punish*, NPR (Feb. 14, 2013), <https://www.npr.org/2013/02/14/171822608/the-drug-laws-that-changed-how-we-punish>; Elizabeth Jonhson, *Trends in the New York State Prison Population, 2008-2023*, Data Collaborative for Justice, <https://datacollaborativeforjustice.org/wp-content/uploads/2023/07/PrisonPop.pdf>.

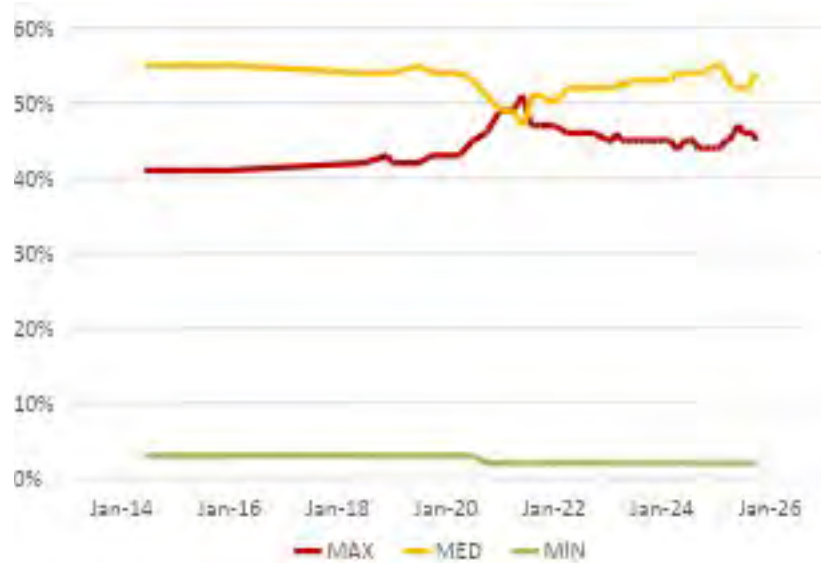
¹¹⁶⁸ CANY, Under Custody Reports Data Dashboard (last updated October 2025), <https://www.correctionalassociation.org/data/dashboard-under-custody>. The percentages reported are accurate as of March 2026 (45%, 54%, and 2%). Percentages are rounded to the nearest whole number. As a result, totals may not sum to exactly 100%. The reported proportions are for DOCCS' total population, not delineated by an individual variable (e.g., gender).

¹¹⁶⁹ CANY, Mapping Closures, Capacity, Staffing, and the Evolving Landscape of NY's Prisons (Nov. 13, 2025), <https://www.correctionalassociation.org/prison-map>.

¹¹⁷⁰ Patricia L. Hardyman & James Austin, *Objective Prison Classification: A Guide for Correctional Agencies*, p. 15 (U.S. Dep't of Justice, Nat'l Inst. of Corr. 2d Ed. 2021).

¹¹⁷¹ CANY, Under Custody Reports Data Dashboard (last updated October 2025), <https://www.correctionalassociation.org/data/dashboard-under-custody>.

Figure 39 DOCCS Total Population by Security Classification



Currently, DOCCS re-classifies incarcerated individuals every six months and discusses their security classification during routinely scheduled interviews with an assigned Offender Rehabilitation Coordinator (ORC).¹¹⁷² Facility staff gather documents related to behavior, investigations, and separation status.¹¹⁷³ Ultimately, the ORC sends information on the incarcerated individual's progress (including recent incidents or misbehavior) directly to the DOCCS central office in Albany, and the central office decides whether to reclassify or transfer the individual.¹¹⁷⁴ Per DOCCS' policy, if the incarcerated individual is dissatisfied with the decision or has information that could influence their security classification, they can submit an appeal in writing to the Supervising Offender Rehabilitation Coordinator (SORC).¹¹⁷⁵ If the facility Guidance and Counseling staff conclude that the incarcerated individual's classification

¹¹⁷² Directive #4020 (Oct. 23, 2025); Directive #4401 (Aug. 21, 2020).

¹¹⁷³ [Cite Redacted].

¹¹⁷⁴ [Cite Redacted].

¹¹⁷⁵ Directive #4020 (Oct. 23, 2025).

should be corrected, they forward the appeal to the central office.¹¹⁷⁶ The facility then submits the appeal to the central office with supporting documentation.¹¹⁷⁷ The final decision on the appeal rests with the central office.¹¹⁷⁸

Marcy facility staff indicated that they did not feel that the information they provided to the central office had any impact on the ultimate reclassification decision.¹¹⁷⁹ In general, facility staff said they felt isolated from the reclassification process.¹¹⁸⁰ They indicated that more involvement from ORCs and incarcerated individuals would be beneficial.¹¹⁸¹ Staff also stated that, in practice, the appeals process is “non-existent.”¹¹⁸²

We recommend that DOCCS modernize the DOCCS classification system by conducting a thorough revalidation that integrates both risk factors and behavior-based criteria. This process should use current research and best practices to ensure that housing decisions accurately reflect individuals’ needs and risks. DOCCS’ current classification system is at least 30 years old and has not been revalidated since its inception.¹¹⁸³ Revalidation is necessary to ensure accuracy of classification, to account for the changing demographics of the incarcerated population, and to align custody levels with national trends. Revalidation entails a comprehensive assessment of the current system, detailed statistical analyses of current policies, procedures, risk factors, the relative weight of risk factors, custody scale, and mandatory and discretionary override factors, as well as implementation of the updated system.¹¹⁸⁴ This process will allow DOCCS to

¹¹⁷⁶ Directive #4020 (Oct. 23, 2025).

¹¹⁷⁷ Directive #4020 (Oct. 23, 2025).

¹¹⁷⁸ Directive #4020 (Oct. 23, 2025).

¹¹⁷⁹ [Cite Redacted].

¹¹⁸⁰ [Cite Redacted].

¹¹⁸¹ [Cite Redacted].

¹¹⁸² [Cite Redacted].

¹¹⁸³ [Cite Redacted].

¹¹⁸⁴ Patricia L. Hardyman & James Austin, *Objective Prison Classification: A Guide for Correctional Agencies*, p. xv (U.S., Dep’t of Justice, Nat’l Inst. of Corr. 2d Ed. 2021).

determine what is driving the high rates of maximum- and medium-security classifications and low rates of minimum-security classifications.

DOCCS should also improve processes for including facility staff and incarcerated individuals in reclassification decisions. Allowing staff to provide meaningful input and views on reclassification reinforces the importance of the staff’s rehabilitative role and mission. While staff currently can provide input regarding an incarcerated individual’s classification, improved training could help staff better understand their roles in the process. In addition, including incarcerated individuals in the process gives them the opportunity to make a case for their path toward successful rehabilitation and may positively influence recidivism statistics. For example, informing incarcerated individuals about the behavior-based factors that are considered when making reclassification decisions may incentivize good behavior.

Placement of individuals in specialty housing units—with appropriate staffing, resources, and training—can also support individualized rehabilitation by providing supportive communities and tailored programs for certain subgroups.¹¹⁸⁵ Indeed, DOCCS has already created some such specialty units, including veterans units with puppy programs, through which incarcerated veterans raise and train puppies for adoption.¹¹⁸⁶ At Mid-State, many of the incarcerated IGRC workers come from the veterans unit. With DOCCS’ success with proof of concept thus far, it should explore the possibility of expanding veterans units across all facilities and developing additional specialized housing units tailored to specific subgroups, such as

¹¹⁸⁵ See, e.g., Patricia L. Hardyman & James Austin, *Objective Prison Classification: A Guide for Correctional Agencies*, p. 12 (U.S. Dep’t of Justice, Nat’l Inst. of Corr. 2d Ed. 2021); Kyleigh Clark-Moorman, *Restoring Promise*, NATIONAL INSTITUTE OF JUSTICE (June 5, 2025) (discussing housing units for 18-25 year olds), <https://nij.ojp.gov/topics/articles/restoring-promise#:~:text=Vera%20and%20MILPA%20researchers%20found,mistakes%2C%20and%20resolve%20issues%20together.>

¹¹⁸⁶ [Cite Redacted]; DOCCS, *Incarcerated Veterans Program* (last visited Mar. 6, 2026), <https://doccs.ny.gov/incarcerated-veterans-program>.

emerging adults (i.e., incarcerated individuals aged 18-29) and other individuals with unique needs. DOCCS should develop a formal structure for these units that includes targeted evidence-based programming for each group and establish systems for tracking participation and measuring program outcomes.

17.2 DOCCS Should Update Conflicts and Separation Lists

After the reclassification process is finished, or in situations involving safety, emergencies, or changes in medical or mental health needs, an incarcerated individual may be moved to a different DOCCS facility. As part of the transfer process, assigned correction counselors provide information regarding conflicts between incarcerated individuals.¹¹⁸⁷ The central office manages the process for investigating and determining whether a conflict exists between incarcerated individuals.¹¹⁸⁸ Marcy staff noted that some incarcerated individuals attempt to influence their housing assignments by reporting conflicts with other incarcerated individuals, such as affiliations with rival gangs that may pose a safety risk.¹¹⁸⁹ Facility staff reported that they did not understand the separation process, that they were not notified or informed if the conflicts had been cleared or resolved, and that decisions regarding conflicts or separations between incarcerated individuals are never reversed.¹¹⁹⁰ Therefore, the conflicts and separation lists become unmanageable and can contribute to increased interfacility transports and classification overrides.¹¹⁹¹ We recommend that DOCCS review the status of current “separatees”—individuals housed separately due to conflicts—to identify and resolve outdated or

¹¹⁸⁷ [Cite Redacted].

¹¹⁸⁸ [Cite Redacted].

¹¹⁸⁹ [Cite Redacted].

¹¹⁹⁰ [Cite Redacted].

¹¹⁹¹ [Cite Redacted].

irrelevant conflicts. This review should reduce unnecessary transfers, ultimately improving facility operations and resident outcomes.

17.3 Recommendation Summary

- 17.3.1 Modernize the DOCCS classification system by conducting a thorough revalidation that integrates both risk factors and behavior-based criteria. This process should use current research and best practices to ensure that housing decisions accurately reflect individuals' needs and risks.
- 17.3.2 Explore the possibility of developing additional specialized housing units tailored to specific subgroups, such as veterans, emerging adults (i.e., incarcerated individuals aged 18-29), and other individuals with similar backgrounds. Develop a formal structure for these units that includes evidence-based programming targeted to each group, and establish systems for tracking participation and measuring program outcomes.
- 17.3.3 Review the status of current “separatees”—individuals housed separately due to conflicts—to identify and resolve outdated or irrelevant conflicts. This review should reduce unnecessary transfers, improving facility operations and resident outcomes.

18. MENTAL HEALTH CARE

Mental health care is constitutionally mandated by the Eighth and Fourteenth Amendments to the U.S. Constitution and plays a critical role in both the well-being of incarcerated individuals and the overall safety and functioning of correctional institutions.¹¹⁹² Correctional facilities frequently serve as mental health treatment centers for those with mental disorders. According to U.S. Department of Justice data from 2016, approximately 43% of incarcerated individuals in state facilities have a history of mental health problems.¹¹⁹³ For these reasons, we examined mental health care to assess the adequacy and effectiveness of available services, especially given the high rates of serious mental illness among the populations at Marcy and Mid-State.

As of September 2025, approximately 12% of Marcy's population was identified as having a Seriously Mentally Ill (SMI) designation, about double the percentage of SMI-designated individuals systemwide.¹¹⁹⁴ This designation applies to individuals with a current or recent diagnosis of a DSM-IV Axis I disorder, such as schizophrenia, major depressive disorder, or bipolar disorder, as well as those diagnosed with a severe mental illness or significant personality disorder that results in self-harm or adversely affects mental or physical health.¹¹⁹⁵ It also includes individuals who are actively suicidal or have recently made a serious suicide attempt.¹¹⁹⁶ Marcy operates a 100-bed maximum-security RMHU and a four-cell RCTP for

¹¹⁹² See *Estelle v. Gamble*, 429 U.S. 97 (1976); *Bowring v. Godwin*, 551 F.2d 44 (4th Cir. 1977).

¹¹⁹³ Laura M. Maruschak et al., *Indicators of Mental Health Problems Reported by Prisoners*, U.S. Dep't of Justice, Office of Justice Programs (June 2021), <https://bjs.ojp.gov/media/44841/download>.

¹¹⁹⁴ CANY Data Dashboard, OMH Caseload and Program Census (last updated September 2025), <https://www.correctionalassociation.org/data/dashboard-omh-census-data> (filter by Facility Profile: Marcy).

¹¹⁹⁵ Mental Health Program Descriptions, p. 3, DOCCS Bureau of Mental Health (July 5, 2011), <https://www.op.nysed.gov/sites/op/files/surveys/mhpsw/doccs-att6.pdf>.

¹¹⁹⁶ New York State Department of Corrections and Community Supervision Bureau of Mental Health, Mental Health Program Descriptions, p. 3, DOCCS Bureau of Mental Health (July 5, 2011), <https://www.op.nysed.gov/sites/op/files/surveys/mhpsw/doccs-att6.pdf>.

incarcerated individuals experiencing mental health crises.¹¹⁹⁷ According to a 2022 CANY visit, incarcerated individuals at Marcy reported being unsatisfied with the quality and accessibility of healthcare services at Marcy, including long wait times for services, infrequent therapy sessions, minimal programming, and difficulty receiving psychiatric medication.¹¹⁹⁸

Mid-State is a Mental Health Service Level 1 facility, the highest designation, and has full-time OMH staff on-site.¹¹⁹⁹ As of September 2025, approximately 15% of Mid-State's population had an SMI designation, compared to 6% of the systemwide population.¹²⁰⁰ Like Marcy, Mid-State operates an RCTP for individuals in psychiatric crisis, but with twelve beds.¹²⁰¹ Mid-State also operates a 58-bed ICP for individuals with a serious mental illness who cannot be housed in general population units.¹²⁰²

18.1 Incarcerated Individuals' Perspectives on Mental Health Services

Our survey results indicate that most of the combined 801 incarcerated individual respondents at Marcy and Mid-State rate their access to mental health services as “Neutral” or better. (see Figure 40). However, 31.3% of survey respondents at those facilities rated the quality

¹¹⁹⁷ CANY, October 11-12 Monitoring Visit to Marcy Correctional Facility: Post-Visit Briefing and Recommendations, https://static1.squarespace.com/static/62f1552c1dd65741c53bbcf8/t/64ab93bfab9c7e045ea53c01/1688966083012/2023_PVB-10-Marcy.pdf.

¹¹⁹⁸ CANY, October 11-12 Monitoring Visit to Marcy Correctional Facility: Post-Visit Briefing and Recommendations, https://static1.squarespace.com/static/62f1552c1dd65741c53bbcf8/t/64ab93bfab9c7e045ea53c01/1688966083012/2023_PVB-10-Marcy.pdf.

¹¹⁹⁹ Directive #0080 (Jan. 19, 2024).

¹²⁰⁰ CANY Data Dashboard, OMH Caseload and Program Census (last updated September 2025), <https://www.correctionalassociation.org/data/dashboard-omh-census-data> (filter by Facility Profile: Mid-State).

¹²⁰¹ *Correctional Association of New York (CANY) Statement in Response to Death of Incarcerated Individual at Mid-State Correctional Facility*, CANY (Mar. 4, 2025), <https://www.correctionalassociation.org/press-releases-archive/correctional-association-of-new-york-cany-statement-in-response-to-death-of-incarcerated-individual-at-mid-state-correctional-facility>.

¹²⁰² *Correctional Association of New York (CANY) Statement in Response to Death of Incarcerated Individual at Mid-State Correctional Facility*, CANY (Mar. 4, 2025), <https://www.correctionalassociation.org/press-releases-archive/correctional-association-of-new-york-cany-statement-in-response-to-death-of-incarcerated-individual-at-mid-state-correctional-facility>.

as “Very Bad.” (see Figure 41). Overall, incarcerated individual interview responses indicated that their experiences with mental health services at these facilities varied widely (from very positive to very negative).¹²⁰³

Figure 40 Incarcerated Individual Survey Result

“Overall, how would you rate your access to mental health services at this facility?”						
Facility	Very Bad	Bad	Neutral	Good	Very Good	Total
Marcy	16.7%	12.1%	46.9%	14.4%	9.8%	100.0%
Mid-State	28.4%	11.5%	41.0%	8.5%	10.7%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

Figure 41 Incarcerated Individual Survey Result

“Overall, how would you rate the quality of services here?”						
Facility	Very Bad	Bad	Neutral	Good	Very Good	Total
Marcy	44.6%	19.4%	25.5%	7.1%	3.4%	100.0%
Mid-State	51.4%	16.8%	23.8%	4.4%	3.6%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

18.2 OMH and DOCCS Are Operationally Distinct and Siloed

Currently, New York’s OMH provides all mental health services through a 2016 Memorandum of Understanding (MOU) between OMH and DOCCS.¹²⁰⁴ A revision to the 2016 MOU between OMH and DOCCS is currently underway.¹²⁰⁵ Under the current MOU, DOCCS, in cooperation with OMH, must establish programs appropriate for the treatment of incarcerated

¹²⁰³ [Cite Redacted].

¹²⁰⁴ [Cite Redacted].

¹²⁰⁵ [Cite Redacted].

individuals with a mental illness who are in need of psychiatric services.¹²⁰⁶ OMH is responsible for “[a]ll mental health clinical, administrative and support services” for these programs, while “[a]ll other personnel shall be employees of DOCCS.”¹²⁰⁷ OMH provides psychiatric care to incarcerated individuals via “satellite” mental health clinics inside state prisons operated by the forensic division of the Central New York Psychiatric Center (CNYPC), which is owned and operated by OMH.¹²⁰⁸ OMH primarily concentrates on delivering clinical services within the facility, while DOCCS oversees security and facility operations.¹²⁰⁹ OMH unit chiefs report to the forensic division leadership of CNYPC, not to the superintendent of the DOCCS facility in which the clinic operates.¹²¹⁰ At DOCCS facilities, the assistant deputy or deputy superintendents of mental health are responsible for overseeing mental health concerns; they report to the facility superintendents.¹²¹¹ As a result, DOCCS personnel and OMH personnel operate with two distinct chains of command.

At Marcy, we observed that the DOCCS deputy superintendent of mental health appeared to be directing OMH’s operations within certain parameters (e.g., dictating what times OMH can see patients and in which locations).¹²¹² However, the division of labor between OMH and DOCCS limits DOCCS’ ability to exercise full oversight of OMH’s provision of services because the OMH-DOCCS relationship prohibits clinical information sharing.¹²¹³ For instance,

¹²⁰⁶ [Cite Redacted].

¹²⁰⁷ [Cite Redacted].

¹²⁰⁸ [Cite Redacted]. *see also* OMH, Central New York Psychiatric Center Corrections Based Components (last visited Feb. 24, 2026), <https://omh.ny.gov/omhweb/facilities/cnpc/programs.html>.

¹²⁰⁹ [Cite Redacted].

¹²¹⁰ [Cite Redacted].

¹²¹¹ *See, e.g.*, Facilities Listing, DOCCS Supervision Facilities Management System (last printed Feb. 24, 2026) (listing Deputy Superintendent of Mental Health at Marcy and Assistant Deputy Superintendent of Mental Health at Mid-State), <https://doccs.ny.gov/system/files/documents/2025/03/attachment-1-facilities-map.pdf>.

¹²¹² [Cite Redacted].

¹²¹³ [Cite Redacted].

DOCCS personnel cannot review the hard copy records of OMH treatment plans or mental health records, which OMH keeps separate from DOCCS.¹²¹⁴ DOCCS medical staff's lack of access to OMH records makes holistic treatment of incarcerated individuals virtually impossible. As both DOCCS and OMH move to electronic health record (EHR) systems (as discussed in Section 19, "Medical Care"), we recommend that DOCCS work to integrate DOCCS and OMH records by developing an interface that allows OMH providers to share relevant records for joint DOCCS/OMH patients and vice versa. This integration should facilitate coordinated, comprehensive patient care and improve continuity of care across agencies.

In addition, the siloed nature of the OMH and DOCCS functions can negatively affect the treatment of incarcerated individuals. At Marcy, OMH leadership appears to accept the limitations on patient access and service caused by DOCCS staffing shortages and defers to custody decisions that negatively impact the fidelity of their treatment model and services. For instance, we did not observe OMH staff advocating for patient care in response to DOCCS' limitations on individual call-outs (i.e., permission for the incarcerated individual to leave their cell) for private therapy sessions in the Marcy RMHU and RCTP.¹²¹⁵ On the contrary, OMH leadership made statements to the effect of: "We are not security and they know best."¹²¹⁶ This deference to security staff impacts the ability of incarcerated individuals to receive confidential counseling, especially since RMHU and RCTP cells are adjacent to each other.

We recommend that OMH staff and DOCCS staff receive training on patient advocacy in correctional settings to empower OMH staff and to set expectations within DOCCS. We also recommend that DOCCS foster proactive collaboration between OMH and DOCCS staff by

¹²¹⁴ [Cite Redacted].

¹²¹⁵ [Cite Redacted].

¹²¹⁶ [Cite Redacted].

including OMH facility leadership in periodic standing meetings with DOCCS high-level facility leadership and by establishing joint DOCCS/OMH rounds of mental health treatment areas.

In another example of how siloing between OMH and DOCCS can impact incarcerated individuals, Directive #4944 (relating to use of force) does not have a protocol for involvement of OMH/mental health counselors in planned uses of force (for example, cell extractions) against individuals with a mental illness, which leaves mental health professionals' participation at the discretion of each facility.¹²¹⁷ For instance, an incarcerated individual may refuse to exit their cell due to symptoms of a mental illness (e.g., paranoia and anxiety). Poor coordination between OMH and security staff regarding mental health concerns heightens the risk that correctional staff will resort to force rather than provide the appropriate mental health care. Improved collaboration between OMH and DOCCS security staff could support the establishment of protocols for dealing with perceived noncompliance by individuals with a mental illness when such situations are likely driven by the individual's mental health symptoms.

OMH also operates a grievance program that is separate from the DOCCS grievance program.¹²¹⁸ Incarcerated individuals often submit mental health grievances to DOCCS, which does not act on them because they are not within the remit of the IGP.¹²¹⁹ To address this, we recommend that DOCCS establish a dedicated mental health oversight committee to guide implementation of patient-centered practices, drive continuous quality improvement, and review barriers to providing mental health care in compliance with OMH protocols. The committee would be responsible for reviewing trends in grievances related to mental health care so that DOCCS and OMH can work together to resolve or remedy these trends. Further, as discussed in

¹²¹⁷ See Directive #4944 (Apr. 28, 2023); [Cite Redacted].

¹²¹⁸ [Cite Redacted].

¹²¹⁹ [Cite Redacted].

Section 9, “Incarcerated Grievance Program,” an electronic grievance system would help to expeditiously route medical grievances to OMH.

18.3 Access to Mental Health Care Services and Programming in Marcy’s RMHU and RCTP Is Limited Due to Staffing Shortages

DOCCS’ RMHU (at Marcy) and RCTP (at Marcy and Mid-State) are the primary programs and housing areas for incarcerated individuals with mental health needs at Marcy and Mid-State.¹²²⁰ The built environments of the RMHU at Marcy are conducive to therapeutic services and intervention and assessment services; they offer individual treatment and group space.¹²²¹ To some degree, the RCTP-built environments at both facilities are also conducive to therapeutic services and intervention and assessment services because they include individual treatment space, group space, and a line of sight into the cell fronts.¹²²² However, we observed that, in practice, incarcerated individuals with mental health needs are unable to access care as intended due to a lack of prioritization on mental health care in the midst of staffing shortages.

For instance, Marcy leadership placed memos in the “call-out” log book in late August 2025, noting that, due to the staffing shortage, there would only be four daily call-outs for incarcerated individual therapy; in addition, per the memo, those four call-outs would occur in the RMHU individual visitation cell rather than in the RMHU treatment spaces.¹²²³ This schedule allows for the 15 mental health staff at Marcy to provide only 20 mental health call-outs per week (i.e., four per day, five days a week).¹²²⁴ Although the OMH unit chiefs at both Marcy and Mid-State reported that patients were seen for individualized sessions at least every 30 days,

¹²²⁰ [Cite Redacted].

¹²²¹ [Cite Redacted].

¹²²² [Cite Redacted].

¹²²³ [Cite Redacted].

¹²²⁴ [Cite Redacted].

which is OMH’s internal practice, they counted cell-side sessions in that total.¹²²⁵ We also performed a limited review of records at Marcy, which indicated that incarcerated individuals with a serious mental illnesses were not being seen *more* frequently than approximately every 30 days, which is likely not frequent enough to comport with individual treatment plans for the population with SMI designations at Marcy.¹²²⁶ Our observations indicated that group treatment was also virtually non-existent in the Marcy RMHU between February and September 2025.¹²²⁷

In addition to scheduled individual sessions, the OMH staff make rounds on the Marcy RMHU and RCTP units daily.¹²²⁸ These rounds are designed to function as a conduit to access to care. However, given the diminished staffing levels, a very limited number of individual sessions have taken place since February 2025 with a reliance on cell-side discussions for those requiring individual counseling.¹²²⁹ The use of cell-side sessions should only occur in urgent situations and with a DOCCS/OMH review process; DOCCS should establish tracking of the number of cell-side therapeutic sessions to determine whether cell-side mental health sessions are being overused as a substitute for individual sessions.¹²³⁰

Programming in the RMHU also suffers from a lack of consistency. In August 2025, Marcy facility leadership informed us that programming in the RMHU had resumed to twice per week.¹²³¹ We observed during site visits that programming had not, in fact, resumed, despite the presence of multiple RMHU staff on those days.¹²³² When we raised this issue to facility

¹²²⁵ [Cite Redacted].

¹²²⁶ [Cite Redacted].

¹²²⁷ [Cite Redacted]; *see also* Decision and Order, *Smalls v. Martuscello*, Index No. 903926-25 (Sup. Ct., Albany County July 1, 2025).

¹²²⁸ [Cite Redacted].

¹²²⁹ [Cite Redacted].

¹²³⁰ [Cite Redacted].

¹²³¹ [Cite Redacted].

¹²³² [Cite Redacted].

leadership, one senior leader commented that they were not surprised.¹²³³ In September 2025, after DOCCS faced litigation regarding the lack of programming in the RMHU, facility and DOCCS leadership informed us that RMHU programming had resumed.¹²³⁴ Through the MOU revision process, DOCCS and OMH should significantly rework staffing, programming, and service components to achieve coordinated, collaborative care and quality improvements to mental health care for incarcerated individuals in the RMHU and RCTP.

18.4 Recommendation Summary

- 18.4.1 Foster proactive collaboration between OMH and DOCCS staff by including OMH facility leadership in additional periodic standing meetings with DOCCS high-level facility leadership and by establishing joint DOCCS/OMH rounds of mental health treatment areas.
- 18.4.2 Establish tracking of the number of cell-side therapeutic sessions to determine whether cell-side mental health sessions are being overused as a substitute for individual sessions.
- 18.4.3 Work to integrate DOCCS and OMH records by developing an interface that allows OMH providers to share relevant records for joint DOCCS/OMH patients and vice versa. This integration should enable coordinated, comprehensive patient care and improve continuity of care across agencies.
- 18.4.4 Establish a dedicated mental health oversight committee to guide implementation of patient-centered practices, drive continuous quality improvement, and review barriers to providing mental health care in compliance with OMH protocols.

¹²³³ [Cite Redacted].

¹²³⁴ [Cite Redacted].

19. MEDICAL CARE

Providing patient-centered medical care is foundational to correctional healthcare and contributes to a culture that values the humanity and dignity of incarcerated individuals. However, the ability to do so may be compromised when medical staff perceive a conflict between carrying out their professional duties and the interests of non-medical staff in the facility. Recall, medical staff observed correction officers killing Mr. Brooks, complying with security officers' directions that medical staff stay out of the room while officers beat him to death.¹²³⁵ Access to medical care is an essential standard, and facilities should make the necessary cultural changes and create processes and practices that eliminate or diminish barriers to accessing proper care.

Moreover, medical staff play an important role in providing medical care and documenting medical assessments after use of force events. However, our review of medical records at Marcy and Mid-State shows that medical staff do not consistently document sufficient details regarding the occurrence of a use of force event and/or ensuing medical assessments, which should be reflected in incarcerated individuals' medical records. Consequently, our review team's ability to evaluate the reliability of those medical assessments and the provision of care was limited.

19.1 Failure to Adequately Record Post-Use Of Force Medical Assessments in Medical Records

We reviewed multiple medical and mental health records from Marcy and Mid-State to evaluate the quality and consistency of care services for incarcerated individuals, with particular

¹²³⁵ Sean I. Mills, *Prosecutor: Nurses Will not be Charged in Death of Inmate Brooks*, UTICA DAILY SENTINEL (Feb. 27, 2025), https://www.romesentinel.com/news/marcy-robert-brooks-nurses-not-charged/article_01be8976-f531-11ef-ae01-eb53c059359b.html (“‘The three nurses on duty when Robert Brooks was murdered will not be criminally charged in connection with his death as they were specifically ordered to remain out of the ER during his beating,’ the prosecutor said in a statement.”).

attention to records related to use of force.¹²³⁶ This review showed that there are gaps in medical records related to use of force events that should be more fully documented in order to ensure proper patient care and adequate recordkeeping. To note, we did not observe any use of force events or related medical assessments during site visits.

After a use of force occurs, the incarcerated individual must receive medical attention, which results in a post-force Physical Examination/Treatment Report.¹²³⁷ The timing of those medical assessments at Marcy and Mid-State seems to vary widely. From 2020 to 2025, the time duration between a use of force and the resulting medical examination at Marcy and Mid-State, as noted in the corresponding Use of Force Reports, was between one and ten minutes for 24.7% of incidents, between eleven and 20 minutes for 20.6% of incidents, between 21 and 30 minutes for 9.7% of incidents, and between 31 and 60 minutes for 11.3% of incidents.¹²³⁸ There were 22.1% of Use of Force Reports that recorded that no time elapsed between a use of force incident and the resulting medical examination, which may reflect unreliable data entry.¹²³⁹ Relatedly, we found that the records do not make clear when a use of force occurred (i.e., whether the assessment occurred minutes or days later).¹²⁴⁰ DOCCS Directive #4944 states that staff must seek medical attention for the involved incarcerated individual(s) “as soon as possible.”¹²⁴¹ The ACA requires “immediate medical examination” for anyone injured in an incident and for all instances involving the use of a weapon or a chemical agent.¹²⁴² We therefore recommend

¹²³⁶ [Cite Redacted].

¹²³⁷ Directive #4944, §IV.A (Apr. 28, 2023).

¹²³⁸ [Cite Redacted].

¹²³⁹ [Cite Redacted].

¹²⁴⁰ [Cite Redacted].

¹²⁴¹ Directive #4944, §§III.G, IV.A.3 (Apr. 28, 2023).

¹²⁴² American Correctional Association, *Performance-Based Standards for Adult Correctional Institutions*, p. 84 (5th ed., March 2021 updates).

changing the language in Directive #4944 to require medical attention “immediately,” at the current facility, following use of force events.

According to Directive #4944, medical staff are supposed to take photographs of the incarcerated individual following a use of force incident and must document injuries.¹²⁴³ Common practice in medical settings is for any pictures, drawings, and diagrams of injuries to be included in a patient’s medical record.¹²⁴⁴ However, the medical records we reviewed did not consistently include the required photographs, drawings, and diagrams from post-use of force assessments.¹²⁴⁵ The post-use of force medical records reviewed often stated “no report of injuries,” but the records did not provide any substantiating documentation of this, such as a description of an assessment for broken skin, bruising, or limitations to range of motion.¹²⁴⁶ In some instances where incarcerated individuals reported pain in specific bodily areas after a use of force, the medical record stated that no redness or bruising was noted, but did not further document that assertion.¹²⁴⁷ The review also showed that some notes do not even reflect that a use of force occurred,¹²⁴⁸ or, if there was a use of force and an altercation, the medical records may only document the altercation.¹²⁴⁹ In addition, in most instances after a use of force, medical staff did not schedule follow-up care, and any further response by medical staff was left to the incarcerated individual’s use of the sick call process.¹²⁵⁰

¹²⁴³ Directive #4944 (Apr. 28, 2023).

¹²⁴⁴ [Cite Redacted]; *see also* State of Alabama Dep’t of Corrections, Administrative Regulation Number 630: Suicide Watch (Mar. 8, 2024), <https://doc.alabama.gov/docs/AdminRegs/AR630.pdf> (policy referencing body chart documentation in a different context than UOF but demonstrating use of such tools in a prison system).

¹²⁴⁵ [Cite Redacted].

¹²⁴⁶ [Cite Redacted].

¹²⁴⁷ [Cite Redacted].

¹²⁴⁸ [Cite Redacted].

¹²⁴⁹ [Cite Redacted].

¹²⁵⁰ [Cite Redacted].

These observations raise several issues. They raise questions regarding whether medical staff adequately document injuries following a use of force in the patient's records and whether they conduct any assessment of the patient at all. They also raise concerns about whether medical staff have a full picture of all relevant information relating to each incarcerated patient, including details from a post-use of force assessment, to make sure that proper care is delivered going forward. Overall, these observations implicate the reliability of post-use of force medical assessments and related records, as well as the proper provision of care and/or ability to adequately follow up on patients. Given these concerns, we recommend that DOCCS ensure that post-use of force medical assessments are actually conducted, are standardized, and are included in medical records. Omissions and a lack of detail result in incomplete health records. We recommend that DOCCS ensure that uses of force and related details, including when and where a use of force event occurred, are reflected in patient medical records.

19.2 Staffing Issues

Medical staff at Marcy reported that nurse staffing shortages have been an ongoing issue; as a result, employees have been working extended hours each week, including double shifts.¹²⁵¹ This can exacerbate staffing issues, as nurses experience burnout, choose to take early retirement, or leave for a healthier work environment. Several nurses at Marcy expressed frustration and exhaustion and did not see an end to the staffing crisis; indeed, several stated that they are just waiting for retirement.¹²⁵² Many of the nurses at Marcy have been employed in this setting for years and have been committed to working extra days and extra shifts multiple times per week because of their sense of loyalty to their fellow team members.¹²⁵³

¹²⁵¹ [Cite Redacted].

¹²⁵² [Cite Redacted].

¹²⁵³ [Cite Redacted].

Staffing issues and inefficiencies can also impact patient care. For example, DOCCS executives, incarcerated individuals, and medical and nursing staff reported that follow-up care related to external specialty or diagnostic care had been rescheduled or missed due to staffing issues at the facility that precluded transportation off-site.¹²⁵⁴ In one example, one of these missed appointments resulted in delayed care and a subsequent emergency event that required outside treatment.¹²⁵⁵ The volume of patients can also result in errors in clinical judgments and care decisions. And when staff become overwhelmed with emergencies, many of the routine requirements for a properly run medical unit can be delayed or omitted, as all care becomes crisis care. This can result in missed clinical encounters, backlogs in routine sick calls, delays in treatment, and an increase in grievances.

Given these concerns, DOCCS should adopt a specific recruitment and retention plan for medical staff as part of larger recruitment and retention initiatives.

19.3 Incarcerated Individuals’ Perceptions of Care

The majority of surveyed incarcerated individuals at Marcy and Mid-State rated access to medical care and the quality of care received as “Bad” or “Very Bad.” (see Figures 42 and 43).

Figure 42 Incarcerated Individual Survey Result

“Overall, how would you rate your access to medical care at this facility?”						
Facility	Very Bad	Bad	Neutral	Good	Very Good	Total
Marcy	39.9%	20.9%	25.8%	10.4%	3.1%	100.1%
Mid-State	52.5%	16.8%	22.5%	4.1%	4.1%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

¹²⁵⁴ [Cite Redacted].

¹²⁵⁵ [Cite Redacted].

Figure 43 Incarcerated Individual Survey Result

“Overall, how would you rate the quality of medical treatment at this facility?”						
Facility	Very Bad	Bad	Neutral	Good	Very Good	Total
Marcy	44.6%	19.4%	25.5%	7.1%	3.4%	100.0%
Mid-State	51.4%	16.8%	23.8%	4.4%	3.6%	100.0%

Note: Percentages reflect the proportion of survey respondents, not the total facility population.

In interviews with incarcerated individuals at Marcy and Mid-State, responses about medical services varied. Most individuals expressed an overall negative sentiment and discussed complaints regarding denials or delays in accessing care, consistent with survey data.¹²⁵⁶ These perceptions may reflect broader problems with the provision of care and the culture surrounding it. Some individuals, however, expressed positive experiences with receiving care and named individual nurses who had been particularly helpful.¹²⁵⁷

19.4 Staff Struggle with a Culture of Dual Loyalty

19.4.1 The Dual Loyalty Dynamic

We found that civilian staff, particularly medical staff, struggle with conflict between their professional duties to patients and their shared experiences with, and loyalty to, fellow staff members, including security staff.¹²⁵⁸

Medical staff rely on officers to protect them in high-stress, high-risk situations. This can result in outsized loyalty to security staff and potentially compromise patient care.¹²⁵⁹ For example, medical personnel at Marcy described how correction officers “have [their] back.”¹²⁶⁰

¹²⁵⁶ [Cite Redacted].

¹²⁵⁷ [Cite Redacted].

¹²⁵⁸ [Cite Redacted].

¹²⁵⁹ [Cite Redacted].

¹²⁶⁰ [Cite Redacted].

This includes, for example, during medical examinations after a use of force—when security staff must determine whether it is safe for medical personnel to conduct a private post-use of force assessment or remain in the room to protect the medical team.¹²⁶¹ While familiar relationships are important to a healthy and safe work environment, they can also create a sense of loyalty to the security officers rather than to their patients.¹²⁶² This can make it difficult for medical staff to advocate for the medical needs of incarcerated individuals, especially when correction officers take actions or tacitly encourage behavior that is in tension with medical best practices.

The presence of non-medical staff during medical examinations can also compromise patient confidentiality, which is a key standard in correctional healthcare, and dual loyalty can compound this issue. Dual loyalty makes it challenging for staff to advocate for patients' needs and privacy, especially if breaches of confidentiality become routine. At Marcy, we observed that correction officers had free access to medical spaces and were present throughout the medical unit.¹²⁶³ During our visits, we observed officers walking in and out of medical work environments, where they could easily gain access to confidential health information.¹²⁶⁴ Officers also stood in the room or the open doorway during interactions between incarcerated individuals and medical providers,¹²⁶⁵ and we observed officers and nursing staff freely holding conversations about behaviors of incarcerated individuals who were patients, including via

¹²⁶¹ [Cite Redacted].

¹²⁶² Sarah Glowa-Kollisch, et al., *Data-Driven Human Rights: Using Dual Loyalty Trainings to Promote the Care of Vulnerable Patients in Jail*, 17 HEALTH & HUMAN RIGHTS J., E124–E135 (June 2015); see also Joseph Neff & Alysia Santo, *When NY Prison Nurses Must Choose Between Loyalty to Guards and Devotion to Patients*, GOTHAMIST (May 1, 2025), <https://gothamist.com/news/when-ny-prison-nurses-must-choose-between-loyalty-to-guards-and-devotion-to-patients>.

¹²⁶³ [Cite Redacted].

¹²⁶⁴ [Cite Redacted].

¹²⁶⁵ [Cite Redacted].

unsolicited remarks to our review team.¹²⁶⁶ Furthermore, the medical unit at Marcy for the general population maintains paper charts of patient medical records in an office that is unlocked and accessible by correction officers.¹²⁶⁷ There were times when no medical personnel was present in this office, and correction officers were seated at the office desk.¹²⁶⁸ There appeared to be no expectation on the part of nursing staff that they should create confidential settings for patients.¹²⁶⁹

DOCCS should ensure that medical and DOCCS facility supervisory staff receive joint training to mitigate perceptions of conflict, to promote full autonomy for patients and medical professionals, and to uphold loyalty to and advocacy for patient care. DOCCS should also ensure that medical and security staff receive training to properly balance the provision of adequate medical care and the preservation of patient privacy with security needs. In addition, we recommend that DOCCS take steps to strengthen patient confidentiality, including by promoting privacy while ensuring a secure environment—drawing on existing practices in other circumstances that involve preserving privacy (e.g., mental health care, religious observation, classroom settings). In circumstances that require security to remain in the examination room, medical staff should explain to incarcerated individuals the security reason for the privacy abridgement. DOCCS should also ensure medical records are kept in secure environments that cannot be accessed by non-clinical staff without supervision by medical staff. DOCCS should also design and implement clear, accessible mechanisms for reporting conflicts or dilemmas that arise between medical care requirements and custody or security concerns. These mechanisms

¹²⁶⁶ [Cite Redacted].

¹²⁶⁷ [Cite Redacted].

¹²⁶⁸ [Cite Redacted].

¹²⁶⁹ [Cite Redacted].

should ensure that all parties are able to voice concerns, seek timely resolution, and maintain a balance between health needs and safety mandates.

19.5 DOCCS Should Transition Away from Paper-Based Health Records

DOCCS' current health records are paper-based, a practice that introduces opportunities for gaps in care. DOCCS currently maintains patient medical health records in paper charts that physically follow an incarcerated individual upon transfer to another facility.¹²⁷⁰ At Marcy, medical records are not located in a single location; they are stored in either the general population medical unit or the relevant restrictive housing unit, depending on where an individual is located.¹²⁷¹

This paper system risks failure points for gaps in healthcare such as medical follow-ups, external specialty appointments, timely diagnostic exams, and review of exam results. DOCCS should transition to an electronic health record system, which will allow providers to access or review medical records when called on an urgent matter, as well as chart audits and clinical review of records. We understand that DOCCS is taking steps toward transitioning to electronic health record systems, though it is only in the initial stages of this process.¹²⁷² DOCCS should continue to work to transition all records from the current paper-based system to a fully electronic format.

19.6 DOCCS Should Take Steps to Prevent Siloing of Medical and Mental Health Care

Mental health records are also maintained separately from medical records, which siloes care.¹²⁷³ For example, DOCCS' nursing staff administers psychiatric medication to individuals in

¹²⁷⁰ [Cite Redacted].

¹²⁷¹ [Cite Redacted].

¹²⁷² [Cite Redacted].

¹²⁷³ See Section 18, "Mental Health Care."

general population with mental health diagnoses, but in Marcy’s RMHU, OMH nurses administer psychiatric medication, while all other medications are administered by DOCCS nurses.¹²⁷⁴ Siloed provision of care risks missed care opportunities and decreases in overall quality of care. DOCCS should also develop a collaborative and patient-centered model of care that focuses on improved communication and a team approach to treatment of the whole individual. As part of this model, DOCCS should integrate DOCCS and OMH records by developing an interface that allows OMH providers to share relevant records for joint DOCCS/OMH patients and vice versa. This integration can promote coordinated, comprehensive patient care and improve continuity of care across agencies.

19.7 DOCCS Should Make Additional Improvements to Cultivate a Culture of Care

We observed instances, including with respect to treatment for substance abuse disorders (SUD) and for incidents of suspected substance use, where medical staff expressed skepticism for patients’ treatment plans or a lack of urgency when responding to possibly emergent situations.¹²⁷⁵ This can pose a problem, as medical staff should be responsive to patients. These conflicts between staff attitudes and standards of practice reflect a degradation to the culture of care that, in turn, can affect the care patients ultimately receive.

For example, we observed a lack of staff training and commitment to medication-assisted treatment (MAT) for incarcerated individuals who require treatment for SUD, which DOCCS and facility leaders acknowledged.¹²⁷⁶ MAT provides longer-term stabilization through evidence-based medications for opioid use disorder, which include methadone, buprenorphine

¹²⁷⁴ [Cite Redacted].

¹²⁷⁵ [Cite Redacted].

¹²⁷⁶ [Cite Redacted].

and naltrexone.¹²⁷⁷ DOCCS facilities offer MAT,¹²⁷⁸ and our review of DOCCS’ MAT policies found them to be consistent with best practices in a prison setting.¹²⁷⁹ However, our review identified that some staff may not be aligned with the mission of the MAT program. For example, during site visits, several executive and medical staff members commented as though MAT commonly results in the diversion of medication to others,¹²⁸⁰ when no data supports that assumption. Multiple nurses at Marcy expressed sentiments that incarcerated individuals “don’t need this medicine if they are in prison.”¹²⁸¹ And Marcy staff in other conversations repeatedly referred to MAT as “free drugs.”¹²⁸²

These recurring themes across facilities suggest that staff do not understand or accept the mission and protocols of the MAT program. To address this, DOCCS should reinforce the MAT mission and protocols and improve training for staff about SUD and stigma. These changes could contribute to a shift in the culture of care that is sensitive to patients’ medical needs and aligned with their individual experiences.

DOCCS should lead and support cultural transformations throughout the organization that emphasize ensuring equitable access to medical care and removing or mitigating organizational, procedural, and attitudinal barriers to timely and appropriate medical care for all individuals.

¹²⁷⁷ [Cite Redacted].

¹²⁷⁸ [Cite Redacted].

¹²⁷⁹ See [Cite Redacted]; Kelly E. Moore, et al., *Effectiveness of Medication Assisted Treatment for Opioid Use in Prison and Jail Settings: A Meta-analysis and Systematic Review*, J. SUBSTANCE ABUSE TREATMENT (Apr. 2019), [https://www.jsatjournal.com/article/S0740-5472\(18\)30449-5/fulltext](https://www.jsatjournal.com/article/S0740-5472(18)30449-5/fulltext).

¹²⁸⁰ [Cite Redacted].

¹²⁸¹ [Cite Redacted].

¹²⁸² [Cite Redacted]

19.8 Recommendation Summary

- 19.8.1 Revise Directive #4944 to require immediate medical attention, at the current facility, following use of force events.
- 19.8.2 Ensure that post-use of force medical assessments are consistently and adequately included in medical records, including photographs, drawings, and diagrams from assessments and substantiating documentation of reports of injuries or of no injuries. Ensure that use of force events and related details, including when and where a use of force event occurred, are consistently and adequately reflected in patient medical records.
- 19.8.3 Strengthen patient confidentiality, including by promoting visual and auditory privacy while ensuring a secure environment. Rather than having security staff listening to and viewing medical appointments from inside an examination room, draw on existing practices in other circumstances that involve preserving privacy (e.g., mental health care, religious observation, classroom settings); in circumstances that require security remaining in the examination room, medical staff should explain to incarcerated individuals the security reason for the privacy abridgement. Ensure medical records are kept in secure environments that cannot be accessed by non-clinical staff without supervision by medical staff.
- 19.8.4 Transition all records from the current paper-based system to a fully electronic format, including grievances, Use of Force Reports, and medical and mental health documentation.
- 19.8.5 As part of the larger recruitment and retention initiative, implement a specific recruitment and retention plan for medical staff.
- 19.8.6 Lead and support cultural transformations throughout the organization that emphasize ensuring equitable access to medical care and removing or mitigating organizational, procedural, and attitudinal barriers to timely and appropriate medical care for all individuals. This includes mitigating perceptions of dual loyalty by medical staff, who may perceive tensions between patient care obligations and the perceived and/or articulated interests of security staff.
- 19.8.7 Ensure that medical and DOCCS facility supervisory staff receive joint training to promote full autonomy for patients and medical professionals, to uphold loyalty to and advocacy for patient care (including commitment to medication-assisted treatment for substance abuse disorders), and to mitigate perceptions of conflicts, and that medical and security staff receive training to properly balance the

provision of adequate medical care and the preservation of patient privacy with security needs.

- 19.8.8 Design and implement clear, accessible mechanisms for reporting conflicts or dilemmas that arise between medical care requirements and custody or security concerns. These mechanisms should ensure that all parties are able to voice concerns, seek timely resolution, and maintain a balance between health needs and safety mandates.

20. **RECOMMENDATIONS**

Below are 57 comprehensive recommendations for reform. The recommendations are categorized as Priority, Short-Term (1-2 years), and Long-Term (3-5 years) and are designed to foster rehabilitation and improve systemwide safety and efficiency by enhancing staff training, accountability, technology modernization, grievance processes, facility operations, and mental health and medical oversight within correctional facilities.

Priority Recommendations

- P.1 **Enhance In-Service Training:** Require every facility to conduct mandatory, in-person, scenario-based in-service training for all correction officers, every two years. Training should provide officers with the most up-to-date techniques with a focus on appropriate use of force, the use of force continuum, de-escalation, and the duty to intervene. The training should leverage technology, such as virtual reality, to incorporate scenario-based training. It should incorporate formal testing at the end of each session to ensure comprehension of both lecture and scenario training and DOCCS should maintain records of participation and assessment outcomes. DOCCS should also hire dedicated curriculum consultants to assist in creating and enhancing the in-service training program.
- P.2 **Standardize BWC Review:** Institute a uniform, systemwide protocol for facility leadership to review the use of Body-Worn Cameras (BWCs) to ensure facility-specific compliance with DOCCS' BWC policy. This protocol should include the steps taken and the disciplinary consequences for failure to comply with the BWC policy, including potential mandatory written records of non-compliance, counseling, coaching, or training. The protocol should clearly outline responsibilities for managers, timelines for investigation, and requirements for documentation to promote transparency and accountability at every stage.
- P.3 **Annual BWC Audit:** Establish a structured routine program for auditing camera footage at least annually as a mechanism to assess compliance with DOCCS directives, including the use of force directive. The program should specify the scope of audits, define the criteria for selecting footage, and require documentation of findings and any corrective actions taken, enabling DOCCS to proactively identify and address patterns of behavior before they escalate.
- P.4 **Monitor Facility Compliance with Ten-Day Use of Force Report Reviews:** Track compliance with the ten-day requirement for superintendent review of Use of Force Reports. Identify facilities that are out of compliance, document, and provide corrective action timeframes. If the ten-day review timeframe proves unworkable, consider amending the policy to provide a clear and realistic deadline

that ensures timely oversight while maintaining rigorous standards for review and accountability.

- P.5 **Restrict Assignments for Disciplined Officers:** Continue to develop and implement policies to ensure that security staff returning after disciplinary actions or on disciplinary evaluation periods are not placed on CERT deployments for a period of time proportionate to the severity of the misconduct. DOCCS should also ensure that security staff returning after disciplinary actions or on disciplinary evaluation periods are not placed into other high-risk assignments (e.g., special housing, emergency response, strip searches, planned uses of force, transport, or housing units). We acknowledge that this recommendation would require a legislative change or changes to the collective bargaining agreements with security staff.
- P.6 **Track Informal Interventions for Facility Staff:** Implement a centralized process to document and log instances of informal interventions for facility staff. Use this data to identify staff with multiple instances of informal interventions, analyze underlying trends, and inform targeted interventions or additional training, thereby supporting a culture of continuous improvement.
- P.7 **Expand Commissioner's Disciplinary Authority:** Grant the Commissioner explicit authority to discipline or terminate any employee for serious misconduct. This authority should encompass a transparent process with documented rationale, ensuring fair and consistent outcomes across the organization. We acknowledge that this recommendation would require a legislative change or changes to the Collective Bargaining Agreements with security staff.
- P.8 **Facility Access and Contraband Control:** Screen all individuals (including staff, visitors, and contractors) before entry to any facility to help ensure that contraband and other unauthorized items are not being illegally introduced. In addition, to the extent not underway, DOCCS should use scanning technology to screen all mail for contraband, including legal mail, with due regard to protecting privileged legal communications.

Short-Term Recommendations (1-2 years)

Improve Staffing Efficiencies and Morale

- S.1 Continue investment in the “Recover, Recruit, Rebuild” initiative. Invest in messaging and strategy that reframe DOCCS as a career of public service and rehabilitation; reinforce this messaging through training, marketing, and recruitment.
- S.2 Revise facility staffing (plot) plans to maximize facility efficiencies, while meeting legal obligations and programming requirements. Evaluate plot plans to account for twelve-hour shifts, closures, and other staffing limitations. Continue collaborating with consultants on identifying and executing efficiencies to

optimize safety, efficiency, and compliance while supporting staff well-being in the long term.

- S.3 Address staff morale and retention by developing a quarterly staff recognition program for staff at each facility, including individual public commendations, social media blasts, or personalized notes from leadership.
- S.4 Continue hiring additional OSI staff to lighten investigative caseloads. At the same time, leverage technology and data to identify risk and investigative patterns, facility hot spots, and trends, as well as potential red flags at the facility or staff level.
- S.5 Track staff deployment and prioritize staffing for programming and recreation areas, including tracking closed job assignments (posts) by location and rotating closed posts appropriately to improve opportunities for programming and recreation.

Modernize Technology

- S.6 Expand tablet-based offerings in special housing units, with an emphasis on adding programming opportunities.
- S.7 Review existing technology partnerships to assess opportunities to expand by leveraging each vendor's full suite of applicable technology capabilities.
- S.8 Create a two-year pilot program at selected facilities that relies on approved state vendors for delivery of goods to incarcerated individuals, with appropriate exceptions to ensure access to items to which individuals are legally entitled, including religious items. After the pilot program, evaluate whether the secure vendor program meaningfully reduced contraband at those facilities.
- S.9 Create a task force to develop a work plan to modernize technology related to maintaining electronic records and leveraging data analytics in the longer term.

Detect and Mitigate Risk

- S.10 Implement a multidisciplinary review at the DOCCS agency level for every severe use of force incident. Minor incidents involving the use of force should be reviewed by an appointed individual, such as a regional administrator or designated agency-level staff member. The review should include an evaluation of the facility superintendent's own review, as well as the involved officers' adherence to policies, use of BWC, de-escalation and intervention practices, response protocols, and all other pertinent information. In appropriate cases, OSI and law enforcement agencies may request holds on institutional review for cases referred for criminal misconduct.

- S.11 Ensure that ILC processes at facilities are consistent with the guidance in the existing ILC directive.
- S.12 Revise the Use of Force Report form to require detailed descriptions of compliance with the BWC directive, de-escalation efforts, and other techniques used prior to use of force (such as active listening, encouraging dialogue rather than confrontation, slowing down the encounter, avoiding immediate force, building rapport, creating space, avoiding threatening body language, and using Crisis Intervention Training, religious, mental health, or other wellness resources). Revisions should also include required fields with checkboxes that consolidate and streamline options for key details like the location(s) and demographic data. The form should add optional comment fields and preserve the text boxes to allow officers to provide details as accurately and comprehensively as possible.
- S.13 Require that canisters of chemical agents issued as duty equipment and carried during the shifts are weighed and serialized at least once a month to confirm how much is being used, as well as after each use, to confirm sufficient levels for subsequent use; ensure chemical agent usage complies with policy; and enhance accountability. At the same time, reassess the need for all staff to carry chemical agents by focusing on supervisors, responders, and designated areas.

Revamp Grievance Process

- S.14 Evaluate and develop a plan to improve the grievance process and resolution of grievances, including recommendations for technology improvements, additional staffing, and analysis of trends.
- S.15 Enhance processes and controls around the grievance system, including by ensuring the availability and accessibility of separate, secured grievance boxes with access limited to the Incarcerated Grievance Program Supervisor and designee in living areas and common areas.
- S.16 Revise the Orientation Handbook and Directive #4040 to be consistent with each other regarding the time limit to file a grievance and how to file a grievance, along with ongoing annual revisions to incorporate supplemental memoranda.

Enhance Correction Officer Training

- S.17 Conduct comprehensive review of correction officer training curriculum at the Academy and in-field training to increase use of scenario-based training wherever possible; to emphasize the most up-to-date techniques on appropriate use of force, the use of force continuum, de-escalation, and the duty to intervene; and to incorporate formal testing of covered concepts. At the same time, implement lesson plans to complement the paramilitary approach with state-of-the-art adult learning methods, such as table-top exercises, discussion and dialogue, case

studies and scenarios, and collaborative learning involving group exercises to leverage collective experience and work through complex, real-life challenges.

- S.18 Include in standardized curriculum for all trainees as part of on-the-job training protocols that ensure trainees receive training on all posts conducted by experienced officers, with testing at the end of the field training.
- S.19 Ensure regular (at least every three years) training of supervisors on key issues, including review of Use of Force Reports, evaluations, and other supervisory responsibilities. Incorporate formal testing of concepts covered.

Prioritize Accountability

- S.20 Prioritize hiring for new positions in the Bureau of Labor Relations to more effectively manage review workload.
- S.21 Strengthen mechanisms to prevent retaliation for filing of complaints, including by creating a formal follow-up protocol that includes checking in on subjects after their interview, providing dedicated retaliation complaint forms, and creating a separate intake process for any retaliation complaints that OSI receives. The protocol should also include a formal mechanism for tracking outcomes of retaliation investigations.

Clarify Guidance Relating to Facility Operations

- S.22 Revise Directive #4944 to require immediate medical attention, at the current facility, following use of force events.

Improve Mental Health and Medical Outcomes and Oversight

- S.23 Foster proactive collaboration between OMH and DOCCS staff by including OMH facility leadership in additional periodic standing meetings with DOCCS high-level facility leadership and by establishing joint DOCCS/OMH rounds of mental health treatment areas.
- S.24 Establish tracking of the number of cell-side therapeutic sessions to determine whether cell-side mental health sessions are being overused as a substitute for individual sessions.
- S.25 Ensure that post-use of force medical assessments are consistently and adequately included in medical records, including photographs, drawings, and diagrams from assessments and substantiating documentation of reports of injuries or of no injuries. Ensure that use of force events and related details, including when and where a use of force event occurred, are consistently and adequately reflected in patient medical records.

- S.26 Strengthen patient confidentiality, including by promoting visual and auditory privacy while ensuring a secure environment. Rather than having security staff listening to and viewing medical appointments from inside an examination room, draw on existing practices in other circumstances that involve preserving privacy (e.g., mental health care, religious observation, classroom settings); in circumstances that require security to remain in the examination room, medical staff should explain to incarcerated individuals the security reason for the privacy abridgement. Ensure medical records are kept in secure environments that cannot be accessed by non-clinical staff without supervision by medical staff.

Long-Term Recommendations (3-5 years)

Improvements to Staff Efficiencies and Ethics

- L.1 Build upon New York State’s existing Excelsior Service Fellowship Program by creating a three-year paid developmental training program to attract more talent to DOCCS. We acknowledge that this may require civil service approvals or changes.
- L.2 Establish a five-year formal health and wellness strategy that addresses a comprehensive approach to employee health and well-being and emphasizes confidentiality.
- L.2.1 Track alignment with wellness strategy by using key performance indicators, such as reductions in staff sick time and workers’ compensation claims, referrals to treatment programs, and wellness app usage and downloads, and by analyzing anonymized healthcare claims submitted by employees
- L.2.2 Establish a department-wide oversight committee or task force to review wellness data annually and report that data annually to steer the focus and direction of regional- and facility-level staff wellness committees and initiatives.

Detect and Mitigate Risk

- L.3 Create the position of CRO to oversee a centralized risk function. The CRO would be a senior official responsible for proactively identifying, assessing, and mitigating risks across DOCCS. These include risks to both incarcerated individuals and staff. The CRO should evaluate risk using quantitative and qualitative methods (e.g., demographic dashboard, disciplinary and use of force trends, PREA, federal funding, etc.), develop policies and frameworks to reduce or manage risks, ensure adherence to regulatory requirements and internal standards, communicate risk exposure and mitigation plans to the Commissioner and senior leadership, and prepare an annual report. The CRO should proactively detect and analyze data across functions to reduce silos among DOCCS departments and manage appropriate resource allocation and implementation for

corrective actions. Where these responsibilities presently reside with the OSI and Research departments, they should be reallocated to the CRO.

- L.4 Under the proposed CRO, expand the scope of data collection and analysis regarding trends (e.g., use of force, grievances, OSI, BLR/discipline data) to develop systemwide early warning systems with the capacity to communicate with and correct in real time at the facility level. Implement comprehensive real-time heat maps that track incidents and patterns across regions, individual facilities, and specific locations within facilities, based on Use of Force Reports, OSI data, and BLR data, among other sources. These maps should include data on involved officers, demographic information such as race, and other relevant indicators. Regularly review and update these heat maps to support decision-making and policy adjustments.
- L.5 Continue prioritizing installation of fixed cameras, including installing cameras in high-need areas, such as transport vans.

Modernize Technology

- L.6 Transition all records from the current paper-based system to a fully electronic format, including grievances, Use of Force Reports, and medical and mental health documentation.
- L.7 Procure software that can use data to allow for proactive staff intervention.
- L.8 Fully digitize the performance evaluation process so that facilities can use data analytics to track staff performance.
- L.9 Work to integrate DOCCS and OMH records by developing an interface that allows OMH providers to share relevant records for joint DOCCS/OMH patients and vice versa. This integration should enable coordinated, comprehensive patient care and improve continuity of care across agencies.

Enhance Officer Training

- L.10 Revamp training to include a hybrid approach with regular in-person training on key issues such as use of force, defensive tactics, the duty to intervene, and de-escalation, supported by delivery of virtual classroom learning (where appropriate for the topic) and a Learning Management System for security officers. This recommendation will require access to the necessary technology for computer-based trainings.

Revamp Grievance Process

- L.11 Transition to an electronic grievance process, including through tablets and/or kiosks, to enable more efficient tracking, response, identification of “hotspots,” and auditing.

- L.12 Leverage the plan created from recommendation S.14 (a plan to improve the grievance process) and trends identified through body camera data, and strengthen the grievance system by adding civilian and security staff at the facility level dedicated to the grievance program.

Update Classification and Facility Operations

- L.13 Modernize the DOCCS classification system by conducting a thorough revalidation that integrates both risk factors and behavior-based criteria. This process should use current research and best practices to ensure that housing decisions accurately reflect individuals' needs and risks.
- L.14 Explore the possibility of developing additional specialized housing units tailored to specific subgroups, such as veterans, emerging adults (i.e., incarcerated individuals aged 18-29), and other individuals with similar backgrounds. Develop a formal structure for these units that includes evidence-based programming targeted to each group, and establish systems for tracking participation and measuring program outcomes.
- L.15 Review the status of current “separatees”—individuals housed separately due to conflicts—to identify and resolve outdated or irrelevant conflicts. This review should reduce unnecessary transfers, improving facility operations and resident outcomes.

Improve Mental Health and Medical Outcomes and Oversight

- L.16 As part of the larger recruitment and retention initiative, implement a specific recruitment and retention plan for medical staff.
- L.17 Lead and support cultural transformations throughout the organization that emphasize ensuring equitable access to medical care and removing or mitigating organizational, procedural, and attitudinal barriers to timely and appropriate medical care for all individuals. This includes mitigating perceptions of dual loyalty by medical staff, who may perceive tensions between patient care obligations and the perceived and/or articulated interests of security staff.
 - L.17.1 Ensure that medical and DOCCS facility supervisory staff receive joint training to promote full autonomy for patients and medical professionals, to uphold loyalty to and advocacy for patient care (including commitment to medication-assisted treatment for substance abuse disorders), and to mitigate perceptions of conflicts, and that medical and security staff receive training to properly balance the provision of adequate medical care and the preservation of patient privacy with security needs.
 - L.17.2 Design and implement clear, accessible mechanisms for reporting conflicts or dilemmas that arise between medical care requirements and

custody or security concerns. These mechanisms should ensure that all parties are able to voice concerns, seek timely resolution, and maintain a balance between health needs and safety mandates.

- L.18 Establish a dedicated mental health oversight committee to guide implementation of patient-centered practices, drive continuous quality improvement, and review barriers to providing mental health care in compliance with OMH protocols.

Look for Creative New Ideas for Rehabilitation

- L.19 Establish a working group to promote rehabilitative goals at DOCCS facilities, seeking input and ideas from a variety of stakeholders and outside subject-matter experts—including, for example, subject-matter experts on correctional institutions elsewhere in the United States and internationally.